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Pain

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Abstract

Pain is a subjective experience, albeit common to all human beings. Undertreated pain is a major contributor to suffering worldwide, although disproportionately burdening the global South, which has limited infrastructure and medications for pain control. Even in countries where opioid analgesics are available, regulations for procuring, stocking, and dispensing them are often strict, limiting access to those who need these controlled medications, particularly outside of a hospital setting. The subjectivity of pain is a challenging reality for both patients and clinicians. In particular, the lack of discernable causes in some situations makes pain an invisible condition that is difficult to treat within the contemporary biomedical culture. The inevitable reliance on patients' testimonies and concerns of iatrogenic harm from addictive medications in pain management raises ethical questions of trust, stigmatized identities, and epistemic injustice. Such challenges call for more training and education to help clinicians find safe and effective ways to help relieve their patients' suffering.

Keywords

Access to treatment; Global health; Justice; Opioids; Chronic pain

Introduction

Pain is an individual and subjective experience and common to all human beings. Undertreated pain is a major contributor to suffering worldwide, although disproportionately burdening the global South (International Association for the Study of Pain (IASP) 2012), which comprises of countries with medium and low human development that are mostly located in the Southern Hemisphere. Pain can arise from an injury, illness, medical problem, or unknown cause and can also be a consequence of violence, war, and natural disasters. It may be experienced as acute, chronic/persistent, or intermittent, in combination or separately. Acute pain can last for minutes, hours, or days, and chronic pain may persist 3 months or more. Musculoskeletal disorders, pain related to operations and injuries, and spine problems represent the largest global burden of pain, with low back pain being the leading cause

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of years lived with disability worldwide (Vos et al. 2012). Even in countries where opioid analgesics are available, regulations for procuring, stocking, and dispensing them are often strict, limiting access to those who need these controlled medications, particularly outside of a hospital setting.

Epidemiology

Ethnographic literature and epidemiological studies on pain have been instrumental for scholars and policy makers to understand how pain is understood, experienced, embodied, and managed across various cultures and contexts. Nonetheless, since pain has been defined using diverse methods, questions, and samples from different geographic areas and time frames, comparison on prevalence or impact is difficult. Moreover, as a subjective experience, the incidence of pain is challenging to quantify. While good quality epidemiological studies of chronic pain in the last few decades have raised global awareness of the impact of pain and contributed to understanding of its development and chronicity, this literature mainly comes from the global North, which comprises of countries with high human development, most of which are located in the Northern Hemisphere. With limited high-quality epidemiological data available from resource-poor regions, it is challenging for policy makers to advocate for increased access to certain essential medicines in these settings or to implement pain management recommendations established by international agencies.

Variability in estimates notwithstanding, the epidemiological literature indicates high global prevalence and incidence of chronic pain. The IASP approximates that one in five adults worldwide live with persistent pain (IASP 2012), and the World Health Organization (WHO) estimates that one in ten adults are diagnosed with chronic pain every year (Gureje et al. 1998). A large-scale telephone survey of 15 European countries and Israel (Breivik et al. 2006) revealed that moderate to severe chronic pain occurs in approximately 19 % of adults. Similarly, an Australian study (Blyth et al. 2001) found the prevalence of chronic pain to be 17.1 % in men and 20 % in women. In the USA, chronic pain affects approximately 100 million people – more people experience chronic pain than heart disease, diabetes, and cancer combined (IOM 2011). In Canada, one out of every three Canadians will likely be affected by chronic pain over the next two decades (Moulin et al. 2002). As the population ages and people live longer with multiple chronic conditions, the global prevalence and burden of chronic pain are expected to increase.

Pain can impact everyday activities, quality of life, and well-being. It contributes to the inability to participate in the workforce, strained interpersonal relationships, physical weakness, and social isolation. Pain is often accompanied by a constellation of comorbidities including depression, anxiety, and suicidal ideation (Gureje et al. 1998). While pain cuts across age, sex, gender, income, race/ethnicity, and geography, the prevalence of pain, especially chronic pain, is higher in women, older adults, and those with physically demanding employment and lower levels of formal education (IASP 2012). The higher reporting rate in women might be explained by psychosocial factors such as different tendencies to seek healthcare services in general and for pain specifically.

Despite the worldwide health, social, and economic impact of pain, multimodal and multidisciplinary pain management has historically not been a priority for global health. In the global North where there is more advanced healthcare and public health infrastructure, pain is nonetheless underdiagnosed and inadequately treated. Such global impact and the lack of public policy priority in pain management motivated the US Congress to declare the 2000s the "Decade of Pain Control and Research" and the Institute of Medicine (2011) to call for a "cultural transformation in the way that health care providers and the public view pain, and its treatment" (p. 4).

Historical Definitions and Classifications

The word pain originates from the Latin words *poena*, loosely translated to mean penalty or

punishment, and *dolor*, which denotes pain, grief, ache, and suffering. Despite its negative connotations, pain in human life is critical to survival and flourishing. Pain may be a biological warning sign of tissue damage, infection, or future harm. Its noxious qualities may have the adaptive advantage of motivating organisms to engage in self-preservation, such as moving away from the danger, getting rest, and, for humans, seeking medical attention. From this evolutionary perspective, pain might be construed as a biological good or *eudynia*. Not being able to experience pain, such as having congenital insensitivity to pain, can be dangerous and even fatal.

Pain as a concept is not monolithic. It has historically held various meanings and significance, shaped by the cultural, religious, intellectual, and political communities of the time. In the West, medical and moral descriptions of the experience and significance of pain come from the texts and philosophies of Ancient Greece and Rome. Plato described pain and pleasure as the "twin passions of the soul," and Aristotle suggested that they must be enjoyed and avoided respectively. Aristotle defined pain, or *lupē*, as an emotion that can alter judgment. Along with pleasure, he considered pain as *epithumia* or an "appetite" which disturbs and devastates the sufferer's character.

The Roman physician Galen emphasized the role of sensation and perception in pain. He classified and described different forms of pain based on their types and intensities. The French philosopher Descartes described pain as a reflexive response to a physical, mechanical stimulus – a concept that had wide influence for centuries. The contemporary McGill Pain Questionnaire (MPQ), an instrument that uses descriptive adjectives based on the experiences of pain patients, helps patients to express their pain in metaphorical language familiar to Western cultures. For example, the MPQ uses terms such as "throbbing," "shooting," "stabbing," and "nagging." Today, the prevailing international definition of pain put forward by the IASP (2010) describes "an unpleasant sensory and emotional experience associated with actual or potential tissue damage" (Pain Terms Section, paragraph 1).

Contemporary Pain Models: Definitions and Classifications

The subjective nature of pain notwithstanding, clinically, pain experiences are assessed by duration and source. These experiences are often distinguished by one of two categories: acute pain and chronic or persistent pain. Recent efforts – such as the IASP definition – attempt to provide a definition of pain that encompasses both acute and chronic pains. This entry will adhere to these distinctions, while recognizing the variable definition of pain in the literature.

Acute Pain and Chronic Pain

Acute and chronic pains differ in temporality and in kind. Acute pain may persist for up to 3 months and typically arises from acute injury, surgery, trauma, or severe medical illness. The pain is usually most intense at the onset of injury and subsides as healing occurs. Acute pain may also be recurring with alternating painful and pain-free episodes. It may transition to chronic pain if it is inadequately treated or if there is a diminished responsiveness to some therapies.

Chronic pain, as defined by the IASP, is prolonged and persistent pain lasting at minimum 3–6 months in duration. However, since chronic pain is often incurable, many sufferers endure pain for much longer or even indefinitely. Chronic pain may be associated with chronic tissue pathology (e.g., arthritis, cancer), or the pain may exist beyond an expected healing period for an acute injury. In some cases, the pathology may resolve, but pain persists without clear evidence of nociceptive stimulus. Chronic pain can be further classified as either nonmalignant (e.g., noncancer) or malignant (e.g., cancer) pain and bear different ethical implications.

Chronic Nonmalignant Pain (CNMP)

When pain becomes chronic, it no longer serves an adaptive role. Chronic nonmalignant pain may arise out of injury, a comorbid chronic disease, psychological trauma, and complications from surgery or may lack an identifiable origin or cause.

Individuals with CNMP often report a lower quality of life due to reduced mobility, loss of energy and strength, disrupted sleep patterns, financial burden, loss of identity, as well as other physical and mental health comorbidities (Lamé et al. 2005). Uncontrolled pain can have detrimental physical and emotional impact on sufferers and their loved ones. It also has a considerable impact on public health systems. Individuals with chronic pain seek out care in various public and private healthcare contexts, imposing financial burden on sufferers and societies directly and indirectly. Direct costs may include out-of-pocket expenses, private and third-party insurance, and time costs. The annual costs of chronic pain in the USA are estimated to fall between USD $560 and 635 billion, including USD $61.2 billion per year in lost work productivity, exceeding the combined costs of the six most expensive diagnoses such as cardiovascular diseases and neoplasms (IOM 2011).

Chronic Malignant Pain

Chronic malignant pain is a corollary of life-limiting diseases such as cancer, AIDS, amyotrophic lateral sclerosis, multiple scleroses, end-stage organ failure, chronic obstructive pulmonary disease, and Parkinson's disease. Many people in cancer remission develop excruciating chemotherapy-induced peripheral neuropathy, radiation toxicity due to radiotherapy, and post-surgical chronic pain. In the global South, chronic pain is considered an inevitable symptom of diseases such as cancer, and patients are expected to live with their pain.

Chronic malignant pain management is particularly challenging and ethically fraught for cancer patients at the end of life. One systematic review estimated that 64 % of patients with metastatic or advanced-stage cancer experience pain (Beuken-van Everdingen et al. 2007). Children who are at the end of life may also suffer unrelieved pain, in part because their symptoms are ignored or signs of pain and suffering are not recognized. In a South African study evaluating terminal care among hospitalized children who died of HIV/AIDS, half the patients with pain and distress, including some with a comfort care plan, received no analgesia (Henley 2002).

Biopsychosocial Model of Pain

There has been a paradigm shift in pain theory, research, and practice over the past several decades. Much of the contemporary scholarship on pain has attempted to incite a cultural shift in how pain is defined and understood, moving away from conceptualizing pain as corresponding to an identifiable, objective pathology to focusing more on the subjective nature of pain (IOM 2011). Moreover, the increasing awareness that pain is not only a physiological event or process has led to the adoption of the biopsychosocial model of pain, which situates the subjective experience of pain as emergent from convergent biological, psychological, and social systems.

Nociception is the neurobiological process of encoding noxious stimuli and is the precursor for the experience of pain. The neuromatrix theory of pain asserts that the perception of pain emerges from signal exchanges on at least three levels of the neuroaxis: peripheral, spinal, and supraspinal areas of pain processing. The integration of multiple inputs (i.e., sensory, visual, cognitive, emotional, and signals from the stress regulation system) produces an output of an individual-specific neuromatrix – a neurosignature of nerve impulses. Pain becomes clinically intractable because the nervous system has not returned to a non-pathological baseline state after a brief healing period.

The neurophysiological processes of pain impact emotion and cognition. For example, chronic pain may be accompanied by comorbid cognitive and emotional manifestations such as anxiety, depression, somatization, fear, addiction, anger, and deficits in learning and memory. Chronic pain may produce maladaptive coping mechanisms, feelings of helplessness, hopelessness, and reduced self-esteem (McWilliams et al. 2003).

Culture and Pain

People's views about pain generally, their own pain, pain management, and the meanings of pain and suffering are historically, culturally, and socially based. People of different cultures respond differently to pain. Some cultural groups are more expressive about their pain, while other

groups adopt a stoic stance and avoid vocalizing or exhibiting signs that they are in pain. For example, in many Western cultures, childbirth is regarded as a painful, distressing experience, a perspective that is not universally shared. If a specific or set of pain behaviors is congruent with a societal or cultural view of how people should behave when in pain, sufferers may receive attention, sympathy, compensation, and sick role legitimization. Accordingly, some individuals may learn to display certain behaviors in order to achieve a desired response within their specific sociocultural context.

Diverse cultures around the world have developed their own taxonomy of pain, based on the theories of the body, health, illness, and medicine. The anthropological literature has provided scholarly insight into the diversity of pain experiences. For example, certain Japanese communities classify their pain experiences based on the features of a land or sea animal. Urdu and Hindu communities of North India use local metaphors to describe the sensory, location, intensity, temperature, and other qualities of pain. For people of certain religious or spiritual perspectives, pain may also cause its sufferers to question the meaning of life, as individuals may believe they are being punished by a higher power for some wrongdoing. Such narratives are found in, for example, Judeo-Christian traditions, highlighted in stories such as the *Book of Job* and the Crucifixion. The role of suffering is also culturally varied. For example, among Christians in the Middle Ages, enduring physical pain and suffering was considered a divine experience that brought believers closer to salvation. Conversely, suffering and its liberation are central to religions such as Buddhism. Given varying cultural meanings of pain and suffering across the globe, pain assessment and management remain challenging despite the increasing prominence of the biopsychosocial approach.

Pain, Subjectivity, and Curative Models of Care

The essence of pain is its phenomenological quality. Its reality is based on how it is perceived by the individual experiencing it, and external access to these experiences is often only based on people's self-reports, raising questions of whether or how clinicians may trust their patients' testimony. Two people diagnosed with the same pain condition may experience it differently as intertwining biological, psychological, social, and even spiritual factors mitigate their experience. Indeed, the subjectivity of pain is a challenging reality for both patients and clinicians. In particular, the lack of discernable causes in some situations makes pain an invisible condition that is complex and difficult to treat within a biomedical culture that is based in observation of material phenomena.

The rise of modern medicine in eighteenth- and nineteenth-century France was characterized by the view that all symptoms and health complaints are caused by discoverable, visible pathological lesions. This perspective became central to the contemporary scientific medical approach that privileges observable over unobservable phenomena. Pain, accordingly, is expressed by a predictable relationship between tissue pathology or nociceptor activity and clinical presentation. Pain without objective pathology thus cannot be easily explained from a materialist perspective.

This scientific-materialist framework points to the predominance of the curative model that operates in contemporary healthcare. Accordingly, global health efforts have prioritized the treatment and eradication of disease. As pain is generally considered a symptom of disease as opposed to a *bona fide* condition, resources tend to be allocated toward prevention and treatment and not toward suffering produced by disease (King and Fraser 2013).

Pain and Suffering

Pain and suffering are so often related that these terms are sometimes used synonymously. Many kinds of pain bring about suffering, but the meaning of suffering is difficult to define. Turk and Wilson (2009) describe pain and suffering as two distinct but inseparably linked concepts. While they reason that pain is not a necessary condition for suffering, they acknowledge that suffering can exacerbate pain, pain-related

suffering does indeed occur, and healthcare professionals ought to assess for both.

Cassell (2004) argues that pain is physiological and suffering is existential and the subjective nature of suffering – like pain – makes it challenging to address with the tools of scientific medicine (i.e., the clinicopathological method). Cassell suggests that the experience of suffering is intensely personal: it is perceived "as a threat to persons' continued existence – not merely to their lives but their integrity as persons" (p. 36). These discussions about suffering are inextricably tied to the concept of human dignity, particularly in discussions of end-of-life and palliative care. In such contexts, physicians are understood to have a primary ethical and professional duty to relieve patients' pain and suffering as a means to promote their dignity at the end of life.

The Ethical Dimensions of Pain

Pain has many ethical dimensions, but pain management has been largely underrepresented in bioethics. Where bioethics literature exists on pain management, contributions have tended to focus on ethical issues in end-of-life pain relief, particularly around the ethics of physician-assisted death and euthanasia. Even here, the focus has mostly been on the consequences of these procedures rather than promotion of pain management. The discussions have been generally around whether these are ethically legitimate means to promote dignity, given the expected outcome of hastening or causing the person's death. Indeed, there are questions of whether increasing support of these procedures has sidelined the discussion of promoting better access and management of pain relief at the end of life.

Jeremy Bentham, the founder of modern utilitarianism, argued that it is the greatest happiness of the greatest number that is the measure of right and wrong. Considering the fundamental role of pain and pleasure in human life and happiness, Bentham reasoned that these "two sovereign masters" alone can guide moral actions. Legislative practices, he argued, should have pleasures and the avoidance of pains as the ends. His hedonic calculus measures the intensity, duration, (un)certainty, as well as the nearness or remoteness of the sensation that may be experienced by the person. Other relevant factors in calculating the moral value of an action include whether one can expect the same sensation to follow and whether a pain may be followed by a pleasure and vice versa.

Given that the impact of pain and pleasure extends beyond the individual, Bentham believed that the extent to which others may be affected also needs to be considered. He proposed a classification of 12 pains, which include physical and spiritual types. Despite such variety, Bentham believed that people could sum up all the values of the different kinds of pleasures and pains and then determine the morally right action by choosing the one that brings the greatest amount of pleasure and the least amount of pain.

Critics such as John Stuart Mill have pointed out the inadequacy of this hedonistic value theory by explaining that the value of life is more than a balance of pleasure over pain. These legitimate concerns notwithstanding, Bentham's theory highlights the ethical dimensions regarding access to treatments or relief of pain, given that it can greatly affect people's physical and mental well-being.

Contemporary utilitarian philosophers such as Peter Singer have argued that nonhuman animals are also sentient beings – they similarly have the capacity to experience pain and to suffer. These capacities, Singer argues, demand that animals' interests in not experiencing pain or suffering should be taken seriously by human beings, based on a more general obligation to give equal moral weight to the interests of those affected by human actions. Singer argues that modern factory farming conditions create intense pain and suffering for animals and human beings are therefore ethically obligated to prevent such wrongs by being vegetarians.

Pain Through the Lens of Major Bioethical Principles

Respect for Persons

Kant believed that the shared ability to reason is what makes humans morally significant and

bestows all humans the right to dignity and respect. In the healthcare context, respect for persons demands that patients be treated as moral agents who can voice their pain concerns in a safe therapeutic environment and engage in shared decision making with their clinicians regarding the most appropriate pain management. This last aspect is specifically captured by respect for autonomy, which requires that mentally capable patients be informed about the risks and benefits of treatment, possible alternatives, and likely consequences and allowed to make voluntary treatment decisions accordingly without undue influence from others. In the context of pain management, respect for a person demands that patients' perspectives regarding their pain tolerance and most appropriate form of pain relief be given due consideration in constructing care plans. It also calls for healthcare providers to refrain from being paternalistic toward patients.

Respect for persons and their autonomy is also relevant in pain management because severe pain can diminish people's energy and clarity of thought, which can in turn restrict people's capacity to exercise autonomy. Patients' ability to access appropriate pain control is thus ethically significant. Of concerns are situations where patients may not know how to describe their pain in ways that would allow them to get appropriate help from health professionals or where they may be too worried about being viewed as weak or demanding to even discuss pain relief. In other cases, some patients may feel distrusted or discriminated by the clinician regarding their pain complaints, particularly when their pain may not have a known origin or discernible organic cause. When patients perceive that they can make their own decisions regarding pain management, they are more likely to have better therapeutic or trust relationships with their clinicians.

However, in the discourse of pain management, there have been questions of whether complying with patients' requests for their desired form of pain management in some situations may ironically compromise patients' autonomy. In discussions of physician-assisted death and euthanasia, whose supporters often appeal to the principle of respect for persons and their autonomy, some have argued that these procedures require the help of another person and thus are not simply matters of one individual's self-determination. In the area of CNMP, some clinicians may argue that paternalistic actions toward patients who are (at risk of being) addicted to pain medication are justifiable, if their (suspected) addiction may affect their ability to make autonomous decisions regarding their care management.

Nonmaleficence and Beneficence

Nonmaleficence describes a basic ethical and professional obligation to not inflict unnecessary harm on a patient, whereas beneficence refers to a duty to promote the well-being of a patient, which is a basic tenet of clinical medicine. Symptom relief for patients suffering from painful illnesses, especially when no cure or reversal of the condition is available, is an active concern of medicine. Both ethical principles require clinicians and others to balance the risks and benefits of various procedures, avoid preventable injuries and pain, and promote access to pain relief. Nonmaleficence and beneficence would both demand the clinical team to consider the patient's overall context and priorities in determining appropriate management to the patient in a timely fashion.

Uncontrolled pain is also a significant concern for patients near their end of life, where pain is the single factor accounting for patient experiences of a "bad death." A commitment to nonmaleficence thus requires clinicians to help minimize potential physical and accompanying emotional suffering. Such a commitment at a system level demands governments and public health systems to take reasonable steps to provide access to effective and safe forms of pain relief.

However, while the principles of nonmaleficence and beneficence may have intuitive appeal, how clinicians may fulfill these obligations can be challenging in some situations. Due to the side effect of certain pain management regimens, balancing harm and benefits can be difficult. At a system level, opioid analgesics are controlled medicines – their production, distribution, and prescription are strictly regulated. These legal parameters have tremendous impact on what

HCPs may (not) be able to provide for their patients. In end-of-life care, the concern among some healthcare providers that opioid analgesics may depress respirations and thus hasten death may have contributed to pain being treated inadequately in populations with advanced illness.

The situation is also complex in CNMP due to limited physician education and training in both chronic pain and addiction. Many clinicians therefore find it difficult to determine the most appropriate management, particularly in situations where complaints of chronic pain are not matched with an obvious or a visible clinical explanation for the pain.

Some pain medications, such as opioids, are highly addictive and have limited evidence for the effectiveness of long-term use (Von Korff et al. 2011). Nonetheless, opioids have increasingly been overprescribed in North America over the past two decades, leading to a rise in morbidity and mortality associated with abuse of these prescribed drugs. This epidemic of prescription opioid abuse emerged, in part, because HCPs were relying on faulty evidence and misleading marketing strategies by drug companies about the abuse risk of medications such as OxyContin. Pharmaceutical companies have also been accused of employing physicians to aggressively promote the liberal prescribing of opioids for CNMP to other prescribers. Given this state of affairs, some HCPs are thus rightly concerned about bringing on iatrogenic harm by prescribing various pain medications, particularly for long-term use by patients with chronic conditions. Facing a challenge in balancing the risk of addiction and the harm of inadequately treating the patient's pain, many clinicians adopt a default position of "pharmaco-vigilance" and exercise extreme caution in prescribing certain pain medications.

Justice

As cultural attitudes, social contexts, and resource availabilities vary across the globe, at least two interrelated forms of justice concerns are salient in pain management.

First, justice as a bioethical principle deals with fairness, equality, and equitable treatment of all patients. It emphasizes nondiscrimination of various populations, such as people living with chronic pain, and addresses the association of global chronic pain with various social determinants of health, which are the circumstances in which people are born, grow up, live, work, and age, as well as the systems put in place to deal with illness. Second, in the form of distributive justice, this principle demands equitable allocation of medical and social resources to correct pain inequities, i.e., avoidable inequalities in access to pain management between populations, and to support patients' needs.

Achieving justice may be the biggest challenge in addressing pain management internationally. Regarding fair and equitable treatment, pain patients, particularly those with chronic pain, have reported perceived discrimination and stigma in society and by healthcare professionals. Stigma is important ethically – as it indicates injustice and a lack of respect for agency. Given the subjectivity of the pain experience, some physicians may question the truthfulness of patients' testimonies and their motivations for requesting highly addictive pain medications, particularly if they appear to exhibit drug-seeking behavior. Due to social stigma, patients with chronic pain are often discredited and hold a "spoiled identity" with presumed "blemishes of individual character," as the sociologist Goffman noted. Such default position of distrust and presumptions of credibility deficit may further marginalize chronic pain patients and perpetuate inadequate treatment of pain for this population.

Second, regarding issues of distributive justice, access to pain management is unequally distributed, with widening inequities across the globe. The poor, elderly, mentally ill, children, women, and racial and ethnic minorities bear greatest burdens of persistent and untreated pain. In many countries, chronic pain is associated with factors such as race, socioeconomic status, stress, neighborhood, and level of education. Members of disadvantaged groups are also less likely to access and benefit from effective treatment. They endure various conditions and accompanying pain due to poverty and other barriers in accessing healthcare. Even when access to general healthcare is available, disparity in pain management remains.

In the USA, persons in minority groups often receive less pain medication than their white counterparts in emergency rooms, postoperatively, and in labor. One review in that country shows the persistence of ethnic disparities in acute, chronic, cancer, and palliative pain care across the life span and treatment settings, with minorities receiving lesser quality pain care than non-Hispanic whites (Anderson et al. 2009).

Geographical locations within a country can also contribute to disparate access to few pain specialists and multidisciplinary pain clinics. In Canada, 80 % of multidisciplinary pain management clinics are in major urban centers, with no clinics in the province of Prince Edward Island or the Yukon, Northwest, or Nunavut Territories (Peng et al. 2007). Wait times to see a specialist for CNMP can take several months to a year from the time of referral and are on average longer than 1 year for more than one-third of publicly funded clinics (Peng et al. 2007).

Internationally, factors related to cardiovascular disease, trauma, and cancer rank among the leading contributors to the global burden of disease. With inadequate access to effective treatments in the global South, a lengthy and painful disease progression for these conditions is common and unfortunately expected. While countries in the global North – Australia, Canada, New Zealand, the USA, and the European Union – consume more than 90 % of the global supply of pharmaceutical-grade morphine (United Nations 2009), there is very limited access to nonsteroidal anti-inflammatory drugs (NSAIDs) and opioid analgesics in less resourced regions (King and Fraser 2013), leaving approximately 5.5 billion people in over 150 countries with limited to no access to these medicines (Seya et al. 2011). For example, tens of millions of patients with terminal cancer are estimated to experience moderate to severe pain due to inequitable access to not only aggressive interventions but also pain medicine, since morphine is virtually nonexistent in most regions (Seya et al. 2011). Similar concerns exist for the millions of people living with conditions such as HIV/AIDS, most of whom live in regions with little or no access to treatment for moderate to severe pain. Barriers to pain management in these regions include restrictive policies and laws, limited education in pain management for health professionals, and inadequate healthcare and public health infrastructure. Poor access spans from curative treatments such as surgery, radiotherapy, and antiretroviral therapy to comfort measures such as palliative care. The International Narcotics Control Board (INCB), which has the legal mandate for the international control of both licit and illicit narcotic drugs, implements quotas on the amount of controlled narcotics that countries can obtain. Some scholars have argued that the INCB acts within the confines of the US-led global war on drugs and is a major legal barrier to accessing pain-relieving medications in lower- and middle-income countries (Nickerson and Attaran 2012). They note that the INCB has approved insufficient supplies of controlled narcotics for medical use based on the misestimates of the prevalence and clinical need of pain in certain countries.

It is worth noting that the global problem of pain is not simply due to challenges in availability and distribution of opioid analgesics. Although critically important for pain management, opioids are not a panacea – structural factors such as economics and political and cultural factors shape the experience of pain and contribute to treatment (in)equality. These remarkable inequities raise important questions of global distributive justice and prompted the WHO and the IASP to advocate for access to pain relief to be a human right.

Conclusion

There is a high global prevalence of chronic malignant and nonmalignant pain that incurs substantial social and economic costs on societies. Countries in the global South bear the greatest burdens. Pain is closely linked to the social gradient of health within countries and highly determined by socio-structural factors such as economic conditions. The suffering caused by the epidemic of undertreatment of pain and its sequelae is an international tragedy, impacted by widening inequalities in access to healthcare.

Various ethical theories and bioethical principles point at ethical obligations of providing pain relief, but data demonstrates a global inadequacy. International efforts that propel paradigmatic changes in pain education, culture, laws, policies, and treatment, including disparities in access, at micro-, meso-, and macro-system levels, can help identify pain as a global health priority. In doing so, the global community can work in solidarity to support healthcare providers worldwide in fulfilling their ethical duties to adequately treat pain and minimize suffering.

Cross-References

- Addiction
- Animal Rights
- Assisted Suicide
- Chronic Illness and Care
- Death: Good Death
- Palliative Care
- Stigmatization
- Substance Abuse
- Suffering
- Trust

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Palliative Care

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Abstract

Palliative care has been developing strongly in recent decades and now has a global reach, albeit with patchy coverage. As it has grown into a recognizable field of activity, discernible fissures have opened up in its approach and orientation. Palliative care grew out of a hospice-type response to the needs of dying cancer patients. Its model of care fitted a particular trajectory of treatment and the associated illness experience. Within that model it identified an ethical and moral space in which to promote the holistic care of the patient and family, and it adopted particular strategies for pain and symptom management. Now in many countries, it seeks to meet the complex needs of aging populations characterized by multiple morbidities – in situations where demand for end-of-life care is growing. This requires it to adopt an ethics of public health and to champion the claim that access to palliative care is a human right. In addition to these clinical and service-level challenges, palliative care has to find an appropriate orientation to contexts where assisted dying is legalized. This may require it to modify its historic opposition to assisted death.

Keywords

Palliative; Hospice; End-of-life care; Dying; Assisted dying; Public health; Human rights

Introduction

Palliative care in its modern guise first emerged during the 1960s and 1970s, though its roots and antecedents are much older. It seeks to prevent and alleviate suffering associated with life-limiting illness, and it is particularly associated with care at the end of life. Its principles are holistic and multidisciplinary, focusing on physical, social, psychological, and spiritual concerns in the context of serious illness. To these ends, it engages the skills of medicine, nursing, social work, psychology, allied health professions, family members, and often volunteers and wider communities. It has developed specific expertise in the understanding and management of pain associated with advanced disease, and it provides expertise in relation to other, often complex, symptoms that may occur across the trajectory of illness. Within the organization, delivery, and practice of palliative care, ethical and moral concerns feature significantly.

In some ways, palliative care has the attributes of a single-issue social movement, since it has engaged the interests and commitments of those well beyond the world of healthcare delivery and it is focused around a specific aspect of care. But it is also a multifaceted field of specialization, with associated academic endeavors in teaching and research and a growing recognition among professional societies, scientific funders, universities, and training establishments. It has been drawn to a public health paradigm when considering levels of need, appropriate policies, and services for delivery, together with suitable quality assurance and evaluation. It also makes an expanding claim for recognition as a human right. Increasingly, the reach of palliative care extends to health policymakers, politicians, and global health organizations and discourses.

Palliative care has a close but sometimes complex relationship with hospice care, and this

differs across countries of the world. Many varieties of hospice and palliative care delivery can now be observed in a wide variety of settings. While few of these are supported by proven evidence of success, they do testify to the rich field of activity and the imaginative and diverse approaches that have been developed in different resource, cultural, and healthcare contexts. Palliative care is also seeking to develop an appropriate relationship with other areas of the healthcare system, in particular those primarily focused on curative treatments, rehabilitation, or the management of long-term conditions. It faces specific challenges in the face of changing demographics, epidemiology, and patterns of symptomatology.

The distribution and availability of palliative care globally are known to be highly inequitable, being concentrated in a small number of countries, and has led the World Health Assembly to call on all governments to give it higher priority. It has been estimated that less than 10 % of those who might benefit from palliative care currently have access to it worldwide. Only 20 countries have achieved a significant degree of palliative care development as demonstrated by levels of service provision, education, drug availability, research, financing, and policy recognition. Palliative *medicine* is recognized as a specialist field of activity in around 25 countries worldwide.

History and Development of Palliative Care

The origins of modern palliative care lie in a movement to improve care of the dying that configured around notions of hospice care. From the 1950s and 1960s, activists such as Cicely Saunders in the United Kingdom and Florence Wald in the United States looked to a hospice tradition that had got underway in the late nineteenth-century homes for the dying that developed in France, Britain, America, and Australia. Many of these hospices and homes for the dying were run by religious orders and gave close attention to spiritual care at the end of life.

But the twentieth-century activists were a mixture of people with and without religious inspiration. They sought to reinvent older hospice traditions in a new guise and in so doing to challenge the growing Western tendency to deny death and to sequestrate the dying. The "hospice movement" was a small revolution against prevailing death ways in Western culture. In some instances, it was a reaction against meddlesome medicine and futile interventions. In others, it was stimulated by the medical neglect of the dying in healthcare systems that prioritized cure and rehabilitation over the needs of those close to death.

The development of modern hospice care had many characteristics of a new social movement. It was associated with a single issue and fostered by strong advocacy as well as charismatic leadership. It grew out of communities of interest, communities of practice, and communities of place. In many instances, services were initiated without the endorsement of the formal healthcare system, but with strong local support. This often led to tensions and confrontation as the hospice pioneers sought recognition for their cause in the face of skepticism, indifference, and bureaucratic inertia.

To succeed "at scale" – and thereby meet population-level needs – it became increasingly clear that hospice ideas and practice would need to find a place within the mainstream of healthcare systems and structures. This was recognized in the early 1970s by the Canadian surgeon, Balfour Mount, who also took the view that the work needed a different nomenclature. He coined the term "palliative care," drawing on established medical notions of palliation but also on wider meanings of the word in relation to "cloaking" or "shielding." The first named palliative care services soon began to appear. In the 1980s journals, training programs and major conference came to be dedicated to palliative and hospice care. In 1987, the United Kingdom was the first country to recognize palliative medicine as a specialist area of activity.

Services also diversified, and palliative care began to be delivered in specialist inpatient units and hospices but also in the wards and outpatient clinics of acute hospitals, in the community, in residential care settings, and in people's own homes. Commensurate with this was a growing interest in research, evaluation, and attempts to

develop policies and strategies for palliative care delivery for whole jurisdictions. Palliative care was taking on the characteristics of a "field" which could be recognized alongside other specialties in health- and social care and bore the recognizable hallmarks of an evidence base, standards and guidelines, codes of training and practice, and professional accreditation.

Professional bodies proliferated and emphasized the multidisciplinary aspects of palliative care as a field that involved medicine, nursing, social work, psychology, spiritual and religious support, and the involvement of allied health workers and volunteers. Some of these developed their own specialist journals. Beginning with links between individuals and the services in which they worked, international collaborations began to develop, and these led to major conferences, the creation of international professional societies, and a growing awareness of the global development of palliative care – and the many challenges which it faced.

From the mid 1990s, research studies were conducted which mapped the differential development of palliative care in specific regions of the world and also globally. Understanding grew of the common elements that united the palliative care ethos, but also of the differences within it, the variations in local practice cultures and assumptions, and the specific ethical challenges to which they gave rise.

Conceptual Clarification/Definition

There has been significant debate – and no lack of disagreement – about the various definitions and models of palliative, end-of-life, and hospice care that now exist. The World Health Organization has produced two definitions, in 1990 and in 2002, but many more have been reported in the literature. Definitional problems continue to inhibit clarity of thought and action in the field.

A paper focusing on definitions of the term "palliative medicine" and "palliative care" in two languages found a total of 37 English and 26 German versions, confirming a lack of a consistent meaning (Pastrana et al. 2008) about key terms and approaches. In 2004, a National Institutes of Health State-of-the-Science Conference Statement noted that "There is no exact definition of end of life; however, research supports the following components: (1) the presence of a chronic disease(s) or symptoms or functional impairments that persist but may also fluctuate; and (2) the symptoms or impairments resulting from the underlying irreversible disease that require formal either paid, professional or informal unpaid or volunteer care and can lead to death" (NIH 2004). Surprisingly, a 2012 volume offering an international public health perspective on the subject offered no clarification, often using "end-of-life care" interchangeably with "palliative care" (Cohen and Deliens 2012).

There are similar problems associated with the definitions of assisted dying. McLean uses the term to encapsulate events which might otherwise be described as (physician) assisted suicide or voluntary euthanasia and notes that although mechanically they differ, at the heart of each of them is the positive choice for death (McLean 2007). "Assisted dying" and "euthanasia" are terms that have proved notoriously difficult to define and are subject to multiple interpretations (Materstvedt et al. 2003). In these specific areas, carefully crafted definitions produced by philosophers are often roughly handled by publics and protagonists who use distinct and separate terms interchangeably or with a lack of clarity of meaning. This is a sociological reality which can undoubtedly create a gulf between formal conceptual work and the living world of disputation, action, and decision making. It is easily witnessed in the inexact drafting of bills to legalize assisted dying that come before parliaments around the world.

Ethical Dimension

The Clinical Realm

As palliative care developed into a recognized field of practice, it sought to define its areas of interest and competency. This has remained a matter for debate and contestation.

In the nineteenth century, medical writers such as William Munk focused their attention on "easeful death" as their principal objective, when efforts to prolong life were no longer productive. Munk, in common with his predecessors from earlier in the century, used the term "euthanasia" to describe this. He was referring to the relief of suffering through the judicious use of pain medication, to the management of the sick room and deathbed in achieving dignity and comfort, and to the role of religious solicitude in preparing the dying person for a life to come, surrounded by family and friends. By such means, the idealized "good death" was to be realized.

But as the nineteenth century came to a close, the meaning of "euthanasia" quite quickly began to change. If suffering might be relieved by the administration of strong opiates, such as morphine and diamorphine, which had been synthesized and put into commercial production in earlier decades, might suffering also be *prevented* by more direct intervention? And might indeed death be hastened deliberately by the physician in order to achieve this? Now some doctors began to take a serious interest in this issue, which also became a matter of public debate and controversy. As the public influence of religion in Western culture moved into a phase of decline, attention focused less on the meaning of death and the afterlife and more on the process of dying and the ability to exert control over it. Medicine had become the central influence over the deathbed.

Therein developed a struggle, for if some doctors endorsed the new definition of euthanasia and supported its legalization, others were opposed to this on religious and moral grounds and sought to direct their efforts to improving care in advanced illness and thereby to thwart the need for medicalized killing. This ethical divide became an increasingly marked feature of Western medical culture during the twentieth century and by the start of the twenty-first century was also in evidence in some low- and middle-income countries. It fueled an ethical debate significantly influenced by Christian ideas about the sanctity of life, but over time it was taken up in other religious and cultural contexts. By the second decade of the twenty-first century, it appeared to have become a matter of global interest.

Perhaps the strongest voice in opposition to euthanasia and in support of palliative care was that of Dr Cicely Saunders, who founded St Christopher's Hospice, the world's first modern hospice, in London in 1967. As a lifelong opponent of euthanasia, she argued consistently that demand for assisted death diminishes wherever the principles of good palliative care are in evidence. Her central instrument in support of this claim was the concept of "total pain," which recognized that the suffering of a person at the end of life can be multifaceted – physical, emotional, social, psychological, and spiritual. It was to the suffering at this level that she addressed her attention, with notable success. But for Saunders, the practice of hospice and palliative care was not simply a medical service. It was enriching of society and a measure of its moral worth. This enabled palliative care to find a wide audience, beyond that of healthcare workers and specialists. It attracted volunteers, fund-raisers, and well-wishers, convinced that its ability to control pain and other symptoms and its determination to give dignity to the dying and bereaved were skills and values worth promoting across the whole of society.

The control of pain in palliative care has been the topic of much scrutiny and debate. The principle of double effect has been used to justify the administration of proportionate medication to relieve pain even though it may lead to the unintended, albeit foreseen, consequence of hastening death, for example, by causing respiratory depression. Some have argued that there is little evidence to support the view that such analgesics carry this risk. From this perspective, the belief in the double effect of pain medication perpetuates the undertreatment of physical suffering at the end of life. At the same time, the concept of double effect in the use of opioids has also been used to support the legalization of physician-assisted suicide and euthanasia. This argument typically takes one of two forms. First, it is argued, as hastening death by drugs is already being done and is ethical, medical practice should be extended to allow physician-assisted suicide. The second argument

is that because physicians are already hastening death, euthanasia should be legalized in order to provide safeguards, checks, and appropriate controls (Fohr 1998).

Clinical practice in palliative care acknowledged that at times refractory symptoms, often highly distressing to patients, family members, and staff, could be difficult to manage. Interest grew in using deep sedation (variously termed) as an approach to this problem. Comparative studies showed how rates of sedation at the end of life varied between settings and countries. The notion developed that the dying process for many had come to involve a set of decision-making processes, some of which result in the decision to sedate the patient. This might be on a temporary basis or until such time as death ensued. It might also be at the request of the patient. Palliative care was called on to produce definitions, guidelines, and studies of this practice in order to give clarity and transparency to its procedures. In this context, the focus was not on the use of opioid drugs, but rather on benzodiazepines, such as midazolam. There was also the need to distinguish the practice from euthanasia, usually explained by the process of titrating the drugs to achieve sedation while maintaining respiratory function and not hastening death.

As the clinical practice of palliative care developed through the last quarter of the twentieth century, other issues began to challenge its development. Defined from the outset as a *multidisciplinary* endeavor, nevertheless, the role of medicine appeared to be primus inter pares. There was a tendency for medical perspectives, medical assumptions, cultural practices, and medical solutions to frame the debate about how palliative care should develop. This was often because palliative physicians were looking over their shoulder at other specialties and seeking to keep pace with developments in cognate fields – oncology, geriatrics, cardiology, and respiratory medicine. It led some external commentators to argue that a process of "medicalization" was at work within palliative care that would quickly dissipate its early more holistic intentions. There were also detractors from within, who cautioned against palliative medicine becoming "just another specialty" and for whom an overemphasis on pain and symptom management seemed to be at the expense of attending to the "whole" person – including not only a person's physical problems but also social issues, spiritual, religious or existential matters, as well as the needs of family caregivers, and matters of psychological and mental health. For some this raised concerns that the specialty of palliative medicine might make progress at the expense of the early goals of palliative care.

Palliative care began with an unequivocal focus on patients and families affected by cancer. It seemed that the widespread stigma associated with the disease, the poor prognosis that existed in many instances, the fairly predictable progression and the possibility of severe pain, and other debilitating symptoms all combined to make a perfect testing ground for the palliative care ethos. This came to be known as the "rapid decline" trajectory, and it appeared to fit well with the perspective of hospice care – primarily focused on the final months and weeks of life. Palliative medicine at first locked onto this paradigm and gave particular focus to alleviating the challenging problems associated with specific pain syndromes, as well as breathlessness, anorexia and cachexia, anxiety, and depression – all of which came to be seen as key determinants of "quality of life" in patients with advanced cancer.

Over time there were calls to extend the perceived benefits of palliative care, beyond patients with cancer to those dying from – and living with – other conditions. This meant two things. First, patients started to be seen for whom the medical context was less familiar. Initially, this was people with neurological conditions such as multiple sclerosis and motor neuron disease, but in time it was extended to those with heart failure, stroke patients, people with dementias, as well as those affected by HIV/AIDS. Second, there was pressure to move palliative care interventions further "upstream" in the disease progression, to earlier stages of the illness, where palliation might sit alongside curative treatments and interventions. Now palliative care began to uncouple from the paradigm of terminal illness and eventually to reframe itself as an "extra layer of support"

for patients – right across the treatment trajectory, with its various consequences and side effects.

The distinction between "cancer" and "non-cancer" palliative care was in evidence for a period from the 1990s but became less commonly used over time. This was driven by the growing recognition of multiple morbidities and symptom burden that occurred in medicine and which seemed particularly relevant to palliative care. At the same time, cancer was in many Western countries making the transition into a treatable disease, associated with the new phenomenon of cancer survivorship and of cancer as a "chronic" and/or "social care" issue. Palliative care therefore had to focus on patients with complex multiple problems, which might endure for potentially long periods. Attempts were made to characterize these into specific "trajectories" of dying, but if these seemed to make sense conceptually, they were found not to map easily onto any empirical reality. In the West at least, the epidemiology of many cancers migrated into that of a chronic disease, and at the same time the concept of multiple morbidities became increasingly recognized clinically – meaning that the care of patients became more complex and protracted in a setting where patterns of decline associated with frailty, dementia, and impoverishment became more prevalent.

This shift was also associated with population aging. The "baby boomers" drew attention to the growing reservoir of care needs in later life that resulted from the heightened birthrate in the two decades before the mid-1960s. And at the same time, these people were set to live longer lives, as the twentieth century increased life expectancy due to improved healthcare and public health measures. This in turn became a global issue. Around 58 million deaths occur in the world every year, and the number may rise to 90–100 million by mid-century and beyond. The increase in the number of deaths will be caused by population growth and population aging, in combination. The likely demand this will place on caregivers and services suggests a humanitarian issue – if not crisis – of enormous scale and complexity.

If modern palliative care had begun in the 1960s and 1970s with the certainties of cancer as a "terminal" disease of predictable course, by the second decade of the twenty-first century, it was located in far less predictable settings – sitting alongside curative interventions, closely tied in with the needs of older people, but also subspecialized in pediatrics as well as in other medical specialties. When accreditation for hospice and palliative medicine was achieved in the United States, it was formalized as a subspecialty of no less than ten fields of specialist care. In each case, new knowledge would be required in order to build a palliative care orientation matched to a specialist field of medical intervention and also to the social dimensions of the underlying disease type or the complexities, where they occurred, of multiple morbidities. In short, the medical landscape in which palliative care is required to operate had become much more complex and nuanced by 2015 than it had been 50 years earlier, when the first transatlantic collaborations were developing between the early pioneers.

Assisted Dying and Palliative Care

In the same period that palliative care gained traction, so too did debates and arguments in favor of the legalization of assisted dying – particularly in affluent countries. In the Netherlands, euthanasia was being practiced by physicians in a decriminalized context from the early 1990s onward, using a procedure of notification. The first legal case of euthanasia in the world occurred in the Australian Northern Territories in 1995, though the legislation was quickly reversed. Assisted suicide was approved by voters in the US state of Oregon in 1994 and legalized in 1997; subsequent endorsements occurred in the states of Washington (2008), Montana (2009), Vermont (2013), and New Jersey (2014). Full legalization of euthanasia and assisted suicide was passed in the Netherlands in 2001 – the first national legislation to do so. Belgium followed in 2002 with legalization of euthanasia only. In Luxembourg, euthanasia and assisted suicide were legalized in 2009. In Switzerland, where the subject of "suicide tourism" has attracted considerable debate, assisting a person to die is not an offense in certain circumstances.

Across many other countries, parliamentary debates have taken place, laws have been drafted, and wider public discussion gains momentum around the theme of assisted dying – how, in what circumstances, and with what safeguards might some form of legislation be framed that permits "choice" in dying – allowing persons a measure of control over the manner and timing of their death? Nevertheless, as the European Court of Human Rights has noted, there is no consensus in Europe on the ethics of assisted suicide, and indeed, the Council of Europe (2012) has stated that "Euthanasia, in the sense of the intentional killing by act or omission of a dependent human being for his or her alleged benefit, must always be prohibited."

The mainstream orientation of palliative care toward assisted dying is essentially oppositional. It is rare to see palliative care practitioners endorsing such practices and unusual for them to be viewed as a part of the delivery of palliative care.

In the public consciousness, there might not be so much "clear water" between the two positions however. The furor in the United Kingdom over the widespread use of the Liverpool Care Pathway (LCP), for example, revealed that from the perspective of relatives and patients themselves, the "best palliative care practice" – withdrawing interventions, closely monitoring symptoms, and aiming for the minimization of suffering and discomfort – was seen as a form of backdoor euthanasia. Indeed, the public discussion on this, the review of the LCP, and its eventual withdrawal in 2013 may well have brought to an end the era of what we might call "unconditional positive regard for palliative care" in the public consciousness.

The ideological orientation of palliative care practitioners has been to oppose assisted dying on the grounds that it is not needed when palliative care is fully available. And of course a subset opposes it on moral or religious grounds. This is challenged by supporters of assisted dying who claim that palliative care can never be the sole solution in all cases and contexts. They argue for choice and self-determination, suggest that even the best palliative care cannot always eliminate suffering, and also point out that some "consumers" of services may not be attracted to the ethos of palliative care for various reasons, such as its religious connections, its desire to delve into spiritual and existential areas where it does not belong, or its unwelcome associations with charity or volunteer involvement.

The protagonists have been engaged in a struggle where "dignity" has become the contested space, with each side claiming to provide it. Key to the palliative care ethos in the early days was the dignity of the patient. But organizations practicing or promoting assisted dying have adopted "dignity" within their names, for example, Dignitas (Switzerland), Dignity in Dying (United Kingdom), and Association for the Right to Die with Dignity (Spain). Both groups also have in common an emphasis on the "quality of life." For those in favor of assisted dying, quality of life is a key element in the argument. When it is poor and likely to deteriorate further, then the case for "choice" in dying must be advanced. But from the palliative care side, poor quality of life is seen as a clinical challenge to be overcome and remedied, through the skills and involvement of a multidisciplinary team.

In Belgium, the two viewpoints have been encouraged to sit side by side. Not only is euthanasia legal in Belgium, but at the time of its enactment, palliative care services were already well developed in the country. Indeed, some palliative care teams see euthanasia as a part of the service they offer – the so-called "integral" model of palliative care. Bernheim (2008) found that in Belgium the movement for legalization of euthanasia promoted the improvement of palliative care, and the existence of sufficient palliative care was instrumental in making the legalization of euthanasia ethically and politically acceptable. But this reciprocal approach may only be prevalent in one part of the country. One study aimed to find out how Flemish palliative care nurses and physicians think about euthanasia (Broeckaert et al. 2010). An anonymous questionnaire was sent to all physicians (147–70.5 % response) and nurses (589–70.5 % response) employed in palliative care teams and institutions in Flanders, Belgium. Analysis of the euthanasia questions identified three clusters: (moderate) opponents of euthanasia (23 %), moderate advocates of

euthanasia (35.2 %), and staunch advocates of euthanasia (41.8 %). A majority in all clusters believed that as soon as a patient experiences the benefits of good palliative care, most requests for euthanasia disappear and that all palliative care alternatives must be tried before a euthanasia request can be considered. Since most Flemish palliative care nurses and physicians were not absolutely against voluntary euthanasia, their attitudes may differ from those of their palliative care colleagues elsewhere. However, the attitudes of the Flemish palliative care nurses and physicians are largely contextual. For a very large majority, euthanasia is an option of last resort only.

In the US state of Oregon, where assisted dying has been legal since 1997 and where the number of cases remains low at around 0.21 % of all deaths per annum, claims have been made that there exists an ideal and comprehensive system for end-of-life care. Around one third of those issued with the lethal prescription to end their own lives never use it. Moreover, in a context where hospice and palliative care services are well developed, some 90 % of those availing themselves of the assisted dying legislation are on hospice programs. Could this prove a model for other jurisdictions? A study by Goy and colleagues (2003) found the majority of hospice nurses and social workers noted positive changes in the provision of palliative care by physicians since the introduction of the Oregon Death with Dignity Act, apart from a level of apprehension when prescribing opioid medications.

In the Netherlands, a rather different picture is in evidence. Here, palliative care developed in the context of a country which had decriminalized euthanasia and where such a practice was accepted by the majority of the population. This is the reverse of the situation in many other countries. It meant that palliative care in the Netherlands developed at first in isolation from the wider palliative care community – which paradoxically was often intensely interested in the practice of euthanasia in Holland. It was not until 1991 that the first hospice was established in the Netherlands. Later in the 1990s, significant government funds were invested in centers of excellence for the promotion of palliative care. Meanwhile, problems with the legal framework of euthanasia were in evidence – insecurity for physicians in following at times unclear legal requirements, a growing sense of the right of euthanasia on demand among the public, and a sense of normalization surrounding its practice (Gordijn and Janssens 2004). The high-care hospices in the Netherlands remain centers of opposition to euthanasia; elsewhere, the practice is not criticized, but there is a clear moral argument from the palliative care community that greater availability of their services will reduce the demand for assisted death. Despite this and the growth of palliative care in the Netherlands, the number of cases of euthanasia each year has been increasing.

The "slippery slope" argument continues to underpin some of the debates about whether or not assisted dying should be legalized. Perreira (2011), a professor of palliative medicine, has argued that in all jurisdictions that have legalized euthanasia or assisted dying, there has been disregard for the laws and safeguards that were put in place to prevent abuse and misuse of these practices. Prevention measures have included, among others, explicit consent by the person requesting euthanasia, mandatory reporting of all cases, administration only by physicians (with the exception of Switzerland), and consultation by a second physician. He provides evidence that these laws and safeguards are regularly ignored and transgressed in all the jurisdictions and increased tolerance of these transgressions in societies with such laws represents a social "slippery slope," as do changes to the laws and criteria that followed initial legalization. Although the original intent was to limit euthanasia and assisted suicide to a last-resort option for a very small number of terminally ill people, some jurisdictions now extend the practice to newborns, to children, and to people with dementia. A terminal illness is no longer a prerequisite. In the Netherlands, euthanasia for anyone over the age of 70 who is "tired of living" is now being considered. Legalizing euthanasia and assisted suicide, Perreira argues, places many people at risk, affects the values of society over time, and does not provide controls and safeguards.

From the still small number of countries where euthanasia and assisted dying are legal or decriminalized, it does not seem possible to find evidence that the development of palliative care has been compromised. Nevertheless, the two remain in uneasy relationship. It seems that within the community of palliative care practitioners, there remains higher resistance to legalizing assisted dying than is found among healthcare workers in particular and the wider society as a whole. As the extent of legalized assisted dying increases, this may precipitate a significant practical and moral challenge to the field.

Public Health, Equity, Human Rights, and Palliative Care

The need for palliative care has been heavily defined by the disease status of patients and their particular associated needs. But as the field matured, a new front of exploration opened up associated not with the diagnosis, organ of the body, or disease severity but with the particular places, settings, and social groups that might benefit from palliative care. Specialist interest began to develop in palliative care for prisoners, for homeless people, and for those identifying as lesbian, gay, bisexual, or transgender. Palliative care availability and access were increasingly framed as a matter of "equity" and then of "human rights." After 2000 a number of key developments took place in end-of-life care globally, including a series of important "summit" meetings on international palliative care development and the creation of the Worldwide Palliative Care Alliance. In autumn 2011, two separate declarations emphasized the importance of palliative and end-of-life care. In the first, the United Nations (2011) made reference to the need for palliative care provision in its statement about the care and treatment of people with noncommunicable disease. In the second, the World Medical Association (2011) made its case for end-of-life care improvement, stating that receiving appropriate end-of-life medical care must not be considered a privilege but a true right, independent of age or any other associated factors.

The palliative care community itself has also been active in producing exhortatory charters and declarations, usually promoted at specialist international conferences. These call on governments to develop health policies that address the needs of patients with life-limiting or terminal illnesses and to promote the integration of palliative care alongside other health services. They promote the need for access to essential medicines, including controlled medications, for all who need them and focus on the identification and elimination of restrictive barriers which impede access to strong opioids for legitimate medical use. They also emphasize the importance of the supply line for such drugs, along with appropriate rules and laws governing their distribution and prescription, by properly trained practitioners. Such calls underline the need for appropriate initial and undergraduate education programs for healthcare providers to ensure that basic knowledge about palliative care is widely disseminated and can be applied wherever there is the need in the healthcare system. They also highlight the need for postgraduate and specialty palliative care programs so that patients with complex problems can receive appropriate care. Key to all this – and at the heart of the public health orientation – is the requirement that palliative care is properly integrated into wider healthcare systems with services designed in relation to need and demand.

The claim that palliative care is a human right seems to be but partially founded (Open Society Foundations nd).

The United Nations Committee on Economic, Social and Cultural Rights has stated that it is critical to provide attention and care for chronically and terminally ill persons, sparing them avoidable pain and enabling them to die with dignity. Also, under Article 12 of the International Covenant on Economic, Social and Cultural Rights and Article 7 of the International Covenant on Civil and Political Rights, countries are obliged to take steps to ensure that patients have access to palliative care and pain treatment. Likewise, according to the UN Committee on Economic, Social and Cultural Rights, states are under the obligation to respect the right to health by refraining from denying or limiting equal access for all persons to preventive, curative, and palliative health services. Access to palliative care

is a legal obligation, as acknowledged by United Nations conventions, and has been advocated as a human right by international associations, based on the right to the highest attainable standard of physical and mental health. In cases where patients face severe pain, government failure to provide palliative care can also constitute cruel, inhuman, or degrading treatment. But, of course, governments of many different stripes can flout the law and ignore human rights.

Conclusion

Palliative care has undergone a period of rapid development since the 1970s, yet it remains an evolving field in which it faces several challenges. It must decide if its focus is on an "extra layer of support" throughout the trajectory of treatable illnesses, whether or not life is threatened, or whether its focus is on advanced and life-limiting illness, with a particular focus on end-of-life care. This requires clearer and more consensual definitions of its field of interest. It must also adapt to the consequences of population growth and aging. How can its specialist services be delivered at scale to all who might benefit from them? In turn, how can it adapt to new trajectories of aging and dying associated not with the "rapid decline" of the cancer model but rather with the uncertainties and prognostic difficulties associated with frailty and dementia? Over time it must also find a more sophisticated stance in relation to assisted dying. As increasing numbers of jurisdictions enshrine this in law, the oppositional stance of the palliative care community will become more difficult to maintain.

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Cross-References

- ▶ Assisted Suicide
- ▶ Death: Good Death
- ▶ Double Effect
- ▶ Hospice
- ▶ Palliative Sedation
- ▶ Public Health
- ▶ Slippery Slope

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Further Readings

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Palliative Sedation

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Abstract

Palliative sedation refers to the reduction of consciousness in patients who suffer from refractory symptoms at the end of life. In this entry, the most radical form of palliative sedation, namely, deep and continuous sedation until death, is at the focus of attention. It is one of the most contested medical decisions at the end of life and is generally considered to be a means of last resort, to be justified only when conventional interventions are no longer effective. After the concept of palliative sedation has been introduced, some empirical findings have been described, and a global perspective has been shortly introduced; three main areas of ethical debate are sketched out and analyzed: (1) the moral status of palliative sedation and differences and similarities between palliative sedation and physician-assisted death, (2) the decision to withhold or withdraw artificial nutrition and hydration in deeply sedated patients, and (3) the assessment of refractory symptoms and the expertise in palliative care that is required from the physician. It is concluded that, although guidelines show important similarities regarding these three domains, there are important differences of opinion among experts in the field. The debate on palliative sedation will remain of topical interest in the field of medical ethics and palliative care.

Keywords

Palliative sedation; Continuous deep sedation; Terminal sedation; Palliative care; End-of-life decision-making; Physician-assisted death

Introduction

Palliative sedation is one of the most contested medical decisions at the end of life. It refers to the reduction of the consciousness of a patient who is at the end of life, and it is indicated only if the patient, in spite of adequate palliative care, is suffering from symptoms that cannot be relieved without the lowering of consciousness. The practice can range from only a mild reduction of consciousness to the induction of deep sleep. In the former case, the patient will still be able to communicate with loved ones and caregivers, while in the latter case, consciousness is taken away. It can be administered intermittently or continuously. In the former case, the patient is awake during episodes, for instance, during the day, while in the latter case, the patient is not expected to wake up anymore. Mild and intermittent sedation certainly involves ethical aspects that need to be addressed, but the international debate has focused on one specific type of palliative sedation, namely, continuous deep sedation until the end of life. In this form of sedation, the patient is in deep sleep, not expected to wake up anymore, and expected to pass away on short term. It is this form of palliative sedation that is at the focus of this entry.

In the next section, the concept of palliative sedation will be further clarified. It will be indicated that in the ethical debate on the moral status of palliative sedation, many different definitions have been proposed, and authors attach many different meanings to the concept. The following section addresses some empirical data regarding

the use of palliative sedation in different countries in order to gain insight in current practice. Issues such as the prevalence of palliative sedation, its indications, and the aims of physicians will be addressed. Little is known about palliative sedation in countries outside the North Americas, Europe, and Australia (with Japan as exception). Yet, in a separate section, a global perspective on palliative care, pain relief, and palliative sedation is shortly introduced. The main section addresses the international ethical debate on palliative sedation. Under three headings, the most important ethical aspects of palliative sedation will be addressed. Views that are presented in important guidelines on palliative sedation will be contrasted to different views from the literature. Thus, it becomes clear that guidelines, although reflecting the majority view, presuppose a specific concept of palliative sedation that is not shared by everyone. First, the moral status of palliative sedation will be analyzed, and argumentation strategies to distinguish palliative sedation from physician-assisted death will be compared to strategies pleading for a moral indifference of palliative sedation and physician-assisted death. Second, the decision to withhold or withdraw artificial nutrition and hydration in deeply sedated patients will be analyzed. Whereas guidelines hold that this is a decision that should be separated from the decision to start palliative sedation, arguments against it may also have convincing force. And third, ethical aspects regarding the assessment of refractory symptoms will be addressed and related to the question whether expert consultation is a prerequisite before resorting to palliative sedation. It will be concluded that, although there is agreement between international guidelines on the moral status of palliative sedation, various authors in the field hold different opinions. The ethical debate on the moral legitimacy of palliative sedation is, in this respect, topical and expected to continue.

The Concept of Palliative Sedation

The term "palliative sedation" can be seen as an alternative to the term "terminal sedation" which was coined in the early 1990s by Enck. Authors argued that the adjective "terminal" might be associated with termination and with the physician's intention to shorten the patient's life by administering high doses of sedatives. Instead, the adjective "palliative" indicated that the physician's aim is (and, according to most, should be) to relieve symptoms that could not be relieved by using conventional measures. The use of the "new" term palliative sedation is now increasing in landmark publications and guidelines.

Definitions of palliative sedation vary widely. Definitions may or may not include the physician's intention, the depth of the sedation, the aim of sedation, the prognosis of the patient, and the proper indications of sedation. Depending on the definition that is used, practices of sedation may be included or excluded, potentially leading to major differences in empirical data. An example of a narrow definition that includes many of these aspects was introduced by Chater et al. in 1998. Terminal sedation was defined as: *"*The intention of deliberately inducing and maintaining deep sleep, but not deliberately causing death, for the relief of (1) one or more intractable symptoms when all other possible interventions have failed, or (2) profound anguish that is not amenable to spiritual, psychological, or other interventions, and the patient is perceived to be close to death*"* (cf. Claessens et al. 2008, p. 322). An example of a broad definition was introduced by Morita in 2002. Palliative sedation was defined as "The use of sedative medication to relieve intolerable and refractory distress by the reduction of patient consciousness" (cf. Claessens et al. 2008, p. 316). Contrary to the former definition, this definition does not speak of the depth of the sedation, the prognosis of the patient, and what symptoms might be proper indications. It does not explicitly exclude "causing death," and instead of aiming at deep sleep, it speaks of relieving intolerable and refractory distress to which reduction of consciousness is a means. Finally, the guideline on palliative sedation from the European Association for Palliative Care (EAPC) speaks of: "The monitored use of medications intended to induce a state of decreased or absent awareness (unconsciousness) in order to relieve the burden

of otherwise intractable suffering in a manner that is ethically acceptable to the patient, family and health-care providers" (Cherny and Radbruch 2009, p. 581). What is noteworthy here is that the definition includes ethical acceptability, while the guideline later on addresses practices of "injudicious use of palliative sedation." The question whether injudicious use of palliative sedation is ethically unacceptable and thus not part of the definition remains open. Apart from the many definitions of palliative sedation, another unclarity relates to what caregivers in practice label as palliative sedation. Thus, the use of opioids with potentially sedative side effects is sometimes labeled as palliative sedation as well as the use of sedatives with the explicit intention to shorten the patient's life. In summary, palliative sedation is an umbrella notion that can have many different meanings. Dependent on the definition that is used, practices can be labeled as "palliative sedation" or not. This has important consequences for the representativeness of empirical data on palliative sedation.

Empirical Data on Palliative Sedation

Figures indicating the prevalence of palliative sedation are highly dependent on the definitions used. This may partly explain the high differences in prevalence between different countries. Prevalence figures of continuous deep sedation until death show a range from 2.5 % (Denmark) to 16.5 % of all deaths (UK). Apart from definitions, these differences may also be explained by the health-care system. In countries where palliative care is less well developed, figures may be higher than in countries with specialist palliative care available for all. Caution is required here since the high figure of palliative sedation cases in the UK, with a well-developed specialist palliative care system, strikes as odd. In countries where patients at the end of life die at home and are cared for by their general practitioner, figures may also be higher, since GP's in general are less well trained in specialist palliative care (cf. Ten Have and Welie 2014).

The far-reaching practice of rendering a patient unconscious is considered only justifiable in cases where the patient is suffering unbearably from refractory symptoms. Most empirical studies mention delirium, dyspnea, pain, fatigue, and agitation as symptoms that were present before palliative sedation was initiated. A minority of the studies mention psychosocial and existential symptoms such as anxiety and anguish (cf. Claessens et al. 2008). In the vast majority of cases, more than one of these symptoms were present, and when psychosocial or existential suffering was mentioned, this went almost always together with mention of other physical symptoms. Evidence from Spain suggests that, in countries where patients are typically not informed of their diagnosis and prognosis, psychosocial and existential suffering may be more often seen as a refractory symptom. The patient is unknowing of his/her prognosis, and unconsciousness is seen as the "best way out" (cf. Materstvedt and Bosshard 2009). But the patient may also realize or suspect that he/she has come at the end of life and feel anxious and unable to communicate this anxiety. For these patients, unconsciousness may sometimes be considered the lesser evil.

Typically, but not exclusively, palliative sedation occurs in cancer patients with a very short expected prognosis of days only. Next to cancer, patients suffering from cardiovascular diseases, pulmonary diseases, and neurological diseases may also require palliative sedation. And in a small minority of cases, patients live longer than expected and may remain in deep sleep for 2 weeks or more. In most cases of palliative sedation, artificial nutrition and hydration are withheld although it is suggested that, especially in countries with a strong Catholic tradition, artificial nutrition and hydration are continued (or even started). Another reason for continuing these measures may be the relatively long prognosis of the patient (Miccinesi et al. 2006).

Palliative Sedation in South America, Africa, and Asia

In many countries outside Europe, the North Americas, and Australia, implementation of palliative care is hampered by a variety of factors (with

Japan and South Africa as exceptions). Availability, affordability, and accessibility of services are limited. Governments and health-care systems do not always facilitate palliative care implementation. Education in palliative care, for instance, in the nursing and medical curricula, is often inadequate, leading to insufficient expertise and a lack of trained researchers. Instead of full multidisciplinary palliative care, it is often not more than "partial" palliative care that can be provided.

An important problem in the care for the incurably ill in many developing countries relates to the availability of opioids. Estimates from Latin America indicate that less than 1 % of the terminally ill needing opioids actually receive it (Wenk and Bertolino 2007). Estimates from India show even lower percentages (Rajgopal and Joranson 2007). Restrictive government regulations and prejudices among medical staff (e.g., relating to addictive effects, life-shortening effects, and respiratory suppression), together with a lack of financial resources, make pain relief for a substantial part of the incurably ill suboptimal. Interestingly, evidence from India suggests that prejudices among those caregivers who did receive education in specialist palliative care are virtually nonexistent (Gielen et al. 2011). Specialist Indian nurses and physicians indicate that if opioids are titrated well, there are no life-shortening effects to be expected at all. As long as dosages are administered proportionately, they see no limit in the use of morphine. The only criterion they use is the patient's pain while possible addictive side effects are accepted. One of the African countries where palliative care is developing rapidly is Uganda. With government support as well as support from palliative care organizations, the goal is to implement palliative care throughout the country. Permissive laws regarding opioid use enable caregivers to make morphine affordable and available to all who are in need (Jaqwe and Merriman 2007).

Regarding the prevailing views on palliative sedation in these countries, there is a dearth of information, and more empirical ethical research is necessary. The interview study from Gielen mentioned above indicates that the (specialist) Indian nurses and physicians did not oppose light sedation even though the majority expressed concerns regarding deep continuous sedation. Concerns were related to unacceptable side effects and the fact that daily activities could no longer be carried out and that social contacts and communication would become impossible. Some pointed out that deep sedation would not be necessary in the presence of specialist palliative care. Those who did not exclude deep continuous sedation stressed careful assessment of the patient's needs and prognosis (Gielen et al. 2011). The results of this study cannot easily be extrapolated to other caregivers since the interviewees all received specialist training and are thus part of a small minority. Decision-making processes in many developing countries may substantially differ from North American, European, and Australian perspectives. Especially the role of the family is important in this respect. Many patients are not informed of the diagnosis, and, instead, a family member decides for the patient. In cases of poverty, decisions are complex from a financial point of view, and choices will have to be made by the patient and family between provision of adequate palliative care for the patient, the marriage of a child, or his/her education (Chaturvedi 2008).

Ethical Controversies Regarding Palliative Sedation

In this section, three main ethical controversies regarding palliative sedation will be presented and analyzed. First, the debate on the moral status of palliative sedation as opposed to physician-assisted death will be sketched out. Second, the ethical question of whether palliative sedation should also include a decision to withhold or withdraw artificial hydration and nutrition will be addressed. Finally, the ethical controversies regarding what constitutes "refractory symptoms" will be described and analyzed.

Palliative Sedation and Physician-Assisted Death

Whereas palliative sedation and physician-assisted death (PAD) both are seen as means of last resort, to be justified only if the best available

palliative care has failed to provide adequate symptom relief, national and international guidelines strictly distinguish between these practices. In fact, one of the main aims of the guideline of the Royal Dutch Medical Association is to distinguish palliative sedation as sharply as possible from PAD (RDMA 2009). Thus, the guideline contrasts palliative sedation with PAD indicating the following (non-exhaustive) distinctions: Whereas palliative sedation is normal medical treatment, PAD is not. Whereas palliative sedation is a decision between the doctor, patient, and relatives, PAD requires independent consultancy and notification of a review committee. Whereas the aim of palliative sedation is symptom relief, the aim of PAD is to put an end to suffering. Whereas the means to that aim is the lowering of consciousness in palliative sedation, the means in case of PAD is life termination. And whereas palliative sedation is reversible in principle, PAD is not. International guidelines, such as the abovementioned EAPC guideline but also the American Medical Association's Code of Medical Ethics, hold that palliative sedation is an acceptable part of palliative care and should be considered fundamentally different from PAD. But since PAD in most countries is illegal, the distinctions between PAD and palliative sedation receive less attention than in the Dutch guideline.

This brings us to the question whether palliative sedation can be conceived of as an alternative for PAD. According to many ethicists and palliative care physicians, this seems to be the case as they argue that PAD is not necessary to legalize if palliative sedation is accepted as a rarely needed option in palliative care. Excellent palliative care, together with palliative sedation in exceptional circumstances, makes the legalization of PAD superfluous, according to these authors. The presupposition in this line of argumentation is that palliative sedation is morally preferable to PAD (cf. Raus et al. 2011). In this respect, it should be acknowledged that, in countries where PAD is legal, cases can be conceived of in which patients at the end of life who are suffering unbearably can be offered a choice between palliative sedation and PAD. Some will opt for sedation, for instance, because they do not want to be killed by the physician with whom they have a relation of confidence. Others may opt for PAD, for instance, because this will last shorter and will be less burdensome for the loved ones. In these cases, palliative sedation may be seen as an alternative to PAD. Yet, in the majority of cases, patient characteristics may make them either eligible for palliative sedation or for PAD but not for both. Patients with a long life expectancy may not be eligible for palliative sedation, and patients whose source of suffering is mainly psychological or existential may be eligible for PAD but not for palliative sedation (see below). Patients who are already subcomatose or delirious may not be eligible for PAD, since their competency is diminished, but may be deeply sedated if the physician and the legal representative agree. In short, although in some cases palliative sedation may be an alternative to PAD (in cases where patients have a short prognosis and are competent to decide), in most cases, patients do not really have a choice (because their prognosis is too long for palliative sedation or their competency too diminished for PAD).

Despite the consensus on the moral acceptability of palliative sedation in specific cases, the debate on the moral status of palliative sedation is continuing, and several authors have questioned whether palliative sedation and PAD are as different as guidelines seem to suggest. From a consequentialistic point of view, it is argued that intentions are by no means decisive for the morality of an act. Instead, morality relates to the balance between harms and benefits. In this line of reasoning, the act is morally right if it leads to less harm or more benefit than was the case before the act was performed. Thus, irrespective of whether the effects of an act were intended or merely foreseen, the morality of an act is judged according to its consequences. The responsibility of the actor equally relates to intended consequences and non-intended but foreseen consequences. If this line of reasoning is applied to palliative sedation, differences between palliative sedation and PAD become blurred. After all, the consequence of palliative sedation is that the patient is put in deep sleep and not expected to wake up anymore. In other words, one can say that

the patient is in a state of social and psychological death since consciousness is taken away. The consequence of PAD is that the patient dies within minutes. In both cases, the experience of suffering is taken away. The only differences between the options are that palliative sedation takes longer and that the patient under sedation is still biologically alive. This is why palliative sedation is sometimes considered to be equivalent to euthanasia with other means and in a slower manner; palliative sedation is "slow euthanasia" (Billings and Block 1996). At least from the perspective of the patient, it might make little difference whether she is biologically alive but unconscious or whether his/her life is terminated by a physician.

Also, other argumentation strategies than the strictly consequentialistic strategy can be used in order to minimize distinctions between palliative sedation and PAD (Janssens et al. 2012). At least some argumentations used against PAD seem also applicable to palliative sedation. For instance, one of the main arguments against PAD relates to the slippery slope. It means that once the door is opened for PAD under strict circumstances, these circumstances will be widened, and cases will occur that we would not want to allow. PAD will become a simple solution to complex problems, and as such, it will make the further development of palliative care needless. But similarly, this argument can be used against palliative sedation as well. Why could palliative sedation not also be seen as a simple, one-size-fits-all solution for complex problems regarding pain and symptom management? And do the criteria proposed in guidelines (such as refractory symptoms, short prognosis) not also run the danger of being widened time after time? Empirical evidence suggests that the prevalence of palliative sedation is increasing in several West European countries (e.g., Rietjens et al. 2008), while there is no evidence that the quality of palliative care services is decreasing. Another argument that may blur the distinctions supported by the guidelines relates to the concept of personhood. If the capacity to think is part of one's definition of person (as, e.g., the seventeenth-century philosopher John Locke held), it would imply that not only PAD but also palliative sedation would terminate the person's life. Palliative sedation then should also be conceived as life termination (with or even without the request of the patient). It not only makes "thinking" impossible but takes away all preconditions for "experience." Finally, empirical evidence suggests that physicians carry out acts with more than only one intention. In a substantial proportion of cases of palliative sedation, one of the physician's intentions is to shorten the life of the patient (next to symptom relief). Intention in those cases may be a concept ill-equipped to distinguish palliative sedation from PAD. The concept of intention that is presupposed in the guidelines may be a simplified, "sanitized" concept which has little relevance for practice where different intentions go hand in hand.

As was shown above, the consensus within the expert community that palliative sedation and PAD are fundamentally different is contested. Yet, also for this majority, opinion-convincing arguments can be found. The view that it is at least at first sight (prima facie) simply wrong to take another person's life, independent of the consequences, is solidly rooted in the morality of any civil society. Life has value, not only instrumental but also in itself. And also the patient under sedation is still "one of us," endowed with legal rights and dignity. This conviction only makes life termination and palliative sedation different. Furthermore, arguments favoring the relevance of intention for the morality of an act are convincing. We would all consider the oncologist who likes to make people nauseated and bald as a consequence of his therapy a bad and horrifying caregiver. And if in retrospect it appears that the bad effects of an act came out but the good effects not, we would regret the act, exactly because one's intention had not come out. In other words, one does not want to bring about foreseeable harms, but one accepts them because a proportionately grave reason (e.g., the reduction of suffering) is present. Thus, it seems unconvincing to argue that the foreseeable negative effects of an act cannot be unintended. A more practical objection to the role of intentions is that the intention of a physician can never be assessed by a third party. Physicians who sedate a patient may have the intention of shortening his life (and as was noted

above, empirical evidence indicates that in a minority of cases, physicians do have this intention, exclusively or among other intentions). This is where the principle of proportionality comes into focus. In retrospect, the act itself may give clues of the intention of the physician. Was the dosage proportionate to the desired effect? Did the physician assess whether symptom control had been achieved before increasing the dosage? Or was the dosage increased every hour without assessing the patient's condition? If dosages in retrospect appear to be disproportionate to the goal of symptom relief, then this reveals something of the intention of the physician. At least one of her intentions appears to be life shortening. If, as is argued in the guidelines, intentions matter morally and if it is at least at first sight wrong to take another person's life, there seems to be a sound basis to evaluate palliative sedation differently from PAD. Still, relating to this point, another difficult issue should be raised in this context.

Palliative Sedation and the Withdrawal or Withholding of Artificial Hydration (and Nutrition)

The moral status of artificial hydration and nutrition merits an ethical analysis of its own. Ethicists, especially those from Christian, Muslim, or Jewish background, argue that the provision of food and fluids is part of good care that can never become futile. In the end of the 1980s however, especially in the context of patients who had been in persistent vegetative state for many years, courts ruled that artificial hydration and nutrition should be seen as forms of medical treatment that can in exceptional circumstances become futile. Court rulings showed consistency, e.g., in the cases of Tony Bland in the UK, Terri Schiavo in the USA, and Ineke Stinissen in the Netherlands. Artificial nutrition and hydration were discontinued in these cases, after permission from the courts, and the patients died as a result. Although the ethical view that food and fluid provision is part of good care may have convincing force, the presupposition in this entry is that artificial nutrition and hydration are medical treatments that can become futile, just like all medical treatments under circumstances can be withheld or withdrawn.

Most patients who receive palliative sedation have a life expectancy of days only. Many will have stopped eating and drinking themselves while still conscious. For these patients, the decision to start artificial hydration after they have been sedated would simply be malpractice. There is a risk that hydration will cause more harm than good since edema or dyspnea may occur as a consequence of the administration of fluids. Yet, patients may also suffer from refractory symptoms when they still have a reasonable life expectancy or when the life expectancy is difficult to estimate. In these cases, the decision to withhold artificial hydration may well have an impact on the patient's life expectancy (since withholding of nutrition is less likely to lead to a quicker death, nutrition is not at the focus here). This is why the Dutch guideline specifically states that the prognosis of the patient may not be longer than 2 weeks (RDMA 2009). The EAPC guideline speaks of patients who are in the very terminal stages of their illness with a prognosis of hours or days at most (Cherny and Radbruch 2009). Both guidelines explicitly state that the decision to withhold/withdraw hydration should be made separately from the decision to start palliative sedation. While the EAPC guideline acknowledges a heterogeneity of attitudes toward the decision to withhold hydration, the RDMA guideline speaks in this context of futile medical treatment and of a medical decision, made by the physician, that should be well accounted for. In cases where the patient is still able to take fluids, intermittent sedation can be proposed, not continuous and deep sedation. In this respect, the Dutch guideline goes a step further, compared to the EAPC guideline. The idea is that artificial hydration does not contribute to the quality of life of the patient in deep sleep. It is questionable whether the concept of quality of life applies to the unconscious patient at all.

Still, one of the main reasons to limit the patient's prognosis to a maximum of 2 weeks is that if hydration is discontinued, the patient may well die as a consequence of dehydration. And in these cases, palliative sedation may become

similar to PAD. Since the aim of many guidelines is to distinguish palliative sedation as sharply as possible from PAD, these situations are considered undesirable. From an ethical point of view however, one can question whether this position is tenable. After all, many medical decisions to refrain from medical treatment may have a life-shortening impact. Abstaining from chemotherapy, dialysis, or antibiotics may well have life-shortening effects, while these are relatively unproblematic decisions in certain situations that in no way seem similar to PAD. It is even widely acknowledged that pain medication with life-shortening effects can be morally defensible as long as the aim is to reduce pain. So why would the decision to refrain from artificial hydration make palliative sedation similar to PAD in cases where the prognosis is more than 2 weeks? Is foregoing medical treatment, even if life-shortening effects are taken into account, not fundamentally different from active life termination? If dehydration is accepted in the abovementioned cases of patients in persistent vegetative state (who all died as a result of the decision to withdraw artificial hydration), why would it not be acceptable in patients who are deeply sedated and may die earlier as a result? It can be argued that if patients suffer from refractory symptoms that cannot be relieved by the best available palliative care, palliative sedation may be indicated, irrespective of the prognosis. And since artificial hydration can be considered futile in a patient in an unconscious state, not expected to wake up anymore, this could still be seen as a form of normal medical treatment, even if life-shortening effects should be taken into account. If that is so, one might even argue that the decision not to artificially hydrate the patient is part of the decision to start palliative sedation, thus criticizing the consensus that dehydration is a separate decision. Dehydration and palliative sedation, in this line of reasoning, come in one pack.

Concludingly, among the medical community, there is consensus that withdrawal or withholding of artificial hydration is an acceptable option for patients receiving palliative sedation. At the same time, argumentation strategies may differ, and, furthermore, it should be acknowledged that especially in Christian, Muslim, or Jewish countries, dehydration maybe considered inhuman per se.

Refractory Symptoms and Expert Consultation

Palliative sedation can only be started if refractory symptoms are present. Generally, refractoriness means that conventional means of diminishing suffering (1) are no longer available or (2) require too much time to become effective or (3) have intolerable side effects. The assessment of refractory symptoms therefore requires medical expertise since the physician should have expert knowledge of the available pain and symptom medications, should be able to exclude reversible causes of suffering, should know how long it takes for a means to become effective, and should be able to predict the expected side effects. However, the assessment cannot do without the voice of the patient. What may be unbearable for one patient may not be so for another. What is an intolerable side effect for one patient may not be so for another. In other words, the assessment of refractory symptoms is an interactive process provided that the patient is still able to communicate. Thus, although there is general agreement that refractory symptoms are a prerequisite for palliative sedation, what constitutes refractoriness is open for debate.

Empirical evidence indicates that the prevalence of palliative sedation differs substantially, not only per country but also per setting. This may be partly explained by terminological and conceptual unclarities as was indicated above or by the type of patients that are seen in a specific setting. However, another explanation must be mentioned here. Assessment of refractory symptoms requires from the physician expertise in palliative care. This is why the EAPC guideline speaks of injudicious use of palliative sedation in cases where before resorting to palliative sedation there was failure to engage with clinicians who are expert in pain and symptom relief despite their availability (Cherny and Radbruch 2009). This contrasts interestingly with the Dutch guideline where it is held that since palliative sedation is normal medical

treatment, consultation is not mandatory. Only in cases where physicians have doubts regarding their expertise, the professional norm prescribes expert consultation, according to the Dutch guideline. Furthermore, in exceptional cases where the patient is suffering from refractory symptoms but the life expectancy is difficult to estimate, consultation of an expert physician is considered a requirement (RDMA 2009).

In this respect, it is noteworthy that authors have pointed to the "generalization of palliative sedation," meaning that not only experts in palliative care resort to palliative sedation but also specialists outside palliative care such as general practitioners are more and more using this option (Ten Have and Welie 2014). The fact that the prevalence of palliative sedation is increasing in many countries would provide support for this explanation. What is at stake here is the widely held assumption that palliative sedation is a means of last resort, to be started only when the best available palliative care is unsuccessful in relieving symptoms. Patients who are sedated without resort to the best available palliative care seem to have been treated disproportionately. Considering the far-reaching effect of palliative sedation, namely, taking away consciousness, this seems to be problematic from an ethical point of view. Expert consultation in cases where the treating physician is not or insufficiently trained in palliative care seems to be a moral requirement. It is noteworthy in this respect that only in a small minority of cases in the Netherlands is expert consultation sought. And whereas all guidelines hold palliative sedation to be normal medical treatment, to be decided upon by the physician and patient (or representative) together, normal medical treatment and obligatory expert consultation are by no means by definition mutually exclusive.

Palliative care has always been seen as "total care" acknowledging the intrinsic connectedness of physical, psychological, social, and existential suffering. In some cases, the physical symptoms may be well controlled while the patient is suffering unbearably on psychological or existential grounds. It is well known from empirical studies that fear for future suffering, for increasing dependence, for the possibility to suffocate, etc., is a main reason for many patients to request PAD. It is also widely acknowledged that these forms of existential suffering (fear, anxiety, hopelessness) may be considered unbearable and hopeless. The question now is whether these forms of suffering may also be considered as refractory symptoms, making patients eligible for palliative sedation. There seem to be good reasons to treat these cases differently from cases where physical symptoms are at the foreground. First, physicians may lack expertise in treating psychological and existential symptoms, and multidisciplinary care, at least including a psychologist who has built up a relation with the patient, seems required. Second, the presence of existential suffering does not say much about the life expectancy of the patient. It seems malpractice to deeply sedate a patient with a long life expectancy for psychological discomfort. Third, the severity of such symptoms may vary in time, and it is immensely difficult to make sure that the symptoms are hopeless and conventional interventions ineffective. Again, expert psychologists, chaplains, or social workers are necessary in the care for these patients. There is no consensus at the moment on the right answer to this question. At the least, it is acknowledged that physicians must be more hesitant in resorting to palliative sedation in these cases, and multidisciplinary care is required. Before resorting to deep sedation, light and intermittent sedation should have been tried. Furthermore, the life expectancy of the patient must be short, and, finally, cases where physical symptoms are under control and where the presence of psychological, social, or existential symptoms *only* constitutes unbearable suffering may be considered improper indications for deep continuous sedation until death by a majority of caregivers. Paradoxically, in countries where PAD is legal, these patients may be considered eligible for PAD but not for palliative sedation.

Concludingly, the presence of refractory symptoms is pre-conditional for palliative sedation. In order to assess refractoriness, expertise in palliative care is required, for instance, to exclude

reversible causes of symptoms. There is however no consensus on the question of whether expert consultation is required in the assessment of refractory symptoms before resorting to palliative sedation. And whether also psychological, social, or existential symptoms may be considered as an indication for palliative sedation is still under debate.

Conclusion

Palliative sedation in its most radical form takes a patient's consciousness away. The patient is not expected to wake up anymore and dies in deep sleep. This makes palliative sedation one of the most contested medical decisions at the end of life. According to influential guidelines, it is a morally defensible option in palliative care in cases where symptoms have become refractory in spite of the best available palliative care. However, the ethical debates on the moral status of palliative sedation and the differences and similarities between palliative sedation and physician-assisted death (PAD) will be of continuous actuality. While guidelines argue that palliative sedation should be sharply distinguished from PAD, several authors are of the opinion that, at least from the patient's perspective, palliative sedation is indifferent from PAD. Guidelines hold that the decision to withhold or withdraw artificial hydration and nutrition in a deeply sedated patient is a separate decision. Yet, authors who presuppose that artificial nutrition and hydration are medical decisions may argue that in a deeply sedated patient, artificial nutrition and hydration are futile measures since, from a medical point of view, they may be harmful, while from the perspective of the patient, they do not contribute at all to quality of life. The question regarding the assessment of refractoriness of symptoms is another contested issue in the debates on palliative sedation. Although expertise in palliative care is required, there is no consensus on the moral or legal duty to consult experts in palliative care before resorting to palliative sedation. And differences of opinion exist among the medical community on the question whether symptoms of psychological, social, or existential origin can be considered proper indications for palliative sedation.

Many more topics will remain of interest in the debates on palliative sedation, and in this entry, only some important topics have been addressed. Topics that were only hinted at above but merit more attention include the concept of person and what effects palliative sedation has on this concept, the principle of proportionality and indicated dosages of sedatives, and, finally, the principle of patient autonomy and rights of legal representatives if the patient is considered incompetent.

Philosophers, ethicists, lawyers, physicians, nurses, and other caregivers should uphold dialogue in respect for one another's views so that guidelines in the future can be amended if considered necessary and that care for patients at the end of life is improved. Palliative sedation can be part of good palliative care, albeit in exceptional circumstances only and if criteria are continuously evaluated by all those involved.

Cross-References

- ▶ Death: Good Death
- ▶ Double Effect
- ▶ Euthanasia: International Debate
- ▶ Pain
- ▶ Palliative Care
- ▶ Suffering

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Pandemics

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Abstract

This entry reviews bioethical principles relevant to responding to pandemic threats in general and pandemic influenza in particular. Selected issues are examined to draw out some of the advantages and limitations of various principles or competing bioethical frameworks. The entry considers both examples of issues impinging on civil rights, such as respect for bodily integrity (vaccination) or liberty (quarantine), as well as illustrative examples of issues that raise issues of distributional justice (regional inequalities, priorities for vaccination) or the realization of socioeconomic rights (the right to health). Consideration is given to debates about the responsibilities owed by and to essential service workers (health-care or emergency workers) and the issue of protection of vulnerable individuals or groups. The entry concludes by identifying the ongoing challenges posed by globalization.

Keywords

Health and human rights; Infectious diseases; Influenza; Public health; Quarantine

Introduction

Infectious disease pandemics pose special ethical challenges, as illustrated by influenza pandemics. While other pandemic threats such as Ebola or HIV/AIDS may be characterized by higher rates of mortality, be of longer duration, generate higher levels of public fear, or make greater

demands on medical and other resources, pandemic influenza is emblematic of the main ethical challenges. These challenges include: striking a balance between individual and collective rights and interests, achieving distributional justice, balancing principles of utility and equity, ensuring protection of vulnerable individuals and groups in society, and the need to exercise public health powers with respect for human rights.

The International Health Regulations (IHR) set out countries' obligations in relation to control of infectious diseases. While international sanitary conferences and conventions date back to the 1800s, the International Sanitary Regulations were first adopted by the World Health Assembly in 1951, renamed as the International Health Regulations in 1969, and revised in 2005, with the revised IHR taking effect in 2007 (World Health Organization 2011, p. 6). The revised IHR set out a broader and more flexible list of diseases and public health events of which countries are obliged to notify the World Health Organization (WHO) than earlier versions of the IHR (Gostin 2014).

History and Development

Documentation of influenza pandemics dates back to the sixteenth century, with an average of three pandemics per century since then (World Health Organization 2007). There were three influenza pandemics during the twentieth century. The first, known as the "Spanish flu" pandemic, started in 1918. It is estimated that 20–50 million people worldwide died during this pandemic. The influenza pandemics in 1957–1958 and in 1968–1969 were milder and each caused an estimated one to four million deaths (World Health Organization 2011, p. 14). In 2009 the World Health Organization declared an influenza pandemic from the A/H1N1 virus (World Health Organization 2011).

Conceptual Clarification/Definition

A pandemic is defined as the dissemination of a new infective disease to which immunity has not been developed in a widespread manner across a significant part of the world, encompassing a large region, or several countries. Although often assumed to be associated with high mortality as was the case with the 1918 influenza pandemic, pandemics can be milder events, as demonstrated by subsequent influenza pandemics.

No one pandemic is similar to another, though particular diseases may generate common features. Thus, an influenza pandemic is characterized by: (i) a change in the influenza subtype; (ii) higher mortality rates among younger groups, rather than the elderly traditionally at risk from seasonal influenza; (iii) several waves of the pandemic; (iv) increased transmissibility compared to seasonal influenza; and (v) geographic variation in the impact of the pandemic (Miller et al. 2009). In 1999 the WHO developed a set of "phase alert levels" to assist in clarifying its advice to countries about planning for a pandemic, with the WHO advice updated a number of times since then (World Health Organization 2011).

Bioethical principles have evolved over time, beginning with the enunciation in the US Belmont Report in 1978 of principles of autonomy (respect for persons), beneficence, and justice, popularized by the work of Beauchamp and Childress and later supplemented by a fourth principle of non-maleficence. These principles are relevant to ethical considerations relating to pandemics. Alternative bioethical frameworks, developed to meet criticism of an unduly individualistic focus, place weight on principles of solidarity, communitarian considerations, and interpersonal reciprocity and seek to better accommodate globalization.

Ethical Issues in Pandemic Management

Pandemics by definition are a global health phenomenon, calling for wider global bioethical frames which accommodate values such as community consent and engagement, international travel and trade, and global communitarianism, beyond the classical bioethical values such as respect for individual autonomy or informed consent to treatment so characteristic of earlier

medical or health-care ethics (Ten Have 2011). While various ethical values and frameworks will provide a useful lens for evaluating and balancing differing interests that may arise in planning for and responding to pandemics, their application will necessarily be informed by local factors (World Health Organization 2007). Cultural, religious, political, and economic factors may also be relevant to pandemic planning and responses to pandemics.

The Joint Centre for Bioethics Pandemic Influenza Working Group at the University of Toronto has outlined ten substantive and five procedural values to guide decision-making during an influenza pandemic. The ten substantive values are:

- Individual liberty including the need to ensure that restrictions are proportionate, necessary, relevant, equitable, and the least restrictive
- Protecting the public from harm
- Proportionality (i.e., employing responses that are proportionate to the risks)
- Consideration of individual privacy and protection of public health
- The duty of health professionals to provide care during a pandemic
- Reciprocity and the ethical requirement to support those who bear a disproportionate burden in relation to measures undertaken to protect the broader community
- Equity in access to health care and health services
- Trust and the importance of transparency in maintaining trust
- Solidarity within and between countries
- Stewardship by those with governance and decision-making responsibilities (Joint Centre for Bioethics Pandemic Influenza Working Group 2005)

The five procedural values outlined by the Joint Centre for Bioethics Working Group are that decisions be:

- Reasonable
- Open and transparent
- Inclusive
- Responsive
- Accountable (Joint Centre for Bioethics Pandemic Influenza Working Group 2005)

This statement of substantive and procedural values is a helpful elaboration of the ethical values relevant to planning for and responding to pandemics.

Pandemics and Individual Autonomy

Autonomy is a core principle in contemporary bioethics. Within the context of public health including pandemics, individual autonomy must be balanced against the interests of the broader community. Feminist scholars have critiqued individualistic understandings of autonomy, positing instead the theory of relational autonomy which expressly conceptualizes an individual's autonomy in terms of the relationships between individuals. Theories of relational autonomy thus seek to move beyond individualism toward a greater recognition of the interconnected nature of human society. Conceptualizing autonomy in relational terms may be relevant to infectious diseases in which an individual may both be infected with an infectious disease and may spread the disease to others. That is, when the individual may be both "victim and vector" (Battin et al. 2009). Societies grounded in strong communal values of collective decision-making (as in Asia and Africa) and/or sharing of goods (as also in indigenous communities) require a different approach to implementation of policies such as quarantine of individuals, restriction on the numbers of caregivers, or infection control measures. These communal values or social capital can be both a boon (enhancing the practical capacity to provide care as well as resonating with the so-called ethic of care) and being potentially a bane in pandemic management (in risking exacerbation of stigma of sufferers and presenting a challenge to social distancing measures).

Recognition of the relevance of human rights to health has also been an important factor in contemporary understandings of pandemics. There has been an increasing recognition of the relevance of human rights to health, including in

the context of infectious diseases (Gostin 2014). International human rights law dates back to the period following the World War II and the adoption by the United Nations General Assembly of the *Universal Declaration of Human Rights* (Gostin 2014). Human rights relating to health are now articulated in a number of international declarations and conventions (Gostin 2014). The importance of a human rights approach to health was articulated in response to HIV/AIDS and it was argued that respect for human rights could support human health (Gostin 2014). In contemporary public health ethics and law there is recognition of the importance of implementing public health measures with respect for human rights. For example, the revised International Health Regulations (2005) require in Article 3(1) that they be implemented by member states in accordance with respect for "dignity, human rights and fundamental freedoms of persons."

At times, restrictions on the movement of individuals may be necessary to limit the spread of disease. The theories of autonomy outlined above and the conceptualization of health in terms of human rights have relevance to the limits on the movement of individuals in response to a pandemic. Quarantine and isolation have traditionally been used to control the spread of infectious diseases. Quarantine involves restrictions on the movement of an individual or individuals who, although currently healthy, have been exposed to an infectious disease. Isolation refers to restrictions on the movement of an individual who is infected with an infectious disease (Nuffield Council on Bioethics 2007). "Social distancing" measures, such as closure of schools, may also be used to limit the spread of infectious diseases and are seen as less restrictive of individual liberty than quarantine or isolation (Nuffield Council on Bioethics 2007). Quarantine and isolation were used by some countries in response to severe acute respiratory syndrome (SARS) in 2003 (Gostin 2014; Nuffield Council on Bioethics 2007).

A number of ethical values may be relevant to public health measures that are restrictive of individual liberty. For example, the principle of reciprocity may require that individuals who are placed in isolation or quarantine are provided with appropriate supports, such as food supplies (Joint Centre for Bioethics Pandemic Influenza Working Group 2005). In addition, public communication of information about a pandemic raises delicate issues around respect for the right to know, the maintenance of public trust and cooperation, and the risk of public panic and disorder in the event that public information is too scant. Countries have been shown to differ in the weight given to individual as against collective or communal interests in the management of pandemics, as illustrated by the use of quarantine in the SARS crisis in 2003 (Jacobs 2007).

Respecting Human Rights

Human rights include both socioeconomic or "positive" rights (such as the right to health) and civil rights ("negative" protections, such as freedom from arbitrary restrictions on liberty). The International Health Regulations (2005) include both as an obligation for countries to develop the capacity to detect and respond to public health emergencies (a form of the positive human right to health) and require that the IHR (2005) are implemented in accordance with respect for human rights (a negative right). Many negative human rights are implicated by public health measures apart from liberty of movement impacted by quarantine or travel restrictions. Among the other rights engaged are those of privacy (impacted by surveillance and data collection measures associated with early warning and detection) and due process (such as a fair hearing and independent review).

Thus, the 2007 WHO guidance was shaped by existing human rights principles such as the nonbinding Siracusa Principles developed by nongovernment organizations and adopted by the UN Economic and Social Committee in 1984 (World Health Organization 2007), which guide the exercise of powers such as managing threats to public health and delivering appropriate health care to those affected. The Siracusa Principles insist that the exercise of powers be grounded in the law, demonstrably necessary, proportional,

and not excessive (World Health Organization 2007).

More problematic is the tension between the civil rights enshrined in such documents and the utility principle (greatest good to the greatest number) which is central to public health interventions and realization of socioeconomic rights, such as that to health, the Millennium Development Goals, and the post-Millennium goals. This tension is highly contested and its balance point is not settled (London et al. 2014), not least because the utility principle is so fuzzy. For instance, in managing an influenza pandemic, it leaves open whether the good being targeted is a reduction in the attack rate (related to who is most liable to become infected and transmit the virus), reduction in morbidity (targeting those groups most likely to be badly hit by the virus), minimizing deaths (such as among the young or indigenous peoples), reducing impacts on social functioning (concentrating on those with a defined status such as that of essential workers or public sector leaders), or simply maximizing years of healthy life (favoring younger people or those with healthy lifestyles).

Ethical Obligations of and to Health-Care and Essential Service Workers

There has been debate over the obligations of health-care workers to work during a pandemic. Key issues here are whether working during a pandemic will expose the health-care workers to increased risk and the obligations owed by the society to health-care workers (such as priority vaccination or provision of protective equipment).

The obligations of health-care workers to work during a pandemic may be seen as arising from moral obligations, from professional obligations, from contractual obligations such as employment contracts, and/or from other legal obligations (World Health Organization 2007). Malm et al. summarize the grounds that have been suggested for the duty of health-care workers to provide treatment in a pandemic: expressed consent such as when an infectious disease specialist accepts employment to treat individuals with infectious diseases, implied consent arising from being a health-care worker which may involve treating individuals with infectious diseases, a duty arising from the special training of health-care workers, reciprocity if the health-care worker has benefited from publicly funded education or training, and professional oaths and codes (Malm et al. 2008). However, it has been argued that none of these grounds provides a strong basis for a duty to treat (Malm et al. 2008). It has been suggested that a duty of reciprocity is owed to health-care workers by governments and employers to ensure that the risks faced by health-care workers during a pandemic are minimized through measures such as access to personal protective equipment and possible priority access to medicine and health care to treat influenza (World Health Organization 2007).

Priority vaccination of health-care or emergency essential service workers poses both an issue about the ethical basis for favoring one group over another (weighing the greater good of a functioning health system against the elevated risk of death or morbidity of groups more vulnerable to infection who miss out or experience delay) and issues of whether vaccination should be compulsory and whether it should extend to close contacts of such workers (weighing possible increases to herd immunity against the infringements of bodily integrity and competing uses for scarce vaccines).

Ethical Responsibilities to the Vulnerable

In pandemics the patient occupies both the space as "victim" entitled to care and compassion and as a "vector" (or carrier) (Battin et al. 2009). Because the science of disease spread (and achievement of control) is reducible to sophisticated mathematical risk modeling as the basis for prioritizing certain groups for vaccination or other interventions and because preservation of social functioning will isolate other groups such as essential service or health-care workers as a priority, such utilitarian values will loom large when making

any triage decisions about allocation of such potentially lifesaving resources.

However, the greater scientific good of most rapidly bringing a pandemic outbreak under control, or the public interest in maintaining functioning society, conflicts with ethical principles of equity of treatment ("treating like cases alike") or priority based on need (treating the most severely ill) or vulnerability (e.g., indigenous or other populations at greater risk of infection or mortality). While such dilemmas are perhaps avoidable for say the homeless in resource-rich countries, the dilemma is a serious one in resource-poor countries.

The socioeconomic determinants of health, such as income, class, gender, religion, culture, and geographic location, have been shown to contribute significantly to outcomes in managing pandemics (Kumar and Quinn 2012). Such factors are however poorly addressed in most pandemic management plans, with an analysis of national pandemic plans revealing that scientific, political, and legal discourses play a stronger role in the plans than social, cultural, and ethical discourses (Garoon and Duggan 2008). Neglect of such issues risks contravening ethical principles of prioritizing care of the most vulnerable or of distributional equity.

Distributive Justice and Health Disparities

Economic and infrastructure difficulties facing developing countries pose issues of distributive justice in nations accessing antivirals and vaccines or in distributing them equitably to all members of the population or all geographic regions (Duque et al. 2014). Such issues of distribution of scarce resources, prioritization of certain groups (such as health-care or emergency workers), and transparency of communication to the public (how much information is shared about levels of risk) are crucial ethical conundrums (Bhatia 2013).

Limited health infrastructure, inability to afford to stockpile medications as a precaution, or logistical and geographic barriers to mobilizing a public health response all serve to heighten the triage dilemmas associated with deciding which groups are to receive priority of access to vaccination or other health services in the event of a pandemic.

Poverty and disadvantage magnify the already well-documented socioeconomic disparities in susceptibility to infection, and poor housing, sanitation, or the economic and social demands of maintaining basic sustenance may render it very difficult to implement public health measures. Global initiatives toward funding and distributing essential drugs and medical supplies (such as protective equipment), targeted capacity-building of health infrastructure (laboratories, surveillance capacity), and international aid and WHO cooperation during emergencies provide some limited expression of a global bioethical response, but the ethical challenges continue to be intimidatingly grave.

Global Bioethics and Pandemics

Pandemics present two interesting challenges for standard bioethical frameworks. The first is for bioethics to give greater attention to infectious diseases and other issues that affect the global community. It has been argued, for example, that infectious diseases traditionally have not been adequately addressed in the bioethics literature (Battin et al. 2009). In recent years, however, within bioethical scholarship greater attention has been paid to ethical issues related to infectious diseases.

The second challenge relates to the role of ethics in pandemic planning, whether ethics is "global" and the degree to which ethics is embedded in factors such as culture and religion. These debates are likely to be relevant to any discussion of the application of ethical principles in a pandemic. In an increasingly globalized world, pandemic responses need to respect values such as the communal equality-oriented value systems of the African *ubuntu* by not placing undue weight on individual autonomy and be mindful for members of Jainist, Buddhist, and Hindu communities of Asian *ahimsa* principles of compassion toward and doing no harm to any living being when

addressing measures such as mass culling of poultry to halt spread of avian flu or in animal testing of new vaccines. Similar issues arise for host countries in respect to immigrant populations or cultural minorities.

Conclusion

Bioethical principles pertinent to management of pandemic threats are both contested and in flux. Early bioethical frameworks emphasizing autonomy and realization of individual civil rights are challenged as failing adequately to address communal values of solidarity, the relational nature of social interactions, or global socioeconomic inequality and poverty.

Issues such as realization of the socioeconomic right to health, pursuit of distributional justice, and operationalization of a global bioethics able to engage with differences in regional capacity, governance systems, and competing pressures such as international trade and commerce remain lively domains for scholarly discourse and international debate.

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Cross-References

- ▶ AIDS
- ▶ Autonomy
- ▶ Bioethics: Founders
- ▶ Care Ethics
- ▶ Communitarian Bioethics
- ▶ Epidemics
- ▶ Ethics
- ▶ Global Ethics
- ▶ Health: Global
- ▶ Human Rights
- ▶ International Law
- ▶ Justice: Global
- ▶ Law and Bioethics
- ▶ Public Health
- ▶ Right to Health
- ▶ Risk
- ▶ Solidarity
- ▶ Triage
- ▶ Utilitarianism
- ▶ Vaccination
- ▶ Vulnerability
- ▶ WHO

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Pastoral and Spiritual Care

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Abstract

This entry presents an overview of pastoral and spiritual care with regard to bioethical principles at the personal, communal, and international levels which, it can be argued, are progressively intersecting due to increasing globalization. The contribution of pastoral and spiritual care practitioners toward resolving bioethical dilemmas will be considered, as will the recognition and inclusion of religious, pastoral, and spiritual care, as an integral part of holistic person-centered care that not only benefits the individual, but also benefits communities both locally and globally.

Keywords

Bioethics; Religion; Pastoral care; Spiritual care; Chaplain

Introduction

Depending on the context, pastoral and spiritual care can have a substantial and yet sometimes ambiguous prominence with regard to bioethical issues. This is due to the fact that pastoral and/or spiritual carers tend to be influential given their practical support to patients/clients rather than by asserting any formal authority. What is certain however is that all three domains (pastoral care, spiritual care, and bioethical issues) have interacted for centuries, either directly or indirectly. It is important to note at the outset, that while pastoral care and spiritual care have predominantly been grounded in religious traditions (of one kind or another), neither contemporary pastoral care nor present-day spiritual care accepts the traditional prism of viewing bioethical issues only in terms of defined religious or faith parameters but rather utilizes a holistic paradigm that emphasizes person-centered care as its core frame of reference.

The person-centered approach (also sometimes known as "client centered" or somewhat hierarchically as "patient centered") was first coined by an eminent humanist psychologist Carl Rogers (1902–1987). Rogers was concerned with the power relationship within expert-client therapy and subsequently developed "nondirective therapy," then "client-centered therapy," and finally the "person-centered" approach. Essentially, Rogers shifted the focus of therapy away from professional expertise and advice, to focus upon facilitating the needs, desires, and goals of a person to achieve their own self-actualization.

It is important to note that a person-centered approach is not an ethical position per se but rather a process which ensures the focus is on the

autonomous goals of the person and not purely based upon medical, religious, or other professional determinants. During the twentieth century, pastoral care, and subsequently spiritual care, largely adopted Roger's humanistic person-centered approach as a key methodology for pastoral care and counseling.

Unfortunately, understanding bioethics from a pastoral and spiritual care viewpoint has often been contrasted *against* "value-neutral" ethics – sometimes called secular or humanistic ethics. While some secular ethicists accept that moral judgments (secular or otherwise) can never simply be statements of fact, nevertheless, others subscribe to a perspective which assumes that in applying "scientific principles," it is therefore possible to operate in a "value-neutral" environment.

Such a stance however ignores the reality that it is always humans who are applying ethical principles and they are never value neutral. Contemporary pastoral and spiritual care however, while recognizing the importance of religious and spiritual beliefs, argues for a broader and more inclusive approach to bioethical issues, not limited by any one specific dogma or philosophy – whether it be religious, secular humanist, or otherwise.

Contemporary pastoral and spiritual care is fundamentally multidisciplinary, with the primary aim of ensuring (through various pastoral and spiritual care practitioners) that religious, pastoral, and spiritual tenets are *equally valued* alongside other systems of belief. Pastoral and spiritual care strives to ensure that person-centered care is not just a perfunctory gesture of health-care policy but an effective and inclusive practice that should be assured, regardless of the type of bioethical issues – whether these are predominantly personal, communal, or global.

Hence one of the roles of pastoral/spiritual carers has been to provide support as facilitators during bioethical decisions to ensure that all issues/viewpoints are considered but most particularly that of the key person affected (Carey and Cohen 2008). Thus, it can be argued that while pastoral/spiritual carers strive to ensure that religious and spiritual care is an active ingredient of holistic person-centered care, they nevertheless have a meta-role, beyond the specifics of any bioethical issues, to ensure that bioethics itself, and bioethical decision making, is truly holistic and person centered in its application.

Conceptual Clarification

Within this entry, the term "pastoral care" and "pastoral carers" are recognized as long-established generic terms for religiously specialized work and the personnel designated to undertake that work – either as a result of being officially ordained and commissioned by a recognized religious organization (e.g., as priests, rabbis, ministers, monks, imams, chaplains, or other such bestowed clergy) or as a result of being approved as "volunteers" to assist in pastoral care work (e.g., as pastoral assistants, denominational assistants).

Most "chaplains" and some "chaplaincy assistants" not only have religious training and authorization to undertake religious and pastoral care duties but also are required to have additional specialist training beyond theological schooling so as to increase their professional utility (e.g., having secular tertiary qualifications, clinical pastoral education, and/or other specialist chaplaincy training relating to a specific context). Usually chaplains also have professional association membership/accreditation and the endorsement of a secondary organization (e.g., hospitals/health department, military/department of defense, police/correctional services, universities/schools).

While it can be argued that the term "spiritual" also has a long-established history, nevertheless, the terms "spiritual care" and "spiritual carer" are more recent terms that have predominantly arisen as a result of the shift from the "biopsychosocial" model (which has been largely antireligious) toward a broader and more inclusive "biopsychosocial-spiritual" model (Sulmasy 2012). Given that the definition of spiritual means relating to or affecting the human spirit or soul, as opposed to material or physical things, both pastoral care and spiritual care have a great deal in common, except that pastoral care is more

religiously grounded and focused, whereas "spiritual care" is much broader (as will be defined later) recognizing both traditional religious beliefs and more contemporary spiritual practices. Nevertheless, for the purposes of this entry regarding bioethics, both the terms "pastoral" and "spiritual" are virtually inseparable and will be equated together as "pastoral/spiritual" and "pastoral/spiritual carer."

This entry will provide an overview of pastoral care, spirituality, and the bioethical principles, particularly as these relate to the role and contribution of pastoral/spiritual carers. Consideration will then be given to the different bioethical dimensions in which pastoral/spiritual carers pragmatically engage in bioethical issues – namely, at the personal, communal, and global levels. Finally, the recognition and inclusion of religious, pastoral, and spiritual care will be noted that ensures a truly holistic approach to person-centered care.

Historical Development

Pastoral Care

The word "pastoral" derives from the Latin word for "shepherd" or "shepherding" – meaning protecting, caring for, or curing something (or someone) in need – hence the extension "pastoral care." The act of "shepherding" is strongly emphasized within both Judaism and Christianity, as recorded in historic biblical passages such as Psalm 23 ("The Lord is my shepherd") or within New Testament texts such as the Gospel of John (Chapter 10: "I am the good shepherd"). However, given that sheep farming and the practical caring for sheep have been universal in many cultures across the world, the concept of shepherding and "being a good shepherd" was also utilized or adopted by many faiths such as Islam, Buddhism, and Hinduism.

For centuries, the traditional approach to pastoral care, which involved religious and spiritual leaders caring for their flock of constituents (using their particular established theological dictates), was fundamentally altered post WWII. Within the Western world, from the 1950s, "pastoral care," based on the progressive work of Richard Carbot (1868–1939), Anton Boisen (1876–1965), and Howard Clinebell (1922–2005), broadened from a purely religious/theological concept to incorporate psychological theories, including the development of pastoral counseling as a specialist discipline within pastoral care. Subsequently, pastoral care, rather than being religiously mission orientated, became more person centered, irrespective of a person's/client's religious or nonreligious preference.

By the latter half of the twentieth century, pastoral care, both in definition and practice, extended to become internationally defined as any ". . .helping acts directed towards the healing, sustaining, guiding and reconciling of troubled persons whose troubles arise in the context of ultimate meanings and concerns" (Clebsch and Jaekle 1967, p. 4). This was an important development with respect to bioethical issues, for while religious dictates may or may not have condoned particular bioethical decisions (e.g., abortion, euthanasia), nevertheless the focus of modern post-WWII pastoral care was not to judge or condemn individuals but to provide support to assist with healing, sustaining, reconciling, and, hopefully, guiding individuals toward better health and well-being decisions.

Paterson (2015) noted that such early definitions of pastoral care tended to blame troubled individuals for their own condition, whereas, given the increasing recognition of social determinants that effected the health and well-being of people, pastoral care from the late 1980s started to shift purely from a clinical perspective to reiterate the importance of contextual and communal influences. Subsequently, by the commencement of the twenty-first century, pastoral care had already further extended to consider social and cultural determinants plus the nurturing of ecologically holistic communities within an international/global context.

A review by Hart (2002) indicated that the progression of pastoral care as a discipline, and the practical contribution of pastoral/spiritual carers to bioethics and bioethical issues, has been considerable over the years – particularly within the health-care context. Hart was perhaps

the first to identify the correlation between bioethical principles and pastoral/spiritual care noting numerous ways by which pastoral care, through its appointed practitioners, had contributed to modern-day bioethics and the resolving of bioethical issues at both personal and communal levels. He noted eight key contributions of pastoral/spiritual care practitioners to bioethical issues:

- Developing rapport with patients and staff so as to assist with reaching into the "patient's world" and assessing potential sources of anxiety regarding bioethical decisions and treatment.
- Establishing a common ground among competing interests (e.g., patients, family, community, professional staff, hospital administration) in order to facilitate mediation and resolve bioethical questions and conflicts.
- Ensuring "ultimate concern" for patients in a health-care setting that should not revolve around organizational interests and employee satisfaction, regulatory services, and budgetary restructuring but primarily focuses upon human care of the sick according to highest clinical practice standards and grounded in science and research (ultimate concern is a concept originally developed by Paul Tillich (1886–1965)).
- Assisting health-care organizations with ethical reflection with respect to balancing competing goods and interests and distinguishing means and ends plus a capacity to separate expediency and deliberate or semi-deliberate rhetorical obfuscation from the overall mission of the health-care institution or organization.
- Presenting/speaking to religious and community groups (e.g., congregations, schools, universities, community meetings) about pastoral and spiritual care applicable to health-care and bioethical issues.
- Writing articles for publication (e.g., newsletters, journals) regarding bioethical issues and advocating on behalf of patients/families the bioethical principles of autonomy, beneficence, non-maleficence, justice, and supplementary issues of privacy and confidentiality.
- Serving on commissions, panels, and committees (e.g., institutional ethics committees, human research ethics committees, hospital boards) making certain that the concerns of patients, particularly related to faith and the inner life, become integral to the decision-making process and organizational policy.
- Utilizing tacit knowledge/expertise/research to present at educational seminars (e.g., medical conferences, continuing education seminars) to benefit health-care staff and the wider community about pastoral, spiritual care, and bioethical issues.

A unique twenty-first century contribution to pastoral/spiritual care has been the development of the World Health Organization "Pastoral Intervention Codings" (colloquially known as the "WHO-PICs"), which recognized and designated pastoral interventions within WHO's International Classification of Diseases and Health Related Problems coding system (ICD-10). Some health/welfare organizations and national health services around the world have sought to develop their own idiosyncratic pastoral/spiritual care coding system; nevertheless, a review of the WHO-PICs indicated that the major WHO-PIC coding intervention categories were found to be specific enough to identify the core pastoral/spiritual care intervention yet flexible enough to incorporate a range of sub-elements relevant to varying contexts/situations within both health (e.g., acute, mental, aged care) and welfare settings (e.g., prisons, military) (Carey and Cohen 2014).

While it has been proposed that the WHO-PICs should be revised to be more inclusive and titled the "Religious, Pastoral and Spiritual Intervention Codings" (WHO-REPSICs), nevertheless, irrespective of such possible amendments, the codings have already proven invaluable for recording the involvement of pastoral/spiritual carers in bioethical issues when undertaking (i) assessments, (ii) support, (iii) counseling, and (iv) education plus (v) ritual and worship activities plus related (vi) religious, pastoral, and spiritual administration in response to patient/client,

family, and other personnel needs (Carey and Cohen 2014).

Spiritual Care

Over centuries the concept of spirituality has changed considerably – particularly within the Western world. Muldoon and King (1995) summarized the progression of spirituality, noting that one of the earliest and most detailed notations regarding "things of the spirit" since the commencement of the Common Era was that of Saint Paul's work distinguishing the "spiritual person" from that of the "natural" or "carnal" human (1 Corinthians 2: 15) – suggesting that an individual's perspective and behavior (and thus ethical decisions) were affected by their spiritual status.

By the Middle Ages, however, spirituality tended to emphasize a distinction between those who wrongly put their trust in materiality and those whose persuasion was primarily religious with ultimate concerns. The thirteenth to sixteenth centuries saw this distinction furthered given the progressive separation of religion and state power, whereby religious philosophy, property, and power were distinguished from secular authority and ownership – and thus spirituality primarily became the administrative concern of specialists such as clergy.

From the seventeenth century, however, and one could argue to the present day, there has been a reclaiming of "spirituality" with respect to its personal significance and merit. Muldoon and King (1995, p. 331) noted that while "spirituality" could be used pejoratively to indicate questionable religious enthusiasm or unorthodox forms of religious practice (or even, one could add, the personal criticism of "being too heavenly minded and subsequently of no earthly use"), nevertheless, spirituality predominantly became a term referring to one's interior quality and one's devotion to an affective relationship with a divine being.

By the nineteenth and twentieth centuries, this led to a distinction between those having a "common life of faith" and those striving for perfection through mentorship, spiritual exercises, and the practice of virtues beyond what was ordinarily expected. Subsequently, over the years, given the changing emphasis of "spirituality," there has been considerable deliberation concerning the meaning of spirituality and spiritual care. Fundamentally, the pondering has been due to the term "spirituality" being difficult to define given a multiplicity of confounding descriptions that have ultimately hindered holistic practice, patient-centered care, and appropriate spiritual care interventions.

The first "consensus" definition of spirituality to emerge, which has gained increasing acceptance and largely resolved the debate (at least among some conciliatory academics and pastoral/spiritual care practitioners), predominantly arose out of the palliative care context, defining spirituality as:

> . . .the aspect of humanity that refers to the way individuals seek and express meaning and purpose and the way they experience their connectedness to the moment, to self, to others, to nature, and to the significant or sacred. (Puchalski et al. 2009)

This definition of spirituality automatically advocates for a person-centered care approach and, if accepted, necessitates certain ethical principles to ensure that person-centered care is respected as a legitimate part of a spiritual care approach. Sulmasy's (2012) work is helpful in this respect, noting that there are a number of ethical principles of spiritual care that need to be maintained in order to ensure that pastoral/spiritual care is patient centered – namely, holism, discretion (referral), accompaniment, and tolerance toward those of a different faith belief compared to one's self (see Table 1).

Bioethics

The term "bioethics," from Greek words "bios," meaning "life," and "ethikos," meaning "habit" or "custom," can technically and rather simply be interpreted to mean the application of habits or

Pastoral and Spiritual Care, Table 1 Summary of the five ethical principles of spiritual care[a]

Principle	*Overview summary*
Patient/person centeredness	Spiritual care focuses beneficently upon the patient as the nexus of all relationships (e.g., family, social, and religious/spiritual relationships) and their autonomous right to make decisions (e.g., medical treatment or refusal, spiritual care, or none). It requires listening to the patient history and how it intersects with their illness narrative and necessitates confidentiality
Holism	Spiritual care requires respect for the totality of the needs of patients as whole persons utilizing a biopsychosocial-spiritual model of care which requires a professional team approach involving reciprocal appreciation for the contribution of other disciplines, communication skills, respect, and humility
Discretion	Discretion is required with respect to the level of spiritual care provided to patients and the ability to discern and willingness to make referrals to other professionals when appropriate (e.g., physicians, psychiatrists, nurses, and other allied health professionals). Such discretion also implies reciprocal teamwork by other professionals deferring to professional/certified spiritual carers (e.g., chaplains, clergy)
Accompaniment	Spiritual care recognizes that the journey of illness for a patient can be one of walking alone/feeling isolated and thus spiritual carers through accompaniment and active listening develop a pastoral/spiritual care relationship which helps to prevent feelings of abandonment and alienation. Those providing spiritual care should maintain the relationship for as long as it is desired and helpful to the patient
Tolerance[b]	Spiritual care, in an increasingly globalized pluralistic society, necessitates interfaith tolerance and respect which requires a degree of "epistemic humility" as religious/spiritual beliefs are often based upon faith rather than convincing evidence. Proselytizing violates the principle of interfaith tolerance and, given patient vulnerability, may be acceded due to fear rather than autonomously making an informed choice. Both medicine and spiritual carers are to be respectful and attentive to a patient's particular spirituality (or possibly no beliefs), enabling such professions to be truly holistic and patient centered

[a]Summary table developed from Sulmasy (2012)
[b]Not all religious/spiritual organizations believe in tolerance toward other religious faiths

past customs upon the whole of life. Such a definition however is too broad as it could include almost anything in terms of ethics (e.g., business ethics, political ethics). More current with today's usage, bioethics is generally taken to refer to the "...application of ethics to the biological sciences, medicine, health care and related areas as well as the public policies directed towards these" (Childress 1986).

While there has been some debate about the cogency and latitude of Beauchamp and Childress's (2013) four *Principles of Biomedical Ethics* (*fp* 1977), nevertheless, these have become foundational and warrant discussion with regard to pastoral/spiritual care:

- Autonomy – The principle of respecting an individual deciding his or her own choice
- Beneficence – The principle of acting with the best interest of the other in mind
- Non-maleficence – The principle that "above all, do no harm"
- Justice –The principle of fairness and equality among individuals

The Principle of Autonomy

Autonomy, etymologically, is a compound of the Greek words "autos" (self) and "nomos" (law or rule). In simple definition terms, it means "self-rule" or "self-government." At a macrolevel, "autonomy" was originally used to refer to the independence of city-states from outside control. At a microlevel, it generally refers to a person's ability to make or to exercise a self-determining choice, literally to be self-governing.

It is not possible to discuss all the issues relating to autonomy within this entry (see "▶ Autonomy"). Wheeler (1996, p. 40) however summarized a pastoral care cautionary for all pastoral/spiritual care practitioners with respect to

autonomy. In order for autonomous decisions to be properly appropriated, particularly with regard to bioethical decisions, pastoral/spiritual carers can help to ensure that four distinct requirements are assured, namely, (i) full disclosure to patients, (ii) comprehension by patients, (iii) voluntaryism involving no coercion or cajoling, and (iv) patients being properly assessed as mentally competent and emotionally stable.

Although ensuring a person's autonomy may seem quite straightforward, in actual fact it is not. Religious ethics sometimes mandates a form of constraint with regard to autonomous decision making that can challenge personal morality or acceptable communal consent. For example, within Islam, one's autonomy can be limited or constrained because Muslims are mandated to "submit" to certain "prohibitions" that may severely limit their autonomous options, and their failure to comply may result in some form of familial or social reprimand, possibly with legal or life-threatening consequences depending upon the cultural milieu. Though not quite as arduous, other religions (e.g., Judaism, Christianity, Buddhism, Hinduism) support in one way or another that the concept of autonomy could be a "sinful" assertion if one fails to acknowledge one's dependence upon a divine/superior authority or ultimate life source. Thus the struggle for people between making autonomous decisions and the constraints of their religious belief can amount to interpersonal and intrapersonal conflicts that can form a regular part of a pastoral/spiritual carer's counseling role given bioethical dilemmas.

Non-maleficence

Maleficent means to be "hurtful" toward others, even "criminal" or with intent to be "evil" in the treatment of others. The opposite is *non*-maleficence. Childress argued that the principle of non-maleficence "establishes the duty not to harm others or injure others or impose the risks of harm on them, at least without compelling justifications for doing so" (Childress 1986, p. 425).

The principle of non-maleficence is based upon the maximum or oath purportedly written by Hippocrates of Kos (460–377 BC), *primum non nocere*, that is, "first, do no harm." The Hippocratic Oath became a "professional ethic", specifically developed to distinguish trained physicians apart from other self-proclaimed practitioners who, due to a lack of training or for political or financial gain, acted immorally by causing harm and injustice to their patients (Fletcher 1986, p. 269). During the twentieth century, some ethicists believed that the principle (in its original form) to be "so general that it is useless" on the grounds that it is impossible to avoid all harm or "evil." Indeed if all harm or the possibility of harm was avoided, no health risks for improvement would ever be undertaken and therefore a greater harm may result – namely, death. Hence the argument that the principle of non-maleficence is only appropriate as a professional ethic when there is no compelling reason to ignore it.

However, the fundamental principle and first loyalty noted in the Hippocratic Oath was *not,* in the first instance, about professional integrity per se but the importance of patient-centered care. To state it simply, the Hippocratic Oath was primarily about patient well-being and "patient protection" which was not to be risked by the political and financial agendas of others. While there are obvious limitations in continuing to use the Hippocratic Oath given modern medicine, there is nevertheless continued merit in the oath, not just because it still has relevance for bioethical issues that are still currently debated (e.g., abortion, euthanasia) but because of its essential emphasis upon patient centeredness – that is to say, it is not the words as such but the values implied in the oath that are important.

Indeed over time, priority in favor of the patient has been reechoed in other medical codes such as the "Geneva Convention Code of Medical Ethics (1949)" stating, "The health of my patient will be my first consideration," and the World Medical Association Declaration of Helsinki (Finland 1964) and amendments (Japan 1975; Venice 1983; Hong Kong 1989) stating, "It is the mission of the physician to safeguard the health of the people. His or her knowledge and conscience are dedicated to the fulfillment of this mission." Indeed numerous codes such as the Nuremburg Code (1947), the Nightingale Pledge (1947), the Declaration of Sydney (1968), the Declaration of

Tokyo (1975), and the Declaration of Hawaii (1977) are indicative that non-maleficence and principles related closely to it (e.g., beneficence) have been regular and recurrent concerns among both health professions and the wider community. The focus of pastoral/spiritual carers is to likewise support the principle of non-maleficence by ensuring patient-centered care through the provision of religious, pastoral, and spiritual support and if necessary advocacy, particularly for those vulnerable patients unable to defend for themselves.

Beneficence

Beneficence means "active well doing" or "doing good so that another is able to experience greater good or happiness" (see "► Benefit and Harm"). More specifically, with regard to bioethics, beneficence can mean simply the "duty to care" or "the duty to do the best for an individual patient." While the principle of non-maleficence (mentioned earlier) can be seen to be relatively passive in its aim, requiring "a person to do no harm to another," beneficence demands the intentional and direct application of care for the advancement of someone's health and well-being.

Some argue however that "beneficence," as a result of medical dominance, has become an excuse for "paternalism," "infantilizing," or "creating dependency" of patients upon medical physicians due to their overriding of a patient's autonomy, either by deception or by coercion (supposedly) for the sake of "benefiting" the patient. Examples of this may include failing to obtain the informed consent from a patient, misleading a patient about the risks of surgery, exercising undue influence over the patient's decision, failing to inform or withholding information, lying to a patient about their condition, failing to include the patients (and their families) in decisions about treatment, or altering a patient's treatment decisions without consent (Wheeler 1996).

The corruption of "beneficence" by some unscrupulous medical practitioners has led to the principle of "beneficence" sometimes receiving a degree of "bad press." Thompson et al. argued however that beneficence involves important aspects such as "patient advocacy and defending the rights of vulnerable patients" – aspects which should not be left amiss:

> Beneficence is indispensable wherever there are dependent or helpless people in need of support or urgent care and attention. The reciprocity in our duty to care of one another should make us realize that we all need others to speak for us, to do things for us, or to defend our rights when we are too weak to do so for ourselves. (Thompson, et al. 1994, p. 61)

An important aspect with regard to the beneficent role of pastoral/spiritual carers can be understood with regard to knowledge. Thompson et al. (1994, p. 61) noted that "knowledge is power" and that the power of the true carer is aimed at "sharing knowledge and skills with the vulnerable individual so as to empower that person to reassert control over his/her life. . .." Similar to the role of clinical ethicists and religious representatives on institutional/research ethics committees, pastoral/spiritual carers can have an important nonclinical independent role in helping to ensure that patients and clinicians are reminded of patients' autonomous rights and that patients (and their families) have the opportunity to be fully informed about their options regarding treatment and the minimum requirements for autonomous decisions (as noted earlier, refer to "► Autonomy").

The Principle of Justice

While there is some debate regarding the principle of justice (see "► Justice: Theories of"), generally, most bioethicists would support the definition that the "principle of justice" (or as it is sometimes referred to the "demand for universal fairness") means "the fair sharing" or "fair distribution" of a society's benefits and burdens and of rewards and punitive controls among its members. With respect to pastoral and spiritual care, justice can be divided into three levels of allocation and subsequent action: "micro," "meso," and "macro" justice. Each can be an important area of involvement for pastoral and spiritual care practitioners as each correlates with personal, communal, and global bioethics (noted later).

Micro-allocation concerns the local issue of who should receive scarce resources such as

kidney organs or supplies of a rare blood type or which individual will be given a bed in the intensive care unit, particularly given the limited availability and increased demand. This level also includes decisions concerning the allocation of time via the use of appointments, waiting lists, triage diagnostic, and treatment decisions. It is also an area involving "procedural justice" that considers "how we decide who receives care, how long they must wait for it, and what quality of care they have access to." The local hospital, for example, may have ample resources and staffing to administer lifesaving procedures, but prescribed hospital procedures, hospital decision makers, or even government policy may deny access to some patients (Wheeler 1996, p. 60).

Whether the denial is due to discrimination on the basis of sex, race, religion, age, disease, mental illness, wealth, or the level of health insurance, the consequences of discriminate justice can have disastrous effects for those prejudiced against. It is arguable that pastoral/spiritual carers within health-care institutions have some responsibility in this arena, given their professional mandate to care for all – irrespective of race, creed, or status – and to encourage a more just and hospitable environment, particularly through providing support and advocacy for patients.

Meso-allocation is the allocation of medical resources at a regional level including decision makers such as administrators, hospital managers, and institutional committees, who are responsible for allocating resources to various hospital departments (including pastoral/spiritual care departments), regional competing services, health-care specialists, and allied health services and programs. At this level, pastoral and spiritual carers compete with other departments and programs for funding. Mostly pastoral and spiritual carers have relied on the "goodwill" of health-care administrators for funding. More recently, however, economic rationalism within health-care contexts has meant that pastoral/spiritual carers have had to provide more empirical evidence/research utilizing religious, pastoral, and spiritual care codings to record their services undertaken with patients, their family, and staff (Carey and Cohen 2014).

Macro-allocation primarily occurs at the government level with regard to the proportion of national and state income which will or will not be spent, for example, on health, education, welfare, transport, and defense. Macro-allocation, for example, involves concerns about whether increased allocation of resources should be shifted from more privileged sectors to deprived areas so as to bring greater equality within society and/or whether tax dollars should be restricted with respect to the aged and instead allocated directly to organ transplantation or to clinical research.

As will be noted later, macro-allocation also affects the distribution of resources at a global level that might assist poorer nations struggling with disease. Due to limited resources, any macro-decisions subsequently affect meso- and micro-allocations, particularly in terms of the availability and accessibility of resources within local communities.

Given their training in health care, all health professionals (including pastoral/spiritual carers) concerned for the principles of justice should "campaign for resources" or "defend standards of care" so as to improve services within a nation, state, or community, particularly for those people underprivileged, aged, or disabled. Yet while such issues of justice are of social and pastoral concern to many, particularly to health-care pastoral/spiritual carers, "macro-"decisions seldom, if ever, involve the participation of pastoral/spiritual carers who are often excluded in such policy decisions.

Of course macro-allocation will substantially affect micro- and meso-allocations and thus the availability of pastoral/spiritual care staffing and local resources. This in turn will significantly affect and challenge their role in terms of the level of pastoral/spiritual care they are able to provide and fundamentally can undermine patient-centered care. Thus while the predominant participation of pastoral/spiritual care currently occurs at the microlevel of decision making, nevertheless, meso and macro distributive justice can and will increasingly be as important – particularly given the growing recognition of faith-based organizations/communities

in providing care within regions having limited resources and/or are poverty stricken.

Bioethical Dimension

It is important to note that most of the literature and research relating to pastoral and spiritual care with regard to bioethical issues has been within the health-care context and largely driven by the demands of health care chaplains working within acute care, mental health care, and aged and palliative care services. There is also however literature noting religious pastoral/spiritual carers dealing with bioethical issues that have had, or currently have, global significance (e.g., HIV/AIDS).

From a pastoral/spiritual perspective, all bioethical issues are globally interconnected, in one way or another – whether this be due to social, legal, political, religious, economic, or (more than likely) a permutation of many dynamics. Traditionally, however, and irrespective of numerous influences, it is possible to generalize that the application of pastoral/spiritual care with respect to bioethical issues has tended, for pragmatic reasons, to be interventions at either (a) personal, (b) communal, or (c) international levels – but many of which are increasingly interconnected due to developing globalization.

Personal

A number of bioethical issues and related decisions, while frequently occurring around the globe, nevertheless reflect a personal or familial situation. Thus far the only quantitative and qualitative international studies exploring the role of pastoral/spiritual carers involved in patient bioethical decisions involved Australian (AU n = 324) and New Zealand health-care chaplaincy personnel (NZ n = 100). The results indicated that, irrespective of the population, of all the bioethical issues encountered by pastoral/spiritual care personnel, it was the personal end-of-life decisions such as pain control, withdrawal of life support, and organ donation in which the majority assisted (Carey 2012a; Carey and Cohen 2008).

The qualitative results gathered in the same studies indicated that while a minority of chaplains preferred not to engage in bioethical decisions at all, nevertheless, the majority of chaplains believed that it was part of their role to help patients and their families make health care treatment decisions and that such involvement typically occurred through various pastoral interventions, namely, pastoral/spiritual assessments, ministry/support, counseling, education, and/or pastoral ritual and worship activities, that aided patients, their families, and sometimes staff.

Communal

There are a number of bioethical issues also encountered by pastoral/spiritual carers that could likewise be considered personal yet tend to have more communal impact and involvement. Perhaps the most common bioethical issues that can involve pastoral/spiritual carers at the communal level are matters such as abortion, organ transplantation, palliative care and in-vitro fertilization (IVF).

Abortion, while argued to be a complex issue on a personal level and thus argued by pro-abortionists to be a matter of autonomous right and personal decision, nevertheless, abortion remains a socially controversial issue within some religious and nonreligious communities alike, who predominantly argue that abortion is a maleficent act, demanding "justice" and the "right to life" for unborn infants and emphasizing the sanctity of life with adoption as a preferred resolution, if necessary, for the benefit of the child, those who are unable to have children, and, eventually, the community. Pastoral/spiritual carers, while perhaps embracing strongly held religious and personal views about abortion, nevertheless, in their professional capacity, are required to practice person centeredness and accompaniment (Table 1) by providing a range of religious, pastoral, and spiritual interventions (i.e., assessment, support, counseling, education, ritual, and worship) as required to assist the patient/client *irrespective* of their or the patient's/client's views and decision.

Likewise IVF is also argued by some to be an autonomous right and personal lifestyle choice that can benefit heterosexual, homosexual, and

transgender couples unable to have their own children through their own natural procreation. However, it has also been a socially controversial bioethical issue given the artificial-technological process involved plus the substantial and repetitive costs that are often subsidized by the community and/or governments in one way or another, causing additional strain upon finite resources and technology that could otherwise have been directed to lifesaving infrastructure (e.g., kidney dialysis, cancer treatment).

The fundamental point to note however, with respect to personal bioethical issues, is that no matter where one stands on the spectrum concerning bioethical issues, such as IVF or abortion, and recognizing that people (including pastoral/spiritual carers) are never "value neutral," nevertheless, ethically and professionally, the pastoral/spiritual carer's role is to be "person centered." This requires a holistic approach to be discrete, provide accompaniment, and advocate for acceptance and tolerance of other's beliefs rather than the forceful imposition of beliefs and values on either the patient/client or family.

Of all the communal bioethical issues, organ transplantation (OTP) is perhaps one of the most demanding for pastoral/spiritual carers. While communal bioethical issues such as abortion and IVF may be socially debatable and controversial, there is general communal agreement in most countries regarding the beneficence of OTP – irrespective of community cost. The social challenge and controversy however has essentially been due to the high demand yet limited supply of organs – a phenomenon that is common worldwide and increasingly shifting from a communal issue to a global issue given the emergence of medical tourism in general and the development of organ tourism particularly in terms of further exploitation of poorer countries.

While research indicates that organ transplantation is *not* the most frequently encountered bioethical issue for pastoral/spiritual carers, nevertheless, it involves and requires the most comprehensive forms of pastoral/spiritual care, sometimes demanding preoperational assessment, support, counseling, and education, as well as during surgery (for family and staff) and postoperative support (for patients, family and staff). One unique and identified role of chaplains as part of their assessment, for example, has been to discuss with patients/families about the possibility of potential organ donation given the likelihood/inevitability of a patient's death. The pastoral/spiritual carers' independent nonmedical perspective and the communal/global shortage of organs mean that their role can be sensitively influential in assisting timely patient/family decision making. Given the emotional tenderness of patients and/or families near or at a time of death, this is an awkward task that despite the most diplomatic protocols is sometimes reciprocated with aggressive negativity. Nevertheless, pastoral/spiritual carers are also aware that patients/families can feel quite disappointed if the opportunity of making a lifesaving donation was not sufficiently forthcoming as a possible option.

International

Internationally, there are a number of ongoing bioethical issues that have sorely affected entire nations (e.g., as a result of cancer, SARS, and more recently the Ebola virus) which have necessitated the involvement of pastoral/spiritual carers in one way or another. In particular, the last quarter of the twentieth century witnessed the outbreak and rapid rise of the human immunodeficiency virus (HIV) and acquired immune deficiency syndrome (AIDS), causing substantial morbidity and the death of thousands of people around the globe. The twenty-first century also continues to experience wars and social upheaval, particularly within regions dominated by Islamic theocracy and extremism, resulting in acts of genocide, homelessness, disability, poverty, starvation and consequently generations that will suffer mental and moral injury.

The work of Miller and Rubin (2011) highlighted the important health communication role that local faith-based-communities (FBCs) and faith-based organizations (FBOs) can have in their respective regions when struggling with substantial health and bioethical issues. These nongovernmental organizations (see "► Nongovernmental Organizations (NGOs)") have also been noted to provide a variety of

basic needs and services that have assisted communities in their struggle to survive – needs such as food and water to the more advanced services such as health-care facilities. In summary, Miller and Rubin's work identified seven areas that faith-based organizations/faith-based communities addressed global community bioethical issues:

- Provide and assist with physiological needs (e.g., food, water, shelter, clothing).
- Provide and assist with security and safety needs (e.g., personal, financial, health services).
- Encourage/ensure a sense of belonging (e.g., friendship, family, community).
- Encourage and assist individual and community esteem (e.g., self-respect, acceptance, and consideration for others).
- Encourage and assist with individual and community development (e.g., helping people/communities actualize desired goals such as education, schools, housing, hospitals, and community utilities).
- Provide individual and community health promotion/communication of information suitable to cultural context and religious/community activities (e.g., religious ceremonies/community celebrations) utilizing appropriate language/dialect.
- Provide clinical and supportive personnel/expertise to administer medical and allied health services plus advise, counsel, and guide individuals and communities to negotiate and solve bioethical issues and problems.

The key resolution for international organizations such as WHO (World Health Organization), UNESCO (United Nations Educational, Scientific and Cultural Organization), and the WCC (World Council of Churches), when attempting to deal with global bioethical issues, is to utilize and maximize the potential public health communication role and support of existing local FBOs/FBCs and their respected pastoral/spiritual leaders and/or chaplains. Such an alliance can help to provide accurate information and support to affected communities that, in turn, will help to lessen both ascribed and achieved stigmatization of those affected and ensure justice at all levels (personal, communal, and global) concerning the equitable distribution of resources so as to improve the health and well-being of communities around the world (noted earlier, refer to "Principle of Justice"; see "► Justice: Theories of").

Conclusion

Contemporary pastoral and spiritual care has progressively developed since the mid-twentieth century concurrent with the progression of bioethics. Pastoral/spiritual carers, as part of their everyday role, are compelled to take into consideration the key principles of bioethics (i.e., autonomy, beneficence, non-maleficence, justice) plus the ethical principles of spiritual care, a combination of which makes a very robust framework for all pastoral/spiritual carers to ensure person-centered and holistic care at multiple levels (e.g., personal, communal, and global). Given their past and current degree of involvement in bioethical issues, plus their training and local tacit knowledge, pastoral/spiritual carers should continue to be fully encouraged and endorsed to provide religious, pastoral, and spiritual care interventions with respect to bioethical issues and decisions, so as to ensure ongoing and proper holistic person-centered care.

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Cross-References

► Autonomy
► Benefit and Harm
► Committees: Research Ethics Committees
► Justice: Global
► Nongovernmental Organizations (NGOs)
► Organ Transplantation
► Pain
► Spirituality
► WHO

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Further Readings

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Patenting

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Abstract

Biotechnology has become one of the driving forces of science and technology and an important pillar of contemporary economy. This entry examines the role which patents play in this context and the challenges they are confronted with as regards concerns of ethics and morals.

Keywords

Biotechnology; Patents; History; Rationale; International regime; *Ordre public*; Morality

Introduction

Patents are exclusive private rights in inventions, i.e., intangible or incorporeal objects, granted to inventors and their successors in title by competent state authorities, such as the United States Patent and Trademark Office (US PTO), the Japan Patent Office (JPO), or the State Intellectual Property Office of the PR China (SIPO) or international organizations such as the European Patent Office (EPO). Patents are territorially limited rights, which confer on the proprietor the right to prohibit third-party commercial exploitation of

the patented invention without her consent. This is true even in the case that the third party created the invention independently. Thus, patents are blocking rights because they, unlike copyrights, do not offer protection only against copying. However, a patent is not a license to use the patented invention. This fundamental, however, often overlooked or ignored principle of patent law is best restated in the Directive 98/44/EC of the European Parliament and of the Council on the Legal Protection of Biotechnological Inventions (OJ 1998 No. L212 at p. 13), which reads as follows:

> . . .a patent for invention does not authorize the holder to implement that invention, but merely entitles him to prohibit third parties from exploiting it for industrial and commercial purposes. . . consequently, substantive patent law cannot serve to replace or render superfluous national, European or international law which may impose restrictions or prohibitions or which concerns the monitoring of research and of the use or commercialization of its results, notably from the point of view of the requirements of public health, safety, environmental protection, animal welfare, the preservation of genetic diversity and compliance with certain ethical standards

The use of a patented invention may also not infringe the rights of third parties. Patents are limited in time: According to binding international standards, the term of a patent is at least twenty years counted from the date of filing of the patent application. In order to be patented, an invention must be new, i.e., must not be comprised in the state of the art; it must involve an inventive step, which means it must not be obvious to an expert; and it must be industrially applicable and useful.

History and Development

Patents owe their name to medieval documents issued by a sovereign in the form of "open letters" (in Latin "*litterae patentes*") granting a title, monopoly, and the like. Unlike the "letters close" (in Latin "*litterae clausae*"), which could be opened only by the recipient, open letters' seal was loosely attached to the document, so that it did not have to be broken in order to be read. Since its beginnings in the privileges granted in the early fourteenth century, the patent system has been used as an instrument to generate and acquire more new knowledge and information in order to improve the economic situation of the country. In comparison to continental Europe, England at that time was a developing country. Therefore, her kings attracted foreign craftsmen from Belgium, Holland, France, and Italy to England and granted them privileges limited in time, to perform their business for the express purpose of introducing fellow countrymen to their skills, "*instruendi et informandi causa*," (i.e. to instruct and inform), as expressed in a privilege issued by Edward III in 1331. Industrial and inventors' privileges granted in the course of the fifteenth and sixteenth centuries in Italy, Holland, and Germany were based with similar motives in mind. Also the first statutory regulation, the Venetian Statute of Inventors of 1474, with its limited prohibition of imitating inventions, was intended to stimulate "deep thinkers and ingenious inventors from other regions" to exercise their activity for the benefit and advantage of Venice. Thus, the main aim of privileges, the forerunners of today's patent system, was to promote domestic crafts and industry and especially to introduce to their own country the production of goods which had theretofore been imported from abroad. Even the "real and first inventor," to whom the English Statute of Monopolies of 1623/1624 restricted grant of patent monopolies, did not necessarily have to make the invention himself. Sufficient and decisive was the introduction of new manufacture into the country, and it was totally irrelevant whether he had acquired his knowledge "through travel or research" (Adams 2008).

These technological and economic policy reasons remained decisive also for the further development of the patent system, when the system of privileges became more and more consolidated and finally led, during the late eighteenth and nineteenth centuries, the age of the industrial revolution, to a legally secured institution for the protection of the inventor. Although the first modern patent laws, adopted in 1790 in the USA, where the constitution of the Federation of 1787 stipulated in Article 1 (8).8 that "the Congress

shall have the power. . . To promote the progress of science and useful arts, by securing to. . . inventors the exclusive right to their . . . discoveries," and in 1791 in France, under the influence of ideas of natural law and as a reaction to the misuse of the system of privileges, strengthened the position of the inventor and awarded him a property right to his ideas, the economic policy aim of patent legislation remained unchanged: to improve the industrial development of one's own country and close the economic gap between it and England, the leading industrial nation of the time. The appropriate measure for this purpose was – as George Washington stated before Congress on January 3, 1790 – "[t]o efficiently initiate the introduction of new and useful inventions from abroad as well as the skill and genius at home for the production of these inventions." Similar arguments were presented by the Marquis de Boufflers, the author of the French Patent Act, before the French Parliament on December 30, 1790 (Beier and Straus 1977; Adams 2008). In order to achieve these aims, the patent system since its early days has required a complete disclosure of invention in patent specification. This has best been expressed in 1785 by Judge Buller in the famous case King v. Arkwright, where he stated, inter alia:

> It is clearly stated at law that a man, to entitle himself to the benefit of a patent for a monopoly, must disclose his secret and specify his invention in such a way that others may be taught by it to do the thing for which the patent is granted, for the end and meaning of the specification is to teach the public after the term for which the patent is granted, what the art is, and it must put the public in possession of the secret in as ample and beneficial a way as the patentee himself uses it. . . for the patent is the reward which, under the Act of Parliament, is held out for a discovery; and, therefore, unless the discovery be true and fair, the patent is void. (Wallace and Williamson 1900, p. 187)

In today's patent system, its "informational" function is even much more pronounced because patent applications are published already 18 months after the filing date, i.e., as a rule even before the patent is granted (in fact it may not be granted at all) and any effective protection attained.

The ubiquitous character of an invention, which allows its use at the same time at any place anywhere, made it clear relatively early that instruments were needed to provide for their protection internationally. The first international legal instrument was created in 1883 in Paris, when the Paris Convention (PC) for the Protection of Industrial Property was adopted and the Union for the Protection of Industrial Property established. The main and lasting achievements of the PC for the area of patents have been the principle of "national treatment" or "nondiscrimination," which made patent protection of contracting states accessible to the nationals of other contracting states under same conditions, and the so-called Union priority right, which enables applicants to use the filing date of their first patent application in a Union member state as the effective filing date in other Union member states for applications filed for the same invention within twelve months from the first filing date. The PC which has undergone several revisions, the last in 1967 in Stockholm, thanks to its rule that countries of the Union are allowed to conclude special agreements between themselves for the protection of industrial property, if they do not contravene its provisions, has become an umbrella for all subsequent international treaties dealing with patent protection. This is not only true for the Patent Cooperation Treaty (PCT) of 1970 and the European Patent Convention (EPC) of 1973 but also for the Agreement on Trade-Related Aspects of Intellectual Property Rights (TRIPS) adopted in 1994 in the framework of the then established World Trade Organization (WTO). In the area of patents, the TRIPS Agreement for the very first time in the history set forth internationally binding standards as regards the subject matter eligible for patent protection, content, and scope of the patent right as well as the term of its protection. Central to the TRIPS protection standards is the obligation of all 160 WTO members (June 2014) to make patents available for any inventions, whether products or processes, in all fields of technology (Article 27.1) and exclusions from patentability allowed only under strict conditions as set forth in Articles 27.2 and 27.3. Because under these TRIPS standards also

developing and least developed countries, after certain transitional periods, had to, or as in the case of the least developed countries, will have to (as of July 2021), provide patent protection, inter alia, also for pharmaceuticals, access to patented drugs in those countries is often viewed as an ethical problem and patents causing this barrier as ethically not justified (Sterckx 2004).

The rapid progress of globalization which followed the establishment of WTO in the mid-1990s has also generated steady exponential growth of worldwide patenting activities: The number of patent applications filed worldwide grew from some 1 million in 1995 to over 2.5 million in 2013, after passing the two million mark in 2011 (WIPO 2014).

Conceptual Clarification: Legal Basis

Despite its prevailing economic rationale, according to which patents are granted to recognize the intellectual property of the inventor; to reward the inventor for his useful services; to encourage inventors and industry to invent, invest, and innovate; and to incentivize the early disclosure and wide dissemination of technological knowledge (Machlup 1958; Beier 1980), the roots of, the justification for, and some limitations of exploitation of patents are reasoned ethically. Traditionally, ethical aspects in intellectual property rights, of which patents form an integral part, have been discussed in the broad justification of these rights. Lockean theory of natural rights to the fruits of own's labor and Hegel's personality justification suggesting that the best way of progressing science and arts is to protect scientists and artists from theft, so as to become the basis for learning by others, are such examples.

Consequently, patent law, as other laws, is subject to barriers inherent in the legal system, set by the constitution, by public policy, and by morals. These barriers exist in most countries, in legal orders of regional organization of states, such as the European Union, and international regional multilateral treaties, such as the EPC. As an example, Article 53 (a) EPC excludes from patentability inventions the commercial exploitation of which would be contrary to *ordre public* or morality. This general clause was understood to constitute the necessary gates of entry for overriding social and ethical considerations into the patent law system, which is otherwise neutral in its judgment and entirely devoted to the technical appreciation of invention (Beier und Straus 1986).

At the global level, Article 27.2 TRIPS sets forth that:

> Members may exclude from patentability inventions, the prevention within their territory of the commercial exploitation of which is necessary to protect *ordre public* or morality, including to protect human, animal or plant life or health or to avoid serious prejudice to the environment, provided that such exclusion is not made merely because the exploitation is prohibited by their law

It follows from this, on the one hand, that TRIPS itself does not impose the exclusion from patentability of inventions the commercial exploitation of which would contradict *ordre public* or morality but only allows WTO member states to provide for such exclusions in their national or regional legal instruments under conditions set forth in TRIPS and, on the other hand, that inventions the commercial exploitation of which is allowed cannot be excluded from patent protection under aspects of ethics or morals.

By adopting the notion of *ordre public* instead of "public order" or "public interest," TRIPS adopted a more narrow standard. Thereunder, the notion of *ordre public* comprises only the "major principles of the legal order," such as the inviolability of human dignity and the right to life, physical integrity, and personal freedom, as, for instance, laid down in Article 2 (2) of the Basic Law (Constitution) of the Federal Republic of Germany. Such principles are not necessarily infringed solely because exploitation of the invention is prohibited by law or administrative regulations. The European patent law founders decided in favor of this narrow standard because it was not necessarily in the general interest to deny patentability in all cases where the exploitation of the invention concerned was prohibited, for instance, under a foodstuff law allowing only certain additives. The inventor of a new additive with possibly

less harmful effects should indeed have the possibility of obtaining a patent since it is possible that the prohibition of exploitation may subsequently be removed. Or, in more modern terms, even if the somatic cell gene therapy, or the use of human embryonic stem cells for curing Parkinson's disease, may be prohibited for safety considerations today, which could be cleared by further scientific achievements and developments related to transfer vectors, in say two or three or possibly only in five years, resulting in subsequent removal of those prohibitions, why should an inventor of suitable stem cells or gene constructs, respectively, not be granted a patent, but left empty-handed and others allowed to use his/her invention for free after the removal of the ban? This would not only be unfair but also counterproductive in terms of providing incentives for the necessary investment into innovation. In the chosen construct, account is taken of the fundamental consideration that a patent does not afford its owner a positive right to use, but solely a right to prohibit (Straus 2013).

The phrase "necessary to protect *ordre public* or morality" in Article 27.2 TRIPS further requires that a causal connection exists between the measure taken (exclusion from patentability) and the effect sought (protection of *ordre public* or morality) for which sufficient scientific evidence is necessary. Moreover, if the objective of excluding commercial exploitation of inventions in a certain field of technology can be achieved in a way that does not require excluding inventions from patentability, then that way should always be preferred (de Carvalho 2010). Finally, the last part of Article 27.2 TRIPS, which reads ". . .provided that such exclusion [from patentability] is not made merely because the exploitation is prohibited by domestic law," requires that a WTO member may not base the exclusion from patentability on legal prohibitions unrelated to *ordre public* or morality. In other words, whereas the prohibition of commercial exploitation is a basic precondition for the exclusion, it in itself is not sufficient, if it is not based on reasons routed in *ordre public* or morality (Gervais 2012).

Article 27.2 TRIPS enumerates *ordre public* or morality as two seemingly independent standards for values, the protection of which allows, under the conditions discussed above, WTO members to exclude from patentability inventions the commercial exploitation of which would violate either or both of them. The leading commentaries on TRIPS are quite short as regards the notion of morality. Gervais (2012) observes that morality is a different concept as compared with *ordre public* and that "it seems to correspond to the French concept of *'bonnes mœurs,* '" further that "it naturally depends to a certain degree on particular culture of a country or region." In the German legal doctrine, it is understood that the reference to the notion of morality was necessary to directionally regulate specifically not foreseeable variety of factual constellations. It has also been pointed out that a danger exists that such provisions are applied in a very subjective, eventually discretionary manner resulting in a disproportional loss of legal certainty. Therefore, it was necessary to narrow down this provision according to its purpose and context. The notion of morality, which in the German translation reads as "*gute Sitten*," refers, on the one hand, to extra-legal principles, but, on the other hand, forms part of the legal order. Consequently, what is at hand is to put in effect in a specific case the value standards, which are anchored in the general legal order. A potential violation of the morality standard must be of such weight that it would justify an exception of the fundamentally existing claim or right to a patent. Prohibitions which are not anchored in statutory provisions, therefore, have to be essentially generally recognized as obligatory and have to be limited to elementary rules (Rogge 1998).

Ethical Dimensions

Although the general ethical aspects of the patent system have been discussed in legal as well as economic literature and have, as reported, also been to a certain specific extent reflected in statutory provisions, they have not been a major issue either for the legislature or for patent-granting authorities. That has changed radically following the advent of modern biotechnology in the late

1970s and early 1980s of the twentieth century. Recombinant DNA technology, cell fusion, embryonic stem cells, and many other techniques developed in the area of molecular biology and genetics generated within a relatively short period of time not only genetically modified microorganisms and tissue cultures producing, for instance, human insulin, erythropoietin, and granulocyte colony-stimulating factors, or hepatitis C, HIV, or breast cancer diagnostics but also genetically modified plants and animals. Moreover, human somatic gene therapy and even human germ line gene therapy came within the reach of possible or at least prospectively doable.

The perspective of having patents issued on genes, i.e., the fundamental units of heredity, or on animals and plants has sparked controversial debates at least partly based on ethical concerns, which even after some 40 years of successful biotechnology have not come to an end. Some religious leaders, for instance, have characterized the granting of patents on genes or organisms as representing "the usurpation of the ownership rights of the Sovereign of the universe" (Stone 1995). The slogan "No Patents on Life!" has gained lasting popularity, irrespective of the fact that it entirely misses relevant and fundamental aspects of patent law, namely, that of which constitutes the subject matter of a patent and which rights a patent confers on its owner. For example, the subject matter of a patent which relates to an animal is the invention, i.e., the teaching, for instance, how to improve the content of a valuable substance in the bovine milk. Thus, as explained from the very outset, it is an intangible or incorporeal object. Unlike an owner of a cow, which can milk, sell, or even slaughter it, the owner of a patent which relates to the said invention has no direct power over an animal covered by his patent, if he does not at the same time own the animal. The patentee can only prohibit third parties to produce, sell, etc., such an animal. Whether or not such animals may be produced, sold, etc., is not a matter of patent law but of regulatory provisions dealing with public health, animal welfare, or, for instance, environmental protection.

The responses of the lawmakers in the USA and in Europe to the ethical concerns raised against patents on genes and organisms have differed profoundly. The US Patent Law (35 United States Code – USC), which in its § 101 provides for the issuance of a patent to a person who invents or discovers "any" new and useful "manufacture" or "composition of matter," despite many attempts in the US Congress to introduce amendments, has remained unchanged and does not dispose of any provision which would link the patentability issue to any specific considerations of ethics and/or morals. In its famous 1980 *Diamond v. Chakrabarty* decision (447 U.S. 300, 100 S.ct. 2204 (1980)), the US Supreme Court, first, emphasized that the Congress intended statutory subject matter to "include anything under the sun that is man-made" and clarified that while laws of nature, physical phenomena, and abstract ideas were not patentable, a claim to a hitherto unknown natural phenomenon, thus to a nonnaturally occurring manufacture or composition of matter – a product of human ingenuity "having a distinct name, character, and use" – was therefore patentable. Consequently, the Court found a genetically modified bacterium from the genus *Pseudomonas*, capable of degrading oil spills, patentable. At the same time, however, the Court has refused to deal with objections raised against genetic research and related technological developments, which, as some amicus briefs had asserted, may spread pollution and disease and may result in loss of genetic diversity and in practice could tend to depreciate the value of human life. The Court, in principle, has not put into question the validity of those concerns, but made it clear that such contentions should be addressed to the political branches of the Government, the Congress, and the Executive and not to the courts. As a consequence, the US PTO since the 1980s has issued numerous patents on genomic as well as complementary DNA (cDNA), monoclonal antibodies, human embryonic stem cell lines, plants, plant varieties, as well as animals. The overall impact of this practice on the availability of new drugs has been remarkable: During 2012, the total sales of biologics, i.e., monoclonal antibodies, hormones, growth factors, fusion proteins, cytokines, therapeutic enzymes, blood factors, recombinant vaccines,

and anticoagulants, reached US $ 63.6 billion in the USA (Aggarwal 2014). However, in a 2013 decision in *Association for Molecular Pathology* et al. *v. Myriad Genetics Inc. et al.* (596 U.S. 12–398 (2013)), the US Supreme Court held that a naturally occurring DNA segment is a product of nature and not patent eligible merely because it has been isolated. Under this holding, only the cDNA is patent eligible because it is not naturally occurring. Patentable are also processes for manipulating and isolating genes, as well as new applications of genes. This approach of the US Supreme Court has not been shared, for instance, by the Federal Court of Australia. Deciding on essentially same facts and claims, the Australian Court in *D'Arcy v. Myriad Inc.* ([2014] FCAFC 115) held isolated genomic DNA a patent-eligible subject matter and emphasized that the isolated nucleic acid has "markedly different characteristics from any found in nature" and that *Myriad* did not merely "separate that gene from its surrounding genetic material." *Myriad's* claim, according to the Australian court, properly considered was not, as the US Supreme Court considered, concerned "primarily with the information contained in the genetic sequence [rather than] with the specific chemical composition of a particular molecule."

In order to secure a harmonized patent treatment of biotechnological inventions throughout the European Union and bring it in line with the then practice in the USA and Japan, the Community legislator, unlike the US Congress, decided in favor of a statutory regulation and, after some ten years of tense debates in 1998, adopted the aforementioned mentioned Directive 98/44/EC on the Legal Protection of Biotechnological Inventions. Altogether, ten recitals of this Directive directly or indirectly refer to *ordre public* or morality as the yardstick for patent eligibility of biotechnological inventions. The recitals, inter alia, emphasize that the patent law must be applied so as to respect the fundamental principles safeguarding the dignity and integrity of the person and the human body at any stage in its formation or development including germ cells cannot be patented (Recital 16). It is further emphasized that *ordre public* and morality correspond in particular to ethical and moral principles recognized in a member state and that such ethical or moral principles supplement the standard legal examinations under patent law regardless of the technical field of invention (Recital 39). Under Article 7 of the Directive, a European Group on Ethics in Science and New Technologies has been established to evaluate all ethical aspects of biotechnology. This Group, however, may only be consulted at the level of basic ethical principles.

The wording of the Directive itself generally specifies that:

> The human body, at the various stages of its formation and development, and the specific discovery of one of its elements, including the sequence of a gene, cannot constitute patentable inventions. (Article 5 (1))

Whereas the exclusion from patentability of "the human body at various stages of its formation and development" is clearly aimed at guaranteeing human dignity and integrity, the exclusion of the "simple discovery" of the elements of human body addresses the delimitation between non-patentable discoveries, as such, and inventions. Due to the accompanying clarifying addition that:

> An element isolated from the human body or otherwise produced by means of technical process, including the sequence or partial sequence of a gene, may constitute a patentable invention, even if the structure of that element is identical to that of natural element. (Article 5 (2))

It is clear that unlike in the USA, in Europe, for instance, genomic DNA as well as other "products of nature" are, in principle, patent eligible.

Directly addressed are *ordre public* and morality in Article 6(1) of the Directive, which by and large corresponds to the aforementioned Article 53 (a) EPC, i.e., excluding from patentability in general inventions the commercial exploitation of which would be contrary to *ordre public* or morality. Under the settled case law of the Court of Justice of the European Union (CJEU), this provision allows the administrative authorities and courts of the member states a wide discretion in applying the exclusions from patentability of inventions, which are necessary to take account of the particular difficulties to which the use

of certain patents may give rise to in the social and cultural context of each member state, a context which the national legislative, administrative, and court authorities are better placed to understand than the Community authorities (Case C-377/98 – *Kingdom of the Netherlands v. European Commission and Council of European Union* [2001] ECR I-7079). However, as also clarified by the CJEU, no such discretion of the member states exists as regards the four specifically identified categories of inventions expressly excluded from patent protection for ethical reasons in Article 6 (2) of the Directive, namely, (a) processes for cloning human beings, (b) processes for modifying the germ line genetic identity of human beings, (c) uses of human embryos for industrial or commercial purposes, and (d) processes for modifying the genetic identity of animals, which are likely to cause them suffering without any substantial medical benefit to man or animal, and also animals resulting from such processes (Case C-456/03 – *Commission of the European Community v. Italian Republic,* [2005] ECR I-5335). In applying Article 6 (2) (c) of the Directive, the CJEU (Grand Chamber) defined the term "embryo" not only as any human ovum, as soon as fertilized, if that fertilization is such as to commence the process of the development of human body, but also a non-fertilized human ovum into which the cell nucleus from a mature human cell has been transplanted and a non-fertilized human ovum whose division and further development have been stimulated by parthenogenesis. However, the CJEU also explicitly clarified that "a non-fertilized human ovum must necessarily have the inherent capacity of developing into human beings," in order to be classified as a "human embryo" (Case C-34/10 – *Oliver Brüstle v. Greenpeace e.V.* [2011] ECR I-09821; Case C-364/13 – *International Stem Cell Corporation v. Comptroller General of Patents, Designs and Trade Marks*, ECLI:EU:2014:2451). Under this case law of the CJEU, any invention, the implementation of which required a prior destruction of a human embryo, is unpatentable, even if the claims of the patent do not concern the use of human embryos and even if that destruction is neither described in this specification nor necessary for the working of invention. The Court also held that although the aim of scientific research must be distinguished from industrial or commercial purposes, the use of human embryos for the purpose of research which constitutes the subject matter of a patent application cannot be separated from the patent itself and the rights attached to it. Thus, the exclusion in Article 6 (2) (c) of the Directive covers also the use for purposes of scientific research. Consequently, the CJEU has declared the (legal) destruction of a human embryo from which, like in the *Brüstle* case, the claimed human pluripotent stem cells were (legally) generated as a part of the technical teaching at hand in the application, although it has neither been claimed nor described nor was it necessary for performing/implementing the invention as claimed. Considering that the human embryonic pluripotent stem cells used in the *Brüstle* invention were legally generated from destroyed supernumerary embryos in Israel and imported into Germany, in line with German regulatory provisions, considering further that the working of the *Brüstle* invention is allowed, and, finally, considering that the commercial use of the "products" of that invention is allowed in a number of EU member states allow for the only conclusion to be that in the opinion of the Court the act of filing the patent application has been implicitly classified as the use of the embryo for "commercial or industrial purposes" and constituted a violation of *ordre public* or morality. Under the pretense to protect human dignity, under the regime established by this case law of the CJEU, the *Brüstle* invention remained unprotected, but at the same time stem cell lines of the type used by *Brüstle* for his invention can in a great number of the EU member states be legally generated from supernumerary human embryos, the products of the *Brüstle* type invention can be commercialized in most of the EU member states, and the *Brüstle* invention can be freely copied EU wide. This does not appear to be in line with ethics and morals, or with the basic mission of the patent system, or with Article 27.2 TRIPS (Straus 2013).

For the sake of completeness and in view of the often expressed ethical concerns as regards

patenting of higher organisms, it should be added that under the EU Directive inventions which concern plants or animals are patentable "if the technical feasibility of the invention is not confined to a particular plant or animal variety" (Article 4 (2)). In other words, generic, i.e., generally applicable, inventions concerning plants or animals are patent eligible. The fact that plant or animal varieties are excluded from patent protection in Europe is not based on ethical but rather on legal (existence of a special type of protection for new varieties of plants) and economic considerations. Policy and economic considerations were also responsible for the fact that WTO member states under Article 27.3 (b) TRIPS are allowed to exclude from patent protection plants and animals in general (de Carvalho 2010; Gervais 2012).

Conclusion

The attitude of the contemporary society toward new developments in science and technology, maybe more than in the past, is characterized by a deeply rooted and complex conflict between, on the one hand, high hopes in all sorts of benefit, in particular as regards new drugs for curing or at least treating as yet not curable illnesses and, on the other hand, fears and concerns of harm which the new technologies could cause. To the same extent as the hopes may be and are unrealistic, at least in short or medium term, the fears and concerns may be and, as the past has often proven, have been groundless. Patents as legal instruments whose primary economic rationale is to provide incentives for inventors and industry to invent, invest, and innovate and to protect the effected, in particular in the area of biotechnology high-risk investments, therefore, have been and still are exposed to variably motivated attacks. Those who oppose or are skeptic of research and technological developments and/or are against the use of their expected results, for instance, as regards research with human embryonic stem cells or research and development, as well as the use of genetically modified plants, do not hesitate to attack patenting with contradicting arguments. On the one hand, they may contend that patents are blocking access to the new technology; on the other hand, they may contend that the technology at hand will harm the environment, affect and reduce biodiversity, etc. This is in Europe very often the case in the area of the so-called green biotechnology. In the case of stem cell research, the attacks against patents are primarily motivated with arguments of ethics and morals, because that research is linked to the use of human embryos. Under circumstances, however, in which such research is allowed under stringent regulatory provisions, as is the case in many member states of the European Union, patents actually constitute only a surrogate target, because the real target is the research and development itself. Interestingly, and a possible sign of double moral standards, it has to be observed that nobody has thus far expressed objections against the use of potentially successful drugs for curing diseases such as Parkinson's or multiple sclerosis, developed with such a technology.

Linking patent eligibility of inventions to *ordre public* and/or morality implies a complicated and tense relationship. Although to a different degree, both *ordre public* and morality are subject to unpredictable, or at least difficult to predict, evolutionary changes, as a consequence of scientific and technological developments as well as changes of public acceptance. Granting of patents, which do not confer on their owners any positive license to exploit the patented invention, but only an exclusive right to use it in compliance with regulatory provisions and rights of third parties, and whose main macroeconomic aim is to provide sufficient incentives for investment into risky research and development, ideally leading to badly needed innovation and, via market success, to reward inventors and innovators, should not be made dependent on rules and principles, which are more or less broadly accepted by the public at large and, by all means, almost without exception, subject to unpredictable evolutionary changes. Developments of science, technology, and industry in the USA and in Europe reveal the higher efficiency of the US approach, whose statutes and case law have not established such a link, over that adopted in Europe. If generally accepted ethical concerns in a society exist against specific research and development or the use of results of

such development, it is, as convincingly and correctly stated by the US Supreme Court in the *Chakrabarty* case, a matter and responsibility of the regulatory legislator to act accordingly and take into account those concerns. It should be generally realized that the grant of a patent does not contain any value judgment other than the patented invention meeting all patentability requirements.

Cross-References

- ▶ Bioethics: Medical
- ▶ Biopiracy
- ▶ Commodification
- ▶ Embryo
- ▶ Gene Therapy
- ▶ Genomics
- ▶ Human Body
- ▶ International Law
- ▶ Stem Cells: Embryonic

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Paternalism

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Abstract

Paternalism and especially medical paternalism, the paternalism of physicians and other healthcare professionals toward patients, occupied center stage in the early decades (1970s–1980s) of bioethics, especially in bioethics in the United States. The concepts of paternalism and medical paternalism in bioethics have roots in the general concept of paternalism, the assertion of epistemic and moral authority, and the power such authority justifies, over children by fathers, historically, and parents. Utilizing relevant ethical concepts and principles, the concept of medical paternalism

is elucidated, to identify its individually necessary and jointly sufficient conditions: (1) a physician or other healthcare professional undertakes actions designed to limit the exercise of the patient's autonomy (2) on the basis of clinical judgment and decision-making that are reliably beneficence based about how to protect the health-related interests of the patient from the consequences of the patient's decisions and actions based on them. The presumption in bioethics, especially bioethics in the United States, that medical paternalism was an accepted ethical norm and therefore common clinical practice in the history of medicine is described and then called into question. This discussion suggests that the importance of medical paternalism for the future of bioethics has become an open question.

Keywords

Autonomy; Beneficence; Medical paternalism; Paternalism; Patient; Respect for autonomy

Introduction

This entry provides a conceptual and historical account of two key related concepts in the field of bioethics, paternalism and medical paternalism. The conceptual account places the concept of paternalism in bioethics in the contexts of paternalism more generally, in the form of asserted epistemic and moral authority and power of parents over their children, and the refinement of the concept of paternalism in canonical bioethics literature during the past four decades. Utilizing the ethical concepts of beneficence and respect for autonomy, the concept of medical paternalism is elucidated, to identify its individually necessary and jointly sufficient conditions: (1) a physician or other healthcare professional undertakes actions designed to limit the exercise of the patient's autonomy (2) on the basis of clinical judgment and decision-making that are reliably beneficence based about how to protect the health-related interests of the patient from the consequences of the patient's decisions and actions based on them. The presumption in bioethics, especially bioethics in the United States, that medical paternalism was an accepted ethical norm and therefore common clinical practice in the history of medicine is called into question. The result is that the importance of medical paternalism for the future of bioethics has become an open question.

History and Development

Paternalism and especially medical paternalism, the paternalism of physicians and other healthcare professionals toward patients, occupied center stage in the early decades (1970s–1980s) of bioethics, especially in bioethics in the United States. The concept of medical paternalism in bioethics has roots in the general concept of paternalism, the assertion of authority and power over children by fathers, historically, and parents. The concept of paternalism in bioethics is therefore best explicated in the context of parental authority and power over children.

Parental Authority and Power Over Children

As many scholars in bioethics have noted, the word "paternalism" takes its meaning from the epistemic and moral authority and subsequent power that fathers – and, more generally, parents (hence the use by some of "parentalism") – have and exercise over their minor children. Paternal epistemic authority is the claim to know better than the child what is in the child's interest: the stakes that the child is reliably understood to have in his or her immediate, short-term, and long-term future. Such authority is claimed as a matter of paternal responsibility to protect minor children from themselves, especially when they make and then act on decisions that are likely to have serious, far-reaching, and irreversible consequences for the child and adult he or she will become. The authority claimed is epistemic – to understand a child's immediate, short-term, and long-term interests better than the child does. Such a claim to epistemic authority need not be infallible but should be reliable, i.e., have some evidence base. The strength of the claim to epistemic authority will therefore vary as a function of the evidence

for judgments about what is in the child's interest. There is good reason for parental claims to epistemic authority: younger children and adolescents usually do not have a well-developed sense of and appreciation for their short-term and long-term futures, whereas their parents usually do. Parental moral authority is based on the fundamental moral relationship between any parent and any child: the duty to nurture and protect the child into adulthood.

Parental epistemic and moral authority of parents over their minor children warrants parental power (the ability to make and effect decisions) over their minor child, to make and carry into effect decisions for that child. The power thus exercised may replace either the decision-making of the child with that of the parent or the action of the child with that of the parent or action of the child enforced by the parent. If the assertion of parental authority lacks epistemic authority or moral authority, e.g., if the assertion of power can reliably be predicted to result in net harm to the child and the parent should have known this, then paternalism is not warranted epistemically or ethically. Such paternalism is ethically unjustified.

The ethical justification for the traditional paternalism of parents over their minor child thus has the following components:

1. The parental claim to epistemic authority should be reliable: the parent must have some evidence base, which can vary in its strength, for his or her judgment about what counts as the child's immediate, short-term, or long-term interests.
2. The parental claim to epistemic authority should be transparent: the parent must be able to explain, to himself or herself and to the child, the evidence base for his or her judgment about what counts as the child's immediate, short-term, or long-term interests.
3. The parent's claim to moral authority should be grounded in the fundamental obligation to nurture and protect the child and not in the parent's self-interest or the interests of others.
4. The decision of the parent should be reliably understood and designed to secure the immediate, short-term, or long-term interest (s) judged by the parent to be at stake in the child's decisions.
5. The action of the parent should be reliably understood and designed to secure the immediate, short-term, or long-term interest (s) judged by the parent to be at stake in the child's actions.

There is an implicit assumption in this ethical justification of paternalism: the child's decisions and actions do not command the respect of an adult's decisions and actions. Put another way, by reason of immaturity – especially the relative lack of experience and the insufficiently developed ability to conceive of short-term, intermediate, and long-term consequences of one's decisions and actions – the child does not have the same right to be free to the controlling decisions and actions of another that adults have. By contrast, adults do have rights, especially the negative right to noninterference with their decisions or actions unless those actions pose predictable harm to others, and those others have not consented or should not be presumed to have consented to be exposed to such risk of harm.

The Ethical Concept of Paternalism in Bioethics: Medical Paternalism

Opposition to physician paternalism, or medical paternalism, has been a defining feature of bioethics from its origins in the United States. There is a vast literature on the topic, reflecting the centrality and importance of the topic to the field of bioethics. The ethical concept of paternalism has undergone significant development in the history of bioethics over the past four decades.

In the first edition of the *Encyclopedia of Bioethics*, "paternalism" is defined as "practices that restrict the liberty of individuals, without their consent, where the justification for such actions is either the prevention of some harm they will do to themselves or the production of some benefit for them that they would not otherwise secure" (Beauchamp 1978, p. 1194).

The *Principles of Biomedical Ethics*, from its first edition in 1979 through its latest seventh edition in 2013, has become the most commonly used textbook in college and university courses in

bioethics in the United States and in many other English-speaking countries. Beauchamp and Childress defined "paternalism" in the first edition of *Principles* as: "overriding a person's wishes or actions in order to benefit or prevent harm to that person" (Beauchamp and Childress 1979, p. 155). The seventh edition sets forth a more complex definition:

> . . .*the intentional overriding of one person's preferences or actions by another person, where the person who overrides justifies this action by appeal to the goal of benefitting or of preventing or mitigating harm to the person whose preferences or actions are overridden*. (Beauchamp and Children 2013, p. 215, emphasis original)

Beauchamp, writing for himself, makes the important distinction between the "autonomy-limiting" and the "preference-limiting" definitions of paternalism. They are, respectively, the following:

> . . . the *autonomy-limiting definition*: Paternalism is the intentional limitation of the autonomy of one person by another, where the person who limits autonomy justifies the action exclusively by the goal of helping the person whose autonomy is limited. (Beauchamp 2010, p. 104, emphasis original)
>
> . . . the *preference-limiting definition*: Paternalism is the intentional overriding of one person's known preferences by another person, where the person who overrides justifies the action exclusively by the goal of helping the person whose will or preference is overridden. (Beauchamp 2010, p. 105, emphasis original)

The second definition does not require that the individual whose preferences are overridden has the capacity to make and execute autonomous decisions; the first definition does. Following Beauchamp, this entry accepts the first definition as the basis for its account of the concept of medical paternalism, below.

Relevant Ethical Concepts

The Ethical Principle of Beneficence

In general the ethical principle of beneficence obligates an individual to act in such a way as to result in net benefit for others. To be applicable, this general principle must be specified to different contexts. In the healthcare setting, this specification focuses on the health-related interests of patients: the ethical principle of beneficence obligates physicians and other healthcare professionals to provide forms of clinical management that are reliably expected to result in a greater balance of clinical goods over clinical harms for the patient and thus protect and promote the health-related interests of the patient. Such judgments should be based on the best available evidence about the outcomes of the clinical management under consideration. Forms of clinical management supported by the ethical principle of beneficence are medically reasonable.

The Ethical Concept of Being a Patient

The ethical concept of being a patient was introduced into the history of medical ethics in the eighteenth century by the Scottish physician-ethicist, John Gregory (1724–1773). The concept is beneficence based: an individual human being is presented to a physician or other healthcare professional, and there exist forms of clinical management that are reliably expected to result in net clinical benefit for that individual (McCullough 1998). This concept is not autonomy based: an individual human being can become a patient without his or her consent. Thus, infants and minor children can become patients, as can adult patients in clinical emergencies (life-threatening conditions for which there is safe and effective treatment that must be provided immediately, making it impossible to engage the patient in the informed consent process) or adult patients who have lost the capacity to make decisions, either from diseases such as severe dementia, injuries such as brain trauma, or iatrogenically from sedation or anesthesia. This is also a concept free of metaphysical requirements, especially the requirement that the individual human being also be a person. The ethical concept of being a patient is both clinical and practical, with comprehensive application in clinical practice, innovation, and research, unlike the metaphysical concept of being a person.

The Philosophical Concept of Autonomy

An individual is autonomous when he or she is not under the control of others and is sufficiently in

control of his or her decisions and actions to be considered their origin. One very demanding conception of autonomy is that of Immanuel Kant (1724–1804), who held that to be autonomous required that an individual become the self-legislating lawgiver and therefore not be subject to the law or rule of another (heteronomy), who may be corrupt. An individual achieves such self-mastery by conforming all moral judgments and actions based on them to the categorical imperative to treat others always as ends in themselves (autonomy). For Kant, becoming autonomous is not automatic, e.g., achieving the legal age of adulthood, but the result of sustained, lifelong self-discipline and sacrifice required to act always from duty and not from self-interest. David Hume (1711–1776) and John Stuart Mill (1806–1873) put forth a somewhat less demanding conception. To be autonomous requires that an individual not be subject to external controlling influences. The bioethics literature on paternalism has been dominated by this less demanding conception of autonomy. All adult patients are presumed to have the capacity for autonomous decision-making (decisional autonomy) and action (executive autonomy), putting the burden of proof on the clinician to establish reliably that the patient lacks either decisional or executive autonomy.

The Ethical Principle of Respect for Autonomy

The ethical principle of respect for autonomy obligates the physician to empower the patient, or the surrogate of a patient who is not able to make decisions for himself or herself by reason of decisional incapacity or minor legal status, to make decisions by providing him or her with information that the patient or surrogate decision-maker needs to exercise his or her autonomy meaningfully in decision-making about proposed clinical management: the nature of the proposed clinical management and why it is medically reasonable, the medically reasonable alternatives, and the clinical benefits and risks of each medically reasonable alternative. The physician should attempt to ascertain that the patient or surrogate decision-maker understands this information and that his or her decision-making process is voluntary.

Ethical Dimension

The Concept of Medical Paternalism

Paternalism in the clinical setting, or medical paternalism, occurs when (1) a physician or other healthcare professional undertakes actions designed to limit the exercise of the patient's autonomy (2) on the basis of clinical judgment and decision-making that are reliably beneficence based in order to protect the health-related interests of the patient from the consequences of the patient's decisions and actions based on them. Each of these two conditions is individually necessary for medical paternalism to be justifiably attributed to a physician's or other healthcare professional's actions in patient care. Taken together these two conditions are jointly sufficient for medical paternalism to be justifiably attributed to a physician's or other healthcare professional's actions in patient care.

In the technical language of logic, a necessary condition invokes the argument form of modus tollens (if p, then q; not q; therefore not p) with q as the necessary condition for p: in the absence of q, p does not occur. For the concept of medical paternalism, if either the first or the second condition is not the case, then the concept of medical paternalism does not apply to the physician's or other healthcare professional's actions. Jointly sufficient conditions invoke modus ponens (if p and q, then r; p and q; therefore r). For medical paternalism, if both the first and second conditions are the case, then the concept of medical paternalism does apply to the physician's or other healthcare professional's actions. The minor premise, "p and q," is a conjunction, which is true if and only if both conjuncts are true. This is the logical meaning of "jointly sufficient." Both conditions must be satisfied for the action of a physician or other healthcare professional to be categorized as medical paternalism.

Medical Paternalism: A Core Topic in Bioethics

As a beneficence-based ethical concept, the concept of being a patient includes an intrinsic potential for paternalism. This is because the second of the two individually necessary and jointly sufficient conditions for the clinical application of the

concept of medical paternalism explicitly invokes beneficence-based clinical judgments about how to protect and promote the health-related interests of patients and the professional responsibility to protect and promote those interests in patient care. This includes the responsibility to protect the patient from himself or herself, e.g., by advising against behaviors such as smoking or excessive consumption of spirit beverages that are known to put health and life at risk. Inasmuch as such clinical judgments are both evidence-based and beneficence-based in their evaluation of the evidence, they are expert clinical judgments. Thus, this second condition reflects the claim of parents to epistemic and moral authority over minor children. The epistemic authority of physicians and other healthcare professionals over patients is a function of the reliability of their evidence-based clinical judgments. The moral authority of physicians and other healthcare professions over patients is a function of the reliability of their evidence-based evaluation of the processes and outcomes of clinical management to determine whether the outcome is of net clinical benefit for the patient and therefore medically reasonable and the professional commitment to implement medically reasonable alternatives to protect the health and life of the patient.

The early literature of bioethics in the 1970s in the United States repeatedly identified medical paternalism as a major ethical issue of widespread occurrence in clinical practice and as an accepted ethical norm and practice in the history of medicine. The *Encyclopedia of Bioethics* entry, referenced above, characterized "issues of paternalism in bioethics" as follows: "There are so many individual examples of controversial paternalistic justifications in biomedical and behavioral contexts that only a few selected samples can be treated here" (Beauchamp 1978, p. 1197). This entry conveyed the accepted view at the time: physicians regularly engaged in paternalism in the patient care, without sufficient justification, and had done so throughout the history of Western medicine. The view that medical paternalism is a major ethical concern is not confined to the early period of bioethics. This view can also be found in the literature of the field in its mature period: ". . . paternalism may be the most pervasive moral problem in medicine. . ." (Gert et al. 1997, p. 195).

Bioethicists often traced the roots of medical paternalism as a medical ethical norm and therefore accepted clinical practice back to the Hippocratic Oath and texts and what has been called the Hippocratic Tradition in medical ethics, i.e., a tradition of medical paternalism persisting from ancient Greece down to the present. Veatch castigated the Hippocratic Oath for its "paternalistic Hippocratic patient-benefitting principle" (Veatch 1976, p. 248). The Hippocratic Tradition supported not being truthful with patients and thus violated the patient's "right and duty to consent to their medical treatment based upon reasonable knowledge of their medical conditions" (Veatch 1976, p. 241).

Katz's *Silent World of Doctor and Patient* communicated in its very title the centrality of medical paternalism as a moral problem in clinical practice. Referencing works from the history of Western medical ethics, Katz argued that the paternalism of not being forthcoming with patients about their illnesses and the management of them was an accepted ethical standard by physicians and put into clinical practice:

> The history of the physician-patient relationship from ancient times to the present bears testimony to physicians' caring dedication to their patients' physical welfare. The same history, by its account of the silence that has pervaded this relationship, also bears testimony to physicians' inattention to their patients' right and need to make their own decisions. Little appreciation of disclosure and consent can be discerned in this history, except negatively, in the emphasis on patients' incapacities to apprehend the mysteries of medicine and, therefore, to share the burdens of decision with their doctors. (Katz 1984, p. 28)

The principal form of medical paternalism was withholding information from patients that they have the right to know. Patients have the positive right to be provided information that they need in order to make informed decisions in the exercise of their autonomy. Patients have the negative right not be to controlled by others in their decision-making processes. The physician's obligations to provide such information and not to interfere with the patient's decision-making processes were

understood in this literature to be required by the ethical principle of respect for autonomy.

Medical paternalism in the form of withholding information from patients limits a patient's autonomy by restricting the range of options for the patient to learn about, evaluate, and consider authorizing. This account invokes a concept of decisional autonomy that holds that a decision cannot be autonomous if the decision-maker was not aware of all of the options available to him or her in a particular decision-making context. This form of limiting autonomy is especially insidious in contexts in which expert knowledge and experience are required to identify the options that are open to the decision-maker. Medical care is one such context. The typical patient, considered as a layperson of average sophistication about health and healthcare provided by healthcare professionals, cannot reasonably be presumed to be aware of all of the medically reasonable alternatives for the clinical management of his or her condition, disease, disability, or injury. The typical patient will therefore be unaware that a physician or other healthcare professional has withheld information about medically reasonable alternatives. When information about medically reasonable alternatives is withheld from the patient without the patient's knowledge and when this is done for beneficence-based reasons, i.e., to protect the health (including general well-being) or life of the patient, then the action of such withholding of information satisfies the jointly sufficient conditions for the application of the concept of medical paternalism.

Another form of medical paternalism, of which the typical patient is usually aware, is interfering with the patient's autonomy of action forcing treatment on a patient for his or her own good when the patient has, as a result of the informed consent process, exercised his or her autonomy to refuse such treatment. In such cases, the limitation on autonomy takes the form of selecting for the patient the medically reasonable alternative for the patient and then implementing it despite the patient's express refusal. The patient's decision and action are both overridden. When such overriding of patient autonomy is justified by an appeal to the healthcare professional's beneficence-based obligation to protect the health and, especially, the life of the patient, the jointly sufficient conditions for the application of the concept of medical paternalism have been satisfied.

A paper by Oken (1961) was – and still is – cited in the American bioethics literature as providing evidence for the clinical acceptability and practice of medical paternalism. In a qualitative, descriptive study of a convenience sample of physicians in one hospital in the City of Chicago, Illinois, Oken documented that 88 % of his respondents reported that they withheld the truth about patients' bad conditions and prognoses from them. A closer look at Oken's data reveals that only 8 % *never* were honest with their seriously ill patients. One of the main reasons that this group of physicians expressed to justify not being honest with seriously ill patients was the emotional burden on the physician. These physicians were limiting patient autonomy by not providing patients with the diagnostic and prognostic information that they required for making informed decisions about the clinical management of their conditions. The first necessary condition for the application of medical paternalism was satisfied. The second necessary condition, however, was not: physicians were withholding information, not for the benefit of the patient (the second necessary condition of medical paternalism) but mainly for their own benefit and their self-interest in protecting themselves from the impact on them of giving bad news to patients.

The concept of medical paternalism therefore does not apply, yet it was invoked by bioethicists who cited the Oken article. This suggests that the core objection to medical paternalism for many American bioethicists is that the first necessary condition, limiting the autonomy of the patient, is also by itself the single sufficient condition for the application of the concept rather than one of two jointly sufficient conditions. That such a position results in conceptual confusion has not been recognized by these bioethicists (McCullough 2011). Not giving patients bad news because doing so is emotionally burdensome for the physician is surely a limitation of the patient's autonomy of decision-making and therefore of action,

but this practice is not accurately characterized by "medical paternalism" because the second of two indivudally necessary and jointly sufficient conditions for the application of this concept is not satisfied. That a physician's or other healthcare professional limits patient autonomy is not eo ipso medical paternalism, unless medical paternalism is equated to limiting patient autonomy.

American bioethicists were also mistaken that not being honest with seriously ill patients was an accepted ethical norm and practice. For example, the two giants of British Enlightenment medical ethics, who invented modern professional medical ethics (McCullough 1998, 2006), did not embrace such a norm. John Gregory (1724–1773), the Scottish physician-ethicist, took just the opposite view and argued for the obligation to be honest as an "indispensable" duty of physicians (McCullough 1998, 2011). Thomas Percival (1740–1804), the English physician-ethicist, held that being honest with seriously ill patients was among the "offices" or duties of a physician (McCullough 2011).

Being honest with patients and providing them with information that they needed to participate in decision-making about their clinical care became clinical practice as early as the nineteenth century in the United States. Powderly (2000) has beautifully documented the consent practices of a gynecologist in the City (now Borough) of Brooklyn, Alexander Skene (1837–1900). He routinely provided his patients with information about their diagnoses, both medical and surgical means of clinical management, engaged his patients in the decision-making about the clinical alternatives, and negotiated care plans when patients refused his recommendations. All of Dr. Skene's patients were women. If there were a patient population at risk for being subject to medical paternalism by male physicians (and almost all physicians in the United States at that time were men), it would be female patients. Dr. Skene's anti-paternalistic clinical practice is therefore all the more remarkable counterexample to the supposed "Hippocratic Tradition" of medical paternalism in Western medicine (McCullough 2011). Recent scholarship (Nie 2009) suggests that paternalism in communicating with patients was not accepted as a norm or practice in the history of Chinese medical ethics.

The self-understanding of bioethics in the United States in its early decades was that it provided an ethically urgent corrective to the rampant, accepted medical paternalism of physicians. This self-understanding is reflected, for example, in the entry on paternalism in the first edition of the *Encyclopedia of Bioethics* (Beauchamp 1978), a key passage from which about widespread medical paternalism is quoted above. McCullough (2011) has recently argued, on the basis of a thorough examination of sources such as Oken and the major texts from the history of Western medical ethics, that the creation of the field of bioethics, at least in the United States, was predicated on both historical and conceptual mistakes about medical paternalism. It was not accepted as an ethical norm by major physician-ethicists in the history of medical ethics, and it was not a widely accepted practice.

Conclusion

The concepts of paternalism and medical paternalism have played a central role in the field of bioethics, especially during its formative years in the United States. Recent scholarship in the history of medical ethics suggests that this central role for these two concepts may not have been justified. The role that the concepts of paternalism and medical paternalism should therefore play in the future development of the field of bioethics has become an open question.

Cross-References

- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Bioethics: Clinical
- ▶ Professional-Patient Relationship

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Patient Rights

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Abstract

The right to health and well-being is a fundamental right that influences all aspects of life. The most effective way for health care professionals to fulfill their obligations under the "right to health" approach is to ensure that they provide the highest possible standard of care while respecting the fundamental dignity of each patient. This entry is designed to give the reader a basic introduction to patient rights. It will focus on the doctor–patient relationship and present areas of greatest concerns. Patient rights are those basic rules of conduct between patients and medical caregivers, covering such matters as access to care, respect, communication, patient dignity, confidentiality, and consent to treatment. Patients have the right to be treated and dealt with in a humane and respectful manner. Health care providers are urged to pay careful attention to this vital aspect of clinical management; they must keep the welfare and the rights of the patient above any other consideration. Most countries outline fundamental elements of the doctor–patient relationship in their Code of Medical Ethics.

Keywords

Human rights; Patient rights; Doctor–patient relationship; Medical ethics

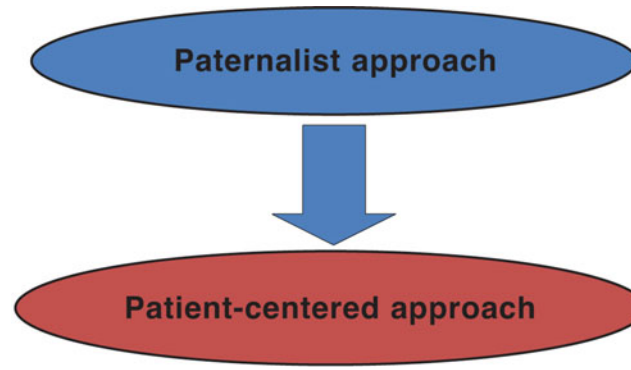
Introduction

Human rights are universal and indivisible rights, possessed by all people, by virtue of their common humanity. The right to health and well-being is a fundamental right that influences all aspects of life. The most effective way for health care professionals to fulfill their obligations under the "right to health" approach is to ensure that they provide the highest possible standard of care while respecting the fundamental dignity of each patient. Governing bodies should respect, protect, account, and fulfill patients' rights. Nevertheless, these rights need further refinement from professional health care providers, patients' relatives and friends, and the patients themselves toward their own bodies.

1. Promotion of health and prevention of disease
2. Cure of disease
3. Maintenance or improvement of quality of life through relief of symptoms, pain, and suffering
4. Prevention of untimely death
5. Improvement of functional status or maintenance of compromised status
6. Education and counseling of patients regarding their condition and prognosis
7. Avoidance of harm to the patient in the course of care
8. Assisting in a peaceful death

* Frequently, many or most of these goals can be achieved simultaneously

Patient Rights, Fig. 1 The goals of medicine



Patient Rights, Fig. 2 Remodeling of the physician–patient relationship

The goals of medical intervention in front of a patient will differ depending on the clinical facts of a case. Medical goals in a particular case may include one or several of the following: promotion of health and prevention of the disease, maintenance or improvement of the quality of life, cure of the disease, improvement of functional status, avoidance of harm, and assisting in a peaceful death (Fig. 1).

Frequently, many or most of these goals can be achieved simultaneously. In all cases, patients and physicians should clarify the goals of intervention when deciding on a course of treatment and should always take account of the patient's own goals. This is what we call the remodeling of the patient–physician relationship with a gradual shift from traditional paternalism to a patient-centered model (Fig. 2).

History and Development

Historically, transition from paternalism to autonomy started in 1914, with a decision issued by the New York Court of Appeals in 1914 following the Schloendorff v. Society of New York Hospital, which stated that "Every human being of adult years and sound mind has a right to determine what shall be done with his own body" (Schloendorff 1914). Another reported event happened in 1976, with Karen Ann Quinlan – persistent vegetative state on a ventilator; the New Jersey Supreme Court recognized the principle of a surrogate decision maker speaking for an incompetent patient (Fine 2005).

More recently, the health and human rights movement has grown with the expansion of the human immunodeficiency virus/acquired

Patient Rights,
Fig. 3 Patients' rights

1- Right for a good quality care
2. Right for informed consent and free choice
3. Right for clear information
4. Right for confidentiality
5. Right of non conscious or incapacitating patient
6. Right to access medical record
7. Right for information and health education
8. Right for human dignity
9. Right for spiritual assistance

immunodeficiency syndrome (HIV/AIDS) pandemic. Then appeared developments in the academic field with the growth of health and human rights training programs, especially in medical and public health schools; the creation of specific professorships at universities; the addition of scientific journals focusing on health and human rights; and international conferences (Mpinga et al. 2011).

There are many challenges in different countries in the world concerning patients' rights, among which are the economy, health systems, and inappropriate vision and/or analysis. It is well known that it will not be possible to provide all forms of health care in every country, as they are different in many respects. Even if it is not affordable at present, a future plan is necessary. In this regard, Mary Robinson from the Office of the UN High Commissioner for Human Rights (OHCHR) stated that the right to health does not mean the right to be healthy nor does it mean that poor governments must put in place expensive health services for which they have no resources, but it does require governments and public health authorities to put in place policies and action plans which will lead to available and accessible health care for all, in the shortest possible time (Asher et al. 2007; Gruskin 2006).

The Patients' Rights

The oldest (more than 2,500 years old) medical code is in The Hippocratic Corpus, otherwise known as the Hippocratic Oath, which contains several elements, emphasizing the commitment to the well-being of patients (Veatch 1997).

The medical ethics that has developed over the centuries has been altered by religion, social change, and behavior of physicians and patients. An additional and newer influence on medical ethics is the human rights movement. A fundamental concept of the human rights movement is that the decisions are made autonomously by informed patients. Human rights are a dominant force in the society and have substantial, positive implications for health care and medical ethics (Sheldon 1998).

They had certainly a positive impact on the development of patients' rights. These rights are summarized in Fig. 3 and are related to the modern application of clinical ethics.

Right for a Good Quality Care

Access to health care services should be provided without discrimination regarding race, religion, sex, national origin, or disability. Patients should also be free from discrimination on the basis of their disease, with respect to both employment and health insurance accessibility. Optimal treatment of a disease should be provided by a team that includes multidisciplinary medical expertise. Patients should also have access to counseling for their psychosocial, nutritional, and other needs. Patients should be offered the opportunity to participate in relevant clinical trials, and should have access to innovative therapies, which may improve their disease outcome.

Right for Informed Consent and Choice of Treatment

Patients should be empowered to participate in decision making, and the health care team should

respect those decisions. Any unauthorized touching of a person is battery, even in the medical setting. The patient's consent allows the physician to provide care.

Consent may be either expressed or implied. Expressed consent most often occurs in the hospital setting, where written or oral consent is given for a particular procedure. In many medical encounters, when the patient presents to a physician for evaluation and care, consent can be presumed. The underlying condition and treatment options are explained to the patient and treatment is rendered or refused. In medical emergencies, consent to treatment that is necessary to maintain life or restore health is usually implied unless it is known that the patient would refuse the intervention.

The doctrine of informed consent goes beyond the question of whether consent was given for a treatment or intervention. Rather, it focuses on the content and process of consent. The physician is required to provide enough information to allow a patient to make an informed judgment about how to proceed. The physician's presentation should be understandable to the patient, should be unbiased, and should include the physician's recommendation. The patient's or surrogate's concurrence must be free without coercion.

The principle and practice of informed consent rely on patients to ask questions when they are uncertain about the information they receive; to think carefully about their choices; and to be forthright with their physicians about their values, concerns, and reservations about a particular recommendation. Once patients and physicians decide on a course of action, patients should make every reasonable effort to carry out the aspects of care that are in their control or to inform their physicians promptly if it is not possible to do so.

The physician is obligated to ensure that the patient or the surrogate is adequately informed about the nature of the patient's medical condition and the objectives of, alternatives to, possible outcomes of, and risks involved with a proposed treatment.

All adult patients are considered competent to make decisions about medical care unless a court declares them incompetent. In clinical practice, however, physicians and family members usually make decisions without a formal competency hearing in the courts for patients who lack decision-making capacity. This clinical approach can be ethically justified if the physician has carefully determined that the patient is incapable of understanding the nature of the proposed treatment; the alternatives to it; and the risks, benefits, and consequences of it.

Most adult patients can participate in, and thereby share responsibility for their health care. Physicians cannot properly diagnose and treat conditions without full disclosure of patients' personal and family medical history, habits, ongoing treatments (medical and otherwise), and symptoms. The physician's obligation to confidentiality exists in part to ensure that patients can be candid without fear of loss of privacy. Physicians must try to create an environment in which honesty can thrive and patient concerns and questions are elicited. Patients should have access to a second opinion and the ability to choose among different treatments and providers.

Right for Access to Information and Clear Disclosure

Patients should receive adequate information about their illness, possible interventions, and the known benefits and risks of specific treatment options. They should have the ability to ascertain names, roles, and the qualifications of those who are treating them.

To make health care decisions and work intelligently in partnership with the physician, the patient must be well informed. Effective patient–physician communication can dispel uncertainty and fear and can enhance healing and patient satisfaction. Information should be disclosed whenever it is considered material to the patient's understanding of his or her situation, possible treatments, and probable outcomes. This information often includes the costs and burdens of treatment, the experience of the proposed clinician, the nature of the illness, and potential treatments.

Even if it is sometimes uncomfortable to clinician or patient, information that is essential to the

patient must be disclosed. How, when, and to whom information is disclosed are important concerns that must be addressed.

Information should be given in terms that the patient can understand. The physician should be sensitive to the patient's responses in setting the pace of disclosure, particularly if the illness is very serious. Disclosure should never be a mechanical or perfunctory process. Upsetting news and information should be presented to the patient in a way that minimizes distress. If the patient is unable to comprehend his or her condition, it should be fully disclosed to an appropriate surrogate. In addition, physicians should disclose to patients information about procedural or judgment errors made in the course of care if such information is material to the patient's well-being.

Right for Confidentiality

Confidentiality is a fundamental tenet of medical care. It is a matter of respecting the privacy of patients, encouraging them to seek medical care and discuss their problems candidly, and preventing discrimination on the basis of their medical conditions. The physician must not release information without the patient's consent (often termed a "privileged communication"). However, confidentiality, like other ethical duties, is not absolute. It may have to be overridden to protect individual persons or the public – for example, to warn sexual partners that a patient has syphilis or is infected with HIV – or to disclose information when the law requires it. Before breaching confidentiality, the physician should make every effort to discuss the issues with the patient. If breaching confidentiality is necessary, it should be done in a way that minimizes harm to the patient and that regards the applicable country law.

Confidentiality is increasingly difficult to maintain in this era of computerized record keeping and electronic data processing, faxing of patient information, third-party payment for medical services, and sharing of patient care among numerous medical professionals and institutions. Physicians should be aware of the increased risk for invasion of patients' privacy and should help ensure confidentiality. Within their own institutions, physicians should advocate policies and procedures to secure the confidentiality of patient records.

Discussion of the problems of an identified patient by professional staff in public places (e.g., in elevators or in cafeterias) violates confidentiality and is unethical. Outside of an educational setting, discussions of a potentially identifiable patient in front of persons who are not involved in that patient's care are unwise and impair the public's confidence in the medical profession. Physicians of patients who are well known to the public should remember that they are not free to discuss or disclose information about a patient's health without the explicit consent of the patient.

In the care of the adolescent patient, family support is important. However, this support must be balanced with confidentiality and respect for the adolescent's autonomy in health care decisions and in relationships with health care providers. Physicians should be knowledgeable about state laws governing the right of adolescent patients to confidentiality and the adolescent's legal right to consent to treatment.

Occasionally, the physician receives information from a patient's friends or relatives and is asked to withhold the source of that information from the patient. The physician is not obliged to keep such secrets from the patient. The informant should be urged to address the patient directly and to encourage the patient to discuss the information with the physician. The physician should use sensitivity and judgment in deciding whether to use the information and whether to reveal its source to the patient. The physician should always act in the best interests of the patient. Confidentiality should be respected during the life and even after death, unless authorization of the patient.

Right for Access to Medical Records

Patients should be permitted to review their medical records and obtain copies for free or for a reasonable fee. Health care providers should be available to explain the contents of medical records to patients.

Medical records and other patient-specific information, including genetic information,

should be regarded as private except to the extent that they are required to be shared for treatment or payment purposes. If access to patient-specific information is necessary for research, patients should be given the opportunity to agree.

Rights of Nonconscious or Incapacitated Patient

When a patient lacks decision-making capacity (i.e., the ability to receive and express information and to make a choice consonant with that information and one's values), an appropriate surrogate should make decisions with the physician.

Ideally, surrogate decision makers should know the patient's preferences and act in the best interests of the patient. If the patient has designated a proxy, as through a durable power of attorney for health care, that choice should be respected. When patients have not selected surrogates, standard clinical practice is that family members serve as surrogates. Some states designate the order in which family members will serve as surrogates.

Physicians should be aware of legal requirements in their state for surrogate appointment and decision making. In some cases, all parties may agree that a close friend is a more appropriate surrogate than a relative.

Physicians should take reasonable care to ensure that the surrogate's decisions are consistent with the patient's preferences and best interests. When possible, these decisions should be reached in the medical setting by physicians, surrogates, and other caregivers.

Physicians should emphasize to surrogates that decisions should be based on what the patient would want, not what surrogates would choose for themselves. If disagreements cannot be resolved, hospital ethics committees may be helpful. Courts should be used when doing so serves the patient, such as to establish guardianship for an incompetent patient, to resolve a problem when other processes fail, or to comply with the country law if it exists.

Right for Information, Health Education, and Prevention

Individuals should be advised with respect to the prevention of diseases (communicable and noncommunicable diseases, cancer, etc.) and should be provided any preventive interventions that are evidence based and available. This can be performed, for example, under the services/programs of health prevention, health protection, screening, and early detection.

Right for Human Dignity

Patients should be treated with dignity at all times (Daher 2010). Quality care in cancer and other debilitating and chronic diseases requires pain, palliative, and supportive care, including the use of opioid analgesics and other symptoms management, for conditions induced by the treatment or by the disease itself (Brennan 2007; European Guidelines for Cancer Patients' Rights).

During the whole process, the health professionals should respect the autonomy of the patient and help him to overcome fear, pain, and suffering and with control of pain and other symptoms in order to provide the best quality of life possible (Daher 2010; Daher and Hajjar 2014). At all times the patient will be asked for informed consent when decisions on the use of life-sustaining treatment are needed (Erer et al. 2008).

Cancer survivors should be provided a comprehensive care summary and follow-up plan at the completion of primary therapy. They also should be systematically monitored for the long-term and late effects of treatment and eventual rehabilitation.

Right for Spiritual Assistance

Religious belief and the teachings of various faith communities are relevant to medical care. Religion offers powerful perspectives on suffering, loss, and death. The majority of people (especially in communities like ours in Middle Eastern countries) profess some form of religious belief. Also, many persons from different cultures are deeply committed to their religious traditions.

Experience reveals the value of religious belief in times of sickness and death. Religious counselors and chaplains have an important role to play in health care. Nevertheless, many physicians respect the tenets of their own religion and allow them to influence their practice of medicine.

Ethical Dimension of Patients' Rights

The traditional physician–patient relationship can be summarized as follows: Sick patients ask physicians to help them get better and physicians profess to be morally committed and technically competent to help the sick. In order to honor this contract, attention to ethical issues in clinical medicine has increased in recent years as a result of profound changes both in medicine and society. Among the many factors causing the increased prominence of ethics in medicine are unprecedented growths in scientific knowledge, expansion in the availability and efficacy of medical technologies, a more equal relationship between patients and physicians, new organizational arrangements in the provision of services, and increased pressure to contain spiraling costs.

It is essential that patients' rights and clinical ethics move in the same direction. The principles of ethics currently embraced by the medical profession include the following: (a) autonomy refers to respect for a person's self-determination, alluding to a patient's wishes regarding treatment choice; (b) beneficence means doing good to patients; (c) fidelity emphasizes faithfulness to a physician's duties and obligations; (d) justice dictates that a physician's decision on patient treatment is made fairly and impartially; and (e) utility implies that a physician's actions should yield good results, achieving maximum benefits for the patient without wasting resources (Pellegrino 1978; Siegler et al. 1990).

Conclusion

Patients' rights as a field of study have gained attention in the recent years worldwide, especially in developed communities. This is not the case in many developing or less developed countries. The literature on human rights and patients' rights in these communities should be addressed.

For adequate fulfillment of patients' rights, much work is required. There are many challenges, including, but not limited to: (a) flexible and creative public policies, (b) greater awareness of patients and their family about their rights and duties, (c) ongoing advocacy, (d) comprehensive education, (e) professional leadership, and (f) continued emphasis on compassion for this vulnerable group of people.

Among the practical recommendations that one can suggest are the following: (1) ensure constitutional endorsement of the right to health, (2) specify government obligations in social welfare, (3) provide health care services with emphasis on vulnerable groups, (4) incorporate a rights-based approach in national policies, (5) collect statistics with monitoring, and (6) report regularly on progressive realization of the right to health and create legal instruments for enforcement (Gruskin 2006; Office of the High Commissioner of the Human Rights 2008; WHO n.d.).

Patients have the right to be treated and dealt with in a humane and respectful manner. Health care providers are urged to pay careful attention to this vital aspect of clinical management; they must keep the welfare and the rights of the patient above any other consideration.

Cross-References

- ▶ Applied Ethics
- ▶ Assisted Suicide
- ▶ Autonomy
- ▶ Bioethics: Medical
- ▶ Clinical Ethics: Teaching
- ▶ Confidentiality
- ▶ Conflict of Interest
- ▶ Consent: Informed
- ▶ Human Dignity
- ▶ Medicine and Ethics
- ▶ Palliative Care
- ▶ Patient Rights
- ▶ Quality of Life
- ▶ Research: Human Subjects
- ▶ Resuscitation (incl. DNR)
- ▶ Truth Telling

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Pediatrics

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Abstract

This entry considers pediatric bioethics in a global context. Like any analysis of a global nature, many of the questions in pediatric bioethics globally – e.g., what constitutes legitimate agency, authority, decision-making, the role of family, and truth-telling – reflect the fundamental philosophical tension between universality and particularity in social and political theory. One question of global relevance that navigates this tension is the moral and legal status of the child, specifically, how changing conceptions of the child have led to a global normative understanding of the child as a moral agent in his or her own right. One striking feature of this understanding is the wide acceptance of two notions that have fundamentally altered – and arguably universalized – the status of the child: the best interests principle, which defends a view of the child as having interests, and the rights of the child, which recognize the child as rights bearer. While the best interests standard has been challenged for its indeterminacy, it remains relevant on account of the flexibility that renders it applicable to a broad range of cultures. Thus, the indeterminacy of the best interests standard, it is argued, is both a strength and a limitation; and it is the strength of its indeterminacy that has led to its globalizing and localizing appeal.

Keywords

Pediatric bioethics; Best interests; Rights of the child; Developing autonomy; Pediatric decision-making; UN Convention on the Rights of the Child

Introduction

This entry considers pediatric bioethics in a global context. Like any analysis of a global nature, many of the questions in pediatric bioethics globally – e.g., what constitutes legitimate agency, authority, decision-making, the role of family, and truth-telling – reflect the fundamental philosophical tension between universality and particularity in social and political theory, a tension that moves between defending universal norms or standards, *a universal ideal*, and the idea that there are distinct, historically constituted cultures, of equal worth, *a respect for the way of life of different cultural contexts*. These questions resonate universally, yet how they play out in particular communities, with their particular traditions, varies considerably across the world and, thus, whether there are genuine universal norms that animate them is open to question. To consider pediatric bioethics globally, just like any question treated on a global scale, one must tread the difficult path between universalizing (or "globalizing") and relativizing (or "localizing") principles.

One question of global relevance that navigates this tension is the moral and legal status of the child, specifically, how changing conceptions of the child have led to a global normative understanding of the child as a moral agent in his or her own right, a universal ideal that, in actual fact, takes on many different flavors in different cultural settings. One striking feature of this global normative understanding is that there is wide acceptance of two notions that have fundamentally altered – and arguably universalized – the status of the child: the best interests principle, which defends a view of the child as having interests, and the rights of the child, which recognize the child as rights bearer. Yet while these notions are firmly entrenched in international conventions and law, thereby implying universality, their implementation is inextricably tied to each cultural context in which they are invoked. Best interests of the child have been described as the prevailing standard in pediatric decision-making (Kopelman 2010); and rights of the child have been steadily "elevated to the level of *jus cogens*" (i.e., fundamental, overriding principles from which derogation is not permitted) in international law (Strasburger 2013). Accordingly, an exploration of these concepts and their relationship to one another provides a window into pediatric bioethics globally through the lens of changing conceptions of the moral and legal status of the child.

This entry examines the historical reconceptualization of the child and how it has taken on universalizing features through the foundational notions of (1) best interests and (2) rights of the child. It then reviews the literature that brings them into dialogue by reflecting on the epistemological tensions between them. Despite the powerful appeal of rights, or what Kymlicka describes as a global "rights consciousness," the best interests principle has remained a central threshold and is in fact a primary consideration in the United Nations Convention on the Rights of the Child (UNCRC), the most widely accepted source of the rights of the child. The grounds on which the best interests standard has been most challenged, its indeterminacy, are also the grounds that carry the most potential for utility on a global level – its flexibility that renders it applicable to a broad range of cultures. Thus, the indeterminacy of the best interests standard, it is argued, is both a strength and a limitation; and it is the strength of its indeterminacy that has led to its globalizing and localizing appeal.

While this entry resists proposing an analysis that claims to derive from universalist principles of bioethics, there is no question that what has emerged as fundamental elements of pediatric bioethics globally privileges a liberal account of the child and of family dominant in Western thought and practices. Critiques of the best interests or rights-based discourses raise important questions for legitimate competing claims. As such, these concepts are not without controversy. Legal rights have been challenged as being insufficient to transform the structured inequalities that shape global health, and a more substantive equality that takes account of social determinants of health is required to effect meaningful change. Similarly, the best interests standard has been challenged on various grounds as being insufficient to guide decision-making. While both

concepts have been critiqued for being inherently based on individualistic notions of agency and rationality, their importance cannot be underestimated. They inform the landscape of pediatrics globally. They have had profound implications for how we conceptualize the role of the child in health-care decision-making as a veritable actor, through the recognition of the child as a unique moral agent with rights.

Pediatric Bioethics

While there is no universally agreed-upon definition of pediatric bioethics, for the purposes of this entry, pediatric bioethics or the ethics of pediatric health care will refer to bringing the perspectives of all relevant parties and interdisciplinary considerations to addressing ethical issues in the health care of children. Relevant parties can include the child himself or herself, the child's surrogate, or decision-makers, the health-care providers, and those taking on additional spiritual, advocacy, legal, or organizational roles. Interdisciplinary considerations can be analyses grounded in science, ethics, law, and where relevant the family's religion or cultural norms.

Pediatric bioethics is grounded in an understanding of bioethics that recognizes (1) the centrality of the role of parents (or surrogates) acting in the best interests of the incapable child and (2) the developing autonomy of the pediatric patient recognized as a distinct moral agent with rights.

Traditionally, parents and physicians made health-care decisions pertaining to children without regard for the child's views or preferences. Key to pediatric bioethics is the ways in which the formal status and the role of the child have evolved along three conceptual developments:

1. The paradigm shift from the child as property to the child as an object of protection that paved the way for the emergence of the best interests standard
2. The rise of autonomy which changed the nature of medical decision-making that leads to new ways of thinking about the developing autonomy of children and the role of children in decision-making
3. The emergence of the child as rights bearer and the institution of basic rights for children as articulated in the United Nations Convention on the Rights of the Child (UNCRC) (United Nations General Assembly 1989) that redefined the child as an object of protection to an autonomous moral agent with rights

Best Interests Standard

From the Child as Property to Interests of the Child

Historically, children were the property of men, bound to the economic, legal, and physical power of the father, who had the authority to make every decision for his children and choose how to treat them. Hobbes (1640) may have said it most starkly:

> Children . . . are in the most absolute subjection to [their parents], that so bringeth them up, preserves them. And they may alienate them, that is assign his or her domination, by selling or giving them in adoption or servitude to others; or may pawn them for hostages, kill them for rebellion, or sacrifice them for peace, by the law of nature, when he or she, in his or her conscience, think that to be necessary.

The Industrial Revolution fundamentally challenged the view of the child as property in favor of a view of the child as having interests, in large part as a result of the separation of work and home that led to new conceptions of family and children. Children, and the notion of childhood, became central to the creation and flourishing of family life, and, as they began losing "the economic value they had held" (Dolgin 1996) in agrarian economies, their economic role shifted from one of participation in production to one of consumption. In this dynamic, children's interests became central in custody and parentage determinations, "displacing the once unyielding principle that always gave fathers custody of their children . . . [and] as fathers no longer had an economic incentive to retain custody of their children" (Dolgin 1996), mothers dominated the home and gained rights to custody.

The view of children as property, and of absolute paternal rights, gave way to the emergence of a new standard in child custody law that increasingly recognized the interests of the child. Parents no longer had the absolute authority to neglect, abuse, or endanger their children and were compelled to make decisions that served the interests of their children. As Kopelman explains: "The best interests standard was introduced to undercut policies that children (and other incompetent persons) were property of their guardians and to give children's interests *some* weight" (2010). As such, the best interests standard was constructed in jurisprudence as a standard to provide for and protect children (Alston 1994; Dolgin 1996; Strasburger 2013). It gave children, and persons lacking capacity, legal rights to a minimal threshold of care.

Today, the best interests standard is entrenched in some form, and has been elevated to a principle, in many national and international legal systems and policy frameworks (e.g., African Charter on the Rights and Welfare of the Child (1990), European Convention on Human Rights (2010), UN High Commission for Refugees, *on action for refugee children* (2008)), and it has important analogues in many cultural, religious, or other traditions (Alston 1994). As Alston observes, "perhaps the strongest evidence of the extent to which the principle has gained general acceptance is the frequency with which it is used in legal analyses at the international level in general" (1994).

Predominant Standard

In bioethics discourse, the best interests standard appears to be the predominant standard of pediatric decision-making (Kopelman 2010; Salter 2012, 2014). Salter (2012) gives many examples where best interests are invoked in a pediatric context: the American Medical Association, in an opinion on pediatric decision-making (2006); the UK Mental Capacity Act (2005); and foundational bioethics and clinical ethics texts (see further reading Beauchamp and Childress 2008 and Jonsen et al. 2006). Appealing to the child's best interests, however, does not provide uncontroversial rules or frameworks for determining what a child's interests are, by whose standards, and to which standard of interests those choosing for the child should be held (Baines 2008). While it is frequently referred to in the medical, legal, and bioethics literature, the concept does not have a universally accepted definition (Kopelman 2010), and it is often presumed that those making health-care decisions for a child in a health-care context understand what it means to act in a child's best interests (Salter 2012). Thus, appealing to the child's best interests does not provide uncontroversial rules or frameworks for determining what a child's interests are, by whose standards, and to which standard of interests those choosing for the child should be held (Baines 2008).

In law, for example, different interpretations of what constitutes best interests have been invoked to support conflicting outcomes or diametrically opposed conclusions (Salter 2012), especially through the lens of different cultural and legal interpretations. As a case in point, assumptions that have underpinned the application of the best interests standard have represented shifting social understandings of children and of the parent–child relationship (Dolgin 1996). In custody litigation, for example, assumptions that have directed applications of the best interests standard have been those that have favored "maternal custody," custody with the "primary caretaker," the "psychological parent," the parent of the same gender as the child, the parent preferred by the child, and both parents jointly (Dolgin 1996). In the health-care context, best interests have emphasized physical, emotional, and social interests. It has reflected among other values a commitment to life, quality of life, relationships, cultural identity, and respect of capable wishes.

Requiring What Is Ideal Versus What Is Reasonable

In bioethics scholarship, there are, according to Salter's analysis (Salter 2012), six key interpretations of best interests whose "critical differences turn on the types of interests emphasized and prioritized" (Salter 2012), that is, what weight is accorded to self- and other-regarding interests.

A commonly invoked interpretation in bioethics scholarship calls on those making

determinations of best interests to aim for what Kopelman (2010) describes as ideal: "The surrogate must try to determine the net benefits to the patient of each option, after assigning weights reflecting the relative importance of various interests affected when subtracting the "cost" from the "benefits" for each option" to achieve the "greatest net benefit" (Buchanan and Brock 1989). As Salter explains: "This characterization is similar to other common interpretations of the standard in that it allows only certain interests into the decision-making calculus: the best interests determination should focus on the individual's "self-regarding" interests" (Salter 2012; citing Buchanan and Brock 1989). In this (ideal) view, the best interests standard is understood as an objective standard of decision-making that does not account for the preferences of the individual or other interests tied to the child (Salter 2014) (e.g., the interests of other family members or of the family as a unit).

Opponents to the view of the best interests standard as requiring what is ideal (i.e., as an objective standard of decision-making) argue that the concept is too individualistic and thus "descriptively inaccurate" (Salter 2014). Like the dominant view of bioethics, according to Salter, it embodies a liberal individualistic view of the child; that is, the interests of the child and the child himself or herself are understood as independent from the interests of "intimate, related others on behalf of their child's best interests." Yet, Salter further argues, "we can't be individualistic in pediatric decision-making because the basic structure of the decision-making standard won't allow it" (Salter 2014). As such, understanding pediatric decision-making as guided by the best interests standard is both ontologically false – for children are necessarily dependent, encumbered, and in relationship – and normatively dangerous (for it promotes the idea that the ideal moral agent is fully independent) (Salter 2014).

Kopelman agrees that the view of the best interests standard as requiring what is ideal is too individualistic. But, she argues, "it is a mistake to conclude that it is excessively individualistic and excludes all other people's interests other than the needs of the patient. This standard would be incoherent and self-defeating if it required the best for everyone" (Kopelman 2010). Viewing the best interests standard as always requiring what is ideal necessarily calls for what is at times impossible and "is divorced" (2010) from the historical context in which it arose and how it ought to be used in practice. She states, "when we consider how this standard is used to solve practical problems, however, it does not require what is ideal but what is reasonable." To that end, she proposes a relational view of the best interests standard, that of requiring what is reasonable, defined by three conditions, to render it a viable decision-making standard: (1) the prima facie duty of decision-makers to maximize the person's overall or long-term benefits and minimize burdens, (2) the condition to meet a minimum threshold of acceptable care, and (3) the condition to make choices compatible with established moral and legal duties to persons lacking decision-making capacity.

Even on the view that the best interests standard requires what is reasonable, and not necessarily what is ideal, many have argued that the child's *best* interests are today often understood in terms of "the child's equality and liberty interests" (Cherry 2010), a view itself underpinned by the idea that the rational goal when making decisions involving children is to develop a child's independence or his or her capacity for autonomous self-determination. The catalyst for this view was the recognition of the developing autonomy of the child and his or her increased role and responsibility in making decisions about his or her care.

The Developing Autonomy of the Child

Two key changes have contributed to the involvement of children, and how children are involved, in health-care decision-making (Kenny et al. 2008) in contexts where autonomy is thought to be essential: (1) The recognition that children have evolving, and dramatically differing levels of, decisional maturity or capacity along a continuum that ranges from those that lack decisional capacity, those with developing decisional capacity, and those with developed capacity. (2) Support for the three categories of decisional capacity and the different levels of inclusion of children in

treatment decisions became the norm: from the requirement of parental consent for those lacking decisional capacity (usually infants) to expectations of parental consent, child assent for those with developing capacity, and the child's consent for those with developed capacity (usually adolescents).

Kenny et al. explain that "assent refers to an agreement with a decision or course of action as distinct from consent, which refers to an informed and voluntary choice with respect to a decision or course of action." They continue by explaining that "assent was said to demonstrate respect for the child's developing autonomy, and parental consent/permission was said to protect the child from assuming unreasonable risks" (2008). The question of assent, e.g., what constitutes assent and when are conditions to gain assent satisfied, has been both further advanced and challenged on theoretical and practical grounds. Significantly, however, the view that medical decision-making involving children moves along a trajectory from assent to consent, according to an understanding of the developing capacity of the child, has become an important part of the fabric of the bioethics of pediatric decision-making.

The view of the child as an active participant in making decisions about his or her care was furthered by the conceptualization of the child as rights bearer which effectively reconceptualized the status of the child and expectations around health-care decision-making for and involving children.

Rights of the Child

The Child as Rights Bearer

A multifaceted and international discourse on children's rights (Freeman 1993) evolved into the recognition that children have inherent rights, thereby setting the stage for their articulation in the United Nations Convention on the Rights of the Child (UNCRC). Not coincidentally, the UNCRC was ratified in the International Year of the Child (1989), creating international rights of the child and providing principles guiding States' laws and practices involving children.

The modern view that the child bears rights galvanized the reconceptualization of the child as an autonomous and self-determining agent capable of choice, a view firmly grounded in the dominant liberal view that every individual is an autonomous rational agent who has the authority and ability to make appropriate choices for his or her own sake. Mama summarizes both the philosophical view of the child as an autonomous self-possessed moral agent bearing rights and how that view is embodied in the UNCRC (Mama 2009):

1. Accept that children are people now – not people in the making. This accords all children full human status from birth.
2. Accept that childhood is valuable in its own right and is not simply a stage toward adulthood.
3. Accept that children can be active agents of their own lives.
4. Accept that children should not be treated less seriously just because of their age.
5. Address the special vulnerability of infants and children.

In short, the child is not an adult-in-the-making, or a becoming, but a being with inherent moral worth who has the right, and the evolving capacity, to make conscious life choices.

Central to the UNCRC is that the best interests of the child remain "a primary consideration" (United Nations General Assembly, 1989, Art 3(1)) in what is arguably the "most detailed and comprehensive international human rights instrument (in terms of the rights recognized, as opposed to the categories of persons covered)" (Alston 1994). While many international conventions accord rights to children (e.g., International Convention on the Elimination of All Forms of Racial Discrimination, Convention on the Rights of Persons with Disabilities), we focus on the UNCRC because it is "perhaps one of the most influential documents to ascribe to a best-interests doctrine" (Salter 2012), a notion central to the reconceptualization of the status and role of the child and pediatric bioethics.

The Best Interests Principle Within the UNCRC

The best interests principle is one of the four general principles of the UNCRC, alongside protection, provision, and participation. As such, while the UNCRC was designed to create international rights for children, it affords *both* rights to and interests for children (Strasburger 2013). Art 3(1) of the UNCRC stipulates that "In all actions concerning children, whether undertaken by public or private social welfare institutions, courts of law, administrative authorities or legislative bodies, the best interests of the child shall be a primary consideration." It mandates three kinds of action: "one proposing the principle, one charging the State with enforcing the principle with respect to the child's guardians, and one charging the State with enforcing the principle with respect to State actions" (Strasburger 2013).

Significantly, the best interests principle appears to serve three, qualitatively different, functions in the UNCRC, as (1) a means to "support, justify or clarify a particular approach to issues arising under the Convention ... as an element which needs to be taken fully into account in implementing other rights; (2) a mediating principle to resolve conflict arising between different rights in the Convention (Alston 1994); and (3) a basis for evaluating State laws and practices not governed by the rights of the Convention" (Alston 1994).

Reinforcing its centrality, the concept of best interests appears in several other sections of the Convention: in relation to the separation of the child from the family setting (Art 9), with reference to parental responsibility for the upbringing and development of the child (Art 18), in relation to adoption and comparable practices (Arts 20 and 21), and in the context of the child's involvement with the police and justice system (Arts 37 and 40) (Alston 1994).

Many have observed, and criticized, that the UNCRC does not explicitly define what constitutes, or what factors determine, or what weight should be given to the principle of the best interests of the child. This has continued to fuel the underlying debate surrounding the indeterminacy of the principle. What is made clear, however, is that within the UNCRC framework, it is not parents who are accountable for ensuring that the best interests of the child are considered when making decisions involving children; the obligation to ensure that the best interests of the child are served when in the care of his or her parents is upon the State, who is required to "determine if an act of the parents ... is in the best interest of the child" (Strasburger 2013). This may undermine, too detrimentally Cherry (2010) posits, the authority of parents, a view that bears significant impact on pediatric decision-making.

On the other hand, and what is plausibly in contradiction, the UNCRC clearly affirms that family responsibilities and duties, and the rights of parents to raise their children, should be respected. This raises important questions central to both pediatric bioethics and the application of the UNCRC about the role of culture, what constitutes a family, what authority parents have over their children, who is in authority, and what or whether there are limits to that authority (Iltis 2010).

Pediatric Bioethical Decision-Making

Significance of Children's Rights for Pediatric Decision-Making

Questions arising from the UNCRC about the role and authority of parents and of the family have been raised in the context of the scholarship on pediatric decision-making (Cherry 2010; Engelhardt 2010; Iltis 2010). These caution against adopting the framework of the UNCRC because it does not adequately give weight to the central role of family (Cherry 2010), or articulate foundational premises of what constitutes the family (Engelhardt 2010), or specify normative criteria to inform decisions about what constitutes the best interests of the child (Iltis 2010).

One such view is that the UNCRC undermines the traditional (moral and social) role of the authority of parents in pediatric bioethical decision-making (Cherry 2010). Specifically, it reduces the role of parents "to acting as custodians of the best interests of children" because the child necessarily holds rights "independently of their parents and are enforceable by children over

against their parents through the medium of the state" (Cherry 2010). Echoing a common view (e.g., Iltis 2010; Salter 2014), Cherry reproaches the UNCRC for being too deeply individualistic because it creates rights that emphasize and promote a view of the child's best interests in terms of "the child's equality and liberty interests" (Cherry 2010). In so doing, it articulates a vision that "perhaps inadvertently, drives a legal and moral" gulf between parents and their own children (Cherry 2010). Such a vision, argues Cherry, does not support the conditions necessary for the family to adequately function, and act in the child's best interests, as a moral community. Thus, by presupposing the primacy of human rights, and recasting the child as outside the sphere of the moral authority of his or her family, such a vision acts in tension with many pediatric bioethical decision-making frameworks largely determined by "family-oriented moral and political commitments," shaped within particular religious and cultural understandings (Cherry 2010) that may or may not recognize rights as having legitimacy.

Engelhardt argues that the conflict between parental authority and the aspirations of the UNCRC extends beyond the issue that it gives priority to the child's equality and liberty rights-based interests. It requires a questioning of whether the priority given to rights is in the child's best interests *in the first instance*, a construct posited without justification in the UNCRC. According to Engelhardt, the recasting of the scope and character of parental authority, "by not recognizing parents as an authority over their children, save as trustees of their children's best interests" (Engelhardt 2010), fundamentally alters many conceptions of the nature of family. It embodies a "post-traditional vision" that seeks "to anchor social institutions, including the family, in concerns for liberty and equality, as well as in terms of a libertarian focus on the emerging autonomy of the child" (Engelhardt 2010), a vision that is fundamentally at odds with other understandings of the moral significance of the family. But more than that, it does not recognize that there are foundationally incompatible views of the moral status of the family in most contemporary societies that are "at the heart of culture wars [that] have substantive implications for the bioethics of pediatric decision-making" (Engelhardt 2010). Engelhardt identifies four accounts of "the moral and ontological standing of the family that underlie conflicting views of the child's status" (Engelhardt 2010) and the role of family in pediatric decision-making. Each account – sociobiological, categorical, liberal social constructivist, and libertarian social constructivist – refers to

> a different appreciation of familial integrity, sovereignty, and authority, as well as of the nature of parental authority, the relationship between men and women in the founding of families, the status of children (. . .) the proper relation of children to parental authority and therefore the place of children in pediatric decision-making. (Engelhardt 2010)

These views of the nature of family are grounded in "robustly developed and conflicting worldviews supported by disparate basic premises about the nature of morality" (Engelhardt 2010) that are irreconcilable and intractable. Any attempt, therefore, to forge a global consensus on the status of the child or the moral significance of the family and its bearing on pediatric bioethical decision-making is rendered impossible.

The Indeterminacy of the Best Interests Standard

Alston offers a different view: The UNCRC does not mandate a particular vision or purport to reflect international convergence on the status of the child. He observes that it

> is sometimes presented (or, more accurately, misrepresented) as being a uni-dimensional document that reflects a single, unified philosophy of children's rights and contains a specific and readily ascertainable recipe for resolving the inevitable tensions and conflicts that arise in a given situation among the different rights recognized. Similarly the respective entitlements of the different actors involved, included the child, the parents, the family, the extended family, the local community, the society and the State are assumed to be clearly defined and delineated . . . In fact, the Convention is far more complex and multi-dimensional than any characterizations would imply. (1994)

He emphasizes the extent to which the actual content of the UNCRC, in particular the best interests principle, is inextricably tied to "the cultural context in which it is invoked" (Alston 1994). Indeed, to reach convergence, and perhaps as a kind of built-in safeguard against universalizing how children ought to be treated, a duty was placed on signatory States to respect local customary practices, further bolstered by the preamble that takes "due account of the importance of the traditions and cultural values of each people for the protection and harmonious development of the child." This is an important consideration. While some have interpreted the acknowledgment of the role of culture in the development of a child as an implicit acknowledgment that "the child's welfare may well be 'trumped' at times by 'cultural values and traditions'" (Freeman 1993), a broader view has been to examine the assertion of culture in the context of the UNCRC's emphasis on the importance of family to raising, and how one views the proper way to raise, children.

A still broader conceptual view has been to examine the assertion of culture in the UNCRC in the context of the broader relationship between culture and human rights (Alston 1994). In Alston's view, the best interests principle as reflected in the UNCRC acts as a window on the relationship between culture and human rights; it provides insights on how it may be possible to bridge the schism between culture and human rights and respond to criticisms that have been leveled at the "universalist aspirations of human rights law" (Alston 1994). Notably, he observes that the indeterminacy of children's rights norms, indeed a fundamental characteristic of *all* human rights norms, is not necessarily a limitation. For any arrangement involving "the relationship between the state, the family and the child, must be characterized by a significant degree of flexibility and adaptability." He explains: "If human rights norms in general can be said to be inherently indeterminate, the best interests principle is located by most of its critics at the most indeterminate outer margins even of that body of norms" (Alston 1994). In fact, the drafters of the UNCRC "seem to have been unaware of the controversy over the principle and thus felt no need to defend its open-endedness or to propose elements which might inject some particular content to it" (Alston 1994). But this indeterminacy plays a positive role: "it serves to emphasize the importance of both institutions as a means through which to develop a better understanding of the different cultural dimensions of the relevant norms" (Alston 1994).

No doubt, any application of the best interests standard raises difficult questions about what constitutes a child's "interests" and what counts as "best." This question looms large in pediatric bioethics. Many have argued that the standard is rendered indeterminate on a practical level because it is too ambiguous to provide an objective basis for decision-making (Dolgin 1996). Indeed, it does not, as Iltis (2010) argues, articulate fundamental moral assumptions or "establish who is in authority to determine the fundamental moral assumptions that will inform the principles and norms that govern decisions." Yet, according to Dolgin (1996), the indeterminacy of the best interests standard is precisely why it has survived and flourished. She explains:

> Only because the principle is so broad has it been able to serve this end. In this regard, its limitations for the law (its indeterminacy) has been its benefit for the law (its adaptability) . . . The principle, precisely because of its indeterminacy, has been flexible enough to justify shifting social preferences and to reflect changing visions of the family without requiring an express alteration in the rule of law.

In effect, the strength of the philosophical and practical appeal of the best interests standard has rendered it foundational in law and health-care decisions involving children despite, or perhaps because of, its indeterminacy, which may reasonably be viewed as both a limitation and a strength allowing for flexibility in (cultural) interpretation and application. As Alston ponders: "While it is most commonly argued in liberal societies that the child's best interests are obviously best served by policies that preserve or promote individual autonomy, the bests interests of the child in more traditional societies might be interpreted as requiring the sublimation of the individual child's preferences and views to the interests of the family (and extended family)" (Alston 1994). Similarly,

Solnit (2004) emphasizes that what he calls the bioethics of children's rights is necessarily embedded in the laws and policies of a society and in the values, customs, and practices of the culture that shape them. In short, while the best interests standard may be widely invoked, it is open to different interpretations in different settings.

But there is an important caveat. The best interests principle as reflected in the UNCRC is both informed and constrained by the rights (Strasburger 2013), meaning that it is an important consideration – "an umbrella provision" (Alston 1994) – when implementing the rights, yet it cannot violate a right. In this way, there is what might be considered a reasonableness standard, which is to say that there are vitally important and inviolable standards, and while there are legitimate competing claims, not everything is equally defensible. Changing conceptions of the moral and legal status of the child have led to a global normative understanding of the child as a moral agent in his or her own right, an understanding that both underpins and permeates medical, legal, and policy decisions involving children, globally.

Disharmony between available international frameworks and worldviews opposed to these can raise moral and legal tensions. In the context of pediatric health care, work in global pediatric bioethics is a step in the right direction. Its values of acting in a child's best interests, respecting relevant voices and engaging diverse perspectives through the tools of multiple disciplines, seem to be a formula warranting cautious optimism.

Conclusion

The goal of this entry was to consider pediatric bioethics within a global context. The value of doing so is to appreciate the relevance of pediatric bioethics around the globe as well as locally. The health care of children commonly involves contexts where patients, families, and health-care providers come to the health-care encounter with a range of differing and consistent values. Varied interpretations of concepts as seemingly supported as best interests should be expected in most pediatric health-care contexts. Values foundational to pediatric bioethics may be defined or resonate differently around the world. The pediatric bioethics goal of sustained moral peace in the health care of children – generated through the perspectives of relevant parties and interdisciplinary considerations – seems consistent with best interests and child rights frameworks. Consideration of the concept of best interests, in relation to international frameworks for child rights, provides a window into the nascent field of pediatric global bioethics.

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Persistent Vegetative State

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Abstract

Persistent vegetative state (PVS) is a condition in which a person with severe brain damage is described as being or, more precisely, seeming to be wakeful without awareness. This condition gives rise to many complex ethical and juridical questions, from the moral status of people in this condition and the dignity of their lives to the questions about the possibility of withholding or withdrawing life-supporting medical treatments. People in this condition are regarded as alive and as holding the same rights accorded to every other human being. However, since these people are incompetent, all decisions need to be taken on their behalf. This aspect exemplifies the relational dimension of the question, which involves not only clinicians but also families, people close to the person, and the community as a whole. As for decisions regarding treatments and other wishes, advance directives, although not the same as informed consent expressed in real time, are considered as an important means to reveal one's wishes in case one might develop this condition and to nominate a surrogate (or proxy) to take decisions on one's behalf. However, no advance directives were presented in the most important legal cases involving PVS patients around the world; thus, the decision process proved to be extremely difficult. The three main criteria applied in these cases were, firstly, of the substituted judgment, which depends upon the decision of the family or a close person on the basis of the personality of the person in question; secondly, the standard of clear and convincing evidence based on the wishes of the patient expressed while competent; and thirdly, the best interest of the patient. Uncertainties of diagnosis and prognosis of PVS give rise to doubts in addition to uncertainties regarding the methodologies of inferring wishes never expressed by the person. There are also opponents to the withdrawing of life support on principle. The Indian Aruna case highlights also the problem of avoiding the misuse of legal authority to take decisions on behalf of an incompetent person. Since PVS is, typically, a gray area, every solution is destined to be accompanied by a certain degree of doubt.

Keywords

Advance directives; Best interest; Conscientious objection; Informed consent; Patient's wishes;

Right to life; Right to die; Surrogate decision-making; Withholding/withdrawing life support

Introduction

The definition of a vegetative state (VS) is still a vexing question not only because of the current level of knowledge in the neurological field, which is still growing, but also because the definition of VS involves other controversial concepts like consciousness and awareness. Moreover, uncertainty regarding the exact diagnosis of this condition and prognosis adds more complexity to the issue. This uncertainty explains also the use of the adjective "persistent" instead of "permanent," because there is no consensus on the possibility of affirming the permanency of the VS in the future. The expression "vegetative state" per se might prove to be ambiguous for laymen, because evoking plant and not human life (nor even animal life) implicitly suggests an element of evaluation about the quality of life. According to the Oxford English dictionary, "vegetative" describes "an organic body capable of growth and development but devoid of sensations and thoughts." Therefore, some, for example, the Australian National Health & Medical Research Council Information Paper, proposed in 2003 to substitute the expression with more neutral phrases like "post-coma unresponsiveness." Similarly, the European Task Force on Disorders of Consciousness chose, in 2010, the expression "unresponsive wakefulness syndrome (US)."

"Unresponsive" means that these patients only show reflex movements without response to commands. "Wakefulness" relates to the presence of eye opening – spontaneous or stimulation induced – never observed in a coma. "Syndrome" stresses that a series of clinical signs are assessed (in line with the Australian NHMRC).

Moreover, considering the uncertainty of the prognosis, some propose avoiding adjectives like persistent or permanent and using only expressions like VS for a specified number of months or years.

The term VS was first documented in 1972 when two neurologists reported a clinical description (criteria for definition) of this syndrome (Jennet and Plum 1972). This condition is described essentially as a neurological condition characterized by wakefulness without awareness. Wakefulness and awareness are considered by neurologists as the two main functions of consciousness. The distinguishing characteristic of persistent vegetative state (PVS) is that the brain stem remains alive and functioning, while the cortex has lost its function and activity.

In 1994, the consensus statement of the Multi-Society Task Force summarized current knowledge of the medical aspects of PVS in adults and children. According to this statement, a VS is a clinical condition of complete unawareness of self and the environment, accompanied by sleep-wake cycles, with either complete or partial preservation of hypothalamic and brain stem autonomic functions (wakefulness).

In other words, wakefulness without awareness means that patients in a VS show no evidence of sustained, reproducible, purposeful, or voluntary behavioral responses to visual, auditory, tactile, or noxious stimuli, show no evidence of language comprehension or expression, have bowel and bladder incontinence, and have variably preserved cranial nerve and spinal reflexes. A person in a VS may open their eyes, wake up, and fall asleep at regular intervals and have basic reflexes, such as blinking when they are startled by a loud noise or withdrawing their hand when it is squeezed hard. They are also able to regulate their heartbeat and breathing without assistance, and they may moan or make other sounds especially when tight muscles are stretched. They may cry or smile or make other facial expressions without apparent cause. They may react to a loud sound with a startle.

Recent advances in this field have led to the preference for the expression of "no evidence of awareness" instead of "absence of awareness" as cited in the above mentioned consensus statement. In fact, the absence of awareness has not been proved yet. As it is defined, consciousness – the state of awareness of self and the environment – is an inference that observers make from a person's behavior. Moreover, thanks to modern techniques, like

neuroimaging and electroencephalography-guided transcranial stimulation, evidence of some residual cognitive processing has been reported (Monti et al. 2010).

PVS state is defined as a VS present for a specific period of time following an acute traumatic or nontraumatic brain injury or lasting for at least a certain period in patients with degenerative or metabolic disorders or developmental malformations.

According to some authors, the diagnosis of PVS must be kept distinct from its outcome.

The research evidence is rather limited and difficult to interpret. In general, however, experience suggests that the likelihood of functional improvement for people in a state of PVS is limited, and the longer the person remains in PVS, the lower the chance of improvement. The probability of significant improvement is less for people whose PVS is of nontraumatic origin. There is little data available to indicate whether different types of care or intervention make a difference to prognosis.

At the same time, it is not unheard of for people who have been in PVS for some years to emerge from a severely incapacitated state to a state of some functioning where, although still dependent, they are able to participate actively in their own lives and with others.

Three categories of disorder can cause such a state: acute traumatic and nontraumatic brain injuries, degenerative and metabolic brain disorders, and severe congenital malformations of the nervous system.

A VS is different to a coma which is a condition characterized by lack of both awareness and wakefulness. Usually, people in a VS are people who were in a coma, and their condition evolved into a VS.

Patients in a coma may also evolve toward a different condition called minimal conscience state (MCS). People in MCS prove to be not only wakeful, but also they reveal some signs of awareness, although this may be inconsistent. Furthermore, people in MCS are more likely to recover even after years of being in this condition. In addition to the difficulties in diagnosing PVS, the difficulty in distinguishing it from MCS has been widely documented. In particular, it is very difficult to be certain that a person who has sleep-wake cycles is not responding at all, and it is not possible to be certain that the person is unaware or what level of awareness he or she might experience. In MCS, there may be a delay between a stimulus (for instance, touch, light, movement, voice) and any response, and responses may be ambiguous and difficult to identify.

Another different condition is the so-called locked-in syndrome. People in this condition are aware and wakeful, but they do not have the capacity to show their awareness, because of their incapability of most voluntary movements. However, most of them can move their eyes and communicate through the voluntary movement of opening and closing their eyes.

Although the PVS implies severe brain damage, it does not necessarily correspond to the conditions of cerebral death which equates to complete and irreversible loss of brain function (including involuntary activity to sustain life).

Thus, according to the medical criteria on cortical death, a patient in a VS, in fact, is alive. Evidently, the person needs to be cared for because of his or her state of dependency. Most of the time, they receive only nursing care: to have their airway suctioned regularly to prevent secretions from blocking it, to be turned in bed to avoid bed sores, to be fed and hydrated, and to have attention paid to the excretion tubes. Medical needs may arise from serious infections that require workup and extensive antibiotic treatment. Thus, when PVS is stabilized, there are doubts about the definition of the person as a "patient" on the basis that the person has no "disease" to be cured. Consequently, some propose describing the condition as a severe form of disability and the person as a disabled.

PVS is more common in developed countries where health-care systems are more sophisticated, and patients can be treated with resuscitation techniques that preserve life especially after car accidents or similar.

Ethical and Juridical Questions

The life condition of patients in PVS gives rise to many ethical and juridical questions.

The First Conceptual Framework Regards Issues of Legal Death, Dignity of Life, and Moral Status

(a) Can this form of life be considered life regardless of how the current statutes define death?
(b) Considering the extremely poor quality of life without any apparent relationship with the external environment, the dignity of life in this condition is put in doubt.
(c) Furthermore, for the same reason, it can be hypothesized whether a patient in this state can be regarded as a person or not.

As for the issue regarding the determination of death, there is no general consensus that a patient in this condition is regarded as dead (Winkler 1988). Moreover, the question seems too abstract because it is inconceivable to bury people who have a beating heart and are still breathing. More debatable is the question whether, although their body is alive, they can no longer be considered as a person (Brody 1988). In fact, this can open the door to withdrawing life-sustaining care like nutrition and hydration in order to induce death or allow death to happen naturally.

The position that excludes the definition of a person is linked to the more general opinion that views self-awareness and consciousness as criteria for personhood and consequently refutes attributing "personhood" to fetuses of up to 90 days and to people born with a malfunctioning cerebral cortex, in addition to patients in PVS. However, it is not so easy to determine the measure of lack of self-awareness and consciousness required to exclude the description of an individual as a person. Moreover, the definition of a person based on the criteria of self-awareness may prove to be insufficient in distinguishing a human being from a machine like a computer that may be able to achieve self-consciousness (Goldberg 1991). Thus, it has been proposed to substitute this definition with another based on the capacity for social interaction. Unfortunately, every attempt in this direction seems to be characterized by a materialistic approach which claims to be able to evaluate conscience or capacity for social interaction on an empirical basis or methodology (Shea and Bayne 2010). When and on what basis one should ascribe awareness of others are two of the most vexing questions in the philosophy of mind and in the science of consciousness. Moreover, as mentioned above, a series of recent experiments has put in doubt the assumption of the lack of consciousness of these patients. Ultimately, this is a slippery issue which may result in discrimination against people on the basis of their physical condition. It contradicts the tendency in the history of human rights toward the broadest concept of man, without even considering the problem of the legitimacy of establishing criteria that leads to such discrimination. Ultimately, denying "personhood" seems to be like a shortcut to discriminating against people when it is impossible to say that they are dead.

The acknowledgment of personhood independently from the state of the cerebral cortex and the degree of self-awareness and consciousness does not imply a unique solution, that is, the support of the patient till death is determined according to the common criteria. In fact, while there are opinions in favor of providing care and necessary medical support, there are different opinions which question the concept of dignity because of the poor quality of life, and thus, the door is opened to uncertainty concerning decisions about ending life in this situation. Often reference to the artificial and natural dyad is made since PVS condition is a result of technological progress which in these cases is able to prevent people from dying, but not sufficiently advanced to be able to make significant improvements to their condition.

The Second Framework Refers to the Practical Issues About How to Treat People in a PVS

There are some general principles that are commonly taken into consideration once individuals in PVS are recognized as both persons and indeed alive. A person in PVS must be respected without ignoring any aspects of human dignity that may be implied. The principle of respect for a person is expressly provided for, for instance, in the Italian constitution, mentioned in the Aruna case and strongly underlined in the ethical guidelines adopted by the Australian government. Respect for a person certainly means respect for his or her autonomy, but the former does not correspond

necessarily with the latter especially if autonomy is not interpreted within a framework of solidarity. Otherwise, respect would slide into indifference. This is particularly true when the person is incompetent, and at most, only his or her living will is available for reference. When people are not capable, respect should be the focus of special attention in order to ensure appropriate care is administered to the person, who should be treated as a human being since he or she has worth per se.

This means that he or she should be respected with regard to welfare, rights, and, when ascertainable, beliefs, convictions, perceptions, customs, and cultural heritage. As for rights, they can be ordered into two categories, rights of the persons as individual entities on one hand and rights of individuals as social beings on the other. Included in the category of individual entities are the right to life, liberty, and security; the right to have their religious and cultural identities respected; the right to refuse or have medical treatment withdrawn, including by advance care planning; the right to access health care that is appropriate to their needs; the right to privacy and confidentiality; and the right to approach death peacefully. As for individuals as social beings, rights to maintain relationships with the family and the remaining social environment that the person used to engage with before succumbing to the condition of PVS are of great importance. These latter rights imply practical consequences like easing visitor access to hospitals when the admission of person in PVS to hospital is necessary and promoting the administration of care and treatment at home when possible. Particularly, the family of the person in PVS or people who take care of him or her should not be ignored not only because of the close relationships in play but also because they represent an integral component of the care given to the person. Therefore, they should be provided with accurate and timely information which is fundamental for ensuring the provision of appropriate assistance and necessary if they are to be involved in decisions that affect the person in question. They need also not only to be informed of the person's real situation and future prospects but also receive instruction on the typology and methodology of providing care and assistance, when necessary, in addition to advice on how to deal with their own responses to particular situations.

Experience suggests that the presence of the family, if informed and supported, can prove to be of great help. Some observations of the state of the patient which could be useful for a more accurate diagnosis or any slight improvements reported by families are not always detected or acknowledged by clinicians. This apparent contradiction may in some cases be explained by the difference between the time devoted to the person by the family and the time devoted by clinicians. For instance, the role of the family is reported as being useful in making the difficult distinction between PVS and MCS, given that over an extended period of observation, any delayed responses to stimulus, typical in MCS, are more likely to be detected.

In general for people in PVS, decisions on treatment and care are made on the basis of prognoses that are, at best, imprecise. Therefore, especially at the beginning, these decisions should necessarily be provisional. Life expectancy is difficult to predict and may range from months to decades. The person's condition is not static, sometimes worsening, sometimes gradually improving, since the person over time may emerge to a state of MCS or better.

Because these changes in the person's condition may be excruciatingly slow (over years), continuing review is crucial in the long term. Also some increased functioning may be reported when the person learns to adapt better to long-term disability over time.

Whatever the decision may be, time is a critical factor. Accurate diagnosis often takes many weeks or longer. Similarly, the prognosis becomes clearer only over many weeks or months. Thus, periodic clinical reassessment is essential. This factor and the related problems regarding diagnosis and prognosis should be taken into account when the most important decisions are being faced.

Since the time that the first cases were brought to courts, the most controversial practical issue has regarded the possibility of withdrawing

some forms of care, especially antibiotics, nutrition, hydration, and respiration. Regarding nutrition and hydration, a common vexing question is whether artificially supplied fluid and nutrition (AFN) is a medical treatment or rather a form of mandatory basic care, given that nutrition and hydration generally are not means for combating a disease, but only to provide daily needs as for a normal person. According to leading opinion, they are regarded as medical treatments, considering that the treatment needs some medical skill and sometimes patients with PEG tubes experience complications in the long term due to the tube. Anyway, considering AFN as a medical treatment, the question is as usual whether the treatment can be judged as disproportionate or unreasonably burdensome. In fact, when a treatment is disproportionate, it can be discontinued or simply not administered by physicians. This issue can be solved by referring to either objective or subjective criteria. It is not so easy to select the criteria to solve the question because weighing up the advantages and disadvantages is controversial. For instance, the improvement in quality of life as a possible advantage raises questions about the very meaning of quality of life. Following the subjective criteria, that is, letting the person subjected to, or might be subjected to, the treatment decide means fundamentally that the issue about proportionality is of less importance or of none at all, and what mostly counts is the choice of the person whatever it might be based on. A midpoint between objective and subjective criteria could be that doctors in their weighing up of the pros and cons should take into account the personal views of the person in question expressed while competent.

The issue about withdrawing care is tightly linked with the question of exactly who is entitled to decide if and when to withdraw those types of care.

From the point of view of the inherent conflicting values of these cases, an appeal to rights in both meanings, substantive negative rights, such as the right not to be killed, and substantive positive rights, such as the right to be aided in life-threatening situations, is one of the most critical. These rights correspond to duties and especially, the duty not to kill is of particular significance in every society, so important that it is disputable whether the right not to be killed may be considered as a waivable right like rights relating to economic interests. These cases are often called "right-to-die" cases, although this phrase is a misnomer for two reasons at least. From the legal perspective, the issue in these cases is whether or not to disconnect artificial means, like the feeding tube or the ventilator that keeps the person alive, in order to let them die. The doctors are requested to withdraw the support, not to take action like administering a lethal medicine in order to directly cause death. In fact, what is at issue here is the possibility of extending the well-established right to refuse or withdraw medical treatments to incompetent patients which is based on rights to bodily integrity and freedom from unwanted touching. From a more general perspective, as was said, everyone is going to die sometime, and there is nothing that one can do to prevent an ultimate death. Thus, the only possibility might be to die sooner than one would without any action being directly implemented to cause death. Moreover, the right to die would imply the duty of someone to cause one's death. Therefore, making the distinction between refusal and withdrawal of medical treatment is strongly supported. The US Supreme Court stated that on one hand "when a patient refuses life-sustaining medical treatment, he dies from an underlying fatal disease or pathology; but if a patient ingests lethal medication prescribed by a physician, he is killed by that medication." On the other hand, the same court stated that a ban on assisted suicide is related to legitimate government interest such as, "prohibiting intentional killing and preserving life; preventing suicide; maintaining physicians' role as their patients' healers; protecting vulnerable people from indifference, prejudice, and psychological and financial pressure to end their lives; and avoiding a possible slide towards euthanasia."

Consequently, the issue is better categorized as one regarding procedural rights (the body of law that prescribes formal steps to be taken in enforcing legal rights) such as the right of an incompetent patient to refuse or withdraw health

care. This right could be exercised, according to the American law system, also by a surrogate or proxy (a person who is trusted by someone to make health decisions on his or her behalf if the patient is unable to do so). This surrogate decision-maker can be nominated by the person in question in his or her advance directives (a legal document in which one spells out his or her decisions about end-of-life care, declares which treatments he or she wants in case of persistent unconsciousness, and expresses also other wishes regarding, for instance, organ or tissue donation) which may provide also some instructions. Otherwise, a surrogate has to be appointed by a court complying with certain requirements. When a surrogate is nominated by the person in the advance directives or, in his/her absence, appointed by the court, the problem of what the duties and the power of a surrogate decision-maker are arises (Buchanan and Brock 1989). The values to be taken into consideration are autonomy, integrity of the person, respect of his or her person, and best interest of the person. In Western countries especially under the influence of American culture, a great emphasis is placed on the principle of self-determination or autonomy. From this point of view, a crucial question is that of ascertaining the real wishes of the person especially when there are no advance directives or, even if existing, a particular treatment or set of circumstances are not specifically provided for (Mappes 2003). In fact, although the advance directives should not be confused with a real and present informed consent or refusal of a competent person who is experiencing the particular condition of illness at that moment, they, nevertheless, prove that the person took the issue into serious consideration and tried to determine what would be his or her wishes in that hypothetical situation. Other casual statements or simply the behavior of the person should not be equated with advance directives. Thus, because of the very delicate nature of the matter, there is a particular need for clear and convincing evidence of the wish to not receive life-support treatment or to have it removed after a period of time in PVS. In fact, the principle, at least formally accepted, is that in case of doubt, the decision must be in favor of life. However, in some cases, the de facto principle, *in dubio pro morte*, is followed given that permission to withdraw life-sustaining treatment is granted once a patient is declared permanently vegetative and the desires of the patient are hypothetically constructed on the basis of lifestyle, religious beliefs, and other indications from which one could infer the personality of the person when he or she was capable. This method can lead only to probable wishes that are not established in function of the individual in question, but instead with reference to categories of people who are thought more likely to opt for one particular solution rather than another. Moreover, significantly, these surrogate decisions are usually restricted to cases of PVS, given the extraordinarily poor quality of life, and denied to severely mentally and physically disabled people who are dependent on artificial nutrition and hydration, but, however, judged as not being unconscious.

There is a difference, which is not always well recognized, between self-determination, that means that a person chooses what he wants for his or her body, and the integrity of a person's character which is perceived fundamentally by his or her environment and which is determined by his or her lifestyle, his or her expressed or apparent beliefs, and so on. The concept of self-determination, or autonomy, implies that the focus should be on a statement, documents, or other forms of expression of the will of the person in question. The concept of integrity implies that people should act in line with their beliefs and lifestyle. Consequently, decisions on whether to administer or not medical treatments in the absence of an expressed will should be taken by a surrogate drawing on his or her understanding and interpretation of the personality of the person as was apparent during the time when the person was competent.

According to a renowned opinion (Dworkin 1993), a decision based on personal integrity is related to the principle of the best interest of the patient, but even if this correlation is valid, the application of this principle is premised on only one point of view without taking into consideration any other criteria. The best interest of the patient is established by applying the absolute

idea that the best interest is to live according to that individual's lifestyle till the end of life even if the person in question has never seriously contemplated the notion of being or not being treated in case of PVS and has never decided what his or her preference would be. This idea that the risk of not satisfying the presumed desire for end of life as a result of an erroneous interpretation of the patient's true will is worse than the risk of ending life by underestimating the chances of recovery is indeed a radical one. Keeping a person alive in PVS contrary to his or her probable wishes is explicitly considered as a cause of great damage. The opposing opinion inverts the rationale sustaining that while mistakenly continuing life, contrary to the wishes of the person, may cause a damage that can be remedied, erroneously ending life would indeed be irreparable. This point is clearly similar to the common argument against capital punishment.

Furthermore, reference to the integrity of a person is impossible in some cases, and doubts arise regarding the requisites for applying this criterion in many others. When the decision regards children of young age, the criterion is not applicable, and the question about how old the person should be in order to establish his/her lifestyle is delicate and controversial.

The concept of the standard of the best interest, which comprehends various aspects, is the criterion applied by the English House of Lords in the case of Anthony Bland and also by the Indian Supreme Court in the Aruna case. Substituted judgment is not favored in English law in relation to incompetent adults. Consequently, the criterion is the patient's best interest which represents an application of the principle of beneficence. The doctors have to follow a course of action because it is in the best interests of the patient and is not influenced by personal convictions, motives, or other considerations. In the process of ascertaining the patient's best interest, standard medical practice is of overriding importance, and the opinion of the people close to the patient should also be taken into consideration, although this opinion cannot be considered as the determining factor in the final decision. Even though the result might be practically the same, from a theoretical perspective, the principle of the best interest of the patient implies an assumption of responsibility by the physicians who take the final decision on the basis of many factors, not only with reference to the will of the incapable patient that in most cases is unobtainable and never updated to the immediate present. On the other hand, the criterion of best interest, which is rather generic, even more so than other general principles, requires a minimum of consensus concerning the goals of medicine; otherwise, in the current culture of subjectivism, it might be interpreted in a too varied or even inconsistent way. In line with English jurisprudence, if the justification, in principle, for treating a patient who lacks the capacity to consent lies in the fact that the treatment is provided in his best interests, it must follow that the treatment may be discontinued where it is indeed no longer in his best interests. However, in such cases, an element of particularity emerges in the parallel between providing treatment and discontinuing it, because the obvious consequence of discontinuing life support is death. The main opinion is currently in favor of the crucial distinction between withdrawing life-sustaining treatments and euthanasia, that is, between cases in which a doctor decides to either cease or to continue to administer treatment or care for his patient which could or might prolong his life and those in which he decides, for example, by administering a lethal drug, to actively bring his patient's life to an end. This distinction probably implies that the idea of natural death in its modern understanding is still relevant in this matter. In fact, since administering treatments could be considered as interfering with the natural course of events, withdrawing a treatment could be judged as a way of reestablishing the natural course of events and thus desisting from any attempt to help the patient with treatments that have proved to be useless. On the contrary, within the context of the practice of medicine, actively causing the patient's death means provoking an indisputably artificial death by means of a professional act that should in fact be guided by the opposite purpose traditionally inspired by the Hippocratic Oath, which is to heal and save or prolong life. The distinction between

natural and artificial lies at the heart of the opinion which motivates refusal to life-sustaining treatments on the grounds that an artificial life made possible only, or mostly, by medical treatments would be unacceptable. This distinction is often linked with the concept of dignity of life and that of dying with dignity.

The opposite opinion criticizes the idea of excluding dignity in some conditions of life, making reference to the principle of equality that excludes any discrimination and recognizes the dignity of individuals no matter their condition (Keown 1993). From this perspective, the idea that dignity of death depends on the fact that (the anticipation of) death is chosen or not is objected to. This criticism is particularly pervasive in the opinion based on the principle of the sanctity of life. In line with this principle that is consistent not only with religious beliefs, life is the most important value at stake, and thus, withdrawing life support is judged as immoral and also an action to be legally banned. For pro-life supporters, human life cannot be reduced to a subjectivist value, because the value of human life is of universal importance, and the cases in which causing death is condoned are strictly exceptional. Moreover, they underline the danger of a mistake being made over the reason for ending life (patient's desires, erroneous diagnosis or prognosis, irreversibility of PVS) that can cause irreparable damage, while the opposite mistake is indisputably of less importance. Uncertainties inherent in PVS regarding the current state of knowledge of this condition strengthen this point. Moreover, they often appeal to the danger of abandoning vulnerable people and favor a sort of euthanastic culture which leads people in a vulnerable position to feel like a burden on the family and the whole of society and therefore to think that it is immoral to continue their life in such a state of dependency. The consequence of the neutrality of law about end-of-life choices is judged to be questioning the dignity of life in PVS itself, rendering more difficult the choice for life over time. The concept of moral neutrality of law per se is disputed, because the law necessarily chooses options. Even if the law allows everyone to make their own choice, the law implicitly opts in favor of certain ethical principles, in this case in favor of a social indifference toward the dignity of life which is conceived as solely the result of individual preferences. If something is regarded as of value only because it is chosen, it cannot be considered a true value, and this conclusion is of great importance when the protection of life and dignity of people are at stake. On this point, the influence of the social environment on an individual's choices and the effect of these on the social sphere should also be taken into account.

Moreover, when taking into consideration the actual difficulty in diagnosing VS with certainty and in making a prognosis and the need for accurate treatment of a patient in a comatose state followed by an appropriate rehabilitation, the culture in favor of a quick solution reduces automatically the chances of improving medical knowledge in this field.

In particular, when physicians are obliged to implement decisions made on the basis of nonmedical criteria, conscientious objection may play a role in order to protect a physician's freedom of conscience and avoid a slide toward "hetero-determination" in medical practice. Physicians should not become mere executioners if one wishes to preserve the professional nature of medical practice.

Finally, if care is provided, another issue pertaining PVS concerns the costs of the care, who should pay for it, and in a system with limited resources whether it is preferable to direct efforts and money to other types of medical needs. This can be described as an appeal to the consequences for the patient, for their families, for the providers, and for society as a whole.

Appeals to justice and cost-effectiveness have to be taken into consideration, the former in order to prevent any form of mistreatment of the vulnerable, the latter to avoid waste of energy and money or simply to redirect these resources to other more deserving people in a system with limited quantities of these resources. This problem is particularly important in countries where there is public support for health care and assistance for severely disabled individuals.

Because of their vulnerability, people in PVS need in particular to be protected against abuse, neglect, exploitation, or discrimination.

Issues Arising in Famous Legal Cases

The bioethical issues like the bio-juridical ones have risen in parallel with the cases brought to courts. These issues have prompted heated discussion not only among bioethicists and bio-jurists but also in the public arena. Initially, it should be noted that the cases are necessarily characterized by the lack of a real adversarial nature of the judicial process precisely because of the incapacity of the person in PVS. From a global perspective, the first cases were brought to court in the USA and the UK and ignited worldwide debate. An example not only of the global nature of the debate but also of the influence of American cases is a case decided in Italy (1991–2007), regarding a daughter, lying in a PVS for 17 years, when after a long trial, the father was granted permission to have the doctors terminate AFN treatment. The last case reported below is from India and probably the most famous outside the Western countries and is thus meaningful from a global perspective, although one should not understate the tight links between Indian and English culture which might have played a role. The court's decision highlights and summarizes the most important ethical and legal issues from a worldwide perspective, including the UK Bland case and the most important American cases.

The first renowned case that mentions the phrase "persistent vegetative state" seems to be the Quinlan case which was brought to the New Jersey Chancery Court in 1975 and, in 1976, decided by the New Jersey Supreme Court. This is also one of the first judicial opinions in the USA to mention a living will.

Karen Ann Quinlan was 21 years old when she was found by her friends in a respiratory crises and unconsciousness. She was taken to a hospital where she was diagnosed as being in a comatose state. After some months, she was diagnosed as being in a PVS. The PVS was not said to be properly irreversible by the doctor who treated her, although an evolution of this type was judged unlikely.

One hundred and seven days after her first day in the hospital, the parents asked the doctor to discontinue the ventilator. Their decision was partly motivated by their Roman Catholic belief, explicitly denying they were in favor of any form of euthanasia. They only wanted to make real a presumed desire of their daughter not to be kept alive by artificial means. The doctor refused, because "medical tradition" required continuing life support. At trial at the New Jersey Chancery Court, Karen's father sought a declaratory judgment that he, after being appointed guardian of the daughter, could order her ventilator to be discontinued. Karen's physicians, the hospital, and the county prosecutor and Karen's guardian ad litem disagreed with the discontinuation of ventilation. The trial court found more grounds in the position in favor of the continuance of life support. The trial court also thought that the daughter's prior statements made by Karen when competent as to her distaste for continuance of life by extraordinary medical procedures were remote and impersonal and thus lacking significative probative weight.

The Supreme Court focused on Miss Quinlan's poor condition and prognosis stating that "no external compelling interest of the State could compel Karen to endure the unendurable, only to vegetate a few measurable months with no realistic possibility of returning to any semblance of cognitive or sapient life." They added that they perceived "no thread of logic distinguishing between such a choice on Karen's part and a similar choice which, under the evidence in this case, could be made by a competent patient terminally ill, riddled by cancer and suffering great pain."

The Supreme Court accepted the parent's argument that Karen's right of privacy included the right to decline or withdraw medical treatment: "[p]presumably [the right of privacy] is broad enough to encompass a patient's decision to decline medical treatment under certain circumstances, in much the same way as it is broad enough to encompass a woman's decision to terminate pregnancy under certain conditions."

Moreover, the Court stated that "the State's interest Contra weakens and the individual's right to privacy grows as the degree of bodily invasion increases and the prognosis dims. Ultimately there comes a point at which the individual's right overcomes the State interest."

However, once this constitutional right is recognized, this does not imply that the personal exercise of it can be equated to the exercise of it by a guardian on behalf of the person in question. This right is personal in nature, and the vicarious exercise raises problems because of the uniquely personal relation of this right to the individual himself. This problem was recognized by the court that argued thus: "If a putative decision by Karen to permit this non-cognitive, vegetative existence to terminate by natural forces is regarded as a valuable incident of her right to privacy. . .then it should not be discarded solely on the basis that her condition prevents her conscious exercise of the choice. The only practical way to prevent destruction of the right is to permit the guardian and family of Karen to render their best judgement . . .as to whether she would exercise it in these circumstances." On this basis, the Supreme Court decided to appoint Miss Quinlan's father as guardian of the person of Karen Quinlan with full power to make decisions with regard to the identity of her treating physicians. The court also made clear that the decision of withdrawing life support should be taken, provided that attending physicians conclude that there would be no reasonable possibility of Karen's ever emerging from her present comatose condition to a cognitive, sapient state and provided that the hospital ethics committee or similar body of the institution in which Karen was then hospitalized would agree about the poor prognosis. Miss Quinlan died at the age of 31 years, 9 years after the removal of the respirator, due to pneumonia as a result of the respiratory congestion that had been mounting for several months before death.

Differently from the Quinlan case, the Cruzan case was brought not only to the Supreme Court of the state where the person lived (Missouri) but also to the US Supreme Court, thus highlighting the orientation of the jurisprudence of the most important body of justice in the USA.

The US Supreme Court confirmed the possibility of withdrawing medical life support of a person in PVS, but on different grounds to the Quinlan case. The Supreme Court of New Jersey, in the Quinlan case, argued on the grounds of privacy and viewed the family as the surrogate decision-maker without relying on prior statements made by the person when she was competent and living under very different set of circumstances. In contrast, the US Supreme Court, referring to the Cruzan case, stated that "The Due Process Clause does not require a State to accept the 'substituted judgement' of a close family member in the absence of substantial proof that their views reflect the patient's." Moreover, the US Supreme Court stated that the right of a competent person to refuse life-saving hydration and nutrition "does not mean that an incompetent person should possess the same right, since such a person is unable to make an informed and voluntary choice to exercise that hypothetical right or any other right." In other words, the Cruzan case focuses on a standard of clear and convincing evidence in order "to assure that the surrogate's action conforms as best it may to the wishes expressed by the patient while competent."

From a certain perspective, one might think that the Cruzan case shifts the basis of the possibility of withdrawing life support from the substitute judgment based on the privacy of family to the principle of self-determination.

However, it should be noted that one might think that applying the doctrine of Quinlan implies a "transparent fiction." The court renounced attributing ultimate value to remote and abstract statements and deferred to the judgment of the family, the closest social environment to the person. The family was thought to be in the best position to evaluate the person's lifestyle and hypothesize what the choice of the person might have been if he or she had been competent.

In the Cruzan case, the court focused on the problem of potential abuse of substituted judgment and the fact that a judicial proceeding, in order to determine the wishes of an incompetent person, may very well not be adversarial in nature, therefore lacking the added guarantee of accurate fact finding that this type of process brings with

it. Consequently, the court stressed the importance of clear and convincing evidence. However, what was called a "transparent fiction" might transpire to be a nontransparent fiction. The proof of the existence of an unexpressed will, unlike a precise disposition in a living will, is very difficult to establish, and very often, lifestyle, as described by relatives and friends, becomes the ultimate criterion on which a hypothetical will is established.

The Schiavo case is a good example of this point. At the age of 26 years, Terri Schiavo was diagnosed as being in PVS. Since there were no advance directives (living will), her husband was appointed formal guardian by the court. After a long judicial struggle with Terri's parents, the husband eventually succeeded in obtaining a judicial decision in favor of the removal of the PEG tube, and Terri Schiavo died 13 days after the removal of the PEG tube. The decision was motivated by referral to some casual statements Terri made to her husband and other members of his family. Although the guardian ad litem pointed out that these statements did not represent the clear and convincing evidence standard required by Florida State law, in addition to the fact that the husband may have had a significant financial conflict of interest, the husband remained in charge as guardian of Terri Schiavo till the end. Ten months after the death of Terri, the husband married the woman with whom he had been in a relationship for 10 years and had two children.

The case of Anthony Bland, a man lying in PVS as a result of severe injuries suffered in the Hillsborough disaster, was decided by the House of Lords. The result of the case did not differ regarding the practical consequences of allowing withdrawal of certain treatment from the aforementioned American cases, but it is different in terms of the grounds of the sentence and to whom authority is conferred in order to make the decision regarding the type of treatment. The English judges preferred to refer to the principle of the patient's best interest, and the ultimate decision, supported by his parents, was taken by the physicians in charge of Anthony Bland. "That question (assuming the continued availability of the system) can only be answered by reference to the best interests of the patient himself, having regard for established medical practice. Indeed, if the justification for treating a patient who lacks the capacity to consent lies in the fact that the treatment is provided in his best interests, it must follow that the treatment may, and indeed ultimately should, be discontinued where it is no longer in his best interests to provide it. The question which lies at the heart of the present case is, as I see it, whether on that principle the doctors responsible for the treatment and care of Anthony Bland can justifiably discontinue the process of artificial feeding upon which the prolongation of his life depends." The Lords also made the distinction between withdrawing life-sustaining treatments and euthanasia, calling this border as the "Rubicon which runs between on the one hand the care of the living patient and on the other hand euthanasia – actively causing his death to avoid or to end his suffering. Euthanasia is not lawful at common law. It is of course well known that there are many responsible members of our society who believe that euthanasia should be made lawful; but that result could, (. . .), only be achieved by legislation which expresses the democratic will that so fundamental a change should be made in our law, and can, if enacted, ensure that such legalised killing can only be carried out subject to appropriate supervision and control."

Not surprisingly for a court of common law, the House of Lords made also reference to the American precedents. More surprisingly, though, in the 1997 sentence of the Italian Corte di Cassazione – the highest court apart from the constitutional court – in relation to the Englaro case, one can find references to the American courts and the principle of law applied by them. It should be noted that Italy is not a country that applies common law, and at the time of the case, there were no legal provisions for advance directives nor principle of law that allowed a guardian to make a substituted judgment in such very personal decisions. Thus, the Italian case can be considered as strong evidence of the global influence of American law. The Italian court held that judges can authorize the removal of medically supplied life-sustaining treatment for patients who have been declared incompetent when two

conditions are met: firstly, it is clear that the patient is in an irreversible PVS, according to international scientific standards, and, secondly, it can be determined by clear and convincing evidence that the patient would not wish to be kept alive through artificial means, based on the patient's personality, lifestyle, and moral convictions, as described by the "representative" of the patient. This case became a long judicial struggle which, like Terri Schiavo's case, divided public and political opinion alike.

Aruna Ramachandra Shanbaug is reported to have been in PVS for 37 years as a result of a terrible assault. The case was brought to court by a person who wrote a book about her and claimed to be her closest friend. In the writ petition, it is alleged that in fact Aruna Shanbaug was already (virtually) dead, and hence, by not feeding her body anymore, "we shall not be killing her." However, according to a team of doctors appointed by the court, she was not only technically alive, but also she apparently had some brain activity, though very little. She met most of the criteria for being declared in a PVS, and the withdrawal of life-support treatments was permissible according to the court, but this decision was not supported by the hospital staff, who had been amazingly caring for her day and night for so many long years (in 37 years, Aruna did not develop a single bed sore), who really were her closest friends ("the whole country must learn the meaning of dedication and sacrifice from the KEM hospital staff"). Since the hospital staff had clearly expressed their wish that Aruna should have been allowed to live, this was the decision to be respected. What the court stated about the need of a judicial check in case of decision to withdraw life-support treatments is also interesting. "In our opinion, if we leave it solely to the patient's relatives or to the doctors or next friend to decide whether to withdraw the life support of an incompetent person there is always a risk in our country that this may be misused by some unscrupulous persons who wish to inherit or otherwise grab the property of the patient." Consequently, "in the case of an incompetent person who is unable to take a decision whether to withdraw life support or not, it is the Court alone, as *parens patriae*, which ultimately must take this decision, though, no doubt, the views of the near relatives, next friend and doctors must be given due weight."

Conclusion

In the field of end-of-life matters, PVS is a gray issue which gives rise to many questions from whichever perspective one considers it. Withholding or withdrawing life support from an incompetent person is, by definition, not consented to by the real and present will of the person in question. Nevertheless, the paradox of a condition resulting from modern techniques that can prevent a person from dying but grant him or her only a life of extremely poor quality creates a dilemma. Moreover, not only advance directives written by the person but also statements of people close to the person that life support would have been refused by that same person if he or she were able to make him or herself understood cannot be easily ignored. While a decision taken by physicians in the best interest of the person might be criticized as paternalistic, a mere execution of a wish formally ascertained might well be criticized as a form of indifference toward the person and sometimes seen as an easy way to discharge society from the burden of the vulnerable suffering from this condition. The Aruna case illustrates this ethical and juridical conundrum. From a global perspective, the special care of Aruna provided by the hospital staff for such a long time is a lesson that extends far beyond the borders of India no matter what one might think the best solution of these types of cases should be.

Cross-References

- ▶ Assisted Suicide
- ▶ Autonomy
- ▶ Compassion
- ▶ Conflict of Interest
- ▶ Conscientious Objection
- ▶ Consent: Informed
- ▶ Disability
- ▶ Freedom of Treatment

- ▶ Intensive Care
- ▶ Lifestyles
- ▶ Moral Status
- ▶ Person
- ▶ Quality of Life
- ▶ Resuscitation (Incl. DNR)
- ▶ Right to Die
- ▶ Right to Health
- ▶ Solidarity
- ▶ Vulnerability

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Person

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Abstract

The term person is usually used to indicate some*one*. This meaning is the result of a complex theoretical legacy, which, in many cases, derives from the use of the term person as *nomen dignitatis*. At the crossroads of theological, philosophical, juridical, and political debate, the concept of person, however, is likely to become equivocal. Specifically, to understand the ethical and anthropological controversy present in bioethics, we have distinguished the analogous concept of person, which can be predicated of several entities, from the univocal concept of human person. In everyday language, the term person already seems to indicate a human being, and therefore, it may seem redundant to write the term human person. In fact, as can be seen from the historical reconstruction, the very distinction

between person and human person allows to identify and clarify the meaning of the current debate. The definition of *human person* extends to every human being at all existential stages: it implies that the human being is always some*one*. On the contrary, the notion of *person* indicates a series of qualities which when attributed to an entity make some*one*. Depending upon usage, the notion of person, in its semantic shifts, can therefore favor maximum inclusion or maximum discrimination between human beings. The debate is therefore open: to clarify the terms is indeed the first condition to promote dialogue and ethical discussion.

Keywords

Human being; Someone; Identity; Dignity

Introduction

The term person has a two-thousand-year history. Originally developed in the theological context, to designate God, it has today become a common notion in the lexicon and culture of the Western world, where it usually serves to indicate the human being. It has acquired particular importance in the language of contemporary bioethics. If, in fact, on the one hand, there is almost unanimous consensus in recognizing the prerogatives of the person to dignity, inviolability, and entitlement to moral and social rights, on the other, there is no agreement on a definition of *who* a person is and what the characteristics are in order to ascribe this recognition. Moreover, in some cases, the notion of person has even been compared to that of being human, to which it had been connected for centuries, with obvious repercussions in terms of ethical reflection and juridical perspective. The tendency to replace the term *human being*, a univocal notion, with the term *person*, an analogous concept, also used at times in an equivocal way threatens to undermine the significance of the same *Universal Declaration of Human Rights* where the two notions, of person and human being, do not indicate the same being, that is they do not carry the same "extension" conceptually. In addition, the debate has been further complicated, with the attribution of person also to those animals believed to possess functions similar to those of human beings. Therefore the notion of person requires clarification regarding its meanings and usage.

History and Development

The history of the notion of person is intertwined with the entire cultural condition of the West. The etymological derivation of the word *persona*, of Latin origin, originally refers back to the ancient Greek word *pròsopon* and its generic meaning of "theatrical mask" and the verb form *per-sonare*, the reverberation of the voice of the actor through the artificial face brought on stage. However, more careful linguistic and historiographical analysis indicates that *pròsopon* also takes on other meanings that are, all the same, related to each other. *Pròsopon* is in actual fact, "what is in front," the front, or apical part of something, therefore in an extended sense, the face.

In Stoicism, *prosòpon* is sometimes used to indicate both the individual and the role or the task played by the human being in the cosmos. The reference to the mask, that is, the function of representation, becomes predominant especially in ancient Roman law that distinguishes the human being (*Homo*) from person (*persona*): it indicates the social status of the concrete human being, determined with reference to *libertas*, *civitas*, and *familia*. In modern times, Thomas Hobbes (1588–1679) defines the State as *persona civilis*, and it is on this artificial construct that he develops a theory of representation and authorization. *Person*, therefore, remains in the legal context, and in political philosophy, a term that serves to "endow" someone or something with power, responsibilities, rights, and duties and ends up merging with the contemporary concept of legal entity. In this perspective, the notion of person has no real anthropological significance.

As far as philosophical lexicon is concerned, the concept of person is related to the genuine semantic turning point accomplished by Christianity that comes to use it to talk about God.

The early years of Christianity are characterized by laborious intellectual labor that responds to the need felt by Christians to clarify the peculiarities of their faith. Christians are baptized in the name of the Father, the Son, and the Holy Spirit; but how may one express this faith in God revealed through Incarnation, and still remain strictly a monotheist? The first author that attempts a doctrinal synthesis is Tertullian (born around 160) who introduces the term *Trinitas* and connects it to *person* in order to clarify the Christian concept of God; with the help of various philosophical sources, He indicates in the Father, the Son, and the Holy Spirit the *Tres Personae* of the one God. For the first time in the history of the word, *persona* is used to speak of God. Tertullian does not offer a proper definition of *person*, a term he uses to indicate the real distinction between the Father, Son, and Holy Spirit, but he is careful to distinguish it from the notion of *substance*. Augustine of Hippo (354–430), in *De Trinitate*, reflects the heritage of Tertullian; however, he does not conceal a degree of dissatisfaction with the use of this notion. For the theologian and philosopher to whom we owe the first substantial elaboration of human subjectivity, *person* refers primarily to the concrete human individual, whose singularity is perceived: it seems to him that an analogous use does not place enough emphasis on the relational unity of Trinitarian life. It is the philosopher and theologian Boethius (475–525) who provides the most influential definition of person that survives to this day, with various modifications. Boethius seeks to find a formula that is able to embrace both man, created in the image and likeness of God, and the Triune God and the angels. Person, according to Boethius, is an individual substance of a rational nature (*rationalis naturae individua substantia*). This definition for Boethius indicates a perfection in the scale of beings: it is therefore *nomen dignitatis*. In actual fact, this much appreciated formulation was bound to provoke much criticism. Indeed, from the theological point of view, the attribution of substantial character to person is to undermine the Christian monotheism once preached of God, just as the anthropological angle failed to convey the reference to the corporeal nature of the human being already decidedly present in the Aristotelian definition of *animal rationalis*. Therefore particularly innovative and interesting is the correction made by Richard of St. Victor (110–1173), which replaces *substantia* with *existentia* in order to strengthen the analogous use of the term *persona*: *rationalis naturae individua existentia*. The change is radical because substance refers to *something*, while existence reinforces the reference to *someone*, to subjectivity. But it is definitely Thomas Aquinas (1225–1274) who introduced the most important revision of the concept of *persona*. Defining person no longer as *individua substantia* but *subsistens distinctum* and assigning it *intellettualis naturae* set the conditions which allow for this term to be ascribed to the Triune God without tarnishing uniqueness and unity. However, the most significant contribution, from a strictly philosophical point of view, comes from Aquinas when he clearly distinguishes the concept of person, which is analogous, from that of human person, which is univocal, indicating the concrete human being in his or her unrepeatable individuality. The human person, according to Thomas Aquinas, is "this flesh these bones this soul which are the individuating principles of a man" [these are aspects Aquinas continues,] "which, though not belonging to 'person' in general, nevertheless do belong to the meaning of a particular human person" (Thomas Aquinas). The Thomistic synthesis leaves no doubt: the human person is the subsistence in itself and for itself of a concrete individual, even if this subsistence is not by itself. The identity and subjectivity of the individual human being is not assured only by the spiritual soul, created directly by God, but from concrete corporeal constitution. The human person is created in the body and is open to the world and to God by reason of his rational nature.

A clear separation of the person from the living body is instead carried out by John Locke, the author who influences liberal bioethics more than any other. Locke, who is involved in the theological dispute on the Trinity, addresses the person issue by self-inquiry on the subject of Resurrection after death and the Last Judgement. His intention is not to define the Trinitarian relationship or determine the relationship between

body and soul but to refer the living body to person perceived as a subject to whom one can attribute and ascribe actions and responsibilities. Locke's definition resembles that of Boethius albeit deprived of its substantive statute. What guarantees the continuity of person is neither existence nor substance nor the living body, but it is conscience. For Locke *persona* "means a thinking intelligent being, that has reason and... consider itself as itself, the same thinking thing, in different times and places" (Locke 1690). In this perspective only those who are conscious and have memory of themselves can be defined as person. If Descartes' I is first and foremost a *res cogitans* placed in relation to a *res extensa*, instead for Locke I is consciousness that inhabits a body of which it is the owner. The individual is thought of as sovereign and owner of his or her own mind, body, and person: this is why, in addition to being able to dispose of it, it must also be accounted for. Even Locke's anthropology is profoundly dualistic because it splits the person as a consciential and moral subject from the living body with which there is a proprietorial relationship, as if it were a thing.

Contemporary debates regarding the relationship between mind and brain are largely influenced by both Cartesian anthropology and from Locke's theory of personal identity. Even for Kant, the concept of person loses every empirical connotation and is used as *nomen dignitatis*: the categorical imperative that requires treatment of oneself and others also as an end and never merely as a means strengthens the separation between the phenomenal I and the empirical I. All reference to substance and existence having fallen, person becomes a fluid notion that is always on the verge of becoming ambiguous. The early twentieth century saw the flourishing of different currents of thought which define themselves as personalistic, from the Communitarianism of Emmanuel Mounier, to the spiritualist thought of Charles Renouvier, J. Lacroix, Maurice Blondel and the dialogical thought of Martin Buber. A special mention must be given to the work of Max Scheler who sees in the person the human being's spiritual and intentional center. The notion of person also finds its own particular relational and social coloring in the philosophy of Paul Ricoeur and Emmanuel Levinas and in the communicative ethics of Karl-Otto Apel and Jurgen Habermas. Lastly, given the very articulate use of the term person, the contribution of those authors who consider it necessary to regain possession of the Aristotelian categories of nature and substance in order to speak of personal identity and among these mention should be made to Peter Frederick Strawson, David Wiggins, Bernard Williams, and Robert Spaemann.

The history of the notion of person cannot therefore today be considered closed: the wealth of contributions and speculative additions found in contemporary philosophy highlight how complex it still is for human beings to define themselves.

Conceptual Clarification/Definition

The term person may be used as a noun, tonight I am expecting to have dinner with a person, or as a predicate, God is person. Its meaning may vary, but what prevents it from becoming ambiguous is the constant reference to "someone." The term person as a noun, in anthropology, has the same extension as the term human being. Someone is never something: the human person is someone corporeal that is generated, develops, is born, falls ill, and eventually dies.

Being someone is a necessary but not sufficient condition for the manifestation of intelligence, will, freedom, and self-consciousness, which are substantial attributes of someone. Therefore, the cognitive and volitional activities of someone are not someone but rather one's temporary manifestations which depend on the stages of development and health of someone. With respect to the topic of consciousness, being someone precedes self-consciousness, because it is its ontological condition. From this point of view, being generated by persons, that is, from a human being, is the simple condition for being persons and to be recognized as such. In this perspective it is evident that the body, which constitutes someone's identity, is the first manifestation of the person. Bodily health has a direct influence not on being someone

but on the means of expression of someone. The practices of treatment and care never have as object a generic human life but someone that can be called by name. Compared to the predominant approach that places the identity of someone in consciousness, this theory takes the view that, in reality, conscience does not establish a subject, just as the loss of conscience does not destroy it. Being someone precedes, as it were, the consciousness that one has of oneself, which may or may not be actuated. Even in healthy adults, the experience of sleeping and waking, the dimension of unconsciousness, shows how someone's subjectivity has a continuity that escapes self-consciousness. The phenomena of mental illness, a limitation of cognitive abilities, their temporary or permanent loss, indicate that someone is always vulnerable in the transparency of the self. In this sense, the Cartesian theory which identifies I in cogito can be declared to be illusory and counter-intuitive. Someone, that is, the person, is not confined to the sum of one's intellectual manifestations. It can therefore be said that in all stages of life, in sickness and in health, *someone* is, as long as one's body is alive. Compared to mentalist theories, an ontological approach to the issue of the human person underlines the fact that the functioning of brain activity is a certain physical condition of the expression of someone, but it does not constitute one as such. Recognizing the human being as a person also implies recognition of his intrinsic dignity, since this word has been coined as *nomen dignitatis*. In an ethical perspective, the term person indicates an ideal to be achieved: it is only when morally good actions are performed that one is a person. There are in fact two ways of understanding dignity. In the first, dignity belongs to the being of the person; in the second, it defines behavior that is the object of praise. While dignity, in the first sense, is not lost nor is it acquired, because it is intrinsic in the second meaning, it varies depending on the actions that a human being carries out. There are also other meanings given to the term according to the dimension of its attribution: person as a moral agent, person as a psychical subject, and person as a legal entity. In the latter case, the term person, as a legal construct, can also be attributed to collective entities. In anthropology, those who use the term as a predicate take the view that no one is in itself and for itself a person, but that one can become and lose the quality of person. According to this usage, there are therefore thresholds of personality that are established or exist through social convention or by reference to cognitive and moral activity. In this perspective, person has no substantial or existential dimension. In fact, this usage introduces discrimination among human beings, by excluding from the category of persons those who do not correspond to certain criteria.

The earliest reference of the term person to the mask and role is still present in reference to theatrical or cinematographic characters: there is usage of the expression "impersonate" in the sense of personify, precisely, to play a part. The difficulty of a single definition arises, however, from the same difficulty in defining the notion of subject in a cultural context that largely rejects the notion of substance understood as "subjectum," what lies beneath, what ensures *duration* – in Bergson's meaning of the term – of he who becomes and transforms over time. The most important thorny point of theory for bioethical reflection is therefore determined by the dual use of the term as a noun and as a predicate. If by person one intends a set of qualities or functions that allow consideration of someone as a subject for rights and duties, it must therefore be concluded that the person *does not exist*, is not born, does not get ill, does not heal, and does not die. Those who are born exist and are, in fact, necessarily living and persist in themselves, in spite of the change in their characteristics. Therefore, the notion of the human person, as also used in common language, implies reference not simply to the psychological or moral dimension but to concrete corporeal existence. Differently from the term human being, the notion of person as a noun indicates the concrete individual here and now, he or she who is "nominated," that is called by name: ultimately someone not entirely communicable. In other words, person expresses a specific mode of existence. The term person as a noun is therefore univocal, while the term as a predicate is analogous: the first lends itself to describing and

defining the human being and the second to indicate certain qualities and functions.

Ethical Dimension

The term *person* has assumed centrality in the contemporary era that substantially leaves aside the theological reasons for which it has entered into the cultural circuit of the West. In fact, today it occupies a prominent position in a large part of philosophical reflection and in bioethics in particular. A point of convergence between the different perspectives is constituted by recognition of the inherent dignity of the person and normative value. In some respects, it can be said that the term person has almost replaced the notion of human being in all of the theoretical contexts dealing with rights and duties. However, this unanimity is broken, and controversies begin when one does not think of the term person as a noun but as an attribute. Having been ascertained that the term *person* and *human being* are not, however, purely synonymous, the debate revolves around this question: are "all" human beings persons? Should the answer be not all of them, then what are the legitimate consequences of this in terms of rights, beginning with the basic right to life? Is the human being the holder of inalienable rights, or is it the person? And if only a *person* has dignity, then does this imply that a human being has none?

The peculiar character of the bioethical debate is that it has polarized the questions around this issue, in the belief that it was enough to answer these questions to solve old and new problems that have arisen from the development of science and technology. It is believed that in order to determine whether abortion, stem cell research, transplants, cloning, interventions on the human genome, euthanasia, and so on are morally legitimate or not, one must move through the issue of the person. The other relevant aspect is that ethical issues are traced back to some sort of first axiological principle, represented by the person. In this sense we can say that the term person is taken as an indicator of value from which all other values are derived. There is, in this usage, a similarity to classical thought and the Christian tradition, with a radical difference that in these contexts the meaning of person was attributed first to God and human beings took part in so far as they were created to "His image and likeness." Contemporary ethics places itself, in the majority of its currents, out of any reference to rational or revealed theology: the dignity attributed to the *person* is therefore linked to the quality or functionality that is manifested and not simply to being a human being. The appreciation of rationality, responsibility, freedom, and relationalism of the *person* is therefore the basis for the recognition of dignity. Consequently, determining whether or not all human beings are persons is of fundamental significance. Some authors, such as Hugo Tristram Engelhardt Jr. and Peter Singer, among the most influential and well-known protagonists of the ethical debate, have resumed analogous use of the concept of person to establish that not all humans are persons: at the same time, asserting that it is not necessary for all human beings to be persons. Engelhardt writes that the archangel Gabriel who appeared to Mohammed in the desert and E.T. that crosses a modern city in this century are examples of entities that are persons but not human beings. The latter hypothesis, however, does not authorize another statement, namely, that all those belonging to the species homo sapiens are "human beings," just as one cannot say that all ETs are not "extraterrestrial persons" and all angels are not "angelic persons"(Engelhardt 1986). If the term person does not coincide with the human being, it does not coincide with any other entity but only with certain functions that can be found in many entities. If the term person indicates cognitive and volitional functions, a certain moral capacity, one cannot ignore the fact that these functions are always someone's functions: they do not exist in themselves and for themselves but in reference to something else. If there are some human beings who have those qualities, it is because the human person is a living corporeality that is born, develops, grows, and dies. To reply to the question whether all human beings are persons, there must be reflection on whether the notion of person indicates functions that belong in themselves and for themselves to someone – and so help to define identity – or are they only

transitory and contingent aspects, which in fact do not contribute to making that entity specifically that actual entity and no other. Is it conceivable that an Archangel, an E.T., and a human being may not also be a person (analogous use)? If so, then it is not decisive to attribute those characteristics to those entities, because they would remain Archangels, extraterrestrials, and human beings even without those qualities. Otherwise, it should be said that wherever those entities exist, those qualities also exist, even when they are not manifested. If the human being is removed from underneath the person mask, what remains is pretense. To conclude: if in origin the human being is not a human person, there can be no possibility of becoming one. The classic categories of actuality and potentiality can clarify this argument. The human being is in fact in actuality a human person, and what is in potentiality that depends on factors such as development, health, and environment is the exercise of reason, freedom, and morality. This thesis can be expressed by saying that by nature, every human being is a human person. By nature is to be understood, in the Aristotelian sense as the set of characteristics that are transmitted through human generation. Persons generate persons: development and the environment will affect the manner of existence of each person, but they can never transform someone into something. It does not arouse any wonder that a human person may even not act or express himself as a person in the ethical and psychological meaning of the term: even an apple tree may fail to blossom or bear fruit, but it still remains an apple tree. The approach which separates the concept of person from that of human being has sources such as Locke, Kant, Mill, as well as Roman law. In fact, despite the apparent novelty of these arguments, which intertwine all functionalist theories of the person, they end up by resuming and emphasizing ancient theses in which human life was not considered a primary value and the distinction between human being and person was valid. Singer points out that in the Greek and Roman era, species membership was not sufficient to ensure protection of life. In fact, there was no respect for the lives of slaves, barbarians, and sick or deformed newborn babies who were killed or left to die by abandoning them to the elements. Even Engelhardt, referring to the proprietary rights of Locke and the *Istitutiones* of Gaius, says that until children emancipate themselves, they remain in the hands of their parents who exercise their legitimate *patria potestas*, which means that, for ancient Roman law, children did not have any rights, not even the basic right to life, and they were totally subjected to the will of their parents. According to these premises, abortion, neonatal euthanasia, and even infanticide do not constitute any violation of a right or moral value: these acts would, in fact, be carried out on nonpersons. Those who claim that mankind has value in itself would make the ethical error that Singer called speciesism. Singer believes that speciesism is similar to racism and sexism because, for him, the difference between humans and animals is morally irrelevant and arbitrary: the real difference is between persons and nonpersons (whether they be, in either case, human beings or nonhuman animals) (Singer 1987). The premise of this reasoning is that the human being is an animal and that all animals are equal: for this reason, it would be unjust to prefer human beings to animals solely on the basis of a difference in species. But what matters from the moral point of view is the person, not the human being. Even in this variant person does not indicate any real being but only the contingent situation in which any adult and healthy animal may find itself. It may be argued that to give animals the status of persons, one must, however, greatly reduce the very distinctive characteristics of a person. If person is understood in a moral sense, in fact, it implies the ability of recognition, respect, dedication to others, and assumption of responsibility that is not manifested at all in the relationship that the animals of different species maintain with each other or with human beings. A lion that encounters a gazelle in the savannah will never recognize it as a person to be respected or protected. Even for functions of a cognitive kind, it is necessary to greatly reduce the threshold of their complexity in order to attribute them to monkeys, pigs, and various mammals. In order to attribute the term person to so-called nonhuman animals, one

should ask the question: are these animals in their full cognitive development able to demonstrate the capabilities that a human person is able to demonstrate at the same stage of development and health? If the answer is, as it must be, a negative one, then it must be said that animals should be respected and protected because they are animals, not persons. In fact, only human persons can protect animals and even assign them rights. It should be noted, moreover, that those who defend the value of the human being in no way intended to defend the species, neither do they do so on behalf of the species: what matters on the contrary is precisely the single individual, even if this protection does not improve the species at all.

Some authors, while distancing themselves from Singer's theses on animal rights, believe however that one becomes a person only when one is accepted into a community of recognition: the process of socialization marks the transition from being human to becoming a person. In this case the demarcating threshold would be indicated by recognition from others. It is not always clear whether such recognition is an obliged act in relation to the presence of a human being, in order to include him in the category of persons, or whether for this to occur it is necessary that the human being gives evidence of his being a person by demonstrating certain qualities. Equally widespread is the theory that considers only human beings with a moral life as person: embryos, newborn babies, the mentally ill, and patients in a minimally conscious state or in a coma would therefore be excluded from this definition. As such, the use of this term would not be particularly problematic if it were limited to emphasizing that the exercise of moral life, associated with cognitive activity, varies over time and requires certain conditions, starting from that of being awake. Throughout the history of moral philosophy, it has always been assumed that in order to be moral agents, certain conditions need to exist and that the framework of rights and duties is distributed throughout life. This means that rights that are not possessed as children, for example, the right to education and to drive a car or vote, depend on age. Duties also change in a similar way; the duty of an adult to work ceases to exist with the onset of old age or illness. The only right that remains at the foundations of all other rights and duties is the right to life: rights regard the living, and rights are not preserved without the preservation of life itself which makes it possible for them to be implemented. Being a moral agent, however, does not mean being a good moral agent but only to act responsibly for better or for worse. This means that one should recognize the attribute of person even to those who do not act in a morally good way. One could not, in fact, reprove someone who is not a person.

This facilitates understanding in moral evaluation of the implicit affirmation that a person remains a person even when acting morally badly: therefore, a distinction between being a person and acting like one is presupposed. If it were not so, there would be the absurd situation in which it would be impossible to accuse anyone because, by acting badly, they would cease to be a person. The two principal meanings of person, as a moral agent and as a psychical subject, can never leave aside the anthropological meaning of human person. This applies all the more in all medical practices. One does not treat and care for someone because he or she is abstractly a person but because the individual is a living and corporeal human being. To dissociate the living body from the person is to forget that no action can be accomplished, not even mental actions, without a body. It can therefore be stated that human person is the term that indicates the continuity and duration of someone: one can be a human person even if one does not behave like a person (ethical significance) or does not possess, yet or any longer, psychical activity (psychological significance). The binary and dualistic logic of person /nonperson introduced in the development of the individual human being accentuates the scheme of Cartesian dualism between *res cogitans* and *res extensa*. In these perspectives, the human body is reduced to *res* possessing the same psychical subject as long as it remains conscious and it is always in danger of being handed over to the power of others when consciousness is lost.

There is in these theories a radical devaluation of the human body, thought of as a purely biological object. Despite appearances, there is a revival of ancient Platonic dualism which construes the relationship between body and soul to be like that of the prisoner and his cell.

The bioethical debate around the person is difficult to compose if it remains influenced by pragmatic requirements which tend to change the meaning of the terms depending on the results one wishes to obtain. No philosophical analysis can lose sight of the immediate element of the human conscience which facilitates understanding of the body not as property to dispose of but as the very mode of existence in time of the human person that each individual is.

Conclusion

The concept of person in anthropology is a theoretical invention by which human beings have tried to accomplish two theoretical moves. In the first, they wanted to specify the universal notion of human being to indicate the concrete individual: "this human being here." The history of philosophy bears witness, in the many insights offered in clarification of the characteristics of the person, as to how inexhaustible the discussion is regarding human beings and their relational and developing identity. The second move has been the self- recognition not only of belonging to a species but of possessing value and being the source of recognition of values: the notion of person has therefore had the function of *nomen dignitas*. The word person has for years been custodian of the human, from birth to death. Today there is the danger of it placing radical discrimination both within the individual human being and among them: in the functionalist perspective, nobody can in fact be called person. In some respects, therefore, there is a risk of a radical devaluation of the human being by human beings themselves, who should demonstrate their being a person in front of others in order to have full recognition of their rights. The bioethical debate, in the emphasis on the term person, as a predicate or a noun, runs the risk of impoverishing the understanding of human.

Cross-References

- ▶ Ethics
- ▶ Human Dignity
- ▶ Human Nature
- ▶ Human Rights
- ▶ Identity
- ▶ Law and Bioethics
- ▶ Personalism

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Personalism

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Abstract

To fully develop and be a reliable guide to bioethical issues globally, Global Bioethics needs at least these elements: a metaphysical foundation for the dignity of persons and a theory of persons. These principles require exploring from a global perspective principles of moral living, the transcendent character of persons, their orientation to Cosmic Person (or God) that transcends finite personal living, a moral compass, and a basis for interpersonal relationships and community. Personalism is any philosophy that considers personality the supreme value and the key to knowing reality and human rights. Therein lies the significance of Personalism for Global Bioethics. Personalism can provide the elements required for a defensible Global Bioethics. Outside of Christianity and Judaism, few Personalists have developed elements that aid the development of a Global Bioethics and that offer the potential to impact a broad range of current bioethics problems, such as allocation of finite medical and health-care resources, the use and limits of genetic manipulation in support of desired medical outcomes, informed consent, proper land and water use, death and dying, and family relations. These issues require a careful ethical analysis and an ethical framework from which those decisions can adequately be made. Though few Personalists have worked toward developing a Global Bioethics, Personalism is nevertheless rich in potential resources in its three dominant forms: Indian or Hindu, European, and American. Here these resources are identified from a historical viewpoint.

The American form of Personalism took root in the late nineteenth century, flowered in the twentieth century, and continues its life in the twenty-first century. Its earliest roots can be traced to a European form and back through Western philosophy to the Mediterranean basin. Long before its Western form, Personalism thrived in India through the six orthodox schools of Indian philosophy scattered along the Indus River Valley of the Indian subcontinent, developing parallel to Personalism in the West. Personalism did not develop in other areas of the globe, but Personalist themes can be found in many of them, providing rich resources for the development of an ethical framework for a Global Bioethics.

Keywords

Persons; Personalism; Realistic Personalism; Idealistic Personalism; Global Bioethics; Transcendence

Introduction

Personalists claim that Person is the key in the search for self-knowledge, for correct insight into reality, for the place of persons in reality, and for correct living (ethics). Other than giving centrality to the Person, Personalism has no other set of principles or unified doctrine. Many prominent Personalists have been theists, but that doctrine is not a requirement. There is also no common set of methods or definitions, including the definition of Person. Respecting that caution, Personalists defend the primacy and importance of persons against any attempt to reduce persons either to the Impersonalism of an infrastructure, such as Scientific Naturalism, or superstructure, such as Metaphysical Absolutism. Between the Scylla and Charybdis of either type of Impersonalism, Personalists trace the origin of the concept of Person and the development of Metaphysical Personalism from the ancient world to its flowering in Europe and America. Open to the richness of their philosophical tradition, Personalists trace their origin and development both to the Indian subcontinent and to the West.

History and Development of Personalism

South and East Asian Personalism

India ***Hindu Personalism.*** In India the religious practice and thought spawned by the Vedic (from

Veda, "knowledge," "wisdom") texts from at least 1500 BCE formed the backdrop of Hindu Personalism. The term Hindu is derived from the river Sindhu (the Indus) where various schools of practice and thought were formed.

Broadly conceived, Personalism in India originates within the main goal of Hindu philosophical inquiry, which is freedom from misery. Each system of Hindu philosophy seeks to help persons to that end by giving them insight into the nature of ultimate reality and their place in it. These systems advocate self-knowledge, *atmavidya*, without which the desired freedom is impossible. The nature and destiny of individual persons are the common themes of the six orthodox Hindu philosophical systems: *Nyaya*, *Vaisesika*, *Samkhya*, *Yoga*, *Purva-mimamsa*, and *Vedanta*. The *Vaisesika* school is frequently lumped together with the *Nyaya* (logic) school, and *Yoga* is classically grouped with *Samkhya*. Each system promises self-knowledge, *atmavidya*, that bonds the systems into a single philosophical tradition.

Seeking freedom from misery through self-knowledge, Hindu Personalist schools of thought center on four questions: What is the self? How is it related to the material world? What is the relation of the self to ultimate reality? And, what is the path from pain and misery to liberation?

First, according to each orthodox school, persons are marked by various characteristics, including a permanent and eternal soul (*atman*) that exists behind the veil of empirical consciousness and that possesses a physical body (*jiva*) existing as part of a changing material world. While it is agreed that the *atman* is eternal, unchanging, independent essence, the six orthodox schools differ whether the transcendent I is conscious or unconscious, active or passive. Each school also recognizes that by being connected to the material world, Persons possess other characteristics, including agency, will, thought, desire, free will, intention, and identity.

Second, Hindu Personalists focus on the indissoluble reality of the individual soul and on its relation to the empirical consciousness. The soul, the basic reality in humans and all living things, is a transcendent I (*atman*) and is veiled by a person's empirical consciousness. It cannot be the object of experience. The empirical consciousness, the experience of objects sensed or being sensed, comes to be interpreted as alien, attributive, essential, adventitious, permanent, or temporary. Schools differ on how to correlate the transcendent I and the empirical consciousness. Hindu Personalism views the empirical consciousness as either attributive or alien, monist or dualist.

At one extreme, *Samkhya*, the oldest school, interprets the dualism of the transcendent I and the empirical consciousness as a dualism of spiritual consciousness (*purusa*) and material nature (*prakrti*). *Purusa* is sentient and passive; *prakrti* is insentient and active. As sentient, *purusa* experiences products of *prakrti* and desires emancipation. As passive, *purusa* can be understood as unaffected and secluded. In *Samkhya*, the material world is not an illusion; it is real and stands over against the spiritual person. This dualism is motivated by final beatitude. However, achieving it requires, in theistic versions of *Samkhya*, moral support, compassionate companionship, and guidance from a Supreme Being who possesses perfect knowledge and is capable of perfect action. The *Yoga* thinker Patanjali (300) introduced God into atheistic *Samkhyan* dualism to satisfy the moral demand of the spiritual aspirant. With that addition, the *Samkhyan* school becomes a theistic Personalism.

At the other extreme, monism is represented as *Advaita Vedanta* (non-dualistic *Vedanta*). Sankara (788–820), the leading expounder of Vedanta (literally, the end of the Veda) philosophy, states its principle insight that the self is One with Ultimate Reality (*Brahman*). Thus, the transcendent I includes the empirical many and the Divine One. The triune of Divine One (*Brahman*), the transcendent I (*atman*), and the empirical many is a grand unity. Sankara was the most influential Vedanta thinker who espoused the monistic framework, as defined in the *Upanishads* (*tat tvam asi*) Thou art that or (*atman* is *Brahman*).

The ethical dimension is found in the search for self-knowledge, which is necessary for the soul to be one with *Brahman*. During a person's life cycle, the caste system prescribed in the *Vedas* eliminated the possibility of social movement,

from lower to higher caste. Self-knowledge focused on one's place in the caste system and its accompanying duties. The soul through its life cycle within a caste system seeks virtue, allowing reincarnation in a higher caste. In this way, the virtuous soul achieves release from pain and suffering to Nirvana. Little or no attention is paid to global bioethics, yet in some traditions the person is understood as having inherent dignity, necessary for grounding ethics, rights, and law. Within the unorthodox systems, such as *Ajivikas* (fatalists), *Charvakas* (materialists), *Jains* (who accept the existence of eternal selves but reject the existence of a supreme God), and Buddhism, Personalism does not develop, except in one sense as noted below. *Ajivikas* adopted materialism on the ground that sense-perception was the only valid means of knowledge. The *Ajivika* materialists questioned the validity of theological and metaphysical theories that do not come within the ambit of sense-experience. This explains why they rejected the religious version of *atmavada*, the belief in a metaphysical self.

Buddhism. Buddhists, followers of Siddhārtha Gautama (563–483), rejected the doctrines of *atman* or *purusa*, central to Hindu Personalism, and accepted instead a causal account (*paticcasamuppada*) of the human personality. The Person, for Siddhartha, is a causally connected bundle of psychophysical aggregates (*namarupa*). Kasulis says, "According to Buddhism, therefore, I am not a self-existent being who chooses with what or how I wish to relate to external circumstances" (Kasulis 2002, pp. 62–63; Mortensen 2014). A Buddhist Personalism, the *Pudgalavadins* thought that behind the aggregates or groups (*skandha*) of mental and physical *Pudgalavada*, arose two centuries after the lifetime of Siddhartha. De Smet says, "The conditions observed by introspection, there must a substrate. Had not the Buddha, in a well-known passage, spoken of the 'bearer of the burden' – an expression seeming to indicate some sort of ego underlying the aggregates? At the same time the *pudgala* could by no means be identified with the *atman*, because it was merely an integrating function demanded by the aggregates and expressed by them" (De Smet 2010, p. 38). Its teaching came close to the heresy of the eternalism of the *atman* and soon died out.

China and Japan Regarding China and Japan, in neither country's faiths are persons understood as having an eternal essence or inherent dignity or value. The Chinese, deeply influenced by Confucianism (551–479 BCE), believed that humans elevated themselves to a position through examinations and service, providing little or no help in formulating a Global Bioethics. The Japanese, deeply influenced by Buddhism, understood persons in terms of their relationships with other humans, nature, and the totality of things. They held to a hierarchical dyadic view of persons. Persons continue to perfect themselves within the community of excellence. Parallel to the origin and formation of Personalism in India, and later the failure of Personalism to take hold in China and Japan, Personalism in the West began in the Mediterranean basin and through Christianity spread north to the Atlantic Rim, northern Europe and the British Isles, and America.

Roots of Idealist and Realist Personalism in the Mediterranean Basin

All Western Personalists trace the origin of the ontological status of persons and their supreme importance as key to the understanding of Reality to the confluence of Greco-Roman philosophy and Christian experience and theology. Both made significant contributions to the formation of the concept of Person and the moral life, and Western Personalism can mask contributions central to global bioethics.

In its early uses, the word Person carried no ontological import. Person is first found in Greek and Roman culture. First consider the formation of "person" in the West. The roots of person lie in the Greek word *prosopon* that refers to the face consisting of that around and near the eyes (*pros* + accusative of *ops*). Soon it designated the masks or faces used in the Greek theater. Its Latin cognate is *persona*, probably of Etruscan derivation from *phersu. Persona* referred to a mask functioning as a loudspeaker (*persono*,

per, "through" + *sona*, "resound," "resound thoroughly"). The mask worn by actors on stage aided them by "sounding through" to be heard by an audience. In the Roman theater *persona* meant a character and role in a tragedy or comedy. In Roman society, the personae of individuals gain their identity, status, and responsibilities from their roles in a hierarchical, honor-shame society. For example, *persona* came to be used in reference to the king as king, implying a difference between the important social man and the relatively unimportant singular empirical man. By the end of the second century, *persona* became a judicial term referring to a Roman citizen as possessor of legal rights, in contrast to a slave who possesses no legal rights, a *non-persona*.

Meanwhile, as Pythagorean, Platonic, and Aristotelian philosophical influences continued, individual persons had little philosophical importance. Plato distinguishes between the individual and the universal and thereby understands the individual through the universal. The individual Socrates participates in the universal, Human Being. To understand the individual Socrates, first know the universal, and then one can understand and account for the particular. Pythagoras and Plato used the term *soma*, body, and played on the similarity between *soma* and *sema*, body and tomb. Aristotle, unlike Plato, calls the individual primary substance and the universal, conceptual. First substance is that which stands under (*hypostasis*) a general term referring to whatever stands under something else. Initially, *prosopon* gained no ontological meaning through that understanding of substance.

In the Eastern Mediterranean among the Jews, the Hebrew word, *nephesh*, is sometimes translated as "person." However, in ancient Hebrew life and culture, no word analogous to *prosopon* or *persona* appears. *Nephesh* is more often translated as soul, life, creature, or self. *Nephesh* can refer to the animating principle of a physical entity or the existential quality or state of life. Usually referring to a human being as a unified entity, no distinction is made between immaterial and material aspects. *Nephesh* as a whole is created by God; *nephesh* is not an attribute of a substance. The form/matter and substance/attribute distinctions are foreign to ancient Hebrew thinking. However, beginning with Alexander the Great in the fourth century BCE and continuing through the Roman period, the Eastern Mediterranean became Hellenized. New Testament writers, St. Paul, for example, would have known *nephesh*, *prosopon*, and *persona*, likely aware of semantic tensions that later found their way into theological debates within the Christian church. The different Greek and Hebrew-Christian understandings of person moved into focus in the fourth and fifth centuries as the Christian church attempted to work out a satisfactory understanding of the Trinity and the individual personhood of Jesus the Christ. The details of the controversies that arose are beyond the limits here. However, central to the controversy was the understanding of the individual person. Is an individual person an *attribute* of being? or is an individual person *being*, who, having been created by the free and independent God and who bears God's image, is also free and dependent? If the former, the Greek metaphysical word, *ousia*, expresses the Trinity, as in *una substantia* (God) and *tres personae*, where the Father, Son, and the Holy Spirit are understood as three independent Gods. If the latter, person is not an attribute of *ousia*, but *upostasis*. Earlier both *ousia* and *upostasis* meant substance. Eventually, they were used separately, *ousia* referring to substance and *upostasis* referring to individual person. This means that *persona* is no longer a kind of mask worn by an ontological substrate, *ousia*. The Greek Fathers, particularly the Cappadocians, led by Gregory of Nazianus (329–389 or 390), understood that individual persons are ontologically ultimate, the central thesis of Personalism. However, an understanding of the interior life of persons lay beyond their metaphysical interests given the honor-shame culture in which they lived.

European Development and Flowering

The analysis of the interior life of persons and laying the foundation for Idealistic Personalism fell to Augustine (354–430), yet he continued the

substance view by defining person as "a rational soul using a mortal and earthly body" (*substantia quaedam rationis particeps, regendo corpora accomodata*). Nevertheless, a person is One; a soul in possession of a body does not constitute two persons but one person. His contribution to Personalism was his investigation of the inner experience of persons. Augustine set a new course in philosophy that would not be developed until the modern period. In doing so, he developed an understanding of reality as Person through an understanding of the interior life of persons. Further, he argued that in persons, free will is superior to rationality.

Realistic Personalism

In the late Roman era or early Middle Ages, the Roman Catholic Church adopted Boethius' (480–524 or 525) definition of person as the *naturæ rationalis individua substantia* (an individual substance of a rational nature). In the meantime, the Greek Orthodox Church continued the doctrines of the Cappadocians, particularly Gregory of Nazianus.

Medieval philosophers, such as Thomas Aquinas, modified Boethius' definition, and others such as Scotus and William of Occam were critical of Boethius' substance view of Person. Nevertheless, Boethius' definition became firmly entrenched in Western Catholic philosophy and continues into the twentieth century as Realistic Personalism (resting as it does on Aristotelian and Thomistic foundations). During the modern period and the nineteenth and twentieth centuries, Personalists continued to debate the nature of Persons. Though the meaning of substance changed under the impact of the Renaissance and the influence of modern thinkers such as Locke, Descartes, Hume, and Kant, the thought of Aquinas became known as the best statement of Christian Philosophy. Thomism has continued due largely to its institutionalization by the Catholic Church in 1879 by Pope Leo XIII in his *Aeterni Patris* where he designated Thomas' thought as the official philosophy and doctrine of the Catholic Church. Cardinal Elio Sgreccia (2012), an important contemporary Thomistic thinker, has made important contributions to Global Bioethics.

Idealistic Personalism: Modern Formulations and Debates

As the grand systems of Christendom in the high Middle Ages cracked under the weight of heavy criticisms from Scotus, Occam, Montaigne, and the new science, a new vision arose in the Renaissance: the Emerging Self. Though for many the substance view of persons continued, aided by its institutionalization in the Western church, it was soon challenged. Pico della Mirandola's, Luther's, Descartes', and Locke's critiques and Augustine's careful descriptions and insights into the interior life of persons contributed to emerging selves.

Influenced by pyrrhonistic skepticism, Descartes (1596–1650) searched for a new foundation for society, culture, and knowledge, one that measures up to the ideal of certainty as in mathematics. He argued that one can know for certain that he exists and that he can doubt every other knowledge claim, including those of the external world. In this way, inspired by new scientific investigations, he raised problems, particularly the mind-body problem. Rather than the view that God created ex nihilo substances with a rational nature, or persons are substances using a body, Descartes contends that God created two substances, res cogitans and res extensa, mind and body. Central to his view of mind is freedom of the will and rationale. In freedom of the will, one finds a characteristic of God that is exactly the same as one finds in oneself. God's reason is so far beyond our finite minds that we cannot with confidence claim that God is reason. Descartes' discussion of the interior life of persons in the early seventh century continues Augustine's insights in the late fourth century. However, Descartes' dualism of two kinds of created substances significantly differs from Boethius' view that a person is an individual substance with a rational nature. Though modified, the substance view of persons persisted.

In reaction to Descartes' dualism and Spinoza's monism, Leibniz's (1646–1716) doctrine of monads offered pluralism. Both Descartes and Spinoza assumed that extension implies actual size and shape. Leibniz wondered why we couldn't assume that all things are compounds or aggregates of simple substances. These substances are not the extended atoms of Democritus or Epicurus. Each simple substance is a monad that is unextended and has no size or shape. Each is a metaphysical point that Leibniz called souls to emphasize their nonmaterial nature. And, each possesses its own principle of action within itself and behaves according to its own created purpose; still they work together according to God's preestablished harmony. A person's identity centers on a dominant monad, his soul, whose life is an "unfolding," set from the beginning. Since the basic nature of persons is thought, development through life means moving from murky confused ideas to true ones, the way things really are. In this way, Leibniz was the first philosopher to reject the substance of persons and to adopt an agency view, even if deterministic. Some philosophers, such as Leroy E. Leomker (1900–1985), considered Leibniz the first modern Personalist, where persons are agents, not substances with an attribute.

Kant (1724–1804) and Hegel (1770–1830) exerted major influences on the formation and development of Idealistic Personalism in the West. Kant's distinction between the phenomenal and noumenal world reinforced Berkeley's view of "material" substance and emphasized that the only path to reality is through the practical reason of persons. With his claim that compliance with moral law is the essence of human dignity and personhood, Kant exerted the single most influence on many personalists with his categorical imperative, persons must be treated as ends and never as means only. That influence was transmitted into American philosophy through the work of Hermann Lotze (1817–1881).

Within that background, the modern Idealist Personalist vision was first stated in the context of a philosophical issue lying at the core of eighteenth- and nineteenth-century German philosophy. The issue was raised in the debate between the followers of Spinoza and those of Leibniz. For Spinozists, Being is one and independent. If Being were more than one, it would not be independent; and, if Being is independent it cannot be many. For Leibnizians, reality is a pluralism of monads. The debate between the two systems sharpened to the debate between monism and pluralism. The problem of the One and the Many resurfaced. How can they be reconciled? What philosophical view can do so? At least two alternatives surfaced at the end of the eighteenth century and the beginning of the nineteenth century.

The first alternative was Hegel's. According to a recent interpreter, Hegel sought a "unification philosophy – the need to unify not only life's various and conflicting powers, but especially the opposing human craving – for individuality and finitude, on the one hand, and for the absolute and the infinite, on the other" (Hegel 2005, p. 48). Hegel sought it by blending the organic growth of an Aristotle and the Absolutism of a Spinoza into a dynamic monistic system marked by Idealism, Pantheism, and Rationalism.

In reaction to the Rationalism of Absolute Idealism and to individual realism, a poet and philosopher and older contemporary of Hegel, Friedrich Heinrich Jacobi (1743–1819), offered a second alternative, Personalism. He thought that Personalism is "that form of Idealism which gives equal recognition to both the pluralistic and monistic aspects of experience and which finds in the conscious unity, identity, and free activity of personality the key to the nature of reality and the solution of the ultimate problems of philosophy" (Bengtsson 2006, p. 53). The insight of Jacobi continued in Schelling, in the speculative theists (Fichte, Weisse, Ulrici), to Hermann Lotze, the preeminent philosopher in mid-nineteenth-century Germany. It was primarily through Lotze that Personalism arrived in Boston. When Personalism arrived in Boston, the distinctive modern characteristics of persons were in place: numerically distinct, individual interiority, freedom of choice among genuine options, autonomy, dignity, and agency. As Personalism made its way to Boston and the West Coast, Personalism on the European continent and in England responded to German Idealism, specifically to Hegel.

In the debate between monism and pluralism, the traditional metaphysical problem of the One and the Many resurfaces. But more, metaphysical monism and pluralism lie beneath many modern philosophies that Personalism rejects as impersonal, dehumanizing. Absolute Idealism leaves little room for free will and self-determination and cannot be reconciled with the worth of the singular person. Totalitarianisms see persons as means to an end that exist for the interests of the state. Personalists respond insisting on self-determination, responsibility, and inherent dignity of all persons. Individualism champions the autonomous self as its own lawgiver, making all other persons a means to one's own ends. Personalism contends persons are communal, open to, cooperating with, and possessing rights respects the viewpoints of others. Personalism also rejects any reduction of persons to impersonal, deterministic laws, whether those of society, for example, Auguste Comte (1798–1857), or of nature, Darwinism evolution. Personalists challenge Comtean philosophical positivism and naturalism whether that of Darwinism or Samuel Alexander (1859–1938) by appealing to the dignity of individual persons, their free will, and values.

On the European continent, responses to Hegel led to the development of three schools: Paris, Gottingen/Freiburg, and Lublin. In Paris Emmanuel Mounier (1905–1950), under the inspiration of Charles Peguy, founded French Personalism. He also founded and edited the influential French magazine *Esprit*, the organ of Personalism in France. His work influenced (1) the development of the Catholic Worker Movement, (2) Dorothea Day and Hull House in Chicago, and (3) Jacques Maritain, who helped formulate the 1948 United Nations Universal Declaration of Human Rights and signed it. Mounier's influence continues in the work of the Danish philosopher, Jonas Mortensen, in his book *The Common Good* (2014). In Gottingen and Freiburg, Phenomenology developed under Edmund Husserl (1859–1938) who addressed the question of the relation of objective reality and philosophical reflection. Later he returned to Idealism, precipitating a break with his former students, who, when on their own, made significant contributions to Personalism. These included Max Scheler (1874–1928) who developed a theory of values based on Love and against the formalism of Kant. In Poland, the Catholic University of Lublin continues as a center of Personalist thought, whose foremost representative was Karol Wojtyla (1920–2005), later Pope John Paul II, an interpreter of Scheler, who developed a distinctive theory of man in his major work, *The Acting Person*, where he argued that man is best understood in action.

Idealistic Personalism in North America

Before the mid-nineteenth century, few in North America discussed the thought of German philosophers such as Kant, Fichte (1762–1814), Schelling (1775–1854), Hegel, and Lotze. As demands of academic life increased, young American philosophers completed their philosophical preparation with a year or two of study in Germany. When they returned, no longer fully embracing the old Scottish orthodoxy, their thinking was framed by the Spinoza-Leibniz debate, sometimes understood by theologians and many philosophers as the pantheism-individual freedom debate and by reductions of persons to deterministic laws of society and nature. Deliberating within that framework, they thoroughly discussed the Personalism they learned while studying with Hermann Lotze at Gottingen. Among that group were George Santayana (who wrote his doctoral dissertation at Harvard University on Lotze), William James (1863–1952), Josiah Royce (1855–1916), and Borden Parker Bowne (1847–1910). That conversation grew into what W. Werkmeister (1901–1993) in the mid-twentieth century called the "first complete and comprehensive system of philosophy developed in America which has had lasting influence and which still counts some of our outstanding thinkers among its adherents" (Werkmeister 1981, p. 103).

Those historical roots found their way into American philosophy and formed at least four distinctive branches of Personalism. These four branches are Idealistic (against the threat of naturalism and realism), Realistic (against reducing all to mind or person), Organismic (against Idealistic and Personalistic Realism), and ethical (against

collectivism and reduction of personality to mechanism and the body-brain).

Boston

Harvard University Josiah Royce, who taught at Harvard, was motivated by a religious view of life and reality, with an emphasis on the self and community. He sought to realize his philosophical goals through a synthesis of two traditions: the rationalistic system building of philosophers in the West and the pragmatic emphasis on experience and practice, distinctive of American philosophical activity since the late nineteenth century.

At the root of Royce's system is a concept of the Self. Early in his career, the Absolute appears as the Self who knows all in one synoptic vision. Rejecting realism, mysticism, and critical rationalism, his central thesis is that to be real is to be a determinate, individual fulfillment of a purpose. Later he focused more on mediation and the idea of system. Toward the end of his career, the Self appears as social. He developed a social theory of reality, a community of interpretation. He called this community "the Beloved Community" whose goal was to possess the truth in its totality.

William James, a leading pragmatist, shared with Personalists a key insight. James said, "The more perfect and more eternal aspect of the universe is represented in our religions as having personal form. The universe is no longer a mere *It* to us, but a *Thou*, if we are religious; any relation that may be possible from person to person might be possible here" (James 1955, pp. 27–28).

William Ernest Hocking (1873–1976) believed the universe is independent of human minds and discoverable through Phenomenology. Sometimes thought of as the American Husserl, he studied for three months with Husserl at Gottingen. Through careful phenomenological analysis, Hocking unpacked everyday phenomena and found that "nature can no longer be fully understood from the atoms upward but only from consciousness or selfhood outwardly" (Howie 2004, p. 219). He stressed that Philosophy must be idealistic. Further, values keep emerging as more is learned about the world and ourselves. The mental life has unity, is deep and mysterious, and finally coheres in a single will. The finite person is an imperfect image of the universe. Influenced by Personal Idealism and Royce's Absolute Idealism, the final unity is a Self, infinite in depth and mystery. The Self or Person is also a natural thing that is completely determined. More than a natural thing, the self is free to determine what will be fact in the next moment. Though nature and mind are in opposition to each other, one subject to the laws of nature, the other transcending them, through the ceremonies of religion that opposition is overcome.

Boston University Borden Parker Bowne claimed to be the first Personalist in any thoroughgoing sense, having developed a systematic metaphysics, epistemology, ethics, and psychology. He taught at Boston University from 1876 until his death in 1910. Metaphysically Bowne was a Pluralistic Idealist like Howison. But his theism distinguishes his Personalism from Howison's. God, the Divine Person, is both creator and world ground. Finite selves are created, and nature is the energizing of the Cosmic Mind. As world ground, the Divine Mind is the self-directing intelligent agency that accounts for the order and continuity of the phenomenal world.

Bowne was not only a systematic philosopher but also a caustic critic of Hegel's Absolutism, Spencer's Evolutionism, and all forms of Materialism. These criticisms were expressed in his famous chapter in *Personalism*, "The Failure of Impersonalism" (Bowne 1908, pp. 107–150).

Bowne's teaching at Boston University attracted many young talented philosophers, some of whom formed the second generation of Personalists in America. The most important among them were Albert Knudson (1873–1953), who continued the Personalist tradition in the Divinity School of Boston University; Ralph Tyler Flewelling (1871–1960), who developed the School of Philosophy at the University of Southern California; and Edgar Sheffield Brightman (1884–1953), who led the Philosophy Department at Boston University from 1919 until his death in 1953.

Brightman, in agreement with other Boston University Personalists, sought truth to guide creative living in the most empirically coherent interpretation of experience. Rejecting the skepticism of Descartes, beginning the search for truth within experience, and advancing and testing hypotheses, Brightman developed the distinction between the shining present and the illuminating absent. Pointing beyond itself, the shining present is unintelligible without reference to an illuminating absent. Though the shining present does not prejudice the nature of the illuminating absent, Person is the hypothesis that most coherently illumines the shining present.

Brightman contended that everything that exists/subsists is in, of, or for a mind on some level. He defined Personalism as the hypothesis that all being is either a personal experience (a complex unity of consciousness) or some phase or aspect of one or more such experience. Nature is an order generated by the mind of Cosmic Person. Finite persons are created and grounded by the uncreated God and as such possess free will. Reality is a society of persons.

Brightman's most impressive work is his *Moral Laws*, in which he works out, along lines heavily indebted to Kant and Hegel, a thoroughgoing ethical theory. In *Moral Laws*, Brightman adopted perfectionism that moved from the abstract universal to the concrete universal. In its moral development, value choices should be guided by moral laws, a kind of cultural neutral regulatory system. Understood dialectically, the moral life is a special instance of the relation of the universal to the particular, wherein the personality can achieve both the best possible and *rational freedom* (Deats and Robb 1986, p. 111). In this way, personality, under the guidance of reason, seeks to become the best it can be, a fully integrated personality.

Peter A. Bertocci (1910–1989), following Brightman as the leading Personalist at Boston University, enriched the understanding of person through his work in psychology. Bertocci claims that the person "is a self-identifying, being-becoming agent who maturing and learning as he interacts with the environment, develops a more or less systematic, learned unity of expression and adaptation that we may call his personality" (Bertocci and Millard 1963, p. 217). Bertocci is well known for his work in ethics and his view that the essence of person is time.

California

Howison's position can be succinctly stated in this way: all existence is either (1) the existence of minds or (2) the existence of the items and order of their experience; all the existences are known as "material" consisting in certain of these experiences, with an order organized by the self-active forms of consciousness that in their unity constitute the substantial being of a mind, in distinction from its phenomenal life. Devoted to empiricism, Howison rejected creation, persons simply are, metaphysically they have source. Collectively they move toward their own fulfillment as measured by the eternal standard to God. Later at the University of Southern California, Ralph Tyler Flewelling, a student of Bowne, taught Boston University Personalism enriched into Democratic Personalism.

Central and South American Personalism Personalism in Latin America developed in the twentieth century against the background of Scholasticism (sixteenth to nineteenth centuries) and Naturalism and Positivism (nineteenth century). When Latin American philosophers became aware of North American philosophy in the twentieth century, it was the Personalism of Edgar Sheffield Brightman that attracted them most. The interest was reciprocal. "Brightman established the first graduate course in Latin American philosophy in the United States" (Kruse 2002, p. 149).

Personalist Themes in African Thought Africans held a rich view of persons, but did not develop a Personalism in the Western or Eastern sense of persons, such as Augustine's rational soul using a mortal body and understanding epistemologically, ontologically, and ethically through personality. They held a social view of themselves. Society comes first, epistemologically, ontologically, and morally. Within community a man becomes a man, and that becoming is a

process that takes many years. In this sense, they rejected Western individualism, especially Western Protestant Christianity when presented through the eyes of missionaries who presented the Gospel as calling an individual to right his or her life with God. Furthermore, no one working in these traditions addresses issues in global bioethics.

Personalist Themes in the Near East Some Islamic philosophers developed a form of Personalism, the most notable of whom was Muhammad Iqbal (1877–1938). Though born in British India (now Pakistan), he was a well-recognized poet in Pakistan. Ibal was deeply religious and at an early age enrolled in a mosque to a study of the Koran and Islam. His philosophical thought centered on the belief in a Cosmic Self that the whole world obeys. The whole of life is self-knowledge and self-realization. The self goes through various stages before arriving at perfection, when the self becomes a vice-regent with God. He stressed that self-realization and perfection can only be achieved within the nation. In his poetry he continually emphasized the importance of community.

Conclusion

As noted earlier, few Western Personalists have addressed issues in Global Bioethics. Personalists in America focused on issues current at the time, such as the Other Minds problem and ethics. There are notable exceptions such as Erazim Kohak's (1933–) *The Embers and the Stars* (1984), an exploration of the moral sense of nature. Kohak writes on environmental ethics, a subfield of bioethics, from the perspective of a Personalistic Phenomenologist. The most notable contemporary work by Personalists in bioethics comes from the pens of realists, most of whom are in the folds of the Catholic Church. Christians of all persuasions support a journal published by Oxford University Press, in which issues in bioethics and global bioethics are discussed. Nevertheless, a central contribution of Personalism is its focus on value theory, and the transcendent character and dignity of human life offers the potential to impact a broad range of current bioethics problems, such as allocation of finite medical and health-care resources, use and limits of genetic manipulation in support of desired medical outcomes, informed consent, proper land and water use, death and dying, and family relations. However, few personalists have developed an adequate theory of interpersonal relations, central for an adequate understanding of community.

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Cross-References

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Personalized Medicine

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Abstract

Personalized medicine is a popular term – meaning diagnosis and treatment adapted to a given condition and/or gene defect. Progress in biomedicine is making the analysis of an individual's DNA increasingly faster and cheaper, which has already had a significant effect on diagnosis and treatment and will increasingly contribute to more precise diagnosis of various diseases. Some of them may be "druggable," that is, an appropriate medicament may exist or be developed for them. In the long term, this should be cost-effective and save money spent on useless treatments; on the other hand, currently most of these treatments are expensive, not very many are available, and problems of their equitable distribution are quite obvious. Moreover, there is also the problem of resource allocation – whether the always limited funds for medical care should be invested in possible future treatment or rather used for addressing current patient needs using available drugs and therapies.

Keywords

Personalized medicine; Precision medicine; Targeted therapy

Introduction

For a long time, the dream of physicians has been the magic bullet which would find its target, be specific, and not give side effects. Although initially the concept was developed by Paul Ehrlich for drugs for eliminating pathogens, currently the incredibly rapid development of techniques of sequencing and analyzing DNA has led to the vision of a completely different approach to treatment of numerous diseases – that the therapy would be perfectly adapted to the cause of the disease. There have been some – though not very many – striking examples of application, for instance, the drug ivacaftor is already used for treating a small subpopulation of cystic fibrosis patients with the G551D mutation and more recently has been combined in a trial with another drug to treat patients with the most prevalent cystic fibrosis mutation (Wainwright et al. 2015). This disease is relatively common in Europe (about 1:2500 births). Perhaps the most striking application has been the combination of diagnosis and therapy in one pair of nonidentical twins with a neurological disorder, in whom genome sequencing indicated the presence of two different mutations; the results of using the appropriate two drugs (L-DOPA and 5-hydroxytryptophan) led to great improvements in both patients (Bainbridge et al. 2011).

On the other hand, the term personalized medicine is not perhaps the most appropriate one; some authors prefer the term "precision medicine" as medicine at its best has always been to some extent personalized – from giving blood type appropriate blood transfusions to adjusting the diet of persons suffering from diseases such as phenylketonuria (Jameson and Longo 2015). The term precision medicine may also be better as the target of the treatment can be quite a large group of persons – for instance, those taking statins for hyperlipidemia, blood clotting factors for hemophilia, or a low-galactose diet for galactosemia.

There are other precise or personalized applications which have also been around for some time, mainly in the field of cancer therapy – for example, Gleevec for chronic myeloid leukemia and Herceptin for breast cancer positive for the Her-neu marker (Jameson and Longo 2015). Other important applications go under the name of pharmacogenetics or pharmacogenomics, where the precision aspect is in a way reversed – certain drugs should not be given to persons with certain diseases or certain gene mutations.

History and Development

To find a precise definition of the term personalized medicine (PM), Schleidgen et al. (2013) undertook a literature search and came up with the following: "PM seeks to improve stratification and timing of health care by utilizing biological information and biomarkers on the level of molecular disease pathways, genetics, proteomics as well as metabolomics." This analysis also found the first mention of this term – in a paper as early as 1971; but its use only became common from the beginning of the 1990s, and by 2009 most papers no longer defined the term (Schleidgen et al. 2013). This is interesting as it indicates that the term is now generally understood by the scientific community.

Personalized medicine has also been called 4P medicine – predictive, personalized, participatory, and preventive. The preventive aspect is an interesting one, as the idea here is that the disease – or the risk for it – should be discovered early so that its onset can be prevented or delayed. This of course raises the problem of patient participation and compliance. The participatory aspect is a somewhat separate matter, as it concerns the patient's involvement in his own treatment, which is not an inevitable consequence of personalized medicine, in the sense that it can, but does not necessarily have to, accompany the treatment specifically suited to the disease and patient. Moreover, there seems to be an overestimation in the literature of how a precise diagnosis may or should affect the way the patient behaves in respect to his disease. As obesity and lifestyle diseases are on the rise, it is clear that knowledge of risks and benefits does not necessarily affect the behavior of people who have been informed of the

consequences of their actions. There are various possibilities of influencing the way people behave when they learn that they can take steps to avoid or alleviate a disease, but they do not seem to work very well for lifestyle diseases. It is not yet clear whether diagnosing a similar risk in a more personal and molecular way (say by detecting polymorphisms in the DNA which could contribute to circulatory problems) would lead to better patient compliance.

Gene therapy is also a form of precision medicine, as the genetic defect is removed by introduction of a functional gene; this is described in a separate entry (see: ▶ Gene Therapy). The first medicament based on gene therapy, Glybera, for treating lipoprotein lipase deficiency was approved for the European market in 2012; its price – about one million euro – is a clear indication that precision medicine comes with a cost that cannot be easily borne by many health-care systems or the patients themselves.

There is a more general problem with personalized medicine, as the interest in genes has shifted the focus away from an extremely important component of preventing diseases as well as understanding their causes – environmental effects, a very broad area including pollution, stress, diet, and many other factors.

Ethical Dimensions

The possibilities of precision medicine are enormous, and the ethical challenges which it raises are quite numerous and not always easy to address.

Most of them are not really new, as the growth of increasingly specific DNA-based diagnostics and the problem of analyzing and safe storage of information about patients have been here for quite some time. Moreover, some of the considerations found in papers on personalized medicine (Gefenas et al. 2011) are also not really due to the concept of personalized medicine but to the development of medical diagnostics in general. The problem of patient responsibility for complying with medical recommendations was already mentioned, and the questions of the ethics of gene testing have been discussed without referring to personalized medicine previously – such problems as respect for private life, the wish to be informed or not informed, whether the results of a genetic test belong to a person or should be shared with members of his or her family, who may also be at risk for a disease (Gefenas et al. 2011). Most of these problems are discussed when professionals are trained as genetic counselors, and there is nothing very new here as these problems have been discussed since molecular diagnosis became more common, but they will become more complex as in the future – at least in richer countries – recommendations and/or treatment may become based on analysis of a patient's whole genome. Though progress in sequencing has been remarkable, understanding what every nucleotide means and which ones are really significant for future or current health has advanced much more slowly. It is not currently clear when health recommendations based on whole genome sequencing may become more common; currently, this type of sequencing is used in some institutions, for example, for the diagnosis of neurological diseases in pediatric patients with a success rate of about 20 %. This of course brings the problem of findings which are not directly related to the disease whose molecular basis is sought. The problem of these incidental or secondary findings has been addressed by scientific genetic societies, and there is a general belief that informed consent may need to be in a tiered form – specifying what kind of information should be released to the patient, based on his choice before the test is performed. Additional problems will need to be addressed in the case of minors, but the general rule that children should not be analyzed for incurable diseases which occur in adults should still apply.

The fundamental problem of personalized medicine is economic – the issue of equitable benefit sharing is one which has been addressed by many documents and will not be discussed in detail here, but there has always been a gap between richer and poorer countries, and the more expensive a technology the less likely it is that poorer populations will benefit from it. The costs of sequencing are decreasing dramatically;

however, even though in the long term specific drugs may save money, currently the outlay for sequencing and drug development is nothing if not very large. Personalized medicine has added a new dimension and tough choices for physicians and health-care providers to face in allocating resources, as currently if something is expensive and potentially lifesaving, it is not available for everyone.

Chadwick (2013) and Vogenberg et al. (2010) have indicated another risk that personalized medicine could be responsible for – if stratification or personalization would coincide with divisions which are already in place – whether ethnic, racial, or other, this could also lead to an increase in divisions within a population. As an example, isosorbide dinitrate has already been sold as a "race-based drug" for African-Americans with congestive heart failure, without any real scientific basis (Vogenberg et al. 2010). There are of course certain diseases which occur more commonly in some ethnic populations – but this requires careful testing to ensure that a population which is not necessarily homogeneous can really benefit from a targeted drug.

Rose (2013) has rightly pointed out that progress in medicine has been almost exclusively due to a completely different trend than personalization, namely, "interventions that have been addressed to all – clean air, water, effective sewage systems, pure food, programs of population wide vaccination" (p. 342). This is, however, not completely true. Some personalized approaches, such as screening newborns for treatable diseases, such as phenylketonuria, have already made an immeasurable contribution to the health and well-being of many people. Some forms of cancer treatment, such as Herceptin, in spite of their costs have saved many lives and are continuing to do so; it is, however, very difficult to believe that an equitable access to the highest possible standard of care will be easily available for all.

It is very speculative to discuss to what extent PM could benefit all – the cost/benefit ratio is something which can only be analyzed for a number of diseases, and in many cases the cost is very high. The benefits to individual patients may be long term (as for the gene therapy by Glybera or for cystic fibrosis therapy by ivacaftor) or shorter (as is the case for targeted cancer treatments, in which resistance develops in many cases after a shorter or longer period of time). In most cases, the group which can benefit is a small one – the disease for which Glybera is used is an extremely rare one (one person per million), and in the case of ivacaftor about 1 person in 2500 among Caucasians has cystic fibrosis, and only a few percent can benefit from this particular drug.

Thus, currently the target group is not large, and there are not that many drugs which are already used, but already there may be problems – only a small percentage of lung cancer patients can benefit from a currently available targeted drug, and for patients who would not benefit from this particular treatment, there are no equivalent alternatives.

The main problem again is an economic one – can one million euro be refunded by any medical care system for the one patient per million who can benefit from Glybera when this amount of money could be used for drugs for more common diseases which are much cheaper to treat.

Conclusion

We are living in a period of incredible development of the biomedical sciences. Precise diagnosis of many diseases is now possible and, for increasing numbers of them treatment, either by appropriate drugs or drug combinations or (in some cases) by gene therapy, is becoming available. The concept of personalized – or precision medicine – in which each patient had his needs addressed by a specific, adapted treatment is a wonderful idea, and in richer countries more and more people are benefiting from this. However, this is still in its infancy, although some effects of personalized medicine – such as saving people from the profound effects of phenylketonuria on the central nervous system by early detection and an appropriate diet and other achievements of screening newborns – should be remembered as a real effect of a targeted strategy for a large number of people. It is not clear how and whether these new and expensive treatments

will become available to persons in poorer countries. The fundamental ethical problem linked to this ongoing development of science is that of an equitable distribution of these advances. This is not the only one – most diseases are not due to single causes, and for most of them there is no chance of a magic bullet, though many companies are looking at multiple markers of lifestyle diseases in the hope of finding "druggable" targets.

Cross-References

- ▶ Gene Therapy
- ▶ Genomics
- ▶ Pharmacogenomics

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Pharmacogenomics

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Abstract

This entry explores the ethical questions related to pharmacogenomics not only in the clinical practice but also in the field of research. Since everyone is the titleholder of their own information, it is essential to obtain their informed consent and provide safeguards concerning both confidentiality of and access to this information at all times. At the same time, it is crucial that these scientific advances contribute to a global justice and not to increase inequalities among individuals and communities.

Keywords

Pharmacogenomics; Informed consent; Confidentiality; Research ethics; Genetic tests; Justice

Introduction

There is some degree of confusion surrounding the two terms pharmacogenetics and pharmacogenomics because some people use them as synonyms while others to refer to distinct fields of science. Pharmacogenetics is understood as the science which studies the impact of genetic variation in response to drugs, while pharmacogenomics is concerned with the same objectives but incorporates new tools from the genetic age such as gene expression profiling, proteomics, or bioinformatics. The definition of these two terms endorsed by the International Conference on Harmonization (ICH), the body which comprises the drug regulatory authorities from Europe, the USA, and Japan, coincides only in part with those given above. According to the ICH, pharmacogenetics is "the study of variations in DNA sequence as related to drug response," while pharmacogenomics is "the study of variations of DNA and RNA characteristics as related to drug response" (ICH 2008, p. 2).

The term pharmacogenetics was coined in the mid-1950s to refer to a new discipline which studies genetically conditioned pharmacological reactions. The term pharmacogenomics, which first appeared in scientific publications in 1997, began to evolve along with the technologies which made possible the sequencing of the human genome and set out a more comprehensive and rigorous view of how a person's response to drugs relates to their genetic characteristics. Although both terms are still used as synonyms, it is pharmacogenomics which has upstaged pharmacogenetics and is the more commonly used term.

Pharmacogenomics is a very promising field of scientific knowledge, one which poses ethical questions of enormous relevance in terms of clinical practice as well as for research and which affects individual human rights as well as social justice. There are three international regulatory documents sanctioned by UNESCO which set out the legal principles which should act as a guide in these matters: the Universal Declaration on the Human Genome and Human Rights (1997), the International Declaration on Human Genetic Data (2003), and the Universal Declaration on Bioethics and Human Rights (2005). Noteworthy at a European regional level is the regulating role of the Council of Europe through the Convention on Human Rights and Biomedicine (Oviedo Convention 1997) and its Additional Protocol concerning Genetic Testing for Health Purposes (2008). The section which follows concerns the aim and importance of pharmacogenomics and the ethical problems which it raises regarding how it is applied to clinical practice as well as research and social justice issues. It also takes into account the international regulations mentioned above, not only because states are required to follow them to one degree or another, but because they also illustrate an ethical consensus on this matter at a global level.

Aim and Importance of Pharmacogenomics

Pharmacogenomics provides us with extraordinarily useful information for enhancing medication safety, in that it can anticipate adverse reactions associated with certain genetic characteristics. It likewise increases drug efficacy because it facilitates prescribing those drugs which would be most effective and in the correct dosage on the basis of an individual's genetic characteristics. Accordingly, we can move toward drug prescriptions which are no longer determined by the "one pill fits all" principle and instead prescribe "the right patient with the right drug at the right dose at the right time" (Marshall 1998, p. 1). The potential opportunities it offers are many: predicting a patient's response to drugs; developing "customized" prescriptions; minimizing or eliminating adverse events; improving efficacy and patient compliance; improving rational drug development; improving accuracy when determining appropriate dosages of drugs; screening and monitoring certain diseases; developing more powerful, safer vaccines; and allowing improvements in drug R&D and the approval of new drugs (Issa 2002). This all amounts to major savings in healthcare costs because it prevents those with certain pharmacogenetic profiles from

taking counter-indicated drugs and it reduces hospital care costs in terms of dealing with the iatrogenic effects of certain drugs.

However, the great expectations which pharmacogenomics has generated since the end of the twentieth century, while the human genome was being sequenced, are still a long way from being fulfilled. Those drugs whose effects are known depending on a person's genetic variations are few in number. Furthermore, the knowledge which pharmacogenomics provides itself is not usually sufficient to determine the appropriate pharmaceutical prescription because both pharmacodynamics (the mechanisms of a drug action on the organism) and pharmacokinetics (the processes a drug undergoes from the moment it is administered until it is eliminated by the organism) vary according to many other circumstances and not just an individual's genetic variation (Tutton 2012). As has likewise happened with the Human Genome Project in general, the move from research to clinical practice in pharmacogenomics has proved to be much slower than initially believed (Godman et al. 2013).

Before tackling the ethical questions related to pharmacogenomics, three factors need to be considered by way of a preliminary. First, our knowledge of pharmacogenomics is mainly based on pharmacogenetic tests on individuals. Genetic tests identify mutations associated with diseases and provide a person with decisive information about their future health or procreative options. However while pharmacogenetic tests provide valuable information, this is not always as crucial as other information, because in principle they only provide information about the genes involved in metabolizing the drug (Roses 2000). For those people identified as not suited to a given drug, this information comes as a negative, but even in these cases it is positive for them in that it prevents them from being exposed to a drug which could harm rather than cure them. Nevertheless, these differences in the type of information these tests provide are not always so clear-cut because the pharmacogenetic test can also provide information about a person's state of health or the genetic characteristics of other family members. Given this, when it comes to establishing guarantees which protect the information from one or other tests, we need to begin with the type of information they provide and not simply the type of analysis being carried out.

Second, with the development of the Human Genome Project, a degree of genetic determinism began to hold sway, according to which knowledge about the human genome was going to provide the key to solving health-related problems. The impact of other considerations, such as the importance of the environment and individual lifestyles for taking care of one's health, lost ground to the protagonism given to an individual's genetic makeup. However, since knowledge of the genome did not automatically manifest itself in medical practice, this view began to lapse. Given the risks involved in assigning too much importance to genomics and pharmacogenomics, it needs to be stressed that while these sciences provide the field of medicine with necessary information, it is far from sufficient.

Third, the great expectations at the end of the twentieth century to develop predictive medicine based on genetic science, and which generated enormous investments by the richest states in the world, are a long way from being fulfilled. Meanwhile, millions of people still live without an acceptable level of healthcare, and there has been a brake on assigning resources for less spectacular research projects, but which could have given short-term benefits in clinical practice. It is imperative that resources in the field of biomedical research and healthcare are distributed in accordance with criteria of justice and opportunity and not vested interests at any given moment.

Ethical Questions in Pharmacogenomics

The aim of pharmacogenomics is to obtain information about an individual which is highly valuable but also very sensitive. Since everyone is the titleholder of their own information, it is essential to obtain their informed consent and provide safeguards concerning both confidentiality of and access to this information at all times. The international regulations concerning genetics are unanimous about the individual's right to

self-determination in such matters, embodied in the right to informed consent and confidentiality of information, and to access to their genetic information (Romeo-Malanda and Nicol 2008).

Informed Consent

The aim of a pharmacogenetic test is to assess the capacity to metabolize a given drug according to one's genotype. This information comes as a negative if it reveals that a person cannot benefit from a drug suited to curing a disease they are suffering from. There are times when pharmacogenetic tests can provide information about genetic diseases or the risk of suffering from them because, sometimes, the genetic mutations which affect drug response can cause, or mean that a person is predisposed to, a disease (Buchanan et al. 2002). Therefore, pharmacogenetic tests can also reveal information which can have consequences at a personal, medical, family, or social level, so those who undergo these tests must be duly informed beforehand about these aspects (Robertson 2001).

Before anyone undergoes a pharmacogenetic test, they should be informed, at the very least, about the following: the exact type of information that will be obtained with the DNA sample; the risks and benefits regarding the information the test is searching for, including psychological and social considerations; where the DNA sample will be sent (if it will be stored and under what conditions or whether it will be later destroyed); how privacy will be guaranteed concerning both the sample and the information obtained; and the final destination and future uses of the DNA samples and of the information obtained.

Informed consent in genetic analyses about diseases poses many problems, some of which are common to pharmacogenetic tests. Firstly, genetic information tends to be difficult to understand and handle for test subjects, so it may well happen that they do not exactly know what they are giving their informed consent for. Secondly, we are dealing with sensitive information: it transcends time (since it does not only refer to a single moment in time) and the subjects themselves (because it frequently affects their family, a given community, or racial group). Thirdly, this information says a lot about the subject and could end up having an impact on how they see themselves (Chadwick 2004). To guarantee that consent to pharmacogenetic tests is free and the result of a truly informed decision, it is essential for the subjects to be given appropriate pharmacogenetic counseling, not only after obtaining the test results but also prior to giving consent (Bellver 2002).

Direct-to-consumer pharmacogenetic tests pose specific problems. Given the nature of the information, it seems reasonable that informed consent should also be required and that it should be accompanied by genetic counseling. But in these tests, it is far more difficult to fulfill these guarantees. In any event, the Additional Protocol to the Convention on Human Rights and Biomedicine, concerning Genetic Testing for Health Purposes (2008), requires that "a genetic test for health purposes may only be performed under individualized medical supervision" (art. 7.1).

Confidentiality

Although in principle the information from pharmacogenetic analyses is not of such a transcendental nature as that provided by genetic analyses on disease, it should also be confidential. Pharmacogenetic analyses reveal which patients do not respond adequately to certain drugs. Except when there is a feasible alternative treatment, these patients generate considerable healthcare costs, since it is more difficult to deal with their disease (Buchanan et al. 2002). For this and other reasons, breach of confidentiality can easily lead to social stigmatization or discrimination. Guarantees concerning the protection of confidentiality must be sufficient enough so that people do not refuse to take part in these tests for fear of a possible future breach of confidentiality.

A dilemma which doctors can be faced with when prescribing a pharmacogenetic test is whether they should inform family members of the results of the subject's test when it provides information which could also prove valuable for them. In principle, it is the test subject who is the titleholder of this information, and it should be he/she who gives consent to share this information (Ellul 2015).

On occasions, drug response variability is associated with some genetic variations which are not exclusive to an individual but rather to a group or community to which the individual belongs. In these cases anyone could be informed about the genetic problems of the members of these groups without having to carry out a pharmacogenetic analysis. When such situations crop up, it is common to resort to the protection of communities against risk of discrimination principle (Weijer 1999).

The question has been posed as to whether the community should be informed when the test on an individual could provide information which would affect an entire community. While it may seem reasonable to consult with the corresponding community authorities, we should not lose sight of the fact that the prime moral subject of consideration should always be the person himself/herself (Anderlik and Rothsein 2001).

Pharmacogenetic Research

We are still a long way from these tests being incorporated into habitual clinical practice on a major scale basis. For the time being, pharmacogenomics is mainly being developed at the research level and should be subject to generally accepted ethical principles for such research which are set out in the Helsinki Declaration (1964, last amended in 2013). The following paragraphs only set out some of the most relevant ethical issues regarding pharmacogenomic research.

As is the case in clinical practice, pharmacogenomic research must be carried out with full respect for individual rights. The main mission of the Research Ethics Committees is to guarantee both formal and real respect for these rights. These committees take on even greater importance when the decision is made to disassociate the names of people from their biological samples, since in these cases extreme caution and supervision must be exercised regarding what is done with these samples (Erice Declaration 2002).

There are times when researchers want to make use of these biological samples in further research. This is only possible if the sample subject has given her or his sufficiently broad consent which allows for these new research projects. Although this necessarily involves having broad consent, it is essential that this is not vague and gives the subject an approximate idea of future uses that his/her sample could be used for (Ellul 2015). If not, then we are not talking about real consent.

Pharmacogenetic information means that the number of people participating in clinical trials can be reduced since it can rule out those subjects who would not respond well to the drug. Consequently it can protect the health of people and also lower the costs of developing new drugs. Furthermore, it makes it possible to develop new drugs that had previously been ruled out because they were unsafe for many subjects. However, when it comes to recruiting test subjects, we also need to allow for the fact that excluding subjects with certain genetic variations prevents us from obtaining further information about the drug that could prove valuable when it comes to prescribing it.

Justice Issues in Pharmacogenomics

There are a number of justice issues posed by pharmacogenomics, and the following paragraphs refer to three: (1) relationships between the pharmaceutical industry, healthcare suppliers, and patients; (2) fair distribution of resources within a given society and worldwide; and (3) new training and behavior standards required of doctors and pharmacists.

1. *Relationships between the pharmaceutical industry, healthcare suppliers, and patients.* It can happen that pharmaceutical companies link sales of their products to acquiring the corresponding pharmacogenetic tests for these products. Should we consider this practice acceptable or not? The pharmaceutical industry can provide two solid arguments. First, performing a pharmacogenetic test prior to dispensing the drug guarantees drug efficacy and additionally prevents unwanted effects. Second, if the drug is used by subjects who are not ideal from a pharmacogenetic perspective, this can have detrimental effects on the health of the individual in question and

healthcare costs and is prejudicial to the company's product/brand image. Given this situation for the pharmaceutical industry, the best option is for them to be governed by drug regulation agencies such as the FDA (USA) or the EMA (European Union), who can determine whether it is necessary, advisable, or irrelevant to perform a pharmacogenetic test before deciding whether or not a drug can be prescribed.

It can be the case that it is the healthcare provider that wishes to perform a pharmacogenetic test before making the drug available. What in principle is a cautionary measure can sometimes become nothing more than a defense mechanism against possible future liability. Once again, the solution is for drug regulation bodies to establish clear guidelines.

2. *Fair distribution of resources within a society and worldwide*. Pharmacogenomics allows us to discover which individuals or groups have genetic variations that prevent them from benefiting from a given drug. From the perspective of profitability, the tendency would seem to be in favor of developing drugs which ideally suit majority groups and to overlook the research of drugs which would prove particularly beneficial for minority groups. If the minority group in question has sufficient financial resources, then almost certainly the necessary research would be financed until a suitable drug is obtained. If however the group is an economically disadvantaged racial or social minority, they are in risk of being overlooked.

   However, it does not have to be like this. First of all, pharmacogenetic analyses only forecast what would be discovered as soon as patients with a genotype which is not apt for this drug began the treatment. Therefore, pharmacogenomics in principle does not discriminate but rather is limited to warning of the risks that a drug represents for an individual or group. Secondly, the cost of developing new drugs can be considerably reduced by incorporating pharmacogenetic analyses, making research into orphan diseases more feasible, and as a result the development of orphan drugs which are suited to those who, because of their genotype, cannot use the common drugs (Olivier and Williams-Jones 2011). Finally, governments would have to take steps to provide incentives to developing research whose costs could not be recovered under normal market conditions and, at the same time, ensure through funding that the price of drugs designed for minority groups is not exorbitant.

3. *New training and behavior standards required of doctors and pharmacists*. It is likely that in the not too far distant future, pharmacogenetic tests will become a common tool when prescribing drugs for patients. At that moment, doctors will need to have the necessary competence to incorporate pharmacogenomics into their daily clinical practice. On the other hand, as pharmacotherapies are going to be increasingly linked to a patient's genetic characteristics, pharmacists will also need to have access to this knowledge. Moreover doctors and pharmacists will need to be adequately trained to explain to patients all the necessary information so that they can give their truly free and informed consent to pharmacogenetic tests. They also will need to be trained to be able to give appropriate genetic counseling. Given this immediate future need, it is only reasonable to expect pharmacogenomics to be incorporated into university training of these future healthcare professionals (Gurwitz et al. 2005).

Conclusion

Pharmacogenomics is a science in its early stages which studies the impact of drugs on individuals according to their genetic characteristics. With this knowledge, the aim is to develop the most effective drugs and dosages tailored to the individual. The advent of pharmacogenomics toward the end of the last century generated great expectations; however, transferring the results of advances in pharmacogenomics to clinical practice has been somewhat slow. In part, this is because the variability in drug response does not depend exclusively on individual genetic variation.

Although they contribute different types of information, it has not been possible to establish

a qualitative difference between disease-related genetic tests and pharmacogenetic tests. The self-determination of the subject has to be assured in each and embodied in informed consent, data confidentiality and privacy, and genetic counseling. The requirements of informed consent, confidentiality, and privacy must also be fulfilled when pharmacogenetic tests are research oriented.

It is important for pharmacogenomics to contribute to improving the healthcare of everyone and not to increasing the gap between the rich and the poor. For this to happen, public authorities must motivate the pharmaceutical industry to develop orphan drugs and make pharmacogenetic tests available for everyone. Finally, there is an urgent need to improve training for healthcare professionals in relation to pharmacogenomics so that they can offer a professionally irreproachable service.

Cross-References

- ▶ Consent: Informed
- ▶ Genetic Determinism
- ▶ Genomics
- ▶ Predictive Medicine
- ▶ Research: Data Sharing
- ▶ Stigmatization

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Phenomenology

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Abstract

Phenomenology is a research tradition in German and French philosophy which has had an influence in many other fields and disciplines, recently also in medicine and nursing. The main idea of phenomenology is to study the structure and content of lived experience from the first-person perspective. This entry provides an overview of different ways in which phenomenology has proved useful as a method and inspiration for bioethics so far and how it could do so in the future. Phenomenological bioethics can be carried out either as an integrated part of, or as a critical perspective on, principle-based bioethics. Phenomenology can be used either to inform the application of principles by way of describing the experiences of moral dilemmas or to criticize the contemporary setup of bioethics and offer alternative approaches. The critical alternatives may be more or less radical in nature: offering alternative principles or abandoning the idea of application altogether. Phenomenological bioethics may also be viewed as an attempt to strengthen and thicken the philosophical anthropology implicitly present in contemporary bioethical studies by focusing on themes such as body, psyche, life, death, authenticity, suffering, vulnerability, empathy, compassion, integrity, dialogue, gift, and responsibility.

Keywords

Phenomenology; Embodiment; Empathy; Lived experience; Life world; Authenticity; Responsibility; Narrative; Philosophical anthropology

Introduction

In what ways is the philosophical tradition of phenomenology able to contribute to bioethics? Phenomenology has been rather absent in the bioethical field up to the present date, but some examples of phenomenology are found in the neighboring fields of philosophy of medicine and medical humanities and in qualitative studies of medical phenomena carried out in disciplines such as nursing, medical psychology, medical sociology, medical anthropology, and science and technology studies. Phenomenology has also had an impact in certain subfields of bioethics, such as caring ethics, narrative ethics, and feminist ethics. These subfields have played important roles as alternatives to the principle-based bioethics that has developed into the mainstream tradition in the field. In this entry an overview will be provided of the different ways in which phenomenology could prove useful as a method and philosophical inspiration for bioethics. The possibilities, as will become obvious, are multiple and to a large extent they still remain to be realized.

Phenomenology of Medicine and Health Care: History and Definitions

Phenomenology is a tradition more than 100 years old, exploring and answering philosophical questions by proceeding from an analysis of lived experience; important classics are philosophers like Edmund Husserl, Martin Heidegger, Maurice Merleau-Ponty, and Jean-Paul Sartre (Moran 2000). The starting point for the phenomenologist is not the world of science but the meaning structures of the everyday world, that which the phenomenologist calls the "life world." Contemporary phenomenology has branched out into many different disciplines from the tree that started growing in philosophy with Husserl and his successors. Scholars and researchers of art, literature, psychology, sociology, anthropology, pedagogy, history, and recently, also, nursing and medicine have tried to make use of phenomenology in investigating phenomena of concern in their field.

The main topic of phenomenology of medicine and health care so far has been bodily experiences – experiences of phenomena such as

illness, pain, disability, giving birth, and dying (Toombs 2001; Zeiler and Käll 2014). Everybody has a body – a body which can be the source of great joy but also of great suffering to its bearer – as patients and health-care professionals know more than well. The basic issue that the phenomenologist would insist on in this context is that not only does everybody *have* a body but everybody *is* a body. What is the difference?

That every experience is embodied means that the body is a person's point of view and way of experiencing and understanding the world. Not only can I experience my own body as an *object* of my experience – when I feel it or touch it or look at it in the mirror – but the body is also that which makes a person's experiences possible in the first place. The body is my place in the world – the place where I am which moves with me – which is also the zero place that makes space and the place of things that I encounter in the world possible. The body, as a rule, does not show itself to us in our experiences; it *withdraws* and so opens up a focus in which it is possible for things in the world to show up to us in different meaningful ways. When I speak to another person, I am not attending to the way my body feels and moves, but I am focused upon *her* and the things I am trying to communicate to her; this is made possible, however, by the way my body silently performs in the background. The body already organizes my experiences on a subconscious level; it allows me to experience the things that are not me – the things of the world that show up to my moving, sensing body in different activities through which they attain their place and significance (Gallagher 2005).

Thus phenomenology can be understood as transgressing any dualistic picture of a soul living in and directing the ways of the body like some ghost in a machine. The body is me. But phenomenology is also – and this is even more important – fundamentally from its beginnings an anti-naturalistic project; that is, the phenomenologist would also contest any attempt to reduce experience to material processes only. Experience, to the phenomenologist, must be studied by acknowledging its form and content for the one who is having the experience. Experience is always *intentional* in the sense of presenting something in the world *to* a subject. It is certainly possible to study experience from the third-person perspective of science also – we could study the ways light rays trigger nerve firings in my brain by way of the retina when I look at a person right now (if we hook me up to a technological device), but this picture of my brain in action would not be the experience of "me seeing him/her right now." The picture could catch neither the "me-ness" nor the content of the experience that I am having – this is the first-person perspective of intentionality, which the phenomenologist takes as the starting point of the analysis.

Among important medical themes that can profit from a phenomenological analysis, we find not only embodiment itself but also related phenomena such as illness, disability, pain, dying, and giving birth (Carel 2008; Svenaeus 2000; Zeiler and Käll 2014). One finds elements of such analyses in the works of the major phenomenologists, such as Husserl, Heidegger, Merleau-Ponty, and Sartre, and even more in some less well-known figures, such as F. J. J. Buytendijk, Hans Jonas, Herbert Plügge, and Erwin Strauss, but the idea of a phenomenology of medicine as a distinct field of academic studies is younger than that, maybe 30 years old or so (Toombs 2001). Long before that, however, phenomenology had a certain influence in one specific medical speciality, psychiatry, with scholars such as Karl Jaspers.

Phenomenological bioethics can be thought about as the part of the phenomenology of medicine and health care that focuses on *ethical* dilemmas in addition to and in connection with themes such as the ones just mentioned. Since the major phenomenological philosophers in the process of their explorations of lived experience and the life world of human beings have developed various types of ethical analyses, phenomenological bioethics could also make use of these moral reflections in a more direct way (Drummond and Embree 2002). This has, as mentioned above in the introduction, already occurred to some extent in fields such as caring ethics, feminist ethics, and narrative ethics. The details of phenomenological ethics will be dealt with below.

The ways phenomenological theories have been brought to the domain of bioethics are consequently several, and mostly indirect. Phenomenology has entered bioethics via the philosophy of medicine and medical humanities in studies of themes, such as embodiment, pain, and illness, or via parts of bioethics that go under different names than phenomenology, such as caring ethics, feminist ethics, and narrative ethics. In what follows, an overview will be provided of how the points of connection between phenomenology and bioethics could be systematized and thought about in a global context. The main concern is the question of what it may mean to do bioethics in a phenomenological manner, and the aim is to offer a structure that can encompass many different understandings of phenomenological bioethics by offering a generous interpretation of both concepts. Phenomenology will be considered as a tradition that is related to hermeneutics as well as poststructuralism and such neighboring schools will be included in the overview at certain points. The ways in which phenomenological bioethics is related to older traditions that have influenced not only phenomenology but also bioethics will be taken into account: mainly as concerns Kantian ethics and Aristotelian virtue ethics.

Phenomenological Bioethics

Different Understandings of Phenomenology in Medicine and Health Care

Proceeding from the way the term phenomenology has been used in studies in and of medicine and health care, at least three different understandings of the concept might be discerned. These three, more or less established, understandings of phenomenology are helpful in drawing a map of phenomenological bioethics.

The first, and probably most dominant, understanding of phenomenology in medicine is to give adequate and detailed *descriptions* of experiences and situations of importance. A doctor may speak about the "phenomenology of a case," for instance. This is certainly a correct and important understanding of phenomenology, but as has become obvious from the overview of the phenomenological tradition above, it is insufficient to get hold of the full meaning of the concept. The second understanding of phenomenology, less well-known in medicine, is that phenomenology is a *research program in philosophy* starting out with Husserl in the early twentieth century and including philosophers such as Heidegger, Merleau-Ponty, and Sartre but also names like Max Scheler, Hans-Georg Gadamer, Karl Jaspers, Paul Ricoeur, Emmanuel Levinas, Hannah Arendt, Michel Foucault, Jacques Derrida, and many others. According to such an understanding, phenomenology is not only a descriptive but also a theoretical and conceptual endeavor. The third understanding of phenomenology common to studies in and of medicine and health care is phenomenology in the sense of a *qualitative research method*, a method inspired by the philosophical tradition of phenomenology in which the researcher aims to give voice in an unbiased way to the experiences of research subjects. The phenomenological method can be applied in gathering and analyzing empirical materials consisting of interview transcripts, video recordings, field notes, diaries, etc. This is a common understanding of phenomenology in nursing research.

The three understandings of phenomenology obviously have a lot in common, and they support each other in offering a more complete account of what phenomenology is about in medicine and health care and, also, in bioethics. The conceptual endeavor is dependent upon having access to adequate and detailed descriptions of the type of lived experiences that are to be investigated, and the empirical research method proceeds from some of the points of departure stressed by Husserl and other phenomenological philosophers from the very start.

To see this more clearly, a practical example will be enlightening: the diagnosis of depression. What does it mean to develop a phenomenology of depression in contrast to other ways of understanding this psychiatric disorder? The phenomenologist will not primarily be interested in the ways the brain works when somebody is depressed. Nor will he take his starting point in cataloging typical symptoms of depressions in contrast to the symptoms of

other psychiatric disorders or somatic diseases, the way it is typically done in contemporary diagnostic psychiatry. Instead the phenomenologist will be interested in what Heidegger calls the "being-in-the-world" of the depressed person (Heidegger 1996). What does it feel like to be depressed? How does the world appear to the depressed person and what sorts of thoughts does she have? What does she want to do (or rather not want to do), and how does she communicate and feel related to other persons (or rather not communicate and feel related to other persons)? To investigate these issues thoroughly, the phenomenological researcher needs to talk to depressed (or formerly depressed) people, at least if she is not herself (or has not been) depressed. Through the phenomenological analysis in which the typical structure and meaning of the lived experience of depression should be revealed, the researcher will perhaps come to the conclusion that depression consists in a particular form (or maybe several forms) of suffering in which the autonomy (and possibly also the integrity and authenticity) of the depressed person has become threatened. To be depressed means to live in a world that appears senseless and fearsome and in which the depressed person is losing her bearings and sense of self-worth and self-respect.

Phenomenology and the Application of Principles

Take notice that already in this sketchily developed phenomenology of depression, some of the central notions in bioethics – suffering (the principles of doing good and avoiding harm) and autonomy (possibly also other principles connected to personhood, such as the duties to respect and enhance integrity or authenticity) – are encountered. In principle-based bioethics, the central notions are phrased in the manner of prima facie principles that are applied in situations that demand better moral understanding because of not being sure what is the right thing to do (Beauchamp and Childress 2013).

Let us say that a depressed person finds her life so hopeless and horrible that she wants to die. Should her decision be regarded as autonomous, and is she really right about her current situation and prospects in life? Leaving aside the problem of whether euthanasia should ever be allowed, it is obvious that the answers to these questions depend on the understanding we are able to develop of the lived experiences of the depressed person and of other parties engaged in the current situation (family, friends, health-care workers who have been in contact with the depressed person, etc.). To ask the question of whether the depressed person is to be considered autonomous in her wanting to die, and whether the suffering that she experiences can be remedied in some manner by intervening, is most often phrased as an *application* of the principles to the current situation (other important principles in bioethics besides respecting autonomy, doing good, and avoiding harm are the creeds of justice, empathy, veracity, confidentiality, and fidelity).

In this setup of the practice of bioethics as applying ethical principles to morally problematic situations in order to better understand the situations at hand and finding out what to do in them, one important role for phenomenological bioethics can be detected. Phenomenological bioethics, according to such an understanding, consists in carrying out investigations of the lived experiences – being-in-the-world – of the persons involved in morally problematic situations in order to better see how the main principles of bioethics will apply. It means very little to say that autonomy should be respected if you have not developed an understanding of a particular person's situation and self-understanding. Likewise, it means very little to say that a patient should be helped and not harmed if you have not developed an understanding of the more precise ways this particular person (and possibly other people related to her) is suffering.

Phenomenological bioethics in this form will be a close relative to what is known since the 1990s as *narrative* bioethics (Lindemann Nelson 1997). To provide the phenomenology of an ethically tricky situation in health care means to give voice to the stories of the participants in the drama, throwing light on the experiences and different points of view on the situation and problem at hand. Phenomenology done by way of stories will develop into a hermeneutical undertaking because the embedded character of every lived

experience in culturally narrated patterns will lead to questions regarding *interpretation*. The phenomenological analysis will supplement and strengthen the narrative approach, since phenomenology is proceeding from the *embodied* life-world account which makes out the basis of every narrated life plot in which human life and even personhood itself can take on culturally constructed patterns (Wiggins and Allen 2011).

Phenomenology as a Critique of Principle-Based Bioethics

In the first version of phenomenological bioethics delineated above, phenomenology forms part of applicative ethics by providing the careful and adequate descriptions of the terrain in which the ethical principles are to be applied. Phenomenology takes care of one of the balancing scales of what is referred to as a "reflective equilibrium," the side on which our moral intuitions are taken into account (Beauchamp and Childress 2013). Phenomenological bioethics accordingly studies the lived experience of moral conundrums that are then to be determined via application of prima facie principles originating from ethical theories, such as utilitarianism, Kantian ethics, and liberal-based rights ethics.

In the process of applying ethical principles, there is a mutual process of explication and illumination going on. To apply the principle of respecting autonomy to the case of the depressed patient makes us see new aspects of the situation, but the phenomenological investigation of the lived experience of being depressed also informs the matter of what it means to be an autonomous person. In this process of balancing the understanding of the case and the principles to be applied to it, phenomenology can take on a more critical role than simply providing the details of the case. Phenomenology may be regarded as an attempt to set up bioethics by way of *alternative* principles, or in a manner that is *not* understood as a procedure of application of principles in the first place (Welie 1999; Wiggins and Allen 2011).

Inspiration for such endeavors can be found by proceeding from the ethical theories found in classic phenomenologists such as Max Scheler, Emmanuel Levinas, Jean-Paul Sartre, Hans Jonas, Hannah Arendt, and many others (Drummond and Embree 2002). These phenomenological moral theories have different emphases, but they are all united in questioning the firm distinction between what *ought* to be done (ethical principles) and what *is* the case (the situation we are applying the principles to in the moral analysis). In the next section, brief introductions to some major moral philosophers from the phenomenological tradition will be offered.

Some Major Examples of Phenomenological Moral Philosophers

In the works of Max Scheler, particularly during the 1910s and 1920s, we find an attempt to extract norms from the phenomenological analysis itself, what philosophers refer to as "objective value theory." This is mainly done via an understanding of the *feelings* we experience in situations in which we encounter other persons and important life goals. Empathy, sympathy, compassion, and love are examples of such feelings, which provide a path to the most important values to be realized in a human life. The concept of personhood is interpreted to involve a hierarchically determined set of values involved in this endeavor of living in an ethically respectable or even excellent manner. The challenge to such a phenomenological framework, as we find it in Scheler, is to prove the ethical values to be objective (global) in nature in contrast to just reflecting particular sets of culturally established norms. This is a question that is especially pertinent in the framework of global bioethics, in which moral relativism appears as a constant threat. Scheler's moral philosophy is nevertheless of particular interest to bioethics because of its starting point in strong feelings connected to situations involving suffering, death, empathy, and compassion.

In Emmanuel Levinas, we also find a phenomenological ethics that takes its starting point in the interpersonal meeting, spelling out the norms that are at stake there. However, this time the phenomenology of encountering the other person is interpreted as a radical asymmetry in which the "face of the other" forbids me to do any violence to her. This violence potentially done to the other person in the meeting is understood not only as a

physical violation but also as an imposing on her of any norms that belong to my point of view rather than the other's. Levinasian ethics can be understood as an ethics of integrity, defending the rights of the weak and vulnerable, but in essence it rests on a more radical claim that makes me the hostage of every other person I encounter. The critical questions to ask the advocates of a Levinasian ethics are if such an ethics does not put too high demands on the persons who are to exercise it, and if it is not possible that the other person may have some obligations to me in the meetings we enjoy. Nevertheless, the asymmetrical setup of Levinasian ethics appears to fit the obligational structure of health-care meetings between professionals and patients in important ways and is therefore of particular interest to bioethics.

Levinas and Jean-Paul Sartre developed their moral philosophies at roughly the same time: the 1940s and 1950s, which were also the heydays of existentialism. In Sartre the primary ethical concepts are freedom and responsibility. Not an infinite responsibility for the other person – the way things are framed in Levinasian ethics – but an infinite responsibility for my own life and what I choose to do with it (including my interactions with other persons, certainly). The existentialist ethics of freedom appears to resemble autonomy-based approaches to ethics, such as we find them in versions inspired by Kantian ethics or liberal ethics. However, Sartre is critical of Kantian philosophy, since he does not believe that any universal moral rules can be philosophically defended – categorical duties like the prohibition of killing or the obligation to always tell the truth. For Sartre what makes a decision ethical is that *I* aspire to the action in question no matter what public norms and duties may prevail. This looks suspiciously similar to a liberal, autonomy-based ethics – do with your life whatever you find fitting as long as you do not interfere with the capability of others to do the same – but it really is not, since Sartre understands every choice to be considered not only as a contingent wish but as an expression of who I really am. Sartrean ethics is an ethics of authenticity rather than an ethics of autonomy. The criticism of existentialist ethics could be voiced in many ways. Are people with different histories and abilities free to realize their potentials to an equal extent? In what manner are the perspectives and situations of other persons taken into account in my endeavor to live authentically?

The final question in the previous paragraph is precisely the point of departure for Hans Jonas in *The Imperative of Responsibility*, a work written in the late 1970s in which he takes on the challenge of how to face the threats of modern technology (including biomedical technologies) to make the planet uninhabitable (1984). Jonas, a former student of both Husserl and Heidegger, claims that we have an overarching ethical responsibility to save and protect the earth for future human generations. This responsibility becomes visible with a special acuteness in certain key situations – like when we welcome and care for a newborn child. Jonas's work is of special concern for bioethics, since his endeavors to develop a phenomenology of life and death during the 1960s were of interest to the founding fathers of the field, and he was probably the first phenomenologist to later combine such studies of medical phenomena with the development of an ethics (Jonas 1984). Medical technology is increasingly the topic of various forms of bioethical studies, and Jonas can be considered an inaugurator in this subfield of bioethics.

The most widely read work by Hannah Arendt, friend and colleague of Jonas at the New School in New York for many years, is her philosophical analysis on the "banality of evil" following the 1961 trial in Jerusalem of the Nazi war criminal Adolf Eichmann. Arendt claims that the evilness of Eichmann, who was in charge of transporting millions of Jews to the concentration camps, consists in his inability to reflect upon and take responsibility for his own actions. Eichmann pleaded not guilty in the trial (he was found guilty, however, and was executed in 1962) on the grounds of having only performed his duties as a citizen and soldier at the time, and exactly in this inability, or perhaps unwillingness, to understand what he was really doing, we find his "banal" evilness, according to Arendt. Such a moral perspective, distinguishing between a

philosophically reflected way of life and a life in which you think and do as everybody else thinks and does, we find also in Sartre and before him in the famous book that inspired both of them: Martin Heidegger's *Being and Time*, published in 1927 (Heidegger 1996).

In other works than the Eichmann report, Arendt went on to develop her ideas about a philosophically reflected, authentic life into a political philosophy. This political philosophy of Arendt's is inspired by her readings not only of the phenomenologists, Kant, and Marx but also notably by Aristotle and his idea of a "practical wisdom" central to moral human interaction. In Arendt's phenomenological ethics, we consequently find links not only to Kantian ethics but also to virtue ethics – a tradition that has been voiced as a complement to principle-based bioethics from the very start (Pellegrino and Thomasma 1993).

The most influential figure in bioethics descending from the tradition of phenomenology is without doubt Michel Foucault. Foucault did not name himself a phenomenologist, and in many ways he rebelled against the Husserlian tradition of phenomenology in France (Sartre, Merleau-Ponty), but he belongs nevertheless to the twentieth century continental tradition of philosophy of which phenomenology forms the backbone.

The Foucauldian idea that has been most influential in bioethics – and in what in Foucault's legacy is named "biopolitics" – is that our thinking and our actions in the modern age are governed by oppressive norms that make what is natural, good, and just appear to us in a certain unquestioned manner (Mills 2010). Such norms, which he named "technologies of power," govern the way we handle questions of health, sexuality, productivity, etc. In a Foucauldian analysis, the set of principles applied by bioethicists would form an important part of the contemporary prevailing power technologies used to keep the population under control. The principles thus need to be critically scrutinized from a historical and cultural perspective rather than uncritically applied, if bioethics is to prove to be a liberating endeavor.

Phenomenological Bioethics as Philosophical Anthropology

In the two main versions of phenomenological bioethics outlined so far, the field is understood either as an integrated part of, or as a critical outside perspective on, principle-based bioethics. Phenomenology can be used either to inform the application of principles or to criticize the contemporary setup of bioethics and offer alternative approaches. The critical alternatives may be more or less radical in nature concerning the way bioethics should be done: offering alternative principles or abandoning the systematic setup of application altogether. It is typical of the phenomenological moral philosophies presented above that they rather offer metaethical, or, perhaps, proto-ethical approaches than a normative theory in the sense of utilitarianism or liberal rights-based or Kantian ethics. Ethics is not pursued as a development of rules to guide actions but as a spelling out of the meaning of the good and the just in the first place. Levinas's philosophy is prototypical in this regard – the face of the other is what informs and gives meaning to human existence, not something that appears subsequent to identifying the other human being (and myself) as persons. In the same way, the radical freedom approach of existentialist ethics is built up as a philosophy of personhood that needs to be developed in order to even formulate the questions of what actions are good or just in an institutional framework á la contemporary bioethics. In these regards phenomenological bioethics will be similar to the forms of criticism and alternative ways of doing bioethics found in caring ethics and feminist bioethics (Zeiler and Käll 2014).

The discussion about what type of perspective phenomenology is able to offer opens up a third avenue regarding how phenomenological bioethics could be pursued. The field may be viewed as an opportunity to strengthen and thicken the philosophical anthropology more or less visibly present in contemporary bioethics. The concept of personhood in such an analysis is connected to an understanding of concepts such as body, psyche, life, death, authenticity, suffering, vulnerability, empathy, compassion, integrity, dialogue, gift, and responsibility (this list is not exhaustive but it includes some of the most important concepts).

To be a person is not only to be a rational agent, it is to be an embodied, cultural creature relying on intersubjective bonds formed through what the phenomenologist calls being-in-the-world (Heidegger 1996). Bioethicists need to deal with the gravitational points and details of this phenomenology of human being in their analyses.

Rather than abandoning the idea of a reflective equilibrium, this third understanding of phenomenological bioethics could be viewed as a needy contributor to what is called a "wide" reflective equilibrium (Beauchamp and Childress 2013). As developed above, the main idea of a more narrow reflective equilibrium is that the moral principles should be brought into equilibrium with (be balanced by and cohere with) the moral intuitions that we first have when confronted with the situation under ethical analysis. To judge the situation from an ethical point of view will mean going back and forth between moral intuitions and moral principles, letting them mutually enlighten each other. By way of this procedure, one reaches a more complete picture of what is at stake and, ideally, also a wise judgment about what to do in the situation, a judgment that is philosophically informed but still close to the situation at hand by engaging with it emotionally in a considered sense.

This version of reflective equilibrium in bioethics might be too narrow, however, since the scrutiny of intuitions and principles will soon bring the bioethicist to more detailed theoretical considerations regarding the meaning of concepts that are brought up in the analysis. In the methodological version referred to as a "wide reflective equilibrium," these theoretical considerations are explicitly brought into the analysis. This is achieved not only by taking into consideration full-blown ethical theories from which principles are derived but also theories concerning different aspects of what it means to be human, including philosophical anthropology, developmental and social psychology, sociology, and political theory. As developed above with the example of the depressed patient, such considerations will appear in the ethical analysis directly or indirectly as soon as one starts describing the situation to be dealt with and the way persons think and feel about it.

A phenomenological version of philosophical anthropology will concern not only the meaning of personhood and autonomy but also various aspects of human suffering and the intersubjective aspects of being-in-the-world related to empathy and justice. It could play a fundamental role in a clinical ethics, spelling out the structure of the encounters between patients and health-care professionals, but it could also be used to analyze the borderline situations of human life (coming into existence and dying) as well as various technological practices of contemporary biomedicine. Philosophical anthropology is, indeed, a core part of every type of ethical analysis, whether the moral philosopher acknowledges this to be the case or not, and the phenomenological tradition is a rich source in doing that type of reflective, considered moral analysis. In bioethics this means not only to tell rich and adequate stories about ethical dilemmas but also to provide a conceptual structure in which the well-known prima facie principles can be anchored or critically transformed in a sustained way.

Conclusion

Phenomenology has been brought to the domain of bioethics in several, and mostly indirect, ways. Phenomenology has entered bioethics via the philosophy of medicine and medical humanities in studies of themes such as embodiment, pain, and illness or via parts of bioethics that go under names such as caring ethics, feminist ethics, and narrative ethics. In this entry an attempt has been made to tell this history and to systematize three ways in which to think about phenomenological bioethics in the future.

Phenomenological bioethics can be carried out either as an integrated part of, or as a critical outside perspective on, principle-based bioethics. Phenomenology can be used either to inform the application of principles by way of describing the lived experiences of moral dilemmas or to criticize the contemporary setup of bioethics and offer

alternative approaches. The critical alternatives may be more or less radical in nature as concerns the way bioethics should be done: offering alternative principles or abandoning the systematic setup of application altogether. It is typical of moral philosophers in the phenomenological tradition that they offer metaethical approaches rather than normative theories in their own right. Ethics in the phenomenological tradition has not been pursued as a development of rules to guide human actions but as a spelling out of the meaning of the good and the just in the first place.

The discussion about what type of perspective phenomenology is able to offer opens up a third alternative regarding the characterization of phenomenological bioethics in addition to the two approaches just mentioned. The field may be viewed as an opportunity to scrutinize and thicken the philosophical anthropology implicitly present in contemporary bioethics. The concept of personhood in such an analysis is connected to an understanding of concepts such as body, psyche, life, death, authenticity, suffering, vulnerability, empathy, compassion, integrity, dialogue, gift, and responsibility, to mention some of the most important ones. Some of these concepts deal not only with the essence of human being, e.g., embodiment, but also with the normative nature of encounters between human beings, e.g., compassion, and they are therefore normatively saturated in a very obvious sense.

Cross-References

- ▶ Applied Ethics
- ▶ Authenticity
- ▶ Biopolitics
- ▶ Care Ethics
- ▶ Empathy
- ▶ Hermeneutics
- ▶ Medical Humanities
- ▶ Medicine: Philosophy of
- ▶ Narrative Ethics
- ▶ Person
- ▶ Principlism
- ▶ Suffering
- ▶ Trust
- ▶ Vulnerability

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Placebo

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Abstract

The term *placebo* is widely used to indicate a perceived health-related improvement following administration of a treatment lacking a therapeutically acknowledged active component. For a long time placebo response meaning "I shall please" or "I shall be acceptable or pleasing" was something of a mystery, at best linked to but not altogether explicable within the clinical encounter. Attempts at explanation tended to be psychological, based on the probability that response to a placebo was a consequence of the patient's own expectation of benefit, confidence, and trust in the practitioner or unawareness of the "dummy" nature of treatment. What was lacking was a scientific theory and detailed understanding of the underlying psychology, and more importantly knowledge of just how a patient's thoughts and beliefs might result in biochemical and cellular changes which mimic some of those caused by medicinal drugs. Recent research has begun to unravel important aspects of the phenomenon.

The ethical implications of placebo use either in clinical practice or as a baseline comparator in clinical trials turn on important aspects of duty of care toward patients, autonomy, informed consent, and trust.

Keywords

Autonomy; Clinical trials; Consent; Duty of care; Healthcare ethics; Respect; Therapeutic response; Trust

Introduction

Use of the term *placebo* is commonplace in healthcare. Its origin derives from the Latin expression *placebo*, meaning "I shall please" or "I shall be acceptable or pleasing," and has been widely employed in therapeutics to indicate a perceived health-related improvement following administration of a treatment that contains no generally acknowledged active component. Although there are serious ethical reservations, this phenomenon has sometimes been used to "treat" demanding patients who are lacking symptoms relevant to treatment but who might be given an *inert* placebo. The placebo may reinforce a patient's determination or anticipation of getting better. Much more commonly, an inert placebo arm is frequently used in the evaluation of new drugs. The method employs John Stuart Mill's *Method of Difference* (Mill 1858, pp. 230–233), where use of a placebo for comparison provides a baseline in randomized controlled trials (RCTs) permitting the beneficial or deleterious characteristics of drugs to be deduced. The randomized and double-blinded controlled trial (RCT) has been referred to as the gold standard in drug evaluation.

No potential therapy either physical or pharmacological can be rendered immune from the context in which it is administered. Indeed, to state this more clearly, it can be said that the outcome of all therapeutic interventions may be qualitatively or quantitatively influenced both by the process of intervention and by circumstances beyond the properties of the drug itself or the physical therapy technique. This much has been known since 1799 when John Haygarth, a British physician at the Bristol Infirmary and at the Bath Mineral Water Hospital in the UK, conducted a comparative trial with genuine Perkin's metallic tractors (alloy rods claimed to alleviate a range of painful rheumatic and muscular ailments when brushed over the affected region) and simulated rods made from a variety of nonmetallic materials of similar appearance. The results of the trial indicated that patients reported similar beneficial response from both the Perkin's rods and the fake ones. Haygarth concluded that it was the patients' imagination that was responsible and not a characteristic of either type of rod. Somewhat earlier, the Scottish physician William Cullen had employed the term *placebo* in a lecture given in 1772 describing having given a placebo treatment

to a Mr Gilchrist, whom he regarded as "absolutely incurable" and "hastening fast to his fate" (Kerr et al. 2008).

The role of patient imagination or expectation of a beneficial response can be a powerful element in therapeutic treatment. During the Second World War, an American anaesthetist, Henry Knowles Beecher, observed that in the absence of morphine, soldiers given a saline injection by a nurse but told that they were receiving potent pain relief responded well. Following the war, Beecher investigated further and in his paper "The Powerful Placebo" (1955, p. 1606) asserted that "It is evident that placebos have a high degree of therapeutic effectiveness in treating subjective responses, decided improvement, interpreted under the unknowns technique as a real therapeutic effect, being produced in 35.2 ± 2.2 % of cases." That roughly one-third of successful therapeutic treatment is due to the patient's expectation is generally held to be the case, with the major exception of antibiotic or similar therapy.

It is not unreasonable to suppose that before development of a greater understanding of the nature of disease and the pharmacological basis of drug action, it was not possible to clearly distinguish between a beneficial response due to drug action and the influence of the practitioner and/or a patient's self-belief. Retrospective consideration of some of the treatments known in antiquity indicates that although some, such as those derived, for example, from the opium poppy and Ephedra species, and later cinchona bark and digitalis, have manifest pharmacological therapeutic properties, the vast majority do not (Galen's pharmacopeia included some 820 remedies, mostly of questionable value but which largely dominated Western medicine for 1500 years, Czerniak and Davidson 2012). Indeed, Haygarth's recognition of the importance of the patient's imagination in therapeutic response did little to suppress the availability of many established *medicines* of little or no therapeutic value throughout the nineteenth century and beyond, nor quash the continuing appetite for quack medicines by the members of the public.

The role of the community healer and traditional medicine (TM) is ubiquitous throughout the world with a long history. "WHO (2002, p. 1) indicates that "Traditional medicine" is a comprehensive term used to refer both to TM systems such as traditional Chinese medicine, Indian ayurveda, and Arabic unani medicine and to various forms of indigenous medicine. TM therapies include medication therapies – if they involve use of herbal medicines, animal parts, and/or minerals – nonmedication therapies, and spiritual therapies." As with modern Western allopathic medicine which generally supports the biomedical model of disease, traditional medicine is every bit as dependent on a combination of the active principle(s) within a therapy and the influence of the practitioner and/or a patient's expectation or self-belief.

The Mechanism of Therapeutic Expectation

Is It All in the Mind?

Mommaerts and Devroey (2012, p. 44) suggest that "using the term *placebo effect* puts emphasis on *how* an external event (such as the taking of a drug) gets translated into a discernible internal event (such as feeling better or the lowering of blood pressure). In this, the external event is not the final cause but a precipitating factor inducing something within the person that itself leads to the consequences of the placebo effect." Furthermore, although there is a tendency for the term *placebo* to be associated with medicinal drugs, "placebos can also be objects (syringes, a copper bracelet), rituals (physical examination, anamnesis), places (doctor's office), relationships (with doctors, self-help groups), thoughts (about performances of medicine), and other entities such as touch, words, social interventions, a tattoo, or a surgery scar. In fact, anything that has meaning to someone can act as a placebo" (Mommaerts and Devroey 2012, p. 44). In many respects, this is the common link between Western medicine and traditional medicine. All of the items, objects, and procedures associated with therapeutics have meaning within the relevant cultural context and perspective, East or West, and have potential mind–body consequences.

Following Mommaerts and Devroey, we can consider *how* an external event (such as the taking of a drug or placebo or employment of an *inert* device) gets transformed into a discernible physiological or other internal event. The authors identify that placebo relief may be associated with or influenced by expectation or psychological factors such as personal faith or hope or belief, or alternatively by classical conditioning. Although human beings are unique individuals each with a personal spectrum of hopes, desires, and susceptibilities, a high measure of consistency is to be expected within groups that have a shared culture. Classical conditioning theory is associated with the Russian scientist Ivan Pavlov, and his observations with dogs deprived of food which salivated at the sight of Pavlov's assistant (a conditioned response), whereas expectancy theory indicates that placebo effects are products of expectations mediated by explicit (consciously accessible) expectancies. It is not altogether clear which theory is more important or more relevant, with respect to placebo, and it may be that both play a part according to circumstances. For a fuller discussion see Stewart-Williams and Podd (2004) and Klinger et al. (2007).

Irrespective of these considerations, it is perhaps surprising that there is evidence that the use of open-label placebo (pure placebos without deception) may be of value in some cases. In a randomized controlled trial of placebo in irritable bowel syndrome (IBS), Kaptchuk et al. (2010) found that "patients are willing to take open-label placebos and that such a treatment may have salubrious effects. Further research is warranted in IBS and perhaps other illnesses to confirm that placebo treatments can be beneficial when provided openly and to determine the best methods for administering such treatments." Such findings may suggest that some patients can and do suspend rational judgment under clinical supervision even though advised that their treatment is nontherapeutic.

Does Personality Have a Role to Play in Placebo Response?

This would appear to be an important and potentially fruitful area for research to address one of the fundamental questions concerning variable placebo response. In a limited study, Geers et al. (2005) investigated whether dispositional optimism (a generalized positive outcome expectancy for the future) is a possible predictor of placebo response in healthy college students. Dispositional optimism is associated with active behavioral and mental coping when faced with adversity. Their experimental results indicate that dispositional optimism can influence placebo response to experimental pain. The study supports the broader notion that personality variables alter placebo response and that optimism appears to relate to positive placebo responding, whereas pessimism relates to negative placebo responding. Geers et al. theoretical position is that optimism is a moderator rather than a mediator of placebo responding: "Moderating variables are factors that influence the strength and/or direction of the relation between a predictor and a criterion variable, whereas mediating variables refer to the intermediary processes between a predictor and a criterion variable" (Geers et al. 2005, p. 7).

Jakšić et al. (2013, p. 17) support the need to further investigate the extent to which personality plays a relevant role in the placebo effect. "The idea that people with specific personal characteristics are more prone to placebo response seems intuitive and also easy to investigate. The earlier wave of placebo research suggested that placebo responders were individuals with certain personality characteristics: anxious, emotionally labile, suggestible, dependent on others, and churchgoing." But the majority of investigators claimed that there is no such thing as a placebo-responding personality, that is, placebo response cannot be predicted from dispositional variables and most studies failed to produce strong or consistent findings. For example, they note some researchers have found that an individual with a positive placebo response in one set of trial circumstances will fail to respond in others. There do not appear to be identifiable and universal "placebo reactors" or "placebo non-responders." Nevertheless, Jakšić et al. (2013) suggest that despite such reservations further exploration to provide a better understanding of placebo-related personality traits would facilitate the use of placebo in clinical

practice and improve the methodology of clinical trials.

The healthcare professional may exert considerable influence on the therapeutic expectation of the patient and this is reflected in the well-known term "bedside manner." And autosuggestive elements of therapy are not confined to human personality. Nonspecific effects, for example, from the appearance (size/shape/color) and taste/flavor of solid-dose medicines such as tablets or capsules can contribute significantly to therapeutic expectation.

Changes in Brain Metabolism

It is perhaps a small step from psychological aspects associated with placebo response, at least in theory, to consider what might be identified in the way of detectable changes in the brain. Factors associated with pain reduction are primary and are most clearly understood. Neuroimaging with positron emission tomography (PET) has been used to measure and monitor cerebral blood flow and functional resonance imaging (fMRI) has revealed that placebo *analgesic* response is associated with reduced neural activity in pain-processing areas of the brain and spinal cord (Benedetti and Amanzio 2011). Release of endogenous endorphins, opioids, and nonopioid mediators are implicated in placebo-induced pain reduction and active anticipation of pain may be associated with dopamine release.

These discoveries simply indicate some of the mechanisms and substances that are likely to be associated with placebo response. They do not clarify why there should be individual differences within groups of responders or nonresponders. Recent work in genetic screening suggests the possibility of significant genetic differences that may help to provide a partial answer and the term *placebome* (a set of variants in 11 genes potentially linked to the placebo effect) is coming into use.

Hall et al. (2015) indicate placebo responses are emerging as a legitimate series of biological reactions that must be rigorously characterized to facilitate efficient pharmaceutical development and optimal clinical care. Predicting who will be a placebo responder could be of value to both researchers and patients. In drug development, detecting a difference between active intervention and the placebo control is an underlying goal of RCTs. Being able to identify and exclude individuals who are more likely to respond to placebos could enhance trial designs seeking to find such a difference.

The Ethics of Placebo

Placebo effects are ubiquitous in all therapeutic interventions. The primary reason is that it is impossible to isolate therapeutic treatment from the atmosphere or context in which it is undertaken. This applies to treatment by a health professional or from a family member or by self-medication. We are all conditioned to a variety of health-related systems depending on cultural upbringing and there is an expectation of beneficial effects from treatment.

It is generally accepted that placebos can be classified as either "pure placebo" where the means or substances of treatment are as far as possible totally inert (in practice this is not wholly achievable as widely used lactose, for example, is not acceptable to lactose-intolerant patients) or "impure placebo" where the treatment is pharmacologically active but not for the condition under treatment.

Placebos are employed both in clinical and healthcare practice and routinely as a baseline comparator in the assessment of new therapies. The role of placebo in healthcare practice continues to be controversial. Indeed the question can be asked whether it is ever ethically justifiable to use a placebo in clinical practice. The question would appear to turn on the practitioner's recognition that there is no available suitable treatment for the patient, an assessment of *relative* commitments regarding beneficence and nonmaleficence, and above all, respect for patient autonomy and informed consent. But if the patient is told that they are being given an inert placebo, this may (seriously) undermine their expectation of beneficial effect. So, the question can be further divided, that is, disclosure to the patient that they are being given a placebo or nondisclosure (failure to

respect patient autonomy and obtain informed consent). It seems clear that it is totally unacceptable to give a placebo solely to mollify a patient (disrespect for autonomy, consent, deception, and dishonesty). All in all, it is extremely difficult to satisfactorily justify undisclosed use of placebo treatment (see Asai and Kadooka 2013). An open relationship between patient and practitioner is inconsistent with undisclosed placebo use but is enhanced by frank discussion in which the patient is fully appraised of the situation in which active therapy is unavailable or inappropriate.

The use of placebo controls in (double blind – the "gold standard") clinical studies is similarly a source of contention, though for a rather different reason than with clinical practice. There is no question of justification for participants in clinical trials not being made fully aware of the nature of comparative studies. Their participation is conditional on informed consent, being aware that the placebo-controlled study is comparative and that neither they nor their practitioner should be able to distinguish between active and placebo treatment, apart from at least potentially experienced effectiveness or its lack and possibly adverse effects. Justification for placebo-controlled studies or active comparator studies is based on a *scientific justification* that the effectiveness and/or adverse effect profile of the test drug is unknown but at least potentially may offer advantages over other currently available treatment(s). So where are the ethical problems?

Healthcare practitioners are obligated to prescribe the most effective treatment available, consistent perhaps with local constraints such as affordability. The treatment and health of the patient should not be impeded or put at risk by the possibility of receiving placebo rather than an acknowledge drug. Therapeutic results must be carefully monitored and the duration of treatment must be strictly defined. Consequently, placebo treatments are generally contraindicated in life-threatening conditions. It can be argued that, unless there are particular reasons not to do so, and of course, informed consent and autonomous decision is vital, patients should be generally encouraged to participate in placebo-controlled studies where relevant and where appropriate. There is perhaps an important consideration that for all patients (and this applies to the vast majority of the population), everyone benefits, at least collectively, from advances in health care and therapeutics. Clinical evaluation of new medicinal drugs is a vital part of this, and clinical trial volunteers contribute greatly to a common good of therapeutics.

Conclusion

The term *placebo* is widely employed in health care and therapeutics but perhaps most importantly in the evaluation of new pharmaceutical medicines and other interventions. Although presumed to be entirely biologically inert with no therapeutic effectiveness and therefore suitable as a baseline against which potential therapies might be evaluated, progressive studies have begun to reveal some unexpected properties of placebos and implications. It is now generally accepted that use of a placebo is invariably context susceptible. The very nature of the therapeutic encounter (both practitioner and therapy) is one of expectation of care and benefit, and to an extent, placebo conveys this expectation. Continuing studies indicate that there may be a genetic underpinning of placebo response which may eventually become an important, discriminating aspect in clinical trials. Ethical reservations about the use of placebo are largely associated with questions of beneficence, patient autonomy, trust, and informed consent.

Cross-References

- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Bioethics: Clinical
- ▶ Care Ethics
- ▶ Respect for Autonomy
- ▶ Trust
- ▶ Truth Telling

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Plagiarism

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Abstract

Plagiarism is an ethical issue of considerable practical impact, affecting educational systems and research worldwide, but also influencing art, music, and literature. The academic discussion on plagiarism in basic education mainly concerns its frequency and explanation and potential countermeasures. Ethical aspects more often come to the fore in the discussions of plagiarism in research, particularly concerning plagiarism of text, where there is considerable disagreement. While plagiarism is frequently brought up as an important aspect of scientific misconduct in research ethical guidelines, and a variety of definitions have been offered in this context, such documents rarely reflect a deeper understanding of the concept, its relation to similar concepts, its embeddedness in specific practices, or its normative implications. More research on these issues is needed, although suggestions have been made as to how plagiarism should be understood, demarcated, explained, detected, normatively analyzed, and counteracted. One theme with an obvious global focus concerns the cultural dependence of attitudes toward plagiarism, for instance, whether differences in perception and practice can be explained by differences in relation to authorities. An important point for the future is whether or not present practices regarding plagiarism need to be changed in order to better promote progress in research.

Keywords

Fabrication; Redundant publication; Research ethics; Scientific misconduct

Introduction

Plagiarism remains a persistent problem within both education and research. In relation to the

educational system, the problem of plagiarism is often described as widespread and growing. Several studies show that a majority of students plagiarize during their studies; others show percentages slightly below that. In relation to academic research, plagiarism is listed as one of the core instances of scientific misconduct and has been described as "a crime against academy," even "academic high treason" (Bouville 2008, p. 311). But it is also acknowledged that there is a large span between different instances of plagiarism, ranging from careless accreditation to premeditated fraud.

Plagiarism constitutes a considerable part of scientific misconduct. In a study of retracted misconduct publications internationally by Stretton et al. (2012), plagiarism was the reason for retraction in 41.8 % of the cases and was on the rise. It is, however, unclear whether the increase in detected cases of plagiarism is best explained by an increase in plagiarism or by improved detection.

Several studies measure the frequency of plagiarism among researchers. The numbers vary considerably with location, respondents, and methods of investigation. For instance, in a paper on scientific misconduct among researchers in the USA, less than 2 % of the respondents claimed to have been engaged in plagiarism during the last 3 years. When applying plagiarism detection software, a Chinese journal found that 31 % of examined papers contained plagiarism. In a questionnaire-based article on Nigerian researchers, 9.2 % of the respondents reported having been involved in plagiarism (Helgesson and Eriksson 2015; Okonta and Rossouw 2013). International large-scale studies on the frequency of plagiarism are lacking, among both students and researchers.

There is a notable tension between different perceptions of plagiarism and no full agreement on its normative relevance.

Definition

Journal editorials tend to discuss plagiarism as if there is broad agreement on both its meaning and normative relevance, but several authors acknowledge that there is disagreement and ambiguity in its present uses, across as well as within disciplines (Bouville 2008; Helgesson and Eriksson 2015; Sun 2013; Wager 2014). Indeed, a wide variety of definitions and measures of plagiarism have been suggested.

A few examples of proposed definitions illustrate some of the differences: In the Longman Contemporary English Advanced Learner's Dictionary, plagiarism is defined as "when someone uses another person's words, ideas, or work and pretends they are their own." The Merriam-Webster Online Dictionary explains that *to plagiarize* means "to steal and pass off (the ideas or words of another) as one's own" and "to commit literary theft." The European Code of Conduct for Research Integrity defines plagiarism as "the appropriation of other people's material without giving proper credit." Other kinds of definitions specify a minimum length of word strings, used from another source without proper referencing, as a line of demarcation for what is and is not to be counted as plagiarism (Bouville 2008; Sun 2013).

On a common view, plagiarism involves *theft* (Helgesson and Eriksson 2015; Wager 2014). However, it has been shown that this is not necessarily the case and that theft therefore should not be part of the definition (Bruton 2014; Helgesson and Eriksson 2015). First, while stolen property remains stolen even if one accredits the source, plagiarism is no longer an issue when the source is appropriately recognized (referenced – and quoted, if relevant). Second, an author may willingly give a paper away while encouraging the recipient to publish it as his/her own. If he/she did that, then it would not be theft, but it would still be plagiarism, since the recipient would present another's results or text as his/her own. One can also plagiarize text that is not owned by anyone, which would not involve theft (if it is not owned, it cannot be stolen). As noted by Anderson and Steneck (2011), while plagiarism has two components, (1) taking (in the sense of using) results/words/ideas from someone else and (2) presenting it as one's own, it is the second part that is characteristic of plagiarism.

Must ill-referenced copying be *intentional* in order to be plagiarism? Some of the definitions above include expressions like "pretending" and "stealing and passing off as one's own," which clearly indicate intention, while others do not. In the literature, there seems to be a clear majority for understanding plagiarism in a way that does not presuppose intention, and this also seems to be in accordance with ordinary usage. Thus, if large chunks of text are copied from someone else without proper references, then it is plagiarism regardless of whether or not the person copying is intentionally misleading or is aware that he or she is doing something inappropriate (Helgesson and Eriksson 2015).

As noted above, some have suggested that plagiarism can be *quantitatively determined*, e.g., that if so-and-so many consecutive words are copied without proper reference, then it is plagiarism (Wager 2014). However, such a definition is difficult to defend. It would bring arbitrariness into the definition – why would copying five or six or seven or eight consecutive words be plagiarism, but not copying one word less (Bouville 2008)? More importantly, it does not catch the basic idea of plagiarism: plagiarism involves inappropriately referenced use of ideas, results, or phrases that are someone else's. Very general statements, written by large numbers of people independently of each other, cannot reasonably be said to be anyone's – "they belong to a common pool of expressions and sentences to which no one has an intellectual claim" (Helgesson and Eriksson 2015, p. 96). Plagiarism presupposes originality. In order to determine whether or not a string of words is plagiarized, it is not enough that you find that string in some other source. You have to determine whether that string of words commonly occurs in texts or is unique to one source (or, possibly, a few sources). Thus, a very short word string can be subject to plagiarism, while a considerably longer one may not. However, the larger the chunk of text that is identical to one found in another source, the more likely it is that it is an instance of plagiarism: "The longer passage may uniquely be attributed to a particular author, even though the individual sentences cannot" (Helgesson and Eriksson 2015, p. 97).

Despite the variation in definitions of plagiarism, many definitions proposed by major research ethical guidelines and leading organizations nevertheless are quite similar, in the style of the definition applied by the US National Science Foundation (2012, p. 237): "the appropriation of another person's ideas, processes, results or words without giving appropriate credit." A common variation is exemplified by Anderson and Steneck (2011, p. 90): "Plagiarism is the presentation of another person's words, work, or ideas as one's own." If others' work is presented as your own, then you do not give them appropriate credit, which means that these two kinds of definition mainly identify the same cases as cases of plagiarism.

It follows from these definitions that many aspects that are occasionally brought up in investigations on plagiarism are in fact irrelevant for determining whether or not it is a case of plagiarism, such as who is plagiarized (e.g., an established researcher or a master student), what is plagiarized (e.g., a published article, a nonacademic report, an unpublished student essay, or an oral presentation), the intended audience/purpose (such as whether it is the same or different for the plagiarizing work as for the original work), and the scientific merit value (i.e., what merit plagiarizers can gain from their plagiarism) (Helgesson and Eriksson 2015).

Related Concepts

Some concepts are frequently brought up in relation to plagiarism because they constitute plagiarism, manifest similarities, or are conflated with it. One of them is *self-plagiarism*. Many of those inclined to use the term regard self-plagiarism to be a kind of plagiarism. Indeed there are similarities. Both plagiarism and so-called self-plagiarism involve overlapping publication. Previous results, or text, in whole or in part, are reused in a way that gives the impression that what is presented is novel. Plagiarizers and self-plagiarizers alike therefore also tend to obtain scientific recognition (again) for something that deserves credit only once, and this is arguably accomplished in both cases by being deceptive and dishonest or at least by being unintentionally

misleading (Bruton 2014; Helgesson and Eriksson 2015). However, there are also important differences between self-plagiarism and plagiarism, the most obvious one being that plagiarizers present the outcome of others' intellectual efforts as their own, while self-plagiarizers reuse their own material without telling that it is reused. In other words, self-plagiarism does not involve using others' work while presenting it as one's own, but only reuse of one's own results or text. Therefore, it does not match standard definitions of plagiarism. This has led some to regard self-plagiarism not only as a confusing term but as an oxymoron (Bruton 2014). What is called self-plagiarism is perhaps better labeled *duplicate publication* when the entire research paper is published more than once without proper notification that the material is reused, while *redundant publication* or *inappropriate recycling* might be better terms for non-notified republications of selected passages (Bruton 2014; Helgesson and Eriksson 2015).

Patch-writing is quite often mentioned in articles on plagiarism, i.e., when authors take a little bit from here and a little bit from there and build a text that mainly consists of text fragments from other sources. Some regard patch-writing as a mild form of plagiarism. However, unless texts produced this way fit the plagiarism definition, they are not instances of plagiarism, but simply examples of independent and poor quality academic work.

While the standard perception of plagiarism involves an individual dishonestly obtaining undeserved scientific credit on his or her own initiative, there are instances of plagiarism where others in fact encourage the misconduct. For instance, sometimes renowned researchers are invited to be coauthors on papers to which they have made no substantial research contribution (so-called gift authorship or honorary authorship). By having a renowned name on the paper, the other authors might hope to facilitate publication in a good journal. When taken to the extreme, those who have done the research and writing up of the paper are not included as authors (ghost-researching and ghostwriting), while researchers who have not been involved in the research are asked to pose as authors of the paper. Historically, this has mainly happened when pharmaceutical companies have wanted full control of the research and publication process, while wanting to give the impression that the research was carried out by independent academic researchers (Moffat and Elliot 2007). Posing as an author of a paper researched and written by others is a clear case of plagiarism. Thus, there is a strong connection between ghost-researching/ghostwriting and plagiarism. Also in cases of accepted gift authorship, where the actual researchers are all credited as coauthors of the paper, the person receiving the authorship without having contributed substantially to the paper is committing plagiarism.

Copyright is another term that relates to plagiarism, but one that is concerned with the legal right to intellectual property, such as representations of original creative work. Plagiarizing someone else's intellectual product and infringing on someone else's copyright are, however, two different things. For instance, you may plagiarize a text that is not protected by copyright. You may also make a copyright intrusion without plagiarizing. Plagiarism of an unpublished text is an example of the first case. Publishing a previously published illustration or graph, with proper reference but without asking the copyright owner for permission (when that is required), is an example of the latter. Furthermore, ideas that do not have a "tangible form," i.e., are documented somehow, cannot be protected by copyright, but they can still be plagiarized (Bruton 2014; Helgesson and Eriksson 2015).

Ethical Dimensions

Plagiarism is a value-laden term – apart from a descriptive content, it has a normative flavor. By claiming that an act is an instance of plagiarism, the speaker is thereby (normally) claiming that the act is bad. Frequent suggestions as to why plagiarism is reprehensible are (1) that it involves being untruthful and deceptive – and in a subset of cases dishonest – by what it implies about the creation of the text, idea, image, etc. at hand and (2) that it involves misplacement of appropriate credit for

the plagiarized intellectual product, which makes it unjust. Plagiarism may also have negative consequences: it may bring unfair advantages to the plagiarizers, such as better grades for students or a better career/more research funding for the researcher, while potentially causing harm to the plagiarized person(s), to colleagues, or to readers.

It is one thing to say that plagiarism is bad, yet another to say how bad a certain occurrence is. Not least for normative purposes, it is desirable to be able to distinguish between major and minor forms of plagiarism. Several authors have argued that the severity of plagiarism varies depending on factors like what is plagiarized and how much.

Wager (2014) has suggested a set of dimensions relevant to an evaluation of plagiarism, each with different levels of severity (Table 1). For instance, examples of major plagiarism are cases where the whole papers, important original ideas, or research results (including images containing original data) are plagiarized. It is a matter of minor plagiarism, for instance, if a few sentences have been copied from a typical background description or a description of standard methods and where the plagiarizer has provided a reference but not quotation marks.

This categorization is intended as an overview of central aspects and degrees of plagiarism. However, some of the end points of the listed dimensions do not, or do not necessarily, constitute plagiarism. Most obviously, fully and accurately referenced uses of others' writing do not constitute plagiarism, nor do a few words or a few sentences copied from a part of a paper that does not contain anything original, but only widely used phrases. This is also the case, for instance, for non-referenced reuse of a standardized description of a common method. On the other hand, "borrowing" a few words may constitute plagiarism, even major plagiarism, if an important idea or interesting results are appropriated this way. This means that where in the "extent dimension" a case of plagiarism ends up gives an indication of its seriousness (more is often worse than less), but it is only an indication (sometimes less of something is worse than more of something else). For instance, copying several paragraphs of a fairly standard introductory text without due referencing is a less serious case of plagiarism than using a sentence or two this way if they are describing a new, astonishing idea.

Just like the "extent dimension" helps in thinking about plagiarism, so does the "type of content" category. But nor does this category capture what is essential about plagiarism or its normativity. Granted, since new research does bring new results (whether similar to old ones or not), unreferenced reuse of others' results always involves plagiarism, while reuse of short passages of standardized technical descriptions never does. But it is nevertheless incorrect to claim that you can sort instances on a scale from less serious to more serious depending on type of content. For instance, although in most cases it is worse to plagiarize results than methods descriptions, there are occasions when it is not, namely, when the method is the main contribution of the paper. The seriousness depends on originality rather than on type of content: "Regardless of where the main merits of a paper are located, plagiarism of those

Plagiarism, Table 1 Dimensions of plagiarism

Extent	A few words	A few sentences	Whole paragraph	Several paragraphs	Whole paper
Type of content	Standard method		Describing another's findings		Data/findings
Originality	Widely used phrase/idea		Phrase/idea used by few authors		Original phrase/idea
Attribution	Source fully referenced		Partially/inaccurately referenced		Unreferenced
Intention	No intention to deceive				Intention to deceive

parts is more reprehensible than plagiarism of less important parts" (Helgesson and Eriksson 2015, p. 99).

The type of content matters normatively also in other ways. Plagiarism of data or results may distort meta-analyses by presenting as new that which is not (a negative consequence). Plagiarism of data/results furthermore constitutes fabrication, since previous work of others is presented with the pretense of being new. As noted above, republishing already published results also constitutes redundant publication.

As touched upon when discussing type of content and extent of plagiarism, what to a large extent determines the seriousness of different instances of plagiarism is the originality of that which is plagiarized. Exactly how this *originality value* is to be specified so far remains unsettled. One way to see it is that the value "stems from the novelty and potential of affecting knowledge development within the specific field" (Helgesson and Eriksson 2015, p. 99). This would suggest that what determines seriousness of plagiarism is not so much the uniqueness as the unique contribution to knowledge development. If so, the ultimate yardstick for seriousness relates to the scientific contribution made by that which is plagiarized. This suggestion would fit with the common intuition that plagiarism of ideas, results, and novel means to reach them is more serious than plagiarism of text.

Regarding *attribution*, plagiarism is worse if literal reuse is neither cited nor quoted, compared to when it is cited but without quotation. Also using an idea without giving credit to its source is worse than leaving quotation marks out, while citing, when text is literally reused. However, as with the other dimensions, the guiding light for what makes a case of plagiarism more or less serious is the originality value of that which is plagiarized. If the plagiarized idea is of meager value, then cutting and pasting large chunks of text without proper referencing may be worse.

Above we distinguished between what is wrong with plagiarism as such and consequences of plagiarism. The "intentionality dimension" brings in a third normative aspect. It does not concern plagiarism as such, nor does it concern the consequences of plagiarism. Rather it concerns the morality of the agent, the plagiarizer. There is a difference in the blameworthiness of the individual depending on whether or not the plagiarism is intentional. If intentional, then it involves dishonesty by being intentionally misleading, while an ignorant plagiarizer cannot be accused of dishonesty. However, a remaining criticism can be that the person *should* be aware of proper citation procedures and therefore can be blamed for being ignorant, which in turn may or may not be intentional.

Blameworthiness in relation to plagiarism may depend also on other things, such as seniority and familiarity with the language in which the work is published: the more senior the researcher, the less excusable is ignorance and disregard of proper citation rules; the more proficient in the language of the article or book, the less excusable to take inappropriate shortcuts in writing it.

When it comes to the normativity of plagiarism among students, the following arguments are common: (1) Plagiarizing is bad for those who do it, because it replaces a good learning opportunity, namely, to write the piece of work the proper way. For the sake of quality of education, it is therefore important to stand at bay against plagiarism. (2) Plagiarism is a way of cheating. By not doing the assignment properly, while pretending to do so, plagiarizers act deceitfully toward their teachers and get an undeserved advantage over their peers. The latter since they escape the effort needed to do the assignment correctly – and perhaps in terms of quality, compared to what they would have accomplished without cheating. The act is, thus, unjust and the outcome is unfair. (3) Also in this context, plagiarism involves misplaced attribution of credit.

There might seem to be a drastically different approach to plagiarism in the arts, where a spirit of "no boring rules should restrain the freedom of creative artists" is nurtured. Nevertheless, art and literary works to a considerable extent consist in repetitions and variations of the tradition. Thus, plagiarism along with imitation, parody, ironic retake, and deliberate assemblages is part of the living tradition (Eco 1990). Borrowing from others without explicit reference to the source of

inspiration is both common and encouraged. However, it can be questioned whether the differences are that great. While using ideas from others and integrating them into something new of one's own is cherished, using the ideas of others while passing them off as your own, as in plagiarism of style, is not. It is one thing that artists do not add footnotes with references to their paintings, sculptures, or installations, but quite another not to admit the influence of others when asked. While the career of an artist very much depends on the appreciation of his or her art production, getting the appropriate credit for influencing and inspiring others, and thereby for having an impact on the development of one's craft, matters for the individual's place in the history of art.

Explaining Plagiarism

Some of the explanations why students and researchers plagiarize are the same as those explaining other kinds of misconduct in education and research:

- *Ignorance*: some students and researchers are unaware of what is acceptable practice. Sometimes universities are not clear in their communication about what constitutes plagiarism or about their attitude toward it, and teachers do not always feel they have sufficient time to teach and practice appropriate writing and citing manners.
- *Indolence*: some might regard it pointless to put in the effort when there are easier routes to the same goal.
- *Opportunity*: modern information technology has made plagiarism more effortless because of the massive changes in access to information over the last few decades. For students willing to take shortcuts, there are websites offering ready-made essays on a vast number of topics. And the opportunities to cut and paste from various sources on the Internet are as good as limitless. For researchers with requisite access, most research journal articles are available in electronic form. Image manipulation is also relatively easy thanks to widely available software.
- *Competition*: publications are essential in any researcher's CV, and the pressure to publish, in order to gain positions or external funding, might tempt some to plagiarize. Similarly, some students might think that they increase their chances of getting good grades or finish their assignment on time, by cutting and pasting from other sources rather than by letting the form and content of the text be a result of their own efforts.
- *Poor control*: to the extent that plagiarism control and control of misconduct generally are limited, it will make that option more tempting.
- *Notions of what others do*: those who believe that plagiarism is common and expect their peers to plagiarize tend to be more inclined to plagiarize themselves.

Limited proficiency in English is identified by many as an explanatory factor behind plagiarism, especially among those who learn English as a foreign language. Fear of failure and limited experience with essay writing can also pressure students to plagiarize. Studies further indicate that students plagiarize when they lack the time to accomplish a task or when they are uninterested in the topic (Pennycook 1996; Hayes and Introna 2005; Sun 2013).

It has been proposed in a number of studies that cultural differences explain a variation in attitudes toward plagiarism (Hayes and Introna 2005; Helgesson and Eriksson 2015; Pennycook 1996; Stretton et al. 2012). One important factor is the attitude toward and treatment of authorities. For instance, according to Pennycook (1996), using other authors' words, without rephrasing, is for students in China a way of showing them respect. This cultural practice is deeply rooted. Stretton and colleagues (2012) have argued that cultural differences in familiarity with the notion of individual property may also affect the understanding of plagiarism – unless you have the idea of an individual right to intellectual efforts, rather than simply seeing them as contributions to a common resource, it might be difficult to perceive the normative relevance of plagiarism and therefore the need to distinguish between what others have written and what you have phrased yourself.

Pedagogical differences also contribute to explaining variations in relation to plagiarism. A stress on memorization and recall-type examinations of the studied material, rather than on critical reflection, arguably promotes a writing practice that goes counter to ideas of the need to rephrase. In China, much of the rest of Asia, and Greece, memorization and assumed authority of the authors encountered in the texts are central aspects of the students' learning experience (Hayes and Introna 2005; Pennycook 1996). However, these cultural and pedagogical differences do not fully explain why explicit quotations are not used.

As noted above, Internet has dramatically increased the ease by which plagiarism can be carried out. Perhaps a cultural change has also taken place in its wake. Studies indicate that a cut-and-paste practice, without providing citations or quotations, has become widely established among high school as well as college students as a way of writing assignments (Sun 2013). It has been suggested that Internet has changed the perception of the writing of others among many people, especially in younger generations, in a direction where it is not considered wrong or cheating to plagiarize, because anything found on the Internet does not belong to anyone, does not need to be accredited, and is free to use (Sun 2013).

As part of the explanation to differences in attitudes toward plagiarism, rather than an explanation of the occurrence thereof, it has been pointed out that text plays a partly different role in different research areas (Helgesson 2014). In areas such as history, philosophy, literary studies, and anthropology, text has a central function as mode of presentation of results, while images, graphs, tables, and mathematical formula take on this role in much of medicine and the natural and engineering sciences. This might partly explain a difference in attitudes toward text plagiarism between research fields. Also, different research traditions may vary in their stress of the relevance of eloquence in writing. Finally, plagiarism seems more common in articles with many authors compared to those with fewer, arguably because multi-authorship brings with it a dilution of responsibility (Sun 2013). But it could also be the case that multi-authorship occurs mainly in fields where text plays a less significant role.

Detecting Plagiarism

The traditional approach to detecting plagiarism is that readers who find images or phrases in an article or book that they think they recognize from someplace else try to recall the original source and undertake further investigations. While modern information technology has made plagiarism more effortless, the technology development has also made it easier to detect plagiarism. There is a variety of plagiarism detection software, drawing on large databases, identifying text segments that are matching previously existing text. Such software is used to control student essays, thesis summaries, and academic articles. There is some evidence that the implementation of such software reduces the number of instances of plagiarism. The software has also shown to be useful as a formative learning tool, since it gives students feedback on the grade of independence of their writing (Sun 2013). Thus, it can be used for constructive purposes relating to awareness of plagiarism as a problem and the need for proper paraphrasing and referencing when using sources from the Internet.

However, plagiarism detection tools have some important limitations (Helgesson and Eriksson 2015; Sun 2013). Firstly, what they do, strictly speaking, is not to detect plagiarism, but to detect text overlapping. To judge whether a certain amount or proportion of text overlapping constitutes plagiarism requires evaluation. As pointed out above, using preset lengths of word strings or percentage levels cannot handle evaluation in a normatively satisfying way. Secondly, the software does not distinguish between different kinds of overlapping text, for instance, whether it is general knowledge, descriptions of standard procedures, or something novel or personally phrased. Also this needs to be evaluated by someone using the software as a detection tool. Thirdly, not all sources are included in the software's databases. Fourthly, the software is presently unable to detect plagiarism across languages. Fifthly, plagiarism detection software cannot detect plagiarism of ideas, for

instance, when the original occurrence is not documented (as when researchers share their ideas over a cup of coffee) or when the idea is *used* by the plagiarizer rather than stated. Finally, plagiarism detection tools do not detect plagiarism of images or tables.

Counteracting Plagiarism

Two major alternatives have been discussed when it comes to counteracting plagiarism: (1) exposure and punishment or (2) education and cultivation of a non-plagiarizing culture. In order to expose more than a random selection of cases, there needs to be some kind of systematic detection of plagiarism, such as the consistent use of detection software. And for detection to be meaningful beyond providing statistics, some measures need to be taken whenever plagiarism is exposed. Removal of any advantage gained by plagiarizing is one potential step, such as retracting published articles or failing plagiarizing student reports. Other disciplinary measures are salary deductions, ban from doing research, or temporary suspension from education, from being eligible to apply for research funds, or from being allowed to publish in the offended journal.

Education about plagiarism, proper referencing, and paraphrasing is a proactive approach to counteracting plagiarism. In discussions on plagiarism among students with English as a foreign language, the thrust has been on helping students develop the required language skills, so that they no longer feel the need to take shortcuts in order to manage their assignments. It has further been suggested that courses on research ethics for doctoral students and other researchers should lead up to an ethics training certificate compulsory for anyone doing research requiring ethical approval (Kombe et al. 2014).

As has been noted in relation to education about scientific misconduct generally, there is a risk that research ethics courses have an effect opposite to the one intended; thus, courses covering plagiarism avoidance may in fact remind students about the plagiarism option and clearly hint that they would not be alone if they went along, while indicating what precautions might be needed to avoid being caught.

Cultivating a good study or research environment takes time and is particularly difficult if important incentives are pulling in the opposite direction. Environments where plagiarism will be a less likely choice are characterized by openness as to what the individuals are up to, social control, clear rules, good learning support, and role models focusing on the substance (content) of the activities, rather than on their measurements and outer rewards. The use of mentors might be one way to facilitate cultivation of proper attitudes and the promotion of research integrity (Kombe et al. 2014).

Changing Standards Regarding Text Plagiarism

Plagiarism detection in the academic world, exposing instances of scientific misconduct involving fabrication, redundant publication, and undue acquisition of scientific credit, is relevant to all research areas. But it has been suggested that detection of overlapping text as such is of varying interest, depending partly on the research field, partly on the functions of different sections of a research article. Firstly, in most research articles, some parts cover standard material, such as a presentation of the relevant background or a description of applied methods. Here the point is to present the familiar in an easily recognizable way, in order to then be able to move onto the original contribution of the paper. One could, thus, argue that while originality when it comes to ideas and ways to explore them is highly esteemed, originality in the description of shared knowledge and starting points is at best odd, at worst confusing and misleading. Secondly, as noted above, text plays a partly different role in different research areas. This might justify a differentiation in approaches to text plagiarism, with the underlying idea that the practice best suited to the promotion of knowledge development should be opted for in each research field.

In several research fields, there is already a consensus that references are not required for descriptions of what is taken to be well-established knowledge within the field (Anderson and Steneck 2011). For instance, moral philosophers trained in the analytical tradition are considered free to analyze an idea or

policy suggestion from a utilitarian perspective without citing any literature on utilitarianism, unless the suggested interpretations are unusual or contested and therefore in need of support.

A way to take this a step further would be to allow literal reuse of text containing descriptions of established background perspectives, methods, specifications of the study's limitations, etc. without showing that it is a quotation and, if very general, without referencing (Bouville 2008; Helgesson 2014). However, this proposal might seem to present as new something that is already familiar in areas like medicine and much of the natural sciences. Newcomers in these fields are often encouraged by their peers to reuse their previous methods descriptions or cut and paste from colleagues, without using quotation marks. But the literature shows that there is no unison in practice and a potential for double standards, where research peers may say one thing and journal editors another (Wager 2014). For instance, in a study of the message in journal editorials regarding plagiarism and self-plagiarism, Roig found a great variety in attitudes: while some editors expressed not having any problems with authors reusing, for instance, large portions of methods sections or even entire methods sections, other editors discouraged all such practices (Roig 2014). For a successful change to take place, there has to be a broad and open agreement among journals and research organizations concerned on what is permitted.

There are a number of advantages with treating some types of word strings, phrases, and paragraphs as a general resource free to use literally or free to use if a reference is given (without explicitly quoting), particularly if such a change is supported and facilitated by reporting guidelines (Helgesson 2014):

1. It would avoid the burden of rephrasing for the sake of rephrasing.
2. The potential problem of procedure drift would also be avoided, i.e., the risk that a continuous change in description of research methods will slowly, and unintendedly, alter these methods.
3. It would support researchers with insufficient language skills to find appropriate ways to present their research.
4. It would probably reduce required writing time also for many others, which could be reallocated to reflections on the content of the paper.
5. If this change were supported by reporting guidelines, then it could not only provide preferred ways to describe procedures but also make papers more comprehensive by improving structure and providing checklists for required content.
6. Increased similarity of papers, both in terms of structure and in terms of terminology, would enhance machine readability, a potential advantage in times of big data.

A potential criticism of the suggested practice is that trust in the authors would be threatened. There might also be a risk for a slippery slope regarding what kinds of copying are accepted (Bruton 2014). Against this can be argued that if a sharp line is drawn between what is permissible and not, e.g., copying background material and copying results, then there is no reason to believe that the division could not be maintained (Bouville 2008).

Conclusion

While plagiarism is treated as a paradigmatic example of scientific misconduct in research ethical guidelines, both the concept "plagiarism" and the normative implications of plagiarism have been given fairly limited attention in the bioethics literature. However, it is an issue of considerable practical import for educational systems and for research. While continuous work to inform, influence attitudes, and detect plagiarism is needed, it may also be relevant to reconsider our overall attitudes toward plagiarism in relation to text. Perhaps it would be more conducive to progress in many areas of research if focus shifted from detection of text overlapping to best practices for reporting research results, in order to facilitate comprehension and comparison. To this end, it might be preferable to abandon the idea of a right to acknowledgment for the creation of text.

Cross-References

- ▶ Ghostwriting
- ▶ Integrity: Research
- ▶ Publication Ethics
- ▶ Scientific Misconduct

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Poverty

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Abstract

This entry focuses on the poor or indigent, that is, those persons living in highly adverse conditions who cannot meet their basic needs. It considers our fellow citizens, as well as those living in remote countries.

This article examines what the obligations toward poor persons are. Should persons be beneficent toward them or do they face a stronger duty? If such a duty exists, what kind is it? To this end, this article will present the main ideas that libertarians like Jan Narveson defend. Then, it will analyze Peter Singer's and Larry Temkin's positions regarding individual obligations. A related issue is the proximity to and kind of relationship people have with resource-poor persons: Is it morally relevant that these persons be neighbors or fellow citizens, or people from remote countries? The latter discussion analyzes the difference between domestic poverty and global poverty when considering the redistribution of resources. For this local versus global discussion, this entry will outline some of Rawls's main ideas about domestic and international justice and conclude with Thomas Pogge's strong defense of global redistribution.

Keywords

Poverty (global poverty and domestic poverty); Charity; Negative duties and positive duties

Introduction

In the United Nations (UN) Millennium Declaration of 2000, 191 member states committed to halve, by the year 2015, the proportion of the world's people whose income is less than one dollar a day and the proportion of people who suffer from hunger. Although it could be questioned whether the UN's objectives suffice given the magnitude and urgency of the situation, what the implicit values are in the measurement proposed, or whether the kind of solution offered is empirically viable, it should be first considered if something like a duty to aid the neediest in the world exists.

This entry focuses on the poor or indigent, that is, those persons living in highly adverse conditions who cannot meet their basic needs. It considers our fellow citizens, as well as those living in remote countries.

Although the data and poverty conditions around the world are not recent phenomena, their philosophical treatment is relatively new. However, the philosophical discussion of poverty has become so rich and complex that it is no longer possible to discuss all of its various threads in one entry. This entry will focus on only some of them. It will distinguish related dimensions in the analysis of poverty. First, it will examine which are the obligations toward poor persons. Should other persons be beneficent toward them or do they face a stronger duty? If such a duty exists, what kind is it? To this end, this article will present the main ideas that libertarians like Jan Narveson defend. Then, it will analyze Peter Singer's and Larry Temkin's positions regarding individual obligations. A related issue is the proximity to and kind of relationship people have with resource-poor persons: Is it morally relevant that these persons be neighbors or fellow citizens, or people from remote countries? The latter discussion analyzes the difference between domestic poverty and global poverty when considering the redistribution of resources. For this local versus global discussion, this entry will outline some of Rawls's main ideas about domestic and international justice and conclude with Thomas Pogge's strong defense of global redistribution.

There Is No Duty: The Charity Model

Even if traditional views have analyzed this problem from an ethical stance through the lenses of beneficence and charity, this entry will focus on the contemporary approach to the debate. The current analysis upholds some of the traditional arguments and adds new elements and distinctions.

In light of the question of duties to the global poor and indigent by those that can meet their own needs and those of their families (while enjoying a degree of comfort), one model sustains that no such duty exists. The so-called libertaransi propose the charity model, disregarding the strategies based on claims for justice. For example, Jan Narveson says, "I think it quite clear, indeed, that generally speaking we do not owe the world's poor, as a class, anything special. Many (but far from all) members of that class are people who might benefit from our own charitable concerns, but I really do think that charity is a different department from justice" (Narveson 2004). According to Narveson, it should be accepted that this may be an unfortunate situation though it is not unjust. This distinction is crucial because an injustice imposes a rectification but an unfortunate situation does not. The author points out, "That persons with very low incomes may merit our sympathy is accepted, but sympathy leads to charity" (Narveson 2004). Hence, it would be nice to benefit the needy, but that is a matter of charity and of the heart. This kind of approach leaves the decision of aiding or eradicating this situation up to the private sphere and up to the particular sympathy of each individual; as such, it does not generate any duty.

This position is based on the "traditional" difference between positive and negative duties according to which negative duties are stricter

and must be respected, while positive ones are less stringent. The libertarian proposal claims that people are free to act as they wish so long as they do not violate the right of people not to be attacked. From this perspective, people have the right not to be attacked, but they have no *right* to be aided. Therefore, no unjust act or wrong would be done if people fail to aid the needy (Narveson 2004). Moreover, and this is another crucial point: while people have a strong negative right not to be attacked, they do not have the positive right to be aided. Thus, no one's rights have been violated and no one has been treated unjustly if the needy are not aided.

The charity model allows individuals to act with complete indifference. As was mentioned, the libertarian claims there is no duty or violation of any negative right: aiding depends on the goodwill and heart of persons. As Thomas Pogge signals, "The common assumption, however, is that reducing severe poverty abroad at the expense of our own affluence would be generous on our part, not something we owe, and that our failure to do this is thus at most a lack of generosity that does not make us morally responsible for the continued deprivation of the poor" (Pogge 2005). According to this proposal, persons are mere "innocent passersby" and have no responsibility whatsoever for what happens to people experiencing severe poverty. The libertarian also agrees with the traditional view that has linked the satisfaction of economic, social, and cultural rights to the existence of positive duties, denying them the importance that civil and political rights do have. For critics of this position, the charity model accepts the existing "status quo" and offers a justification that invites self-indulgence.

Individual Obligations: Positive Duties

The proposal that defends the existence of positive duties toward the neediest takes an opposite stance. This section presents an "individual" strategy centering on the obligations and moral duties of individuals.

In his seminal article, "Famine, Affluence, and Morality," Peter Singer argues for a positive duty to aid. He claims that our duties and obligations go far beyond our respect for people's negative rights (Singer 1972). Singer says, "The way people in relatively affluent countries react to a situation like that in Bengal cannot be justified; indeed, the whole way we look at moral issues–our moral conceptual scheme–needs to be altered." He affirms that absolute poverty is bad (suffering from the lack of food, shelter, and medical care) and presents the following principle: "If it is in our power to prevent something bad from happening, without thereby sacrificing anything of comparable moral importance, we ought, morally, to do it" (Singer 1972). "It requires us only to prevent what is bad, and not to promote what is good, and it requires this of us only when we can do it without sacrificing anything that is, from the moral point of view, comparably important." However, he later states that "we ought to give as much as possible, that is, at least up to the point at which by giving more one would begin to cause serious suffering for oneself and one's dependents – perhaps to the point of marginal utility." Singer holds that this principle is the correct one although he also argues in favor of a weaker or more moderate principle in the same article. "If it is in our power to prevent something very bad from happening, without thereby sacrificing anything morally significant, we ought, morally, to do it." With respect to how much persons ought to give if they follow this second moderate principle, Singer states that they would have to give away enough to ensure that the consumer society slows down and perhaps disappear entirely.

Singer explains that these principles have two characteristic features: the first has to do with the second dimension to be considered below: he argues that persons should not consider proximity or distance (it makes no difference whether someone saves a neighbor's son or a Bengali child). The second feature is that it is not important whether someone is the only person that can save him or one among a million. Yet, this second proposal is related to a powerful objection concerning the levels of demand that these principles imply.

Some years later, in the chapter "Rich and Poor" in his book *Practical Ethics,* (Singer 1984) Singer reduces the levels of demand and suggests donating 10 % of a person's income. He devotes a section in the chapter to the objection of whether it is acceptable to support a position that is so demanding that it is almost impossible to put into practice. He wonders whether it would be counterproductive to set such a high standard that only saints would be able to comply. Singer answers, "What would follow from the objection is that public advocacy of this standard of giving is undesirable. It would mean that in order to do the maximum to reduce absolute poverty, we should advocate a standard lower than the amount we really think people ought to give. Of course, we ourselves – those of us that accept the original argument, with its higher standard – would know that we ought to do more than we publicly propose people ought to do [. . .]. There is no inconsistency here, since in both our private and our public behavior we are trying to do what will most reduce absolute poverty" (Singer 1984). Thus, this position significantly reduces the donation to 10 %. Singer argues that this is the minimum and persons do wrong by giving less. However, this discussion did not end there. Years later, Singer lowered the demands even further and proposed donating 1 % (Singer 2003). As was mentioned above, these proposals are linked to the discussion that analyzes to what extent ethics demands can go. It is undoubtedly true that behaving ethically is not easy and that these kinds of duties must take this problem on board. Yet, the issue of moral demand gives rise to an extensive debate that goes beyond this entry.

Some of Singer's innovative proposals have recently been taken up again and complemented. For example, Larry Temkin considers that people can and often behave wrongly even though they do not behave unjustly (Temkin 2004). He offers several arguments to support this affirmation. This entry will focus on only one of them based on the distinction between positive and negative duties.

In response to libertarian affirmations, Temkin shows that some positive duties are not trivial. Temkin highlights that some libertarians mix the category of positive duties with the category of supererogatory acts (those acts that go beyond duty).

He examines Singer's famous example of the duty to save a drowning child: "If I am walking past a shallow pond and see a child drowning in it, I ought to wade in and pull the child out. This will mean getting my clothes muddy, but this is insignificant, while the death of the child would presumably be a very bad thing" (Singer 1972). Temkin asks the reader to assume that John is one of twenty people that are walking along the pond in which a child is drowning. If John can save the child, he has the positive duty to do so; even if it is accepted that the child has no claim to be saved. If one of the other passersby wades in and saves the child, he relieves John of his responsibility to do so. But if nobody else acts, John must do so. Thus, when John lets the child drown to preserve his suit, he is acting wrongly; even if he accepts that he would not be violating the child's rights and, therefore, he would not be acting unjustly.

Temkin uses Singer's case to argue that there are positive duties and these can also be strong. In contrast, he presents the following case: suppose Tom and Tim have been given a box of candy to share. Tom takes one extra piece. He is not just being thoughtless. He knows full well that the piece belongs to Tim, and he greedily does it anyway. Tom is, in essence, consciously violating a negative duty against stealing. Temkin then goes on to say while Tom has consciously violated a negative duty against stealing, and John has "merely" violated a positive duty, John's failure to save the drowning child is morally much worse and open to much more serious moral criticism, than Tom's stealing of Tim's candy (Temkin 2004).

Temkin writes, "So-called 'positive' duties to aid others are often regarded as weak, broad, imperfect, and meritorious while "negative" duties are regarded as strict, narrow, perfect, and unexceptionable. But these divisions are deeply misleading. Positive duties are still *duties* and people acts wrongly if they *fail* to fulfill them. Moreover, a positive duty can be as strict or compelling as a negative duty. Likewise, failing to fulfill a positive duty can be significantly more

open to moral criticism than failing to fulfill a negative duty, even if the latter involves a rights obligation, and hence an injustice, while the former does not" (Temkin 2004).

Temkin seeks to escape the rigid classification of positive and negative duties. He proposes taking into account circumstances in specific situations, in order to evaluate the moral weight of their fulfillment or failure. Temkin's position reflects the moral intuition of the obligation to aid when the gravity of the situation is relevant. This appears to be an acceptable strategy when no contradictions exist between following negative or positive duties or even when the negative duties are trivial. Temkin adds another argument that further strengthens Singer's premise. Extreme poverty is bad and there is something wrong in its persistence in a rich world; even though people are not blameworthy, they cannot remain indifferent if they can help.

Institutional Obligations: Domestic Versus Global

Another strategy introduces another sphere in the discussion of poverty. Unlike the proposals in the previous section, this strategy is not based on individual duties but on institutions. Institutions must be just. Since institutions may vary, another dimension appears; they may be local or global. This discussion centers on the kind of relation or proximity persons have toward those without resources and the role institutions play toward them.

For John Rawls in *A Theory of Justice* (Rawls 1971) or Thomas Nagel in "The Problem of Global Justice" (Nagel 2005), local institutions are responsible for the distribution of justice. In this line, the foundational work of John Rawls can be read as taking care of local poverty. A full analysis of Rawls's work goes beyond this entry, so this article will merely sketch some of the main points that are relevant to it. In *A Theory of Justice*, Rawls (1971) argues that his theory is only to be applied to the basic structure of society which is understood as the major social institutions and the ways they structure the rules of association and allocate the advantages of cooperation (Rawls 1971). This entry will outline Rawls's idea of two principles of justice, which he argues represent the principles of justice persons would accept as governing the "basic structure" of our society. These principles deal with political justice and with what fairness would demand of the rules allocating persons to positions within the basic structure but they also constrain inequality. For the so-called difference principle, the only inequalities of primary goods that can be justified are those shown to benefit the least advantaged. This principle constrains the types and degrees of justifiable inequalities. But these principles only apply within the context of the domestic state.

There is broad agreement that when Rawls deals with the issue of international justice, it is in a manner markedly less reformist than when he approaches domestic justice. This difference in his treatment of international justice and local justice was contested by many philosophers. For these interpreters Rawls defends a weak cosmopolitanism.

In *The Law of Peoples* (Rawls 1999) Rawls considers the international situation. He argues that a just international regime would involve states agreeing to treat one another fairly in their mutual interaction but this does not entail any sort of distributive consideration or economic justice. In this reading, Rawls's approach is international and not global. Rawls explicitly seeks principles that will regulate the interactions among territorially defined political, corporate agents with a monopoly on the legitimate use of force, called "peoples" (Rawls 1999). He does not attempt to derive complete principles of global justice.

Rawls presents eight principles peoples could reasonably endorse. For example, the first one says: "Peoples are free and independent, and their freedom and independence are to be respected by other peoples"; the second principle says "Peoples are to observe treaties and undertakings," and so on. The eighth principle is the one that concerns this discussion: "Peoples have a duty to assist other peoples living under unfavorable conditions that prevent their having a just or decent political and social regime" (Rawls 1999). In Rawls's proposal, nations will not demand that

inequalities among them be justified by improving the lives of the least well-off persons or groups. In the ideal case where international society is fully composed by well-ordered states, there are no distributive requirements at all. And this is so even if some peoples may be very rich and others quite poor (Blake and Taylor Smith 2013). However, there are some distributive elements. For example, as these authors also point out, if deep material inequality among peoples undermines their ability to be considered equals in the negotiation of treaties or tempts peoples to inappropriately intervene in the domestic affairs of other peoples, there will be a reason to eliminate that inequality. But for Rawls, the most important determinant of wealth is the political culture and institutions of peoples. Rawls believes that the causes and sources of wealth of peoples lie in their political culture and in their religious, philosophical, and moral traditions as well as in the laboriousness and corporate talent and he gives the examples of Japan (which with very few resources can organize itself quite well), and Argentina (a country with abundant resources can confront great difficulties) (Rawls 1999). Nonetheless, Blake and Taylor Smith also assert as a distributive element that all members of the Society of Peoples must respect human rights and that guarantees to every person a minimum level of material prosperity and physical security (Rawls 1999).

In order to understand why interpreters of Rawls consider him a weak cosmopolitan, the dissimilarities between the difference principle and the duty of assistance to burdened societies can be evaluated. While the difference principle is meant to continuously apply through the basic structure of society to the flow of social and economic interactions, the duty of assistance has a target and cutoff point: when the society is well ordered. In addition, while the difference principle is about the distribution of primary goods, the duty of assistance might have very little to do with the distribution of wealth (for example, well-ordered peoples will have to provide more technical support, information, institutional assistance than financial support). Hence, Blake and Taylor Smith claim that Rawls argues for a two-tiered theory of distributive justice: the domestic context demands an egalitarian redistribution of primary goods, but equality in the international context requires that each person finds herself in a well-ordered society. This interpretation of *The Laws of Peoples* includes some redistributive elements though absent from this is the difference principle or anything like it. Thus, local poverty seems to be addressed but not the global one.

Another reading of Rawls is the one pointed out by Samuel Freeman as a stronger cosmopolitan. He considers Rawls's position regarding the saving principle and says: "Given the strong parallel that Rawls draws with the just saving principle, it appears that the duty of assistance to burdened peoples to meet basic needs must be satisfied (like the just savings principle). The duty of assistance to burdened peoples then should have priority over the difference principle and duties of distributive justice to the members of one's own society." Freeman goes on to say that Rawls is then giving a kind of importance to meeting basic human needs worldwide. And he finishes claiming: "In this regard, and also given the potentially exacting demands that the duty of assistance can place on advantaged peoples, Rawls's 'weak' cosmopolitanism would seem to be stronger than his cosmopolitan critics allow" (Freeman 2007).

Even if Rawls's foundational and groundbreaking work on justice should be acknowledged, it seems fair to recognize that one is the position expressed by Rawls of *A Theory of Justice* and his original justification of local justice; and quite another is Rawls of *The Laws of Peoples* where he seems to fall to the international status quo. Even Rawls himself states that there is a strong contrast between the rights of peoples and the cosmopolitan perspective (Rawls 1999).

Negative Duties

Thomas Pogge challenges the above mentioned positions. He is a disciple of Rawls. However, he considers Rawls a weak cosmopolitan and questions Rawls's international views. For Pogge,

domestic justice is not enough. On the one hand, Pogge considers Rawls a defender of the thesis of the "purely domestic causation of poverty" which holds that the causes of poverty in poor countries are internal and that, therefore, the eradication of poverty depends on the modification of internal factors. Pogge accepts that there might be internal factors of poverty, but he argues that these factors are frequently caused, modeled, and supported by external factors which cannot be denied (such as the international economic order) (Pogge 2005). Thus, there should be an adequate answer toward the global poor situation. On the other hand, regarding the first discussion, Pogge goes a step further than Singer and Temkin. He does not challenge the distinction between positive and negative duties. He concedes that point to the libertarian. He accepts the importance and force of negative rights as libertarians do. But unlike libertarians, he argues that there is a negative right to compensate the poor. In contrast to Singer and Temkin, who assume that those who have the duty of assistance could be mere innocent bystanders, Pogge claims that they are responsible, that people from industrialized countries are responsible for the global institutional arrangements of their governments. Also responsible are the elites of poor countries with economic resources who often produce the poverty conditions of their fellow citizens. Thus, to persuade the world's "affluent" to help the poor, Pogge bases his strategy on the previous fact that they have harmed the poor. They have given rise to the situation that the needy confront and, hence, a compensation and rectification duty exists.

Pogge's bases are supported by three independent arguments. The first invokes the common and violent history through which existing radical inequality has developed. That is, it targets colonial history, genocides, and past abuses.

The second argument considers that in the state of nature people are entitled to a proportional share of the world's natural resources (Pogge 2002, 2005) and he contends that the existing distribution is unacceptably unjust. Even if Pogge recognizes that the baseline for establishing what would be a just distribution is imprecise, he notes that it cannot be realistically conceived as involving the scale of suffering and deaths that exists today.

The third states that the unjust global economic order is preserved even if there is a massive incidence of avoidable severe poverty. Pogge criticizes the international organizations that allow wealthy countries to establish arrangements with asymmetric rules through tariffs, quotas, subsidies, and so on. He objects to the imposition of absolute property rights on drug patents or the "indirect" support of de facto governments, enabling what Pogge denominates international borrowing privileges, resource privilege, and so on. Pogge argues that this implies that global institutional arrangements are causally implicated in the creation and reproduction of severe poverty, and that the citizens of the most powerful countries are responsible for the global arrangements that their governments have negotiated in their names. If it is possible to show that such harm and responsibility exists (which some philosophers question (Risse 2005)), then Pogge's proposal is undoubtedly very powerful.

Pogge centers his attacks on the failure of the international community and its global organizations, while bearing in mind the value of Article 28 of the Universal Declaration of Human Rights – Everyone is entitled to a social and international order in which the rights and freedoms set forth in this Declaration can be fully realized. In a very original way, Pogge not only takes up the moral notion of human rights but also links these to global institutions. He offers an institutional interpretation of human rights but only demands negative rights.

For Pogge, then, people in industrialized countries are responsible for the global arrangements that their governments have made in their international negotiations (as are the *elites* in countries without resources) and he focuses on those global institutions, their functioning, and their responsibility in the face of global poverty. So, to persuade the world's "affluent" to aid the poor, Pogge bases his strategy on the fact that harm has been done to persons without resources by their governments and the global institutional order that maintain them. They have created the situation in which the poor exist. There is a harm and a positive duty

(unlike Singer's and Temkin's proposals) and, hence, there is a duty to compensate and rectify.

According to Pogge, the individuals of affluent countries, the elite of countries without resources, and global institutions are morally responsible for the extreme conditions under which the global poor live. In this sense, his proposal offers a theoretical justification for the international vision of human rights and broadens the scope of agents of justice in that it not only points to national States but also the international community as a whole.

Conclusion

There are different philosophical positions on the plight of the poor and obligations due to them. These, however, embed some weaknesses. Singer could be criticized for his continued reduction in aid to the point that the percentage appears to be arbitrary. Temkin could be questioned when he says that strict or compelling positive duties exist since he seems to "qualify" duties. Not all positive duties are trivial; nor are all negative duties relevant. But what is the criterion to qualify duties as compelling? Must it be an urgent or desperate situation? Likewise, Rawls's response to domestic and global poverty could be objected and some inconsistencies be indicated. As well as the historic reading that Pogge formulates could also be criticized or the degree of responsibility that supposedly everybody would have.

However, if it is believed that this world is very unjust and that the chance of being born in one country or another, or into one family or another, cannot definitively seal a person's destiny, and that every person ought to do something, these proposed types of responses present interesting lines of argument. Thomas Pogge offers a lucid critique of the current functioning of global institutions, as well as an interesting acknowledgement of human rights from an international stance. Pogge's proposal helps explain strong, negative duties, those for which persons are causally responsible and links them to human rights. This double strategy proves to be most interesting when it comes down to addressing the problem of global poverty.

And yet, does this proposal suffice? In light of the difficulty of implementing these rights in a strongly skeptical and indifferent society, it is worth considering a possible complement. In this sense, even if it cannot be established as the only answer, the proposal based on an individual strategy is redeemable: broadening personal duties à la Singer. Pogge himself would not reject the idea though it is not part of his main concerns (Pogge 2005). According to the above, when it comes to alleviating global poverty, the availability of different tools can prove very fruitful.

Cross-References

- ▶ Equality and Equity
- ▶ Human Rights
- ▶ Justice: Global
- ▶ Justice: Theories of
- ▶ Public Health
- ▶ Social Ethics
- ▶ Solidarity

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Precautionary Principle

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Abstract

Precautionary reasoning has deep historical and wide cross-cultural roots in the ethics of health, health care, and medical research. As in general ethics, however, this side of bioethical thinking has not been the subject of focused critical analysis until recently. The emergence of the precautionary principle (PP) in general environmental and technology policy debate has, after an initial period of confusion, resulted in a range of possible ideas about the value of precaution and what sacrifices it may be worth. This has indicated some need for developments in ethical as well as decision theory. In bioethical debates, this process has left only vague traces, however. Although many issues exist where precautionary reasoning has a place, this is either often left unnoticed or arguments developed suffer from elementary flaws. Environmental and general public health ethics, the ethics of evidence-based practice in research, as well as clinical decision-making, management of normative or factual uncertainty, and the nature of clinical ethical virtues are all areas where precautionary ideas seem to have a place. Such reasoning moreover has specific relevance for global approaches to bioethics and health policy issues in a number of ways.

Keywords

Clinical research; Decision-making; Decision theory; Emerging technology; Environmental health; Evidence; Ignorance; Risk assessment; Technology assessment; Uncertainty

Introduction

Precaution as a concept in ethics is about the moral reason for and value of taking extra care in the face of uncertain or unknown outcomes of decisions. Historically, this theme may be traced back to both eastern and western moral philosophy and religious ethics, such as the roles of courage, patience, and self-control in the ethical ideals of Aristotle, as well as similar notions in earlier Buddhist, Taoist, Hindu, and later Islamic traditions. Outside of this virtue ethical domain, related themes figure for instance, in David Hume's classic defence of private property, or the generic ideas of negligence and recklessness as forms of legal culpability in the philosophy of law. The more specific notion of a precautionary *principle* (PP) is more recent, however, and its wide impact has its roots in the U.N. Rio Declaration of 1992 and its mention of a "precautionary approach" to be applied in policy making on environmental and technological issues. This launched the notion of precaution as a central ethical theme, besides sustainability. As a consequence, there are now different policies in different countries and areas, mandating decision-makers to take preventive action in the face of unknown, uncertain, or probable dangers.

The background motivation for PP is the experience of how seemingly valuable and promising practices may lead to seriously adverse results in spite of lack of solid evidence to that effect. Thus,

PP emerged as a clearly *global* concept (with global historical roots), distinguishing it from many central notions in state-of-the-art bioethics, such as the "four principles", with a distinct European or North American legacy. At the same time, the underlying value conveyed by PP, about being careful in the face of uncertainty and danger, is an age-old motif of health care institutions and the ethos of medical professions – visible whenever needs of garnering information to make responsible decisions are emphasised, such as in African rituals of divination and the Hippocratic dictum of "primo non nocere" – and provides a core rationale for medical research.

This entry first presents the general philosophical ethical debates on PP and precaution. Second, it describes how issues of a precautionary nature and PP-type ideas and arguments occur in different strands of bioethics discourse. On this note, some weaknesses and theoretical gaps are pointed out. Third, some specifically *global* aspects of precautionary thinking in bioethics are set out. The concluding section summarizes the main points and sketches some suggested directions for future work.

General Ethics and Philosophy Debates

Within general ethics and philosophy, PP has been criticized on three main fronts: for lack of clarity, for lack of practicality, and for ethical implausibility. The discussion has shown that PP may be interpreted in a multitude of different ways, some of which convey ethical ideas that may accommodate much of common criticism (Munthe 2011). Building on a seminal analysis by Per Sandin (1999), followed by similar suggestions by Manson (2002) and Gardiner (2006), the generic idea of PP offers a variety of possible interpretations of interest to many fields of study. The most interesting aspect from an ethics standpoint is the suggestion that "in the face of an activity that may produce great harm, we (or society) have reason to ensure that the activity is not undertaken, unless it has been shown not to impose too serious risks" (Munthe 2013). This prescriptive or strong idea needs to be distinguished from what has been called the weak or argumentative idea behind PP (Gardiner 2006; Roeser et al. 2012) which merely says that acting on weak evidence may be acceptable when much seems to be at stake. Further sharpening the understanding of PP requires a basis for assessing

1. What determines whether an activity *may* produce *great harm*
2. What determines whether or not some risk is *too serious*
3. What is required to *show* that too serious risks are not imposed

It is far from clear that these issues can be satisfactorily attended to using ready-made standard models in decision theory, ethics, and/or political philosophy. In particular, the recurring idea of understanding PP as a formal decision rule, such as the maximin strategy, has proven highly problematic (Munthe 2013; Roeser et al. 2012).

The claim that PP is impractical holds out its prescriptions as impossible to satisfy. This is because it is either assumed that the standard regulating what it takes to show that some activity does not impose too serious risks is very demanding (McKinney and Hill 2000) or that PP generally bans (very) risky action, in effect forbidding us to do any of the things we can do, including the option of "doing nothing" (Harris and Holm 2002; Sunstein 2005). This argument connects to the idea that PP cannot take into account variations in probability, outcome value, and so-called opportunity costs of options and therefore violates basic tenets of standard risk-cost-benefit analysis (RCBA). However, these arguments may instead be taken to show merely that a desideratum for a sound version of PP is that it avoids such extreme implications (Munthe 2011).

Claims that PP is generally ethically implausible have come in three forms. One assumes that PP arbitrarily exempts the option of preserving the *status quo* from scrutiny, thereby faulting its own rationale (McKinney and Hill 2000). Another assumes that taking precautionary action in the face of potentially risky activities cannot take into account the costs, risks, and uncertainties of this very action (McKinney and Hill 2000; Harris and Holm 2002; Sunstein 2005). The third

objection conjectures that political implementation of PP would have to break with political values such as science-based policymaking or individual liberty (Sunstein 2005). Sunstein has insisted that the only alternative to PP is standard RCBA, essentially expressing the decision theoretical idea of maximizing expected utility and that PP therefore has to express deeply irrational sides of human nature. Such general attacks become less convincing in view of the many ways of interpreting PP, given by answering the questions (1–3) posed above. PP may be as applicable to the option of preserving *status quo* as to other options, and the answer to question (1) may include a threshold that restricts the choices or options to which PP is applicable, although the form and basis of such a threshold has been debated (Roeser et al. 2012; Munthe 2013). One idea is some variation of the classic *de minimis* risk notion from decision theory (where very small probabilities are excluded from a decision calculus); another is to restrict PP to options where some of the possible outcomes are especially sinister –often described in terms of catastrophe or irreversible damage– or when the evidential basis of decision-making is particularly weak. The answers to questions (2) and (3) may furthermore allow versions of PP that take opportunity costs and degrees of riskiness and uncertainty into account, without making it identical to standard RCBA (Munthe 2011, 2013; Roeser et al. 2012).

Within the frame of avoiding the criticism just described, there is plenty of room for arguments concerning which more precise PP may be ethically justified. This encompasses three distinct challenges (Munthe 2011):

(a) Supporting and clarifying the idea that lack of precaution has a moral price (i.e., that we have some reason not to impose or allow impositions of risks)
(b) Acknowledging that precaution also has such a price (since any measure to prevent or reduce a risk will bring costs and risks of its own)
(c) Balancing (a) and (b) in a way that provides reason for believing this version to prescribe an ethically acceptable price of precaution

The general idea of (a) goes well with common sense opinions in health-related policymaking (e.g., traffic rules, safety assurance of pharmaceuticals, and communicable disease management), as well as in private life (inconsiderate, reckless, or negligent behavior imposing significant risks to the well-being or liberty of others). Although modern ethics or moral philosophy have not until recently focused their attention on such phenomena (Asveld and Roeser 2009), they are an obvious part of human moral thinking, institutionally present within such spheres as law and constituting an essential part of medical ethical thinking. It is important to note that the balancing of (a) and (b) is not only about weighting predetermined risks and chances of predefined options according to a predefined decision rule (thus, not only actualizing standard RCBA thinking). Rather, it likewise involves the question of how to assess the ever present option of delaying decision-making to improve the basis of information or knowledge that underlies any risk assessment. This issue becomes especially complex in cases where the activity itself is pondered for precautionary or risk-reducing reasons; a common enough situation in the settings of health care, medical research, and public health.

The requirements by this approach to ethically clarifying and justifying PP has been conjectured not to be met by standard solutions in decision theory or ethics (Asveld and Roeser 2009). The idea of modifying classic approaches in ethical theory to this effect has been assessed with skeptical results (Munthe 2011). Rights- and justice-based ethical theories *assume* rather than provide reason for the claim that risks are ethically important in their own right. Consequentialism encounters problems in justifying how the imposition of risks and chances is to be balanced against the occurrence of actual harms and benefits. Natural law theories struggle to ground an even elementary coherent notion of how risk impositions may be absolutely morally wrong as such. Justifying a specific PP may thus require some ethical theoretical innovation, observing the formerly mentioned desiderata implied by the criticism against PP. This generates difficult problems for attempts to recreate absolutist ethical ideas in the form of

an ethics of risk, as that of Hans Jonas (Jonas 1979). The need for having a version of PP that is practically useful provides further reason to the same effect. A *virtue* of precaution may have a place in an ethics of risk making sense of general precautionary reasoning (Sandin 2009), but personal qualities of decision-makers do not seem central to assessing policy decisions or regulatory frameworks of the sort primarily targeted by PP. An adequate theory thus needs to be *gradualist* rather than absolutist and primarily focus on *decisions* and *risk impositions*. Virtue ethical renderings of suitable elements of precaution to include in particular contexts may follow as a secondary outcome.

A central problem for any such theory relates to the basis upon which decision-making may or may not be halted in order to improve the quality of the information underlying a risk assessment (which itself is a decision to which PP applies). Making PP sensitive to the probability and outcome variations of options, as well as their opportunity costs –thereby creating compatibility with the basic form of an RCBA– requires that the combination of the benefits and harms of possible outcomes and the likelihood of these outcomes are accounted for but *not* that these are the *only* factors that matter. Thus, it has been proposed that making a decision on the basis of information that could be improved is a moral downside in its own right (Munthe 2011). Moreover, an ethics of risk meeting these standards need not –as is the case in standard RCBA– observe *risk neutrality*. Risks and chances need not be evaluated on the assumption that harms and benefits of equal magnitude balance each other out perfectly from an ethical point of view, independently of the surrounding context. Instead, an idea of an *increased moral weight of evil* may be applied: harms and benefits of equal magnitude are assessed differently in terms of moral importance or seriousness, so that risks involving the possible occurrence of worse consequences become more difficult to justify in terms of compensating benefits in a way disproportionate to their comparative magnitudes. This idea has been defended on the ground that the choice between two lotteries, with equal probabilities for winning or losing but with radically different stakes (say, losing a modest amount of money or winning the same sum, as opposed to losing all your material possessions or doubling them), is not indifferent from an ethical point of view, although this would be implied by the idea of risk neutrality and RCBA (Munthe 2011). Another critical issue is if the underlying justifying basis of PP should take the form of a *moral right* not to have risks imposed by others (Asveld and Roeser 2009). This may be problematic, since many risks relevant from the perspective of PP are the results of many individual actions taken together, thus making the responsibility for securing a plausible price of precaution a collective matter and, ultimately, the value of precaution into a *public good* (Munthe 2011). To secure defensible precaution for all (e.g., in the management of a serious pandemic or in organizing a health care system providing equal access to a certain level of care relative to need), it may be necessary to expose single individuals to loss, harm, or risk that cannot be balanced by any contribution to the prevention of any such downside for a particular individual, albeit all people may share in the resulting increase of safety. This mirrors well-known tensions within bioethics and public health ethics between traditions and topics where more individualistic stances may seem apt and those where more collectivist or community-oriented approaches are held out as primary.

Precaution and PP in Bioethics Discourse

Within bioethics, references to PP or the value of precaution occur primarily in four contexts. First, when more traditional bioethics topics pertaining to the use and conduct of health science are linked to broader policy concerns, for example, those that have far-reaching environmental and technological impact, such as environmental health management or primary public health policy and safety system infrastructure management (Resnik 2013). In this area, PP-inspired arguments from the environmental and technology policy areas apply to issues such as the use of pesticides to battle insect-borne diseases, the effects of engineering solutions to tackle the impact on health of climate change in coastal areas, global use of genetically modified crop to tackle

undernourishment, and other public health challenges. Other scenarios include disseminating (what is hoped to be) human enhancement drugs into the drinking water, common foods, or the atmosphere. Controversies may here be predicted to be similar to those arising in general environmental and technology policy, and the importance of formulating a PP capable of accounting for varying opportunity costs and evidence levels as well as the point about precaution as primarily a public good is therefore salient. Although there are a few exceptions, just as the general area of environmental health ethics has not yet been in focus among bioethicists, this more general way of framing bioethical discussions as parts of more overarching policy projects actualizing precautionary concern needs development.

Second, the issues of when available information is sufficient to make decisions, when one should delay decision-making and pursue more solid evidence, and how risks and chances should be balanced in view of the options and stakes present once the evidence is good enough often arise in clinical care, medical research, as well as the transition in between them. One example of this is the issue of so-called stopping criteria for clinical trials (Malmqvist et al. 2011), when apparent outcomes seem to affect the stakes and/or risks considerably, albeit the evidence base may still be weak. A classic example occurred when new treatments for AIDS were tested in the early 1990s and some centers wanted to discontinue (and give the new treatment to all patients) in view of apparent good results, although that would delay the time to the level of evidence for effectiveness targeted (and required by licensing authorities) considerably. A similar question arose more recently, due to the 2014 Ebola outbreak in parts of West Africa, as discussions around applying unproven drugs gave rise to debates, eventually shifting to the benefit of prioritizing more proven clinical and public health strategies. Another example comes from high-tech biomedical research, where the general discussion of the idea of changing the human genome and recent plans to attempt so-called mitochondrial replacement involve debate on how to assess the long-term uncertainties implied by these and many other reproductive medical procedures. Similar discussions apply to other instances of emerging medical technological procedures with potentially wide impact coming out of areas such as nanotechnology and synthetic biology. In all such areas, one may predict debates on the proper "price of precaution", as theoretically possible health gains are set against problems in reducing uncertainty and ignorance with regard to efficacy and negative side effects (Munthe 2011). In such debates, the issue of how to assess "the weight of evil" in a precautionary argument will have critical importance. For instance, a position of increased "weight of evil" means that precaution is worth less of a price when affected parties have less to lose by taking risks to improve their situation (depending on the gravity of conditions as well as the nature of alternative procedures). This issue moreover appears regularly as part of the core ethical aspects of assessing treatments for orphan diseases, due to the difficulty of meeting standards of evidence in research that makes it very difficult for these to have "been shown not to impose too serious risk" on similar conditions as other procedures (Vos et al. 2004). Likewise, critical discussions of how much evidence is needed in order to motivate public health measures in general have been highlighted lately (Attena 2014). A further research ethics area of interest from a precautionary standpoint relates to the so-called dual use problem of how to assess the development of technologies which may be used for bad as well as good purposes (Kuhlau et al. 2011). Finally, this issue seems to be what underlies current critical debates around research ethics regulation on the basis of its possibly doing more damage by forestalling or delaying medical advances than good by protecting patients against infringements (Wilson and Hunter 2010).

Third, precautionary areguments are recurring in various debates where the issue of moral status takes center stage, such as abortion, embryo research, preimplantation genetic diagnosis, and animal studies. It is, for instance, a common argument of supporters of prolife positions with regard to practices involving the destruction or killing of fetuses or embryos, to argue that the mere

possibility of these entities possessing a moral status that would ground a moral ban on destroying or killing them is reason enough to support bans or restrictions (Mahoney 1984; Ford 1990). Similar lines of reasoning have been proposed regarding the use of animals in research (Bradshaw 1998). Another variety of this type of precautionary reasoning, found across a large span of bioethics discussions, does not link to explicitly moral uncertainty but instead holds out expectations or a possibility of some outstandingly serious effect (good or bad) as silencing all other reasons –pro or contra– some procedure. A special case of this variant has recently occurred in discussions of so-called existential risks or "ultimate harms", where the mere possibility of outcomes such as humanity being eradicated some time in the future is held out as a reason in itself to, for instance, prioritize research on the prevention of such eventualities. All of these cases, however, exemplify uses of PP or precautionary reasoning which are vulnerable to the standard objections regarding impracticality and ethical implausibility earlier described. While a version of PP which avoids such criticism would certainly not disregard the possible outcomes mentioned (e.g., that destroying an embryo in an IVF lab is a morally banned act of murder), neither would it disregard all other normatively relevant possibilities, such as not acting according to the reproductive wants of IVF couples being a grave wrong, while embryos possess no moral status whatsoever. Just mentioning *one* possible downside or upside of an option –however sizable– will not settle the issue from a precautionary standpoint. Neither will it by itself determine what price of precaution in terms of risky actions and lost benefits of alternative ventures is justified. Thus, developing precautionary arguments of this sort into a convincing form would require quite a bit of revision of currently existing versions to fit underlying plausible theoretical models of PP.

Finally, while the notion of precaution is seldom explicitly mentioned in these contexts, precaution is a salient part of health professional virtues or ethoses. Traditional virtues, such as courage, temperance, conscientiousness, prudence, and beneficence, can all be partly spelled out in terms of precaution in clinical decision-making. Precaution spelt out as a virtue of clinical decision-makers (doctors, nurses, pharmacists, and others) should be possible to design with all the nuance and sensitivity to varying likelihoods and outcome-values, quality of evidence, and opportunity costs required to avoid obvious implausibility (Sandin 2009). It is a general weakness, however, of virtue ethical discourse to usually avoid making concrete issues of balancing competing reasons in decision-making more precise, and this applies also to the medical field, as virtue approaches to bioethics tend to take a rather abstract route, with unclear implications for clinical decision-making. This suggests the need to develop the analysis of what clinical virtues there are, what they imply concretely for practical clinical decision-making, and what role a nuanced idea of precaution may play in that context.

Precaution and PP: Specifically Global Aspects

As mentioned at the outset, the basic ethical concept of precaution has far-reaching historical roots across many cultural traditions. At the same time, as just mentioned, it remains to spell out how a theory of clinical virtue provides room for nuanced and defensible precautionary thinking. Working that out may plausibly start off in a practice-oriented approach where the starting point is studies of how decision-making is actually experienced, handled, and judged within different clinical professions across countries and cultures, for instance, with regard to the issue of what degree of ignorance is acceptable and when to delay decision-making to pursue further information. Similar studies with regard to the way in which clinical decisions are institutionalized in the form of formalized ethoses, clinical guidelines, or policies of responsible authorities may also be of interest in this respect. Such work may possibly build on and interact with existing literature on the psychology of decision-making and risk and may then be analyzed and critically scrutinized from the point of view of normative ideals of professional responsibility in clinical health care, illuminating the issue of the price of

precaution. Due to the cross-cultural roots of the idea of precaution as a virtue, such efforts may hold some promise of transcending present conflicts around "principlism" versus various antitheory approaches, Western/Eastern/Northern/Southern ethical imperialism, individualist versus collectivist framings of bioethical problems, and so on. All positions in these debates assume the presence of a solution to the issue of the proper price of precaution, which remains to be explained.

A further area of special interest regards the bioethical implications for debates on truly global environmental and health policy issues. This regards the bioethics of, for instance, climate change and related environmental policy, global trade regulation with regard to health-related goods (food, drugs, infrastructural materials, arms), the management of migration following violent conflict, environmental destruction, or poverty. It also extends to the aftermath of all of these: the intergenerational distribution of goods and capabilities, among these many related to health (Coggon and Gola 2014). Such inquiries connect bioethics directly to the very roots of PP and will address the politics of bioethics and public health ethics on its grandest scale. Although there are some recent attempts to deal with this angle on precaution from the starting point of political philosophy and theory, as well as environmental ethics (McKinnon 2011; Resnik 2013), much remains to be done to achieve a coherent view on the issue of to what extent general theories of *global* justice or environmental ethics would support some version of PP and what that would imply in bioethical terms.

A third, and final, global aspect of precaution from a bioethical standpoint is revealed by recent theoretical research on the practical implications of a PP –or precautionary policy– for global distributional issues (Munthe 2011). That is, rather than having a concrete regulative PP or some precautionary policy be supported by bioethical arguments or general theories from ethics or political theory already assuming particular views on global justice, the ethics of risk underlying a specific version of PP may by itself provide support for specific conclusion on global bioethical distributional issues. The opening for this comes out of the possibility of having a PP avoid the standard criticism and be compatible with basic RCBA tenets while at the same time applying additional requirements on the evaluation of options in view of risks, uncertainties, quality of evidence, and involved stakes. In particular, the idea of an increased moral weight of evil may favor the idea of a reduced price of precaution for parties with bad stakes and an increase in such price for parties who are already better off and thus have much to lose. In effect, the latter have lesser reasons for spending resources on additional (risky) improvements, while the former have much more reason to embark on such ventures– implying that the resources controlled by the latter should rather be spent on improving the conditions of the former. The additional risks thus created are simply less of a problem in view of the value of the additional chances of benefits thus attained for those worse off. This in contrast to privileged societies, where more than decent conditions may in most cases be upheld without new measures which could be applied if resources are so allocated and the risks that would bring. In terms of opportunity costs, the latter will simply be a waste of resources compared to investing in improving the conditions of those worse off, where there is less reason for caution in view of the prospects of improvement of health or other life conditions. In essence, this type of reasoning seems to link well to standard lines of thought around so-called compassionate use of unproven procedures, where a distinctly precarious situation of a patient and lack of alternatives is thought to be able to motivate procedures otherwise ruled out due to lack of evidence. However, even if this or the foregoing notion is denied, there are attendant consequences for the distributional outcome of applying PP at a global level. How PP is specifically designed in this respect will thus form a necessary point of focus for critical assessment.

Conclusion

The precautionary principle engages the moral reasons for and value of taking extra care in the

face of uncertain or unknown outcomes of decisions and has deep historical and wide cross-cultural roots in the area of the ethics of health, health care, and medical research. As in general ethics, however, this side of bioethical thinking has not been the subject of focused critical analysis until recently, and in particular, the central underlying issue of what price it is justified to pay for precautionary measures has been left unprobed. A sound PP needs to avoid absolutism and allow considerations of the quality of evidence, likelihoods, and outcome value of options, and must further imply a sound answer to the underlying ethical issue of what is the proper "price of precaution." Further specification of a PP requires an ethical theoretical defence, which may necessitate innovation and revision of standard ideas in moral philosophy and decision theory and serve to complement existing theoretical models in bioethics.

Nevertheless, precautionary reasoning is abundant in bioethics, albeit sometimes positions defended are about too much of precaution being applied or about precautionary measures having too high a price –as in recent criticism against clinical research regulation. Four general areas have been explored in this entry, as well as a wide variety of examples. Likewise, the idea of a PP and precautionary reasoning offers some specifically global aspects in relation to bioethical issues –from cross-cultural aspects of the further analysis of health professional virtues to overarching policy issues of the global distribution of goods and capabilities related to human health and well-being.

The under-researched nature of the relevance of precautionary thinking in bioethics offers ample opportunities to work on issues with greater latitude than usual for developing fresh ideas and solutions, including the development of sounder theoretical starting points than those applied in previous attempts. Thematic topics for such work include (but do not exhaust)

- The transitional area between research ethics and clinical ethics, in particular with regard to the issue of how much evidence is sufficient for defensible application of a procedure
- Ditto with regard to public health measures and policies
- Virtues of clinical decision-makers and professionals: the handling of uncertainty, risk, and possibility of improving evidence
- Policies, institutions, and regulations for the introduction of emerging medical technologies with potentially far-reaching impact
- Cross-cultural mapping and analysis of precautionary thinking in clinical situations, health policy, and medical research
- The role of bioethical values and norms in precautionary reasoning connected to large-scale global policy issues with regard to the environment, violence, migration, poverty, and similar topics

Cross-References

- ▶ Agricultural Ethics
- ▶ Animal Ethics
- ▶ Bioethics: Environmental
- ▶ Climate Change and Health
- ▶ Dual Use
- ▶ Embryo
- ▶ Genetic Modification (GMOs): Animals
- ▶ Genetic Modification (GMOs): Food
- ▶ Genetically Modified Organisms (GMOs): Human Beings
- ▶ Health Policy
- ▶ Moral Status
- ▶ Nanotechnology
- ▶ Public Health
- ▶ Research: Clinical
- ▶ Risk
- ▶ Sustainability
- ▶ Synthetic Biology

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Further Readings

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Predictive Medicine

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Abstract

In the last part of the twentieth century, predictive medicine has gained currency as an important ideal in biomedical research and health care. Research in the genetic and molecular basis of disease suggested that the insights gained might be used to develop tests that predict the future health state of asymptomatic individuals. The desirability of this ideal and the technologies developed to realize it have been the subject of much ethical debate and analysis. This contribution discusses the most important ethical concerns about predictive medicine and points out how these have coevolved with scientific and technological developments in the field. The focus is on genetics- and genomics-based predictive testing of individuals for health purposes, but many issues are pertinent for nongenetic predictive testing as well. Early on, many ethical analyses asked how knowledge of one's future health state might impact an individual's quality of life, family relations, and social relations. Later discussions shifted toward the interpretation of test results, to what should be considered as a benefit of predictive testing and who should define this. In addition, there have been recurring worries about potential shifts in attributing responsibility for

one's health and about the impact of predictive testing on society.

Keywords

Asymptomatic testing; Autonomy; Direct-to-consumer testing; Family relations; Genetic discrimination; Geneticization; Predictive biomarker testing; Predictive genetic testing; Psychosocial impact; Responsibility for health; Whole genome analysis; Whole genome sequencing

Introduction

Prevention has been a long-standing ideal in public health policies around the world. In the past, preventive interventions were usually low tech and targeted at populations. Recent developments in biomedical research, however, seem to open up possibilities to predict health risks of individuals and thus for "personalized prevention." In particular, the surge of biomedical research in genetics, genomics, and molecular biology is often justified in terms of the "predictive and preventive medicine" it might enable. This ideal has invited much ethical reflection. Is the practice of predictive medicine as desirable as suggested?

After having sketched the historical background of predictive medicine, this contribution discusses a number of the attendant ethical issues while also pointing out how ethical concerns have shifted with developments in science and technology. The focus is on predictive testing of individuals for health, not on prediction for reproductive purposes. Although the examples mainly refer to prediction based on genetic and genomic testing, many issues also pertain to predictive testing with nongenetic molecular markers.

History and Development: Background

Predicting the future of individual patients has always been part and parcel of medicine. Providing a prognosis of a patient's future was an important part of Hippocratic medicine, and prognostication has remained a central task of medical professionals ever since. With the nineteenth century rise of statistics and the increasing interest of states in furthering public health, a different form of prediction emerged. By collecting data about a population's health state and linking these to its other characteristics (like lifestyle or environment), subgroups "at risk" of future health problems could be identified. Such epidemiological knowledge was used to justify and design public health interventions targeting these subgroups of the population (like smokers or obese people). Prediction in prognostics is usually part of clinical care, since it focuses on the diseased individual and is about the expected disease course in this particular person. Predicting the future in epidemiology, in contrast, is part of public health because it focuses on populations and is about health risks and prevention of disease in general. For a long time, these two predictive practices were clearly distinct, not only in aim but also with regard to the professionals involved and the type of knowledge and tools used.

This gradually changed in the late twentieth century, with the growth of biomedical interest in genetics and the start of the Human Genome Project. The increasing awareness that genes play an important role in disease causation brought along the idea that identifying a person's genetic makeup might enable predictions about his or her future health. If population studies would succeed in linking genes with disease outcomes, genetic testing of individuals could indicate what the future has in store for a particular person. Such predictions might subsequently propel an individual to take preventive action. Thus, the preventive aim of public health merged with the orientation at the individual typical for prognostics and clinical care in general.

By the beginning of the twenty-first century, the growing body of research in medical genetics had shown that the assumption that genes are linked in a clear and unambiguous way to states of health and disease was way too optimistic. The search for reliable predictors of disease continued, but attention shifted, first from single genes to combinations of genetic variations (targeted in

genomics research) and subsequently to almost any bodily molecule (like RNA, proteins, enzymes, metabolites, or neurotransmitters, targeted in transcriptomics, proteomics, metabolomics, and the like). At present, huge efforts and amounts of money are spent on the identification of so-called molecular biomarkers. These biomarkers may actually serve many different functions; the subset of predictive biomarkers measures a change in molecular bodily processes that is associated with later emergence of disease. Changes in the concentration of the tau protein in cerebrospinal fluid, for example, are now thought to precede the emergence of symptoms typical of Alzheimer's disease. Testing for such biomarkers thus seems to enable individualized predictions in a way analogous to predictive DNA testing. It is also liable to many of the ethical concerns voiced in the context of predictive genetic testing.

Conceptual Clarification

Genetics and genomics are both scientific fields studying the genetic constitution of organisms. Whereas genetics investigates the functioning and composition of the single gene, genomics addresses all genes and their interactions, in order to identify their combined influence on the growth and development of the organism.

The *genome* is the entire set of genetic material (DNA) of an individual; the *exome* is the subset of this genetic material that has a coding function. Current estimates suggest that about 1–2 % of the whole genome has a coding function. A *single nucleotide polymorphism* is a difference in a single DNA base pair found among individuals. SNPs are the most common type of genetic variation.

On the other hand, a *biomarker* is usually defined as a characteristic that is objectively measured and evaluated as an indicator of normal biological processes, pathogenic processes, or pharmacological responses to a therapeutic intervention. A gene or a set of genetic variations can be a biomarker; however, biomarkers may also be based on other types of molecules.

Ethical Dimensions

When the rise of genetics and the start of the Human Genome Project in the late twentieth century fueled the expectation that predictive genetic testing might become a reality, most ethical questions centered around its potential impact. Is it truly desirable to learn what the future will have in store for you with regard to health and disease? What good is to be expected from knowing that somewhere in the future one will contract bowel cancer, sickle cell anemia, or cystic fibrosis? On the one hand, such anticipatory knowledge might help the person tested to prepare for the disease. On the other hand, in particular, if nothing can be done, such knowledge may be very depressing and have a huge negative impact on a person's life and the lives of relatives. Moreover, the information that particular individuals are prone to a specific disease may invite other parties to use that knowledge for their own purposes, leading to discrimination and exclusion. Such worries were colored, moreover, by the (mis)use of genetic knowledge for eugenic purposes in the nineteenth and first half of the twentieth century, when in many countries whole families with purportedly undesirable hereditary characteristics were sterilized or even extirpated to "breed" a better human race (Kevles 1985).

Predictive Testing for Monogenetic Disease

One of the earliest predictive genetic tests that became available targeted the gene for Huntington's disease. "Huntington" is a progressive neurological disorder associated with motor and cognitive problems as well as personality, mood, and behavioral changes. Symptoms in the majority of cases become manifest at the age of about 40, and patients die relatively young. The disease is clearly hereditary; individual members of a "Huntington family" usually know they are at risk of the disease, but in the past they had to wait whether or not it would strike them personally. The identification of the gene for Huntington (in 1993) enabled predictive testing of healthy individuals. It was expected that the test result would have a positive impact on the quality of life of those proven not to carry the mutation,

relieving them from worry. In contrast, the quality of life of those who do carry a mutation might decrease, as they would learn that they would become a patient and probably die early. Similar concerns were voiced about predictive testing for other diseases, like breast/ovarian cancer, thalassemia, or hypercholesterolemia, even though the availability of effective treatment for some diseases might tip the scales in favor of testing. In such cases, however, predictive testing followed by treatment might still have far-reaching consequences for the person involved. Women carrying a mutation for breast/ovarian cancer might have to subject themselves to regular screening or opt for preventive removal of breasts and ovaries. In the case of hypercholesterolemia, individuals might have to take drugs for the rest of their lives. And in case of thalassemia, knowledge that both parents are mutation carriers may influence the decision whether or not to start a family.

In view of its potentially far-reaching psychosocial impacts, both genetic professionals and ethicists argued that predictive testing should be embedded in extensive counseling procedures before and after testing. Such counseling, moreover, should aim for autonomous decision-making by the client, not for testing as many individuals as possible. Individuals have a "right not to know" or, as some have formulated it, the "right to an open future," unbothered by predictive knowledge about what will happen later on (Chadwick et al. 1997). This approach fitted with the professional ethos of most clinical geneticists, in which nondirective counseling had been promoted as a means to stay away from the specter of eugenics. The international guidelines to regulate predictive genetic HD testing, drawn up in 1994 by the International Huntington Association and the World Federation of Neurology, clearly reflect this ethical reasoning. They stressed the importance of autonomous, informed decision-making and of nondirective counseling to facilitate this (MacLeod et al. 2013). Even though there is much debate as to whether nondirectiveness is really possible, these guidelines soon became a model for guidelines concerning other predictive tests and for good clinical practice regarding predictive genetic testing more generally. They were recently updated to accommodate evidence and novel technologies that have emerged since 1994, but the ethical principles informing the guidelines have not really changed (MacLeod et al. 2013).

Predictive Genetic Testing, Autonomy, and Family

One of the problems with an ethical approach that stresses autonomy, however, is that the meaning of "autonomy" is not straightforward in the context of genetic testing. Since genetic diseases run in the family and the test targets a gene that may be (or have been) passed on to next generations, both the decision-making process and the test result have implications for family members. If most of your relatives decided to test, it is probably harder to stick to your desire not to know. And if a parent knows he/she is a carrier, should he/she inform his/her children, and if so, when? What if an (adult) child wants to be tested, but the parent does not want to be informed of his or her carrier status? After all, if the child is a mutation carrier, the parent knows he/she is a carrier too.

In particular, the question of whether children should be tested for late onset diseases has received a lot of attention (Borry et al. 2006). Even if we grant that adults should have the right to decide for themselves whether or not they want to be tested, the principle of autonomy is harder to apply in the case of minors. After all, they are (supposedly) unable to make well-considered decisions yet. Moreover, the impact of predictive knowledge may be bigger at a young age. The internationally endorsed view has been, therefore, that children should be tested only for diseases when treatment or prevention is available and that all other testing should be postponed till the child can decide for himself or herself. This view is based on the idea that testing should be allowed only if the child clearly benefits and on the view that a child's future options should remain open till he/she has matured. There is more controversy regarding testing for childhood-onset diseases for which no intervention is available. In these cases, after all, it is unclear whether the child actually benefits (Borry et al. 2006).

Even in the case of predictive testing of adults, however, putting autonomy center stage does not solve all ethical issues. Leaving the decision to the individual does not help resolve potential tensions between personal values and those of family members or between the desire (not) to know and the value of good family relations (Rehmann-Sutter and Müller 2009). As several ethicists have argued, the isolated individual that is so often assumed in autonomy discourse does not seem to be a fitting point of reference for the ethics of genetic testing. Hermeneutical, communitarian, and care ethics views pointing out that the self-respect and development of an individual are always dependent on his/her relations to others may be more to the point (Rehmann-Sutter and Müller 2009). Even if counseling aims for individual autonomy, it seems wise not to interpret this as an invitation to relegate all deliberations to the private sphere. Individuals deliberating whether or not to test or how to deal with results might profit from firsthand stories about how genetic testing is dealt with in other families. Such stories show that actual deliberations and ways of doing include a broad array of values and that people often have inventive ways to accommodate different, seemingly conflicting values and interests within a family.

Social Impacts of Predictive Genetic Testing

In addition to psychological impacts, predictive testing may have broader social impacts for the tested individual. Knowledge of carrier status (e.g., for Huntington's disease but also for other serious diseases) may be relevant for employers and insurance companies, among others. This has engendered fear of genetic discrimination: "the differential treatment of asymptomatic individuals or their relatives on the basis of their real or assumed genetic characteristics" (Otlowski et al. 2012). Mutation carriers might, for example, be refused a job or life insurance, or the costs of health insurance might be increased. The use of genetic risk knowledge by insurance companies and employers may sometimes be justified by their legitimate interests, for example, when a client wants to contract life insurance for a very large amount of money when he knows he will die young, or when the risk concerns a disease that is closely associated with the job at hand. However, many ethicists agree that treating mutation carriers differently in most cases is unjust and in case of insurance violates the value of solidarity and risk sharing that is underlying the idea of insurance in general. Several countries have drawn up some form of regulation to prevent genetic discrimination in the public and sometimes also for the private sector. Nonetheless, recent research showed that genetic discrimination is a common and global phenomenon (Otlowski et al. 2012).

Predictive genetic testing may also affect society as a whole. Several critics have pointed out that predictive genetic testing might contribute to the "geneticization" of society: the tendency to interpret all individual differences in terms of genes (Hedgecoe 2009). This concept and the critique it puts forward are reminiscent of earlier, more broad-ranging critiques of the "medicalization" of society, which targeted the practical, cultural, and political implications of labeling a phenomenon as a "disease." In comparison to the medicalization critique, geneticization discourse focuses less on the power shifts entailed by the labeling processes; it mainly addresses identity issues and their political implications. Geneticization is considered a problematic phenomenon for two reasons. First, labeling differences as "genetic" suggest that these are innate and therefore hard to change, and secondly, a genetic explanation (like a disease label) invites biological remedies where social solutions might be more fitting. Attributing health differences to genetic variety, for example, may deviate attention from inequalities in income or housing and thus depoliticize health problems. The geneticization thesis has been hotly disputed, in particular by social scientists who question its empirical adequacy (Hedgecoe 2009). The thesis seems to ignore the active role of individuals themselves in *assuming* a genetic identity and how such an identity invites them to *take* responsibility toward their own and others' lives (Novas and Rose 2000). In practice, members of families with Huntington disease, for example, take an active role in the way they deal with the

(possibility of) predictive testing. In doing so, they shape life strategies that they think are responsible, develop their own expertise, and start co-shaping science, for example, by lobbying for specific types of research (Novas and Rose 2000). Finally, the geneticization thesis hinges on the idea that genes are viewed as automatically causing disease (and other phenotypic traits). This view has been put into perspective since the concept of geneticization was coined.

Predictive Testing for Multifactorial Disease

As indicated above, predictive genetic testing for Huntington's disease has been a paradigm case for ethical reflection on predictive testing in general. Since its emergence in the 1990s, however, it has become clear that the "one gene, one disease" model that makes genetic testing for Huntington very informative does not apply for most other diseases. A distinction is now made between "monogenetic disease," caused by a mutation on one gene, and "multifactorial disease," which is caused by the interactions of (possibly multiple) genes and nongenetic factors like lifestyle and environment. The majority of diseases is now thought to be multifactorial. This implies that most predictive genetic tests indicate an increased (or decreased) risk of disease, not a certain fate. This disenchantment with genetics brought about a shift in the search for predictive markers. Hopes were now set on genomic and on nongenetic biomarkers. Genomic markers try to predict future health by combining many small variations in the genome. Nongenetic markers usually are based on molecular changes implied in "real time" (but presymptomatic) disease processes. In practice, however, these technologies also indicate increased or decreased risk only. Predictive testing, then, whether focusing on genetic, genomic, or other molecular biomarkers, produces probabilities, not certainty.

As a result, many of the ethical issues raised at the dawn of predictive genetic testing seem to lose some of their urgency in case of multifactorial diseases. If the test results are not announcing a dramatic, certain fate, their impact is probably less dramatic than anticipated. This does not mean, however, that predictive testing for multifactorial diseases is ethically unproblematic. Novel, ticklish issues abound, often closely related to the interpretation of test results: How to make sense of the probabilities offered, and how to relate this knowledge to existing identities, ways of life, and the values embedded in these? How to deal with the uncertainties surrounding the knowledge produced? Such questions require attention to the details of predictive testing.

BRCA-testing is an instructive example here, even though it is usually seen as a test for monogenetic disease. BRCA1 and BRA2 are two genes known to be implicated in hereditary breast and ovarian cancer. Women carrying a mutation in either gene have a significantly increased, but not a 100 %, chance of both diseases. Moreover, the two BRCA-genes explain only a limited set of all hereditary breast and ovarian cancer (suggesting that other, unknown genetic factors may be involved). This poses a problem when healthy women from families with a clear pattern of hereditary breast and ovarian cancer apply for predictive testing. If no mutation is found, it is not clear what this means: the woman may be free from a mutation that runs in the family, but she (and her relatives) may also have mutation(s) in one or more other genes that were not tested or have not been linked to breast/ovarian cancer. In both the UK and the Netherlands, this uncertainty is dealt with by asking healthy women who opt for a predictive BRCA-test to engage their diseased family members in the testing process (Boenink 2011). Testing one or more (former) patients first helps to interpret an eventual test result of the healthy person. This procedure, however, turns predictive testing into a family affair and assumes that healthy women are willing and capable to involve their relatives. In the USA, in contrast, BRCA-testing is offered to healthy individuals directly (even the involvement of medical professionals may be quite limited). This means, however, that the test result is often much less informative, because of the uncertainties at stake (Boenink 2011). It seems, then, that there is a trade-off between the value of individual autonomy and the value of informative results and that the actual trade-off depends on the "testing architecture": the way technical and social elements

hang together in a testing practice. This example also shows that speaking of "the" BRCA-test (or of "the predictive test for X" in general) may be misleading; depending on the architecture, predictive testing can mean different things and therefore have different impacts in different settings.

Dealing with uncertainty is a major issue in predictive tests for multifactorial diseases in other ways as well. Since the test produces probabilities, individuals tested have to decide when (if ever) a probability is high enough to justify action. Angelina Jolie apparently considered a lifetime risk of 87 % to get breast cancer and of 50 % to get ovarian cancer sufficient to justify preventive mastectomy and ovariectomy. In cultures where the presence of breasts is experienced even more strongly as an integral part of female identity, the "options" offered by BRCA-testing may cause serious moral dilemmas. However, it is important to be aware that in many cases probabilities associated with genetic mutations are much lower than the ones mentioned above, both in an absolute and a relative sense. A slippery slope may emerge of ever smaller risks being considered relevant, in particular since many people are prone to "anticipated regret." This way of thinking implies that it is preferable to act now to avoid a small chance of an adverse event in the future than to be sorry when the event actually occurs. It has been suggested that, instead of focusing on "risk" and striving to reduce all risks to zero, it may be more fruitful to accept the continuous vulnerability of human health as a given and to strengthen resilience (Palmboom and Willems 2010).

Direct-to-Consumer Testing

When genetic research became booming, the issue of gene patenting attracted a lot of ethical and legal discussion. Several companies, but also academic institutions, started to file for patents on specific genetic tests and even on genes or genetic sequences themselves. The American company Myriad, for example, for years tried to patent both the BRCA-1 and BRCA-2 genes, as well as several elements of BRCA-analysis in both the USA and Europe. This led to huge controversies and a series of conflicting legal judgments, which seems to have finally ended (at least for the USA) in June 2013 when the US Supreme Court ruled that segments of DNA making up the human genome are not eligible for patenting because they are a product of nature (Kesselheim et al. 2013). Reviewing the full debate on gene patenting goes beyond the scope of this article. In practice, the opportunities for profit turned out to be smaller than expected anyway because, as discussed above, most predictive genetic tests were less informative than anticipated. Hopes to commercially profit from genetics (or rather genomics) were nonetheless refueled, when in the late 1990s science changed track and started to perform so-called genome-wide association studies (GWAS). Instead of focusing on single genes, these studies determine the correlation between a large number of genetic variations, usually single nucleotide polymorphisms (SNPs), and the occurrence of disease in large populations. The idea is that a large number of variants, each with a very mall effect on disease risk on its own, in combination might explain the difference between diseased and disease-free groups and thus predict individual risks. Chips were developed that screen individual samples of blood or saliva for sometimes more than a million SNPs. With the knowledge produced by GWAS, it has become possible to test an individual sample and produce a "risk profile," indicating an individual's increased or decreased risk for a number of diseases at the same time.

This possibility was taken up by newly emerging companies like deCODE Genetics, 23andMe, and Navigenics, which from 2007 started offering direct-to-consumer (DTC) predictive testing via the Internet. Customers can order a "spit kit," send in saliva, and access their results online a number of weeks later. Next to testing SNPs related to health conditions, many companies offer tests for other phenotypic traits (like alcohol flush or eye color), drug response, ancestry, and/or paternity. Providers claim that this information empowers users to make their own choices and take responsibility for their own health (or even life). DTC-testing has caused a huge ethical controversy and is the subject of ongoing regulatory

attempts in many countries. Partly as a result, this is a highly volatile market. Although many commercial initiatives have emerged, success is in no way guaranteed. Providers frequently change their products, their business model, or marketing strategy and rapidly emerge and disappear to sometimes reappear in a different country (Tutton 2014).

The ethical concerns related to DTC-testing are twofold. First, there are several issues with the validity and interpretation of results, suggesting that customers may be deceived for profit's sake. In addition, there are worries about the impact of providing results without a professional intermediary (Tutton 2014). With regard to the first, there are doubts concerning companies' claims about the clinical validity and utility of their tests. The commercial interests of the companies offering such testing often seem to lead to inflated promises. Many critics point out that knowledge about the meaning of SNPs is limited and that therefore the interpretation of the variation found is challenging. GWA studies as yet have not produced many results that are clinically useful, and many have not been reproduced. Moreover, the far majority of populations studied in GWAS have a white European bias, so results may not be applicable to people with different ancestry or ethnicity (thus adding to the injustice caused by the fact that most predictive testing predominantly targets diseases prevalent in affluent societies). In practice, different companies look at different sets of SNPs and use different GWAS and different sets of reference populations to interpret test outcomes. As a consequence, the same client may receive very different results from different companies (Tutton 2014).

Questions have also been raised regarding the impact of the test results and the associated health advice. Customers might be unnecessarily frightened or erroneously relieved about their risk when no professional counseling is provided. It might even cause them to use health-care services without clear clinical need. Several commentators have argued that to prevent harm, tight regulation of DTC-testing is warranted. They suggest that the norms used to regulate clinical genetic testing should apply. Others argue that DTC-testing should not be put on a par with clinical genetic testing, because the utility of such tests goes beyond the medical domain. They can provide self-knowledge and entertainment, in addition to empowering individuals to make their own life choices. Whether or not a test is useful, this argument goes, can and should be decided by individual clients alone. In response to attempts at regulation (e.g., by the American FDA), many companies have indeed started downplaying claims about medical utility, stressing the "personal utility" of their tests. Others have started to involve medical professionals (Tutton 2014).

Framing DTC-testing as "medical" or as "personal" indicates the relative weight given to ethical principles of "protection against harm" and "individual choice." The usefulness of both principles has been questioned, however, because they presuppose boundaries between medical and nonmedical information, experts and lay people, patients and donors, academic research, health care, and commercial companies that are rapidly eroding (Prainsack et al. 2008). Increasingly, DTC companies offer their services in combination with options to actively participate in research. The company 23andMe, but also noncommercial organizations like DIY Genomics, encourages clients/participants to provide medical and lifestyle data in addition to their genetic profile to set up large databases that may enable further research. Their claim is that clients will personally profit from the diagnostic and/or therapeutic applications these biobanks may generate *and* contribute to the public good at the same time. In "crowdsourced" projects, participants may be offered a say in what should be investigated and in the interpretation of results. In such cases, decisions about the meaning and utility of results are no longer the prerogative of a limited number of experts, nor left to individuals; they are becoming a collective effort of (both commercial and academic) geneticists, bioinformaticians, clients, and (web) communities (Prainsack et al. 2008).

Whole Genome Analysis

The most recent development of note is that sequencing the whole genome is becoming so

cheap that it can be applied almost routinely. The "thousand-dollar genome" may become a reality soon. Many ethical issues raised before are relevant to whole genome sequencing and analysis (WGS/WGA) as well. However, there are also ethical questions more specific to WGS/WGA. First of all, sequencing in principle opens up all "raw" genetic data of an individual. In practice, however, filters are often applied, sequencing only those parts of the genome that have a coding function (the "exome") or that are most relevant to the knowledge aimed for. To make sense of the raw data, moreover, the sequence has to be analyzed. Such analysis, again, can be performed on all data or on filtered data only. The choice whether or not to apply filters (and if so, which ones) is crucial for the knowledge gained and the potential impact of this knowledge (Bredenoord et al. 2011). This is all the more relevant since WGA may reveal information with regard to health and disease but also about other characteristics, like personality or cognition. It is crucial, then, who decides how broad or targeted analysis and subsequent disclosure of results should be. For research settings, it has been suggested that enabling participants to choose among different "packages" of genetic information with different clinical utilities and/or different significance might be a good way to steer between the values of autonomy and clinical utility or actionability (Bredenoord et al. 2011). A similar procedure could of course be used outside research settings.

WGA, when it is not targeted at a specific disease or the explanation of a specific problem, can be considered as a form of screening (Dondorp and de Wert 2013). The basic idea is that WGA results may inform choices of lifestyle, prevention, and treatment, as well as reproductive decisions. As in other cases of screening, potential drawbacks of GWA are that one may receive unwanted or unsought for information, that it may increase worries about both negative and uncertain outcomes, and that it may lead to unnecessary use of health-care services. A complicating factor is, again, that WGA is likely to reveal much information of uncertain significance. Typical moments to perform a WGA could be at birth or in early adulthood. At present, there seems to be broad international consensus that it should not be performed in newborns unless it is used to clarify a disorder, hence for diagnostic purposes. Although some have argued that predictive knowledge about late onset disease may be of value to (parents of) children, most ethical bodies judge that the knowledge gained would infringe on the child's future, anticipated autonomy (Dondorp and de Wert 2013). As for WGA in adults, some argue that the value of such an analysis will soon be evident because of the anticipated possibilities for tailoring prevention and treatment to one's genomic profile. Others suggest that, regardless of the quality of the information gained, individuals have a right to know about their genetic constitution. For the moment, however, many seem to agree that in the absence of valid, reliable, and actionable predictors, such a right is rather premature (Dondorp and de Wert 2013).

Finally, analysis of the genome is of course dependent on the state of knowledge regarding the link between genetic variations and health status (or other characteristics, for that matter). Since this knowledge is evolving, any interpretation of sequencing results has a temporary character: future insights may change the meaning of what was revealed. This raises issues of where and how to store one's genetic sequence and who (if anyone) is responsible to communicate novel insights that might affect its meaning (Dondorp and de Wert 2013).

Individualizing Responsibility and the Erosion of the Common Good

Next to the myriad of issues related to interpretation of the predictive information different technologies produce, there are ethical and social issues regarding the responsibility to act on this information. If risk for disease is increasingly framed as a bodily characteristic, how does that affect thinking about responsibility for health? Whereas a genetic mutation for a monogenetic disease may be interpreted as an excuse for one's health state (since the fault is in your genes, you cannot be blamed), probabilistic predictions seem to increase one's personal responsibility. Now you can know what diseases you are prone to, you can

also be expected to take precautionary measures. Increasing "empowerment" in that case means increasing individual responsibility. Whether or not individual responsibility for one's health gains in strength as a social norm, and whether this is associated with a concurrent decrease in collective responsibility for health, is as yet unclear. It should at least be noted that these are not automatic, unavoidable effects of the availability of predictive tests (Juengst et al. 2012). The conditions under which such tests are implemented can make a lot of difference. And even if the architecture of a predictive test invites individual "responsibilization," users may respond in unanticipated, creative ways, for example, by bringing together those with similar risks and taking collective action to improve health care for this group.

It has been pointed out, finally, that the central role currently assigned to (genetic and nongenetic) biomarker testing in predictive medicine does shift the aim and meaning of prevention. Whereas prevention in the past was seen as a crucial pillar of collective policies to improve public health, current thinking about prevention tends to focus on improving the health of individuals (Dickenson 2013). This shift may encourage a stance toward prevention in which personal interest, not the common good, is paramount, This in turn could contribute to an erosion of the notion of public health and decrease support for and willingness to participate in interventions targeting public health (e.g., in vaccination policies). In view of the frequently limited benefits of individualized prediction and prevention, moreover, spending public funds on predictive testing may be much less cost-effective than spending it on collective measures like environmental infrastructure, vaccination, and health education (Juengst et al. 2012; Dickenson 2013).

Conclusion

Biomedical research into genetics, genomics, and molecular medicine in the late twentieth century has brought along a vision of predictive medicine. In this vision, testing of an individual's molecular functioning enables predictions about his or her future health state. Such predictions, it is thought, facilitate disease prevention and early treatment but also help to prepare for the future and to make life choices. Fueled by the high expectations about genetics and the Human Genome Project, ethicists early on were concerned about the potentially negative impacts of such predictive knowledge on an individual's quality of life, on family relations, and on social relations. The right to have an open future emerged as an important argument to balance against the potential benefits of predictive testing. Moreover, autonomous decision-making was put forward as an important condition for ethically acceptable predictive testing, even though it may be hard to achieve in case of genetic information. When it became clear that most predictive testing (whether based on genetic, genomic, or other biomarkers) results in probabilistic information, ethical attention increasingly focused on issues related to the interpretation and understanding of test results. Awareness has grown that the conditions under which such tests are offered, used, and interpreted are crucial to their actual impact. Moreover, it is now acknowledged that the rapidly evolving knowledge base in molecular medicine means that interpretations of test results evolve as well, complicating the assessment of their impact. Next to these concerns about testing practice, worries remain about potential shifts in thinking about responsibility for health, as well as about broader impacts of predictive testing on society and culture.

Cross-References

- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Biobanking
- ▶ Biological Citizenship
- ▶ Consent: Informed
- ▶ Discrimination: Concept of
- ▶ Discrimination, Ethics of
- ▶ Disease
- ▶ Genetic Determinism
- ▶ Genetic Screening
- ▶ Genomics

- Lifestyles
- Personalized Medicine
- Pharmacogenomics
- Public Health
- Quality of Life
- Respect for Autonomy
- Responsibility: Individual
- Responsibility: Social
- Risk
- Solidarity

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Further Readings

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Prenatal Genetic Screening

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Abstract

This entry focuses on the ethical issues of prenatal genetic screening. Genetic screening is a generic term that refers to a set of diagnostic techniques for sample collection and analysis with the aim of detecting fetal anomalies in utero. These different methods will be briefly described and assessed in terms of their ethical aspects. As one of the basic outcomes of prenatal screening is to offer parents a termination of pregnancy in case of a diseased

fetus, and also because the sociocultural consequences of prenatal diagnostic technologies are highly controversial, the entry also briefly touches upon debates that relate to disability rights, eugenics, and geneticization.

Keywords

Informed consent; Reproductive choice; Genetic testing; Genetic counseling; Eugenics; Geneticization; Disability rights; Risk communication

Introduction

Prenatal screening occurs in the first and second trimester of pregnancy usually aiming at risk assessment for common aneuploidies and fetal abnormalities. Traditionally, the aim of prenatal screening was to detect Down syndrome, but other chromosome abnormalities including neural tube defects (such as spina bifida), cleft palate, sickle cell anemia, thalassemia, cystic fibrosis, fragile X syndrome, Tay Sachs disease, and muscular dystrophy have increasingly entered into the purview of prenatal genetic screening. This has been made feasible with the advent of new genetic technologies that allow a wider set of genetically influenced conditions to be detected on the DNA level. This has likewise shaped the ethically relevant issues that have been connected with prenatal screening.

Some of the ethical issues and their background debates are more or less settled, but new issues are coming up at the horizon. There were some technological advances and some changes in the screening strategies that still have an ongoing effect on the ethical landscape of prenatal genetic screening. The rapid advances in ultrasound diagnostics, the further development of molecular techniques, and the pending introduction of noninvasive prenatal diagnosis are all transforming the ethically relevant issues. Some might argue that the dynamics of these developments could even challenge the existing normative framework (Dondorp and van Lith 2014).

Usually the ethical issues are interpreted within the doctor–patient fiduciary relationship, so the normative framework refers to the basic principles of biomedical ethics including autonomy, beneficence, nonmaleficence, justice, and some more specific norms like proportionality and equity of access. Within this framework, the explicit aim of prenatal screening services should be to enable parents to make meaningful reproductive choices or in case of testing for fetal abnormalities to avoid children with serious disorder or disability (de Jong, et al. 2011). As prenatal screening creates the potential for selective abortion – which is a highly sensitive moral issue in several countries – this constitutes the major ethical issue. However, disability rights activists also perceive the very aim of prenatal screening services as highly troublesome and having discriminatory effects because it reinforces the idea that life with disability is something negative that can/should be avoided. The global usage of prenatal screening is low, but increasing. Prenatal screening is frequently used in the developed world, and it is either offered as a market-driven commodity (United States) or as part of public health initiatives (Europe, Canada, and Australia).

Conceptual Clarification: Screening Techniques and Methods

It needs to be mentioned that some authors differentiate between prenatal screening, testing, and diagnosis. For them, the term prenatal screening entails the whole testing process that is offered to pregnant women; prenatal diagnosis refers to follow-up testing, while prenatal testing constitutes a generic term. In this entry, genetic screening, testing, and also prenatal genetic diagnosis (PND) will be used interchangeably.

In the medical literature, the term screening is frequently used to refer to a public health diagnostic intervention for detecting disorders that have a relatively high prevalence in order to provide early treatment options. Prenatal screening programs however differ, ethically from traditional forms of population screening programs because the needs of individual patients (e.g.,

whether or not they want to abort or keep a genetically "sick" baby) are central as opposed to societal needs (as obtained in typical public health scenarios). During prenatal genetic screening, a set of diagnostic techniques are used which parallel three options. These are medical ultrasound devices (which provide clearer real-time pictures about the bodily outlook of the fetus), invasive techniques (which acquire a sample of amniotic fluid or a tissue sample from precursors of the placenta which then can be used for diagnosis by a variety of analytic methods including karyotyping), and noninvasive tests (which acquire fetal DNA from a simple blood sample taken from the pregnant woman and is analyzed using molecular biological techniques).

Each diagnostic method differs in context, applications, and associated risks. For instance, amniocentesis was developed for detecting Down syndrome for women with advanced maternal age, especially those over age 35. Chorionic villus sampling (CVS) is used in the cases of high-risk pregnancies, assessed on family history of genetic disorder, first trimester screen results or abnormal ultrasound findings, and maternal age. Its risks include miscarriage, amniotic fluid leakage, and infection. Compared to amniocentesis, it is regarded as advantageous, because it can be performed earlier in pregnancy and also because of the shorter waiting period for results. Prenatal ultrasound is usually done in the second trimester of pregnancies that might have a wide range of possible outcomes. These scans can indicate structural defects with different severities or unknown clinical consequences and can also reveal "soft-markers" that show a risk of a wide range of genetic disorders. These findings usually require further diagnostic procedures and more detailed screening. Lastly, noninvasive prenatal testing (NIPD) is an emerging technology in which the so-called cell-free fetal DNA/RNA is obtained from the pregnant woman's blood as early as 4 weeks of gestation.

From a global perspective, it should be noted that the issues concerning prenatal genetic diagnosis will differ across countries and continents due to factors including cultural nuances, technological accessibility, and legal norms.

Ethical Dimensions

In some EU countries with a stronger catholic tradition where elective abortions are restricted, PND and screening are more restricted. Elsewhere, screening programs are more established and more frequently used. There are some data that in countries of emerging economies, like India and China, the middle classes have already adopted some of these technologies; for example, ultrasound scans are widely available and used for the identification of disabilities and also for sex selection, especially in India (Crawford 2012). The use of PND for sex selection is however prohibited in Europe and a significant number of other Western countries, with the exception of serious genetic diseases.

In those countries where prenatal screening practices are widely available, there has been an emerging standard and an internationally acknowledged normative framework for the provision of prenatal genetic screening. Basically, this framework refers to widely used principles and norms of health-care ethics like nonmaleficence, respect for persons, beneficence, justice, equity of access, proportionality, and informed consent. Although the explicit aim of prenatal genetic testing is to provide pregnant women with meaningful reproductive choices, this framework sees informed consent as a necessary but not sufficient condition.

One of the basic principles of biomedical ethics, respects for persons, requires to treat human beings as autonomous moral subjects (Beauchamp and Childress 2013). This principle demands that decision making regarding participation in the screening process – at all stages of offering various testing and diagnostic services to the clients – must be based on a proper informed consent, on adequate information, and voluntariness. According to the principle of beneficence and nonmaleficence, it is important to clearly assess the potential benefits and harms of the testing procedures. The provision of testing is only justified if the potential benefits for clients clearly outweigh the risks and burdens of testing. According to the principle of justice, prenatal genetic screening programs should be embedded

in health-care services in a way that equity of access is ensured. These programs should be based on the fair distribution of benefits, burdens, and resources. As one might observe, the above-sketched generic ethical framework for genetic screening services could be derived from the more general set of principles for population screening as well. However, the current biomedical limitation (in terms of treatment options or prevention of diseases screened via prenatal genetic screening) makes fetal abortion an only option and shapes the extent to which PND allows clients to make meaningful reproductive choices. Against this background, the more specific aspects of the ethical issues associated with PND will now be briefly addressed.

Meaningful Reproductive Choice

Most countries offering prenatal genetic testing and screening services have formulated the requirement that the test results should be provided to the patient by an expert genetic counselor. To be sure, genetic counseling is also stipulated to precede testing to ensure that both the potential test results and their clinical consequences should be clearly explained in a plain language in order to facilitate informed choice. Counseling should be nondirective and supportive to provide the pregnant woman with opportunities for meaningful reproductive choice. Beyond the main content of informed consent in the case of ordinary medical procedures, the counseling process in genetic screenings is guided by the idea that screening results might yield new information that could force some into uneasy decisional situations (about marriage, adoption, abortion). One of the major challenges of counseling and informed consent in such a setting comes from the great variety of tests, findings, and further testing that may follow. Moreover, as a consequence of the "right not to know" women should be given the opportunity to indicate which specific outcomes they do not want to be informed about.

There is considerable agreement on the above-mentioned ethical requirements as these are embedded in professional guidelines and recommendations, like the Nuffield Council's report on Genetic Screening (1996), or in the position statement of the International Society of Prenatal Diagnosis (2013). The online database of the Geneva Foundation for Medical Education and Research contains a global list of recommendations and guidelines related to the ethics of of prenatal genetic screening (Geneva Foundation for Medical Education and Research).

In order to facilitate meaningful informed choice, some have argued for filtering out all "irrelevant" information and focusing on the most important and significant findings of the tests. It has also been argued that it would be more feasible to follow a generic consent model in genetic screening. This generic consent would be similar and might be compared to obtaining consent to perform a physical examination, where one finding might generate other necessary diagnostic procedures that also might generate additional follow-up and also some difficult choices that were not foreseeable.

Another challenge is the uncertainty of the findings, especially when the results are presented in statistical terms of likelihoods, chances, and probabilities. Except from some cases where the results are quite clearly indicating a certain clinical consequence for the future child, most findings are not having such strong predictive value. Some empirical literature already shows that both doctors and patients can be generally regarded as statistically illiterate. This certainly indicates that interpreting and communicating the results of the emerging new testing services will further complicate the process of reaching meaningful reproductive choices (Sivell et al. 2008). In this regard, risk communication experts offer some insights on how to communicate difficult statistical information. Two of their recommendations are certainly relevant in the context of genetic counseling. One is related to the use of decision aids, especially illustrations and pictures, while the other is the use of natural frequencies (3 persons from 100 persons) instead of probabilities [3 %] (Gigerenzer 2002).

According to one of the systematic empirical studies conducted in Australia, 31 % of the study participants did not know that miscarriage was a possible consequence of diagnostic testing

subsequent to an increased risk of screening result, and only 62 % correctly identified that the termination of pregnancy would be offered if Down syndrome were to be diagnosed. These and other studies also suggest that we are still quite far from the ideal of meaningful reproductive choice (Green et al. 2004). In an ideal scenario, the pregnant woman receiving pretest counseling would be "left alone" to make her decision whether to undergo screening or take a specific test. Some have argued that this scenario is very unlikely because of the emergence (in the western context particularly) of wrongful life/birth lawsuits as doctors are increasingly interested in obtaining their clients acceptance to undergo genetic screening. Clinicians might be worried that even if the woman does not wish for screening now, in the future she might argue that she would have wanted it, a possibility which might make such doctors liable. Such incentives could ultimately interfere with both the process of informed choice and nondirectiveness as doctors tend to be more persuasive.

Reassurance or Anxiety

In most developed countries, prenatal screening is offered not just to "high-risk women," but to "low-risk women" as well. As a result, most pregnant women will undergo some forms of diagnostic techniques, which have certainly transformed the experience of pregnancy in these countries. Because the proper clinical assessment of test results might take days or weeks, pregnant women could experience significant anxiety. Indeed, it is not an exceptional scenario for some women to go through a waiting period for a result that necessitates testing for another condition that initiates another waiting period. On the other hand, some literature focusing on the psychological effects of the screening process have emphasized the positive phenomena of reassurance that pregnant women could experience when receiving favorable diagnostic results (Green et al. 2004).

Equity of Access and Proportionality

In some countries, the access to prenatal screening or a specific diagnostic test will depend on the woman's risk status. Risk status is mainly determined in relation to a specific condition (like Down syndrome, sickle cell trait, Tay Sachs, cystic fibrosis) on the basis of ethnic origin, known family background of a genetic condition, and on maternal age. In the case of high-risk women, prenatal screening could be offered that involve the use of various tests and a given follow-up trajectory. This approach leads to more undetected Down syndrome pregnancies in cases of younger women. On the other hand, there is an emerging trend that women bear children at later ages. This has not been limited to developed countries but increasingly occurs in developing nations due to factors including career and educational needs which tend to foster late marriages. This social change creates a significant increase in the number of amniocentesis performed, an increase in the rates of procedure related to miscarriages and a concomitant rise in the economic costs of testing. Moreover, if the access to prenatal screening services is determined by the age of the woman, then it could create an unacceptable form of health-care rationing.

Geneticization

There are some overreaching ethical issues that concern the overall project of prenatal diagnostic services. One such issue relates to the geneticization thesis. This is a social theoretical concept that basically claims that as a result of the emergence of the new genetics and the concomitant technological developments, contemporary societies are increasingly relying on genetic information. According to this view, the introduction of new genetic test and the emergence of genetic services foster a gradual shift from our more complex biopsychosocial view of human life to one in which differences among individuals would be increasingly reduced to their genetic characteristics.

In the context of genetic testing, this means, for example, a change in how we as society evaluate the role of genes in human life and development. This has an overall effect of how the social context of prenatal genetic testing is constructed, and more precisely how the setting for individual autonomous choice is constructed. As Lippman

argued, the claim that women themselves need or choose prenatal screening is something constructed by the context of testing (Lippman 1991). The current context of prenatal tests therefore increasingly portrays screening services as being a normal part of routine care and reinforces the idea that to opt for screening is the responsible course of action.

Disability Rights Critique

To be born with a certain disability probably appears bad to most people, especially for people who have been raised around "normal" people of their kind. Consequently, to prevent the birth of a disabled child will be considered good by most people. In the light of PND, it would thus be good to offer prospective parents the possibility of not giving birth to a nondisabled child (by eliminating potentially disabled ones in utero). Scholars supporting the disability rights movement however view the entire project of genetic screening differently. This difference should be approached by the background understanding of the concept of disability. Criticizing the widely shared medical model of disability that rests on the representation of disabilities as departures from species typical functioning, they emphasize the social construction of disability. This social model of disability asserts that the opportunities lost to impaired people come from environmental design, not from biology itself (Wasserman et al. 2005).

According to their critical perspective, the entire project of prenatal genetic testing and the accompanying selective abortion based on disability is dangerous. The supporting argumentation usually rests on three claims. The first two holds that selective abortion based on prenatal testing is morally problematic. The third suggests ways on how this practice is driven by misinformation regarding disability. The claim that selective abortion is morally problematic can be supported by two arguments. The so-called expressivist argument focuses on the fact that discriminatory attitudes against disabled people are expressed by the practice. The parental attitude argument claims that using prenatal tests to select against some traits indicates a problematic conception regarding parenthood. The expressivist argument have been criticized on several grounds, basically with reference to the diversity of potential parental intentions to abort a fetus that certainly does not have an intent to send a message or express their views on disability or hurt the dignity of disabled people. One might decide for abortion based on the prenatal genetic test result because she feels that her own life will be significantly compromised. The parental attitude argument rests on the idea that parents can guarantee or create perfection for their children. If parents would have a genuine understanding of what they should seek in parenting, then they would see the particular traits of their children as relatively unimportant. Additionally, they would see the disabled trait of their children as relatively unimportant and leave behind the "selective mentality" that focuses on some specific traits.

Modern societies have a rather bad record of pervasive discrimination against people with disabilities. Although, some might refer to laws and regulations prohibiting or ameliorating discrimination based on disability, there still exists some misinformation and stereotypic thinking regarding what it means to be disabled for individuals and families. There are many widely shared beliefs and perspectives regarding the quality and overall experience of what life with disability is for children and their families. However, these views are not evidence based but anecdotal, and most of them fall into the category of prejudices. Some studies suggest that many members of the health professions view childhood disability as predominantly negative for children and also for their families. These results can be sharply contrasted with the findings on the life satisfaction of people with disabilities and their families.

Eugenics

The question of eugenics is a central issue in discourses assessing the ethical and social impact of developments of genetics, especially on issues related to genetic testing. At the heart of these discussions is the explicit fear that there will be social return or a "backdoor to eugenics" (Duster 2003). Some even regard the contemporary

human genetic practices as a continuation of population policies of the early twentieth century, social cleansing and racist programs. Modern definitions of eugenics are varying, like the concern with the genetic improvement of mankind, the attempt to improve the population through selective breeding, or the promotion of reproductive options favoring desired human genetic traits, and attempts to improve hereditary qualities through selective breeding. Although such definitions will incorporate genetics, they might also involve ordinary acts of human reproduction.

Some critical arguments against policies that promote the widespread use of prenatal genetic screening are referring to such practices are eugenic. But what is eugenics and when can we rightly make the claim that something is eugenic? For many, the eugenic attribute has a stigmatizing effect as it attempts to judge a practice as morally wrong. But on what conditions can such an evaluation be justified? Eugenics has been described as a social movement, as a scientific ideal, as a set of practices, as a doctrine, or as an ideal. Looking beyond the British, the American and the German "versions" of the eugenics movement and the politics of racial hygiene, the eugenics idea has a very diverse history that resulted in very different consequences worldwide. The contemporary uses of eugenics in bioethical and policy discussions are also rather diverse and usually very obscure. As one scholar observed, eugenics is a word with nasty connotations but an indeterminate meaning (Paul 1998). Some would consider a practice as eugenic if it has the intention to improve the gene pool of society, but for others even unintended consequences might imply that a practice is eugenic. Others would argue that it is not the goal of eugenics (which is to improve the health of the population) that is really problematic but the means (like coercive sterilization) to achieve those goals. Against the eugenics conceptual background, the morality of prenatal genetic screening clearly suggests some measure of caution.

Future Challenges: Noninvasive Prenatal Diagnosis (NIPD)

Identified in the maternal blood in 1997, cell-free fetal DNA (cffDNA) offers another avenue for genetic diagnosis of the fetus. While there is still a need for larger validation studies to be conducted for a variety of uses, some studies already demonstrated that with the NIPD using cffDNA, the whole genome of the fetus can be sequenced and analyzed. This raises the possibility that in the not too distant future samples for testing might be available for any risk factors and mutations in a simple, noninvasive way. This future scenario certainly poses a further challenge on the established justification of prenatal screening programs that emphasize meaningful reproductive choice (Dondorp and van Lith 2014).

Conclusion

In most developed countries, prenatal screening for fetal abnormalities is offered as part of publicly funded public health programs. However, these programs are not aimed at prevention of disease, or reducing the disease burden in society as in other screening programs. Except in a few cases, where therapy is available and feasible, usually the clinical consequence of prenatal testing is abortion of the fetus with a genetic condition. Prenatal genetic screening programs aims at enabling couples make meaningful autonomous reproductive choice. This paradigm of autonomous choice is reflected in the relevant guidelines and professional society's recommendations stressing nondirectivity in genetic counseling and the significance of balanced information provision. Screening should be presented as an option while avoiding direct suggestions that women should take the test and how to proceed after the test results. This paradigm of autonomous choice – as being the paradigm of prenatal screening programs – attempts to avoid the charges of trivializing abortion decisions, of being discriminatory against the disabled, and opening a backdoor to eugenics. This is a substantial challenge on the current practice of prenatal screening, because as a response it should prove that as an aim for the program, individual autonomous choice is present in all dimensions of the practice: in the way prenatal screening is presented, offered to pregnant women, organized, and evaluated. In

the final analysis, there is a significant discrepancy between the stated aims of PND and the clinical reality, which leaves the ethical framework open to future criticisms and modifications.

Cross-References

- ▶ Autonomy
- ▶ Commodification
- ▶ Consent: Informed
- ▶ Disability
- ▶ Genetic Screening
- ▶ Reproductive Ethics

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Principlism

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Abstract

The term "principlism" designates an approach to biomedical ethics that uses a framework of four universal and basic ethical principles: respect for autonomy, nonmaleficence, beneficence, and justice. It is presented and defended in Beauchamp and Childress' *Principles of Biomedical Ethics*. The basic principles state prima facie (or non-absolute) moral obligations that are rendered practical by being specified for particular contexts. Moral problems arise when principles or their specifications come into conflict with each other. The conflicts are resolved by further specification or balancing judgments. Principlism

justifies moral reasoning by appealing to the method of reflective equilibrium and to the common morality. Principlism is committed to a global bioethics because the principles are universally applicable, not merely local, customary, or cultural rules. They are correlative to basic human rights and set limits to what is ethically acceptable in all societies, but they are also sensitive to particular conditions in societies and cultures that may account for legitimate differences in the ethics of medical research and practice.

Keywords

Coherence theory; Common morality; Human rights; Principles of bioethics; Principlism; Reflective equilibrium; Specification

Introduction

The term "principlism" designates an approach to biomedical ethics that uses a framework of ethical principles that are both basic and global in application. Principlist theory concentrates on the philosophical and practical roles that these principles should play in bioethics. Principlist theory emerged in the 1970s, and the name "principlism" was bestowed in 1990 by K. Danner Clouser and Bernard Gert (Beauchamp and Childress 2013). A principlist theory is committed to a global bioethics because the principles are universally applicable, not merely local, customary, or cultural rules. This entry examines the origin of principlism, its basic framework of principles, and its application to concrete moral problems in biomedical research and practice. The nature of principlism as a global, cross-cultural, and context-sensitive biomedical ethics is emphasized.

The Historical Origin and Development of Principlism

The objective of a theoretically sound and practical framework of basic moral principles in healthcare and biomedical research has a short history. Throughout the long history of Hippocratic medical ethics, a framework of basic principles never played a significant role, despite its commitment to the maxim, "Above all [or first] do no harm."

Basic principles have often been attributed to the Nuremberg Code Trials of war criminals (1948–49) (1947) and the Declaration of Helsinki (1964), two documents that were established in response to inhuman medical experimentation of Nazi physicians using as their subjects inmates of concentration camps. However, neither document presents a framework of basic principles. The so-called Nuremberg Code was part of the verdict in trials against Nazi doctors, held by a US military tribunal in the city of Nuremberg, Germany, immediately after World War II. The code had no prior history and contained ten universal moral requirements for medical research, but no basic moral principles. In 1964, the World Medical Association (WMA) first released the Declaration of Helsinki, which is set out in terms of "principles," but not basic general moral principles. Instead, the Declaration offers an internationally applicable body of practical guidelines for clinical research ethics (Declaration of Helsinki 2015) . They have been correctly described by the WMA as "recommendations as a guide to every doctor in biomedical research involving human subjects (World Medical Association 2008)."

Basic ethical principles that can be understood with relative ease by a broad public first came to prominence in bioethics in the 1970s. As scandals in biomedical research were accumulating, new government and international guidelines were needed, and frameworks of general principles were presented in a manner that allowed them to be readily understood by people with diverse forms of professional training. Two published works became the primary sources of interest in principles of bioethics.

1. *The Belmont Report* of the National Commission for the Protection of Human Subjects in the United States was published in the US *Federal Register* in 1979. The purpose of the *Belmont Report* was to ensure that basic ethical

principles would become embedded in the US research oversight system so that meaningful protection was afforded to all research participants. This Commission advanced three principles as basic and universal, though the Commission did not address global issues or reach explicit conclusions about universality. The three principles are respect for persons, beneficence, and justice (National Commission for the Protection of Human Subjects 1979).

2. *Principles of Biomedical Ethics*, also published in 1979, was written by Tom L. Beauchamp and James F. Childress at the same time the *Belmont Report* was being drafted. Whereas the goal of the National Commission was basic principles for research ethics, *Principles of Biomedical Ethics* presents a set of basic principles suitable for application to ethical problems in medical practice, research, public health, and the like. This book proposes that medicine's traditional preoccupation with a beneficence- and care-based model of medical ethics be augmented by a principle of respect for the autonomy of patients and by wider concerns of social justice. It also has the objective of bringing order to various debates on bioethical issues that at the time the book was originally published lacked a systematic conceptual and moral framing, both in public discussions and in academic settings.

The moral framework in *Principles of Biomedical Ethics* Beauchamp and Childress (2013) has been under continuous discussion and modification since 1980 and is still today, in its seventh edition, considered the main resource for principlist theory. The presentation here focuses on its framework of principles and supporting arguments.

The Framework of Principles

Constitutive for principlism is the assumption that morally relevant aspects of many real-world situations cannot be reduced to a single, supreme moral principle but need to be captured by a plurality of irreducible principles. The principles in the principlist framework are grouped under four general categories: (1) respect for autonomy, (2) nonmaleficence, (3) beneficence, and (4) justice. Analysis of the meaning and moral demands of each of these principles is the core of principlism:

Respect for Autonomy

The two basic conditions of being autonomous are liberty (the absence of controlling influences) and agency (self-initiated intentional action). Personal autonomy is self-rule free from both controlling interference by others and from limitations that prevent meaningful choice, such as inadequate understanding. The autonomous individual acts freely in accordance with a self-chosen plan, analogous to the way an independent government manages its territories and sets its policies. The principle of respect for autonomy protects the rights of individuals to hold certain views and to make certain kinds of choices. It contains both a negative obligation and a positive obligation. As a negative obligation, autonomous actions must not be subjected to controlling constraints by others. As a positive obligation, autonomous actions must be treated respectfully, and individuals must be appropriately informed, educated, encouraged, and assisted if needed to foster their autonomous decision making. Respect for autonomy obligates professionals in healthcare and research to procure informed consent for significant procedures by disclosing relevant information, ensuring understanding and voluntariness, protecting confidentiality of personal information, and promoting autonomous decision making. True respect therefore requires more than mere noninterference. It includes, at least in some contexts, building up or maintaining others' capacities for autonomous choice while helping to allay fears and other conditions that destroy or disrupt free agency.

Nonmaleficence

The second principle, nonmaleficence, requires that actors abstain from causing harm or risk of harm to others. Not "harming" others means not

thwarting, defeating, or setting back their interests. The term "interest" does not refer to what a particular individual seeks, desires, or happens to be interested in. Rather, it refers to that which is genuinely in an individual's interest, which is a welfare condition or a welfare advantage. Typical welfare advantages are those of health, mental and physical fitness, and social inclusion and opportunities.

Beneficence

The principle of nonmaleficence requires *not acting*. By contrast, principles of beneficence require *acting* – in particular acting to benefit others. These principles are concerned with the well-being of individuals, but nonmaleficence and beneficence are notably different in their demands – the one requiring individuals to avoid taking actions and the other requiring that actions be taken. Principles of beneficence require that we prevent harms from occurring, remove harming conditions that exist, and promote the good of others. Beneficence in the care of patients has traditionally been the centerpiece of medical ethics. It is presented in principlism as a foundational value, but not *the* sole foundational value in healthcare and research ethics. Many specific duties in medicine, nursing, public health, and research are expressed in terms of a positive obligation of beneficence to come to the assistance of those in need of treatment or in danger of injury.

Justice

Principles of justice make up the final part of the framework. The broad term "justice" refers to fair and appropriate treatment in light of what is due or owed to individuals, whereas the narrower term "distributive justice" refers to fair and appropriate distribution of benefits and burdens determined by norms that structure the terms of social cooperation. There is no single principle of justice in principlism, because no single principle is capable of expressing the diverse array of obligations of justice. Basic obligations in social justice are to provide each individual in society with the means necessary for the realization of core aspects of well-being, such as health. In principlism many issues of justice are about the distribution of primary social goods such as essential health benefits and fundamental political rights and also about the distribution of burdens such as bearing the risks of participating in medical research. A basic ethical problem in every society, and certainly for principlism, is how to structure healthcare systems and research systems so that burdens and benefits are fairly and efficiently distributed and a threshold condition of equitable levels of health and access to healthcare is in place.

This principlist framework is understood as part of (but not the whole of) a larger body of universal and basic moral requirements that all impartial, morally committed persons accept – what is called "the common morality" in principlism. The framework is regarded as complete in the sense that it is capable of covering all morally relevant aspects in medicine, public health, health policy, and related fields such as animal research, pharmaceutical research, and healthcare insurance.

Rendering Principles Practical Through Specification

The meaning and the general moral content of the basic principles in principlism are relatively straightforward, but it is often unclear how to bring them to bear on concrete cases and issues because the principles underdetermine their application. That is, there is too little content in abstract basic principles to determine many practical rules, judgments, and recommendations that are essential in particular contexts. Practical policies that guide action in healthcare and medical research do not follow directly from highly abstract concepts and principles such as "respect for autonomy" and "beneficence."

Principlism therefore holds that its principles must be specified to give concrete guidance about when and how to obtain an informed consent, how much information must be disclosed, how to maintain confidentiality, and the like. Specification is not a process of producing, explaining, or defending general principles, but of making them relevant and useful in specific contexts.

Specifying the norms with which one starts, whether the basic principles in the principlist framework or norms that were previously specified from those principles, is accomplished by adding context-specific clauses that enrich the content and narrow the scope of the norms. As Henry Richardson puts it, specification occurs by "spelling out where, when, why, how, by what means, to whom, or by whom the action is to be done or avoided (Richardson 1990, 2000)."

Specification entails that every instance of a specified norm is also an instance of the general norm. For example, every instance of obtaining informed consent in healthcare is also an instance of respecting autonomy, but not vice versa. The specification relations link the abstract four principles to concrete moral problems, cases, and issues of policy. Another example of a specification of respect for autonomy is "respect the autonomy of competent patients when they become incompetent by following their advance directives." Often, a string of progressive specifications is necessary to establish a transparent connection between a basic norm and a particular moral issue or case. The moral authority of the basic norm is transmitted step by step all along the way to the final, specific judgment.

The *formal* requirements of specification – adding clauses to and narrowing the scope of a norm – do not determine the substantive content of how norms are to be specified. The content is determined by a context-specific interpretation of the norm being specified, based on a judgment regarding adequate clauses for the particular context. Thus, the method of specification requires the exercise of substantive moral deliberation, and therefore more than one line of specification of principles is commonly available when confronting practical problems and moral disagreements. Different persons or groups may justifiably offer conflicting specifications. The variability in the particular moralities of different communities, cultures, and institutions can be seen, at least in part, as the result of differences in the specification of basic principles that are themselves shared in each of the groups.

Principles as Statements of Prima Facie Obligations

Oxford philosopher W. D. Ross developed a theory that has deeply influenced *principlism*. He contrasts prima facie obligations with actual obligations. All obligations are prima facie, which means that an obligation *must* be acted upon unless it conflicts with another obligation that is of overriding importance in the situation. One's actual obligation is determined by an examination of the respective weights of the competing prima facie obligations. No obligation is overriding in all situations (Ross 1988).

Likewise, in principlism basic principles and their specifications claim universal validity in the sense that they constitute good moral reasons for or against certain acts in any case to which they apply. However, all moral norms can be justifiably overridden in some circumstances by other moral norms with which they come into contingent conflict. For example, a physician might withhold some diagnostic information from a rehabilitation patient in order to make the treatment regimen go maximally well and also might justifiably disclose confidential medical information about a person in order to protect the rights of another person or the reputation of an institution. Principles, duties, and rights are not absolute (or unconditional or unbreachable) merely because they are universally valid, and there are justified exceptions to all basic principles.

Moral Conflicts and Their Resolution

In principlism, moral problems arise because of conflicts of prima facie duties. Classic examples are problems of paternalism where physicians seek to provide health benefits by overriding a patient's autonomous choice. These problems involve conflicts between prima facie duties of beneficence (providing proper care) and prima facie duties of respecting autonomy when, for example, a patient refuses a lifesaving amputation of his/her leg.

Principlism proposes either dissolving moral conflicts by further specification or deciding the

conflicts through balancing judgments. The first strategy consists in critically reviewing the conflicting norms and, upon due consideration, determining a need for a specification that dissolves the seeming conflict. In the amputation case, imagine that the patient rejects amputation of his/her leg although he/she has been properly informed and understands that the intervention is the only way to save his/her life. This situation creates a conflict for the physician between the prima facie duty of beneficence to save the patient's life and the prima facie duty of respecting autonomy by respecting the patient's informed refusal of the amputation. Imagine further that the patient suffers an acute depression that makes his/her focus, with a controlling emotional attachment, on his/her leg and blinds his/her to remaining life opportunities. This blindness is a reason for critically rethinking the specification of the principle of respect for autonomy in this case. Arguably, the patient's refusal is not adequately autonomous because his/her judgment is overwhelmed by an emotional distortion. Therefore, the appropriate specification of the principle of respecting autonomy would be to respect the refusal of a patient who is adequately informed, adequately understands, and is not controlled by an emotional attachment. This specification allows an overriding of the patients' refusal because it would not violate the principle of respect for autonomy. Here the seeming conflict between the duties of respecting his/her refusal and saving his/her life dissolves.

In other cases, in which conflicts between prima facie duties cannot plausibly be dissolved, "balancing" the duties may be pursued effectively. No supreme principle or rule is available in principlist theory to balance conflicting obligations. Rather moral judgments have to be made that identify the best moral outcome, all things considered. For example, in the case above, a judgment is required to balance the risks of the amputation against the benefits of saving the patient's life. Only if the benefits outweigh, all things considered, the harm can amputation be justified, and the moral conflict between harming and benefitting can be decided in this manner.

Balancing judgments are not mere expressions of intuition. They are the result of careful reasoning that involves evaluation of evidence, assessment of consequences, a search for alternatives, consideration of possible biases and partialities, and other heuristic measures. In everyday life, thoughtful and impartial people agree on most balancing judgments. For instance, as in the case above, the risks of many surgical procedures are commonly judged to be outweighed by their expected benefits. However, in some cases, thoughtful and impartial persons do disagree, even after thorough reflection, on the moral weight of conflicting prima facie obligations. In principlism these cases are not expected to have only one definite ethical solution, as there may be a (limited) plurality of acceptable solutions. The only question is how well one *justifies* one's solution, another issue principlism treats.

Justification in Principlism: Reflective Equilibrium and Common Morality

The selection of basic principles, their specification in various contexts, and solutions of moral problems by further specification or balancing are in need of justification, given the potential for competing judgments. Thus, principlism is not complete without a theory of justification, i.e., a systematic account of how to support one's judgments.

Principlists argue that moral judgments are justified by the evidence and plausibility they gain in conjunction with related moral and nonmoral beliefs and ultimately by the coherence of the belief system as a whole. The coherence of a system of beliefs (whether in the form of a theory, the rules of institutions, or the beliefs of individuals) is established through a reflective testing, specifying, and revising of its elements with the goal of making them as consistent, mutual supporting, and unified as possible – a method of reflective equilibrium originally devised by John Rawls (1999) and extended in theoretical bioethics by Norman Daniels (1996). The goal of reflective equilibrium is not merely rendering particular views and theories coherent and plausible, but to

seek a "broad" equilibrium, taking under consideration all relevant and plausible beliefs, whatever their source or level of abstraction. Accordingly, in the ideal a diverse array of particular judgments and views, rules, concepts, data, background theories, and the like are identified as relevant resources for moral reflection to be brought into equilibrium or rejected as being incapable of contributing to the coherence of the moral belief system. In principlism, the method of specification is emphasized to achieve reflective equilibrium by producing coherent strings of norms that connect basic principles, derivative norms, and context-specific judgments.

The formation of reflective equilibrium requires that initial judgments – the starting points of deliberation and justification – are available to be specified and rendered coherent. Rawls refers to these initial beliefs as considered judgments, i.e., judgments that are least likely affected by conflicts of interest and other distorting influences and, therefore, appear at least provisionally acceptable on their own. Principlists follow his lead and take its basic principles to be the main considered judgments at the roots of medical morality. These principles are considered as part of "the common morality," which is a set of basic moral norms that, in the course of human history, have proved to be of fundamental importance for the prevention of harm to, and the general flourishing of, communities and societies.

In contrast to Rawls, who does not refer to the concept of the common morality, principlism takes its basic considered judgments not only to be provisionally acceptable as starting points but as a solid moral base for the formation of reflective equilibrium. In principlism, a bare coherence of beliefs is not sufficient for justification, because the body of substantive judgments and principles that cohere could be merely a system of prejudices and ill-considered opinions. Normative views are often wrong not merely because they are incoherent, but because they irreparably conflict with basic moral principles. Therefore, in principlism, the principles function as the primary considered judgments and are justified by being basic to common morality. Particular judgments are justified by being shown to be both in accordance with the basic principles and to make a contribution to the coherence and plausibility of the overall structure of moral beliefs.

Because basic principles have a foundational place in the common morality in principlism, the theory of justification is not purely coherentist, despite its clear commitment to coherentist methods of deepening a system of moral beliefs. In this way both foundationalist theory and coherentist theory are given appropriate places in the principlist theory of justification.

There is no reason in principlist theory to expect that the process of rendering specified norms coherent will come to an end or be perfected. The process of reflective equilibrium is viewed as a continuous work in progress – a relentless process of creating and improving moral norms and increasing coherence. Medical morality, like all of morality, is not a finished product. Principlism also holds that moral systems such as research ethics and clinical ethics can be rendered coherent in more than one justifiable way, though not if basic principles are disregarded. Multiple particular moralities such as those found in medical institutions can be expected to present coherent ways to specify universal principles, but it is also expectable that some form of incoherence will be a ubiquitous problem in all such particular moralities. Normatively, principlists demand no more than that agents faithfully specify the universal principles with an attentive eye to overall coherence.

Using Principles in Practical Deliberation

Principlism approaches practical deliberations by embedding its framework of principles in a structured procedure of discussing cases and issues by what may be called a heuristic method for putting principles to work. The structure of the procedure may be adapted to different fields and types of moral problems in bioethics. The following five steps have proved useful in the context of clinical ethics in particular:

1. The ethical problem and the options for action are described as precisely as possible.

2. All facts and circumstances that seem morally relevant for the problem at stake are documented and explained. These involve medical aspects (e.g., diagnostic findings, treatment options, risks and side effects, etc.) as well as broader circumstances such as personal backgrounds of people involved, societal conditions, societal perceptions of unfairness, etc.
3. The basic principles are specified to become relevant for the circumstances of the case and the solution of the moral problem. This is the core work of principlism in practice and should be done in a detailed manner. Strings of specification may be needed, with attention to the formal requirements of specification as well as to the plausibility and coherence of the specified norms' content. In clinical settings, it is useful to first specify the principles of beneficence and nonmaleficence and to balance them with regard to the overall well-being of the patient. This deliberation is related to the determination of the "medical indication" for a particular treatment. Next, the patient's choice and competence to choose are determined, and the principle of respect for autonomy is specified independently of considerations of well-being. Finally, it is specified how third parties such as physicians, relatives, and society are affected, including their preferences and well-being.
4. Once the proposed specifications are determined, the problems of the case or issues are worked out by examining any conflict of norms that may be present. For a solution of a prima facie conflict, the norms must be critically revised, further specified as appropriate, and balanced against each other, as described previously.
5. The resulting recommendations are critically assessed by addressing the strongest objections and the reasons favoring and disfavoring these objections.

Principlism as a Global Bioethics

Several features of principlism make it a global, cross-cultural biomedical ethics.

Global Acceptance of Basic Moral Principles

Principlism claims that its basic principles (and other principles in the common morality) are acceptable in all cultures and communities around the world, despite profound differences in their particular specified moralities. Moral differences in communities usually descend from different specifications and balancing of the basic norms, which can be accounted for by different living circumstances, historical experiences, religious and secular beliefs, and other particular cultural and political sources of different communities and societies.

Although principlism was first developed in the United States, its basic principles are not considered American or even Western products, but as drawn from the common morality that is shared globally among persons committed to morality. It has been claimed by some critics that principlism represents distinctly American values and that other basic moral principles are needed elsewhere – for example, European values such as dignity, precaution, subsidiarity, and solidarity are said by some to work better in Europe (Rendtorff 2002). Principlism does not deny that other moral norms such as solidarity are available that are useful in some contexts. But principlists argue that its basic principles are not parochial and are the best basis of an ethical framework for biomedicine in every society worldwide (cf. Gillon 1993).

The Convergence of Moral Theories on Basic Principles

Principlism assesses its basic principles as acceptable for all major ethical theories that are committed to universal obligations and rights. Deep and perhaps irresolvable differences exist between types of ethical theory such as Kantian, utilitarian, and virtue ethics with regard to ethical reasoning and justification, and they all converge to acceptance of a similar set of substantial moral norms that should govern conduct in particular contexts. Proponents of many ethical theories with different philosophical, cultural, and religious backgrounds have accepted the principles featured in principlism, even if they sometimes prefer another terminology and support them for different

reasons. They may to some extent interpret, specify, and balance the principles differently with regard to particular cases and issues, but they share with principlism a commitment to similarly formulated universal moral norms.

The Principlist Framework as a Basis for Cross-Cultural Moral Judgments

The acceptability of the basic principles across cultures and theories will not, of course, eliminate all substantive moral disagreement. Principlism serves only as a common basis for cross-cultural and cross-theoretical controversies in biomedicine. Medical practices and opinions from all cultures and societies are to be judged within the framework of the basic moral norms. No healthcare professional from any culture or community can plausibly deny that avoiding causing pain or other harms to patients, providing them with health-related benefits, respecting their freedom of choice, and treating every patient and patient population fairly are of central moral importance. Therefore, the principlist framework is a legitimate basis for judging – and, as appropriate, criticizing – morally relevant features of healthcare and research with human subjects in foreign cultures as well as in one's own culture.

Research Ethics as an Example of Global Standards

Principlists share the goal of developing globally accepted standards of bioethics that are justified within the principlism framework. A good example is the research ethics that govern human-subjects research in biomedical research centers. Research ethics is today a body of worldwide standards that are no longer in serious dispute in any country in which research with human subjects is conducted. Even if the rules vary to some extent from one country to the next, every community in which scientific research with human subjects occurs has accepted a body of universally valid rules. Examples are requirements to disclose all material information to subjects; requirements to obtain individual, voluntary, and informed consent; requirements to protect subjects in research against excessive and unnecessary risk; and requirements that ethics review committees criticize and approve research protocols. These rules grounded in general moral obligations of not harming other people, helping them in need, respecting their freedom of choice, and giving what is owed to them. The Declaration of Helsinki is one of many examples of the formulations of such rules (though, in contrast to principlism, it contains no arguments about how the rules are grounded).

The Place of Context Sensitivity

The prima facie status of moral norms and their potential to be specified allow for precise and subtle judgments in different contexts, appreciating all morally relevant differences between societies that may account for their different practices and moralities. It would be false and inappropriate for a global bioethics to claim to be able to locate the same particular obligations for every community regardless of their environments. As examples, legitimate claims of people in need of care may well be different in nomadic tribes and highly developed cities, and the demands of rules of informed consent will need to be adjusted for illiterate and literate societies. Specific moral requirements are the result of specific natural, economic, historical, social, and cultural conditions, including religious beliefs and conceptions of healthy environments.

The Rejection of Relativism

However, there are limits to what is to be tolerated and what constitutes reasonable judgments and justifications. The bare fact that a particular medical practice has a long tradition behind it in some societies is not by itself a sufficient justification for the practice. Principlism rejects the claim of moral relativism, according to which every society (or even every person) has its own morality that cannot be judged and criticized from outside. Medical practices that cannot reasonably be justified by and made coherent with the principlist framework of universal norms are to be rejected. For instance, female genital mutilation cannot be justified in principlism, irrespective of cultural practices and despite its presence in several countries. Injuring and hurting children severely for no medical benefit by using physical force are serious

violations of basic principles that cannot reasonably be outweighed by culturally specific ideas of chastity, aesthetics, or family honor. This form of relativism is entirely unacceptable.

Acceptance of a Moderate Pluralism

Principlism does not reject all forms of moral pluralism. Many responsibilities, ideals, attitudes, and sensitivities found in, for example, cultural, religious, and professional guidelines can be justified. In some situations, different practices are morally justified, fulfilling moral requirements equally well. For instance, healthcare systems may be organized differently but perform equally well with regard to health outcome, fair financing, and general service quality. Moreover, conflicting prima facie principles might be of equal or incomparable moral weight in particular situations such that each alternative is acceptable from a principlist point of view. Many difficult decisions in clinical practice can be interpreted in this way, e.g., when the chances and the risks or the opportunity costs and the benefits of a particular treatment are judged as having equal moral weight. Well-informed, impartial, and morally committed persons may well disagree, upon due consideration, about morally relevant facts (e.g., chances and risks of therapies, presumed preferences of patients, and societal consequences of particular practices) or about how to specify and balance moral obligations in particular situations.

Principlists expect a pluralism of theories and moral positions in bioethics, but they draw a threshold line at what is tolerated: All justified particular moralities share the general norms of the common morality with all other justified particular moralities. These norms are universal, not local, and so can only be specified, not abandoned.

The Relation Between Universal Principles and Human Rights

A human rights approach has in recent years become the most widely used way to express foundational moral norms that cross national boundaries and that support the statements of international law and policy made by international agencies and associations. Assuming that they have this status, how are the basic principles of obligation in principlism related to human rights? And how might they jointly constitute a global bioethics of universal norms?

A human rights approach, like principlism, provides standards that transcend morally problematic norms and practices in particular cultures. These rights are interpreted in principlism as prima facie rights that must be specified in order to become practical moral, political, and legal norms, just as in the principlist account of basic, universal norms of obligation. Without careful specification(s), a human right such as the right to be told the truth will leave us uncertain what the right does and does not cover. Even such fundamental human rights as the rights to life, freedom of religion, and property can be justifiably overridden in particular situations if they come into conflict with other human rights or important public interests. For example, some cases of self-defense, bans of extremist religious groups, or expropriation of private land for important public purposes may be justified.

Principlists regard the grounding of human rights as in one respect identical to that of the basic principles of obligation. Both are basic norms (basic considered judgments) of the common morality. Principlists also see basic principles (and the norms that specify those principles) as uniformly translatable into correlative rights; likewise, basic rights are always translatable into correlative basic principles of obligation. For example, the basic principle that one should not cause pain or suffering to others (the principle of nonmaleficence) directly translates to the right to not be caused pain or suffering by others. Likewise the principle of respect for autonomy directly translates into the human right to have one's autonomy respected.

This thesis of the correlativity of rights and obligations does not hold that universal rights are identical in meaning to universal principles. Rights in general are *justified moral claims* to something that individuals or groups can legitimately assert against other individuals or groups. Human rights in particular are those that all

humans possess. What rights language adds to the language of principles of obligation is the part about "justified moral claims." A right gives its holder a justified claim *to* something (an entitlement) and a justified claim *against* another party. The latter party is the holder of the correlative obligation. Claiming is a mode of action that appeals to moral norms that permit persons to demand, affirm, or insist upon what is due to them because a correlative obligation requires that the claim be honored. "Rights," then, are justified claims that can legitimately be asserted against persons or institutions who are the bearers of obligation to these individuals or groups. The right to receive an adequate level of healthcare, for example, is correlative to a communal obligation to provide the necessary resources. If this right is a human right, then all humans are owed a proper distribution of available resources, a demand of justice.

Conclusion

Principlism presents a global bioethics. It has basic moral norms that are acknowledged by morally committed persons across all cultures and societies, and it offers a moral framework that is universally binding and that allows for a systematic assessment of moral issues in biomedicine. It both sets limits to what is ethically acceptable in all societies and is sensitive to particular cultural, economic, and natural conditions in societies that may account for legitimate differences in medical research and practice.

This approach requires tolerance and mutual respect in global bioethics, without spreading moral indifference. If there is no one definite ethical solution to every moral problem, but a limited plurality of acceptable solutions to some moral problems, then conflicting opinions and practices in different cultures need not be based on partiality, ignorance, or irrationality.

Cross-References

- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Bioethics: Founders
- ▶ Clinical Ethics: Methods
- ▶ Codes of Conduct
- ▶ Consent: Informed
- ▶ Ethics
- ▶ Global Ethics
- ▶ Human Rights
- ▶ Justice: Theories of
- ▶ Paternalism
- ▶ Professional Ethics
- ▶ Respect for Autonomy

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Prisoners

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Abstract

Although imprisonment limits many rights of a person, a prisoner still retains the right to health. Upon imprisonment, the health care for prisoners becomes a State obligation. International guidelines recommend that health care in prison must be administered independently of the reasons for imprisonment and governed by the same medical ethics that exists outside prisons. The minimum standard of care in prison that must be respected universally is the principle of equivalence of care. In this entry, two basic medico-ethical concerns relevant in prison health care are discussed: confidentiality in seeking health care and dual loyalty of medical professionals. The latter includes dual loyalty in difficult situations such as torture, solitary confinement, and hunger strike. Even today, health care personnel in prisons work in conditions where they are unable to respect the basic principles of medical ethics. Resources and conditions to allow these professionals to respect the basic principles are critical and should be provided. Whenever these conditions and resources are unavailable, their lack should be reported. Prisoners are entitled to receive medical care which is equivalent to the care that any other patient who is not incarcerated receives.

Keywords

Prisoners; Ethics; Confidentiality; Health care; Human rights

Introduction

The number of prisoners has increased in the last decade. It is estimated that a total of 10.2 million persons are incarcerated in penal institutions worldwide (Walmsley 2013). In this entry, a prisoner is any person deprived of liberty and incarcerated in one of the following correctional institutions: remand prisons, penitentiaries, and police stations.

Imprisonment rates vary between different regions of the world as well as within a region. The country with the highest number of prisoners is the United States with 2.24 million imprisoned persons and an incarceration rate of 716 per 100,000 persons (Walmsley 2013). In Europe the average incarceration rate is at 98. In Demark, it is 73 prisoners for every 100,000 inhabitants, while that of Spain is 147.

The prison population includes minority subgroups such as juvenile prisoners, female prisoners, and older prisoners. Female prisoners tend to constitute less than 10 % of the total prison population, according to the International Center for Prison Studies. Among these different subgroups of prisoners, the number of aging prisoners is increasing significantly (Human Rights Watch 2012). Furthermore, ethnic and racial minorities are overrepresented in the prison population.

As a closed environment, one of the main goals of a prison is to separate those it incarcerates from the general population. In doing so, it intends to prevent these persons from committing future crimes and thus to limit the danger they pose to society. Prison structures are generally designed for young and healthy persons and do not address the needs of vulnerable subgroups, such as aging prisoners as well as those with health problems.

As such, the prison environment poses an additional burden on the health of different group of prisoners. Furthermore, poor prison conditions, such as overcrowding, insanitary services, imposed promiscuity, poor diet, and limited opportunities to exercise, pose severe challenges to ensuring good physical and mental health.

Both the physical health and mental health of prisoners are found to be poorer than those of individuals of the same age and demographic characteristics in the general population (Fazel and Baillargeon 2011; Wilper et al. 2009). Aging prisoners carry a high disease burden (Aday 2005–2006; Fazel et al. 2001). Psychological health is of great concern among all prisoners because of the stressful prison environment and the inherent characteristics of the prison population itself: prisoners often belong to the lower socioeconomic strata exposing them to poor health behaviors and increasing their chances of suffering from illnesses.

Once incarcerated, prisoners lose the right of free movement and decision-making about their daily life. At the same time, the responsibility of care for these persons is transferred to the prison administration. This means that the State is responsible for answering to the needs of prisoners. It must ensure that adequate health care is provided so that they do not suffer undue harm. Studies have shown, however, that the State often falls short in guaranteeing prisoners' right to basic care and health. The poor living conditions make prisons an ideal breeding ground for infectious diseases such as tuberculosis, HIV, and hepatitis B and C. Another important health effect of prison life is accelerated aging. Prisoners are reported to age 10–15 years faster than their peers in the general population (Loeb and Abudagga 2006).

In light of the many health problems that prisoners face, this entry will focus on concerns that are directly related to biomedical ethics, that is, on those that pertain to health care provision to prisoners. Thus, this entry first presents the standard of health care prisoners should receive in light of existing guidelines. Next, it discusses some of the main ethical concerns that arise in the provision of medical care to prisoners. Criminological questions related to the justification of imprisonment will not be covered.

Standard of Health Care for Prisoners

Universal medical ethics and human rights guidelines stipulate that persons should not suffer unfair discrimination based on legal status or other characteristics, such as gender, age, religion, political opinions, and race (*see* Council of Europe's Resolution (73)5 Standard minimum rules for the treatment; United Nations (UN) Standard Minimum Rules for the Treatment of Prisoners; UN for the Protection of All Persons under Any Form of Detention or Imprisonment; UN Basic Principles for the Treatment of Prisoners; United Nations Educational, Scientific and Cultural Organization (UNESCO) Universal Declaration on Bioethics and Human Rights). Therefore, health care provided to prisoners should be of good quality and at par with what is provided to patients in the general community. These guidelines also underscore that prisoners should have access to primary health care services as well as specialized professionals based on their needs and in a timely fashion. Moreover, health care must be provided to prisoners in a manner that respects their basic human dignity and their rights as individuals and as patients.

Health care personnel working in prisons are expected to implement their roles as care providers in the best interest of the patient by using their best judgment and independently of external loyalties (*see* European Committee for the Prevention of Torture and Inhuman or Degrading Treatment or Punishment (CPT Standards)). The standard of care for prisoners is based on the principle of equivalence (*see* UN Principle of Medical Ethics) which is also the standard principle in European ethical and legal frameworks. It is explained in detail in the Council of Europe "Recommendation No. R (98)7 of the committee of ministers to member states concerning the ethical and organisational aspects of health care in prison." The recommendation has the status of a "soft law." It demands that health care provided to prisoners is equivalent to what a non-imprisoned person receives as part of the national health policy. It also underscores that the doctor-patient relation in prison should be guided by the same standard of professional ethics which is in force

outside prisons. This means that a prisoner-patient has the same autonomy rights, such as informed consent, medical confidentiality, freedom of choice and decisions, and the right not to be harmed, as those that apply to the general population (*see* CPT Standards; UN Principles of Medical Ethics; UN Basic Principles; World Medical Association (WMA) International Code of Medical Ethics). The World Health Organization in its Health in Prisons Programme also points out that equivalence of care includes a minimum level of care that prisoners should receive. This standard has been incorporated in professional recommendations of organizations such as the Swiss Academy of Medical Sciences and the Australian Medical Association.

Irrespective of this minimum standard of health care, prisons fail to adequately address the care needs of inmates (Fazel et al. 2004; Wilper et al. 2009), and the principle of equivalence of care is not met. This raises the question as to how health care professionals should act if they notice the noncompliance with ethical standards. Should medical associations prohibit physicians from working in such unethical conditions and therefore increase the pressure on the State to adapt the conditions or should they try to avoid the worst consequences for prisoners and do their best in unethical conditions? It is important also to point out that several scholars state that the equivalence standard may be misleading as it cannot properly respond to the complex health care needs of prisoners (Niveau 2007; Jotterand and Wangmo 2014). These observations are in part the result of the fact that there is no clear definition of equivalence of care within the prison context. Still, despite these doubts, this principle remains the most widely accepted minimum standard of care that prisoners must receive.

Medical Ethics in the Care of Prisoners

Prisoners are not different from any other patient in the medical care context, and the place where the physician cares for his patients should not affect medical ethics. According to the WMA Declaration of Lisbon as well as UNESCO Universal Declaration on Bioethics and Human Rights, patient rights apply to prisoners as well. These basic medical rights include, among others, the right to autonomy, ability to provide informed consent, right to their information being kept strictly confidential, right not to be harmed by the health care personnel, and health care personnel working on their best interest. However, the inherent prison context exerts specific threats to these rights. This is particularly true when physicians providing mental health care are asked to also provide expert opinion on their patients to the judicial authority. Such a situation brings forth two critical ethical concerns: breach of confidentiality and dual loyalty. Health care personnel working in correctional settings must be particularly sensitive to those violations of patient rights and prevent them actively. Furthermore, physicians must be vigilant not to confound the roles of a treating physician with that of an expert witness. Finally, with a number of limited resources available in prisons, health care professionals may not be able to address each single prisoner's need, thus bringing forth the issue of inadequate care. These medico-ethical situations that are characteristic of the prison context are discussed below.

Confidentiality

Ensuring that medical confidentiality is upheld in the prison is highly important for the doctor-patient relationship. International guidelines such as the CPT Standards and WMA International Code of Medical Ethics conclude that prisoners should enjoy the same medical confidentiality as any other patient. In prison the trust relationship is based upon the belief that the (sensitive or incriminating) information that prisoner-patients share with their physicians will not be transmitted to others. However, in certain situations, confidentiality is breached without the consent of the patient (Pinta 2009). In these cases, it is recommended that the breach of confidentiality is limited, justified by medical ethics, required by law, and that patients are informed about it (Appelbaum 2002). This recommendation has been endorsed by professional organizations such as the World Psychiatric Association and the Royal College of

Psychiatrists. They state that breach of confidentiality is justified if required by law, if it applies in the same way to prisoners and non-prisoners, and if breaches occur to curb danger to third parties.

Prisoners suffer from many acute and chronic health problems, including transmittable diseases and mental health disorders. Their health status may undermine their rights in prison to equal treatment and freedom from harm and violence. Knowledge that someone suffers from a stigmatizing disease such as HIV-AIDS or a serious mental health disorder can lead to discrimination from both the prison staff and other inmates. The discrimination, subtle and obvert, could result in feelings of isolation if other prisoners do not wish to communicate or be in the company of the affected person or even lead to violent reactions including murder. In the case of infectious diseases and communicable illnesses, it is important not only to prevent other non-concerned persons from accessing medical information but also to make sure that such information is not obvious and cannot be deduced by other types of avoidable behavior. For example, when medications are dispensed by prison staff and the names of the medications are clearly announced or if it is known that a certain size or color of the medication denotes certain diagnoses, it is the obligation of health personnel to make those signs less obvious and to find solutions to protect confidentiality adequately.

Considering mental health illness, a prisoner may disclose information about crimes and fantasies that may include (past or future) harm to third parties. A Swiss study with mental health professionals concerning confidentiality in prison highlighted that several of these professionals were uncertain when confidentiality should be breached (Wangmo et al. 2014). From an ethical and legal point of view, an assessment as to whether the new information changes the risk evaluation and implies important harm to others that cannot be averted otherwise than by violating confidentiality was correctly identified as the most important factor to justify not maintaining medical confidentiality. The same study also found that prisoner-patients were informed that confidentiality rules did not apply in the case of expert consultation where the mental health therapy was court-ordered (Elger et al. 2015).

The dilemma arises as to what should be the standard process of informing prisoners about the limits to confidentiality. Routinely informing patients that shared information regarding actions that could harm another person or the patient himself will not benefit from confidentiality rules could compromise the mental health treatment the patient is seeking (Elger et al. 2015). If the past crime of the patient is very much linked to the treatment sought and if disclosing them means further incarceration, it is unlikely that a prisoner-patient would disclose such information (Wangmo et al. 2014). This in turn means that those patients may never get the indicated therapy (which is itself a prerequisite to treat the underlying mental disease and to avoid future harm) since the health care professional remains ignorant of all facts. Thus, a well-argued and evidence-based balance is needed between the right of third persons not to be harmed and the prisoner's right to health care.

Dual Loyalty

In the example discussed above, mental health professionals reported informing their prisoner-patient that in the case of a court-ordered therapy, everything that is discussed will be transmitted to the responsible organization (Elger et al. 2015; Wangmo et al. 2014). Such a direct approach makes it explicit that the loyalty of the physician lies elsewhere. All prison physicians working as therapists must be transparent and explain to their prisoner-patients that their well-being is their main goal as stipulated by medical ethics. At the same time, physicians working in detention facilities have to respect the security rules of the prison. This means that health professionals are often caught between the need to maintain confidentiality toward their patients and the pressure by the justice system or the State to respect the laws (International Dual Loyalty Working Group 2002).

In cases where physicians accept the role of an expert witness, the change of loyalty has to be made transparent to the prisoner who is no longer their "patient" but the person to be examined at the

request of third parties, in this case the judicial authorities (Konrad 2010). This is one of the difficult cases of dual loyalties where a treating physician is expected to provide information to the judicial authorities as part of mandatory treatments. Such situations require clear rules. The Council of Europe's Recommendation No. R(98) 7 states that physicians carrying out a therapeutic role in prisons have the ethical obligation to refuse forensic evaluations of their prisoner-patient's health to determine his or her dangerousness or the ability to stand trial. If no other physician is available and the patient actively requests the treating physician to make an expert statement, they may do so, but it should be clear to the patient what the consequences could be. The reasons for refusing to be an expert witness are multiple. First of all, evaluations of dangerousness need expert knowledge that goes beyond the general knowledge of a treating physician and should remain in the hands of specifically trained physicians. Second, treating physicians have a role conflict of obligations, as the primary goal to act for the benefit of their patients contradicts the role of an expert to provide impartial evaluation.

In order to avoid as well as reduce situations of dual loyalty, it is further recommended by the Council of Europe's Recommendation No. R(98) 7 that prison physicians are independent and not employed by the justice and police departments. If the independence of health care personnel is guaranteed, prison physicians are able to respect the same ethical and legal framework as their colleagues outside correctional setting. To ensure that health care personnel who treat prisoners have the best interest of their patient at the fore, in addition to the recommendations of the Council of Europe, various guidelines underline the significance of professional independence (*see* CPT Standards; UN Principles of Medical Ethics). Discussed below are three situations of dual loyalty more in detail since they are unique to the prison context.

Participating in Torture

In some countries, health professionals are asked to conduct a health assessment of a prisoner in order to carry out torture. In these instances, the physician is demanded to perform the task as required by the State who is in most cases the physician's employer. Irrespective of what is asked from physicians, the British Medical Association underlines that medical ethics dictates that physicians refuse tasks that are not compatible with the goals of medicine. Health care providers should never become party to violations of human rights and participate in any treatments of torture or humiliating practices against a prisoner (*see* International Dual Loyalty Working Group 2002; UN Body of Principles for the Protection of All Persons under Any Form of Detention or Imprisonment). In order to ascertain that physicians act according to their Hippocratic Oath of do no harm, the WMA Declaration of Tokyo "Guidelines for Physicians Concerning Torture and other Cruel, Inhuman or Degrading Treatment or Punishment in Relation to Detention and Imprisonment" states that physicians should not participate in torture and provide any information or tools to facilitate torture. This declaration, again, underpins the need for complete clinical independence of doctors in order to provide the appropriate health care to their patients. The need for better training of doctors in medical ethics becomes apparent if one looks at the history of human rights violations where health personnel participated actively. A recent example of doctors aiding in the process of torture and inhuman acts against prisoners is from Guantanamo Bay (Miles 2007).

Certifying Solitary Confinements

Solitary confinements are used in many jurisdictions as a measure to punish, discipline, or interrogate a prisoner. The European Committee for the Prevention of Torture finds it medically, ethically, and legally questionable whenever prisoners are held in such confinements. It thus "pays particular attention to prisoners held, for whatever reasons (for disciplinary purposes; as a result of their "dangerousness" or their "troublesome" behaviour; in the interests of a criminal investigation; at their own request), under conditions akin to solitary confinement" (*see* CPT Standards, p18). In light of the harm that such a confinement can do to the mental and physical health of a prisoner, it argues that solitary confinement should be proportional and that medical

examination of the prisoner must be done without delay to record the health of the prisoners and to present the consequences of continued confinement. Furthermore, the European Committee for the Prevention of Torture goes on to state solitary confinement could amount to inhuman and degrading treatment in some circumstances. Hence its duration must be limited. Physicians in no means should be involved in declaring a prisoner "fit" to be kept under solitary confinement or "fit" for continued confinement (*see* Council of Europe Resolution (73)5 Standard minimum rules for the treatment of prisoners).

Hunger Striking Prisoners

Hunger strikes are used by prisoners for political or personal reasons as an expression of discontent or as a means to gain certain outcomes (Andorno et al. 2015; Sebo et al. 2004). When a hunger striking prisoner dies, it not only draws negative media attention but also raises important issues related to the respect of human rights. Thus, prison administrations in their attempt to avoid the death of an inmate may wish to use force-feeding and may seek the aid of the physician in doing so. The WMA Declaration of Malta on Hunger Strikers states that physicians must respect the informed decisions made by the prisoner. The issue of informed consent is crucial in these situations because medical ethics requires respect for a detained patient in the same way as for any other patient. The duty of the physician in these cases is to ensure that the hunger striking prisoner is competent and understands the consequences of food refusal. Physicians should take sufficient time to understand the motives and wishes of the prisoner and ensure that he or she has given valid informed consent and was not under any sort of duress when doing so. If the physician is uncomfortable accepting the decision of his patient, the prisoner should be referred to another physician who is able to respect the autonomous choices of the patient. In any case, force-feeding a competent patient in response to the request of the prison administration is not compatible with medical ethics. Respect for human rights and ethical sensitivity are also required when health care personnel is asked to intervene in the case of hunger striking prisoners (Andorno et al. 2015; Sebo et al. 2004). Force-feeding when there is an explicit valid informed consent not to do so amounts to inhuman and degrading treatment and unauthorized invasion of the private space.

Reacting to Inadequate Care

How should health care professionals react to the fact that many prisons fail to address the care needs of prisoners? In light of the lack of adequate living conditions, basic health care, respect for patients' rights, and human dignity, international "soft law" and ethical guidelines claim that it is the duty of the State to ensure prisoners a minimum standard of care (*see* UNESCO Universal Declaration on Bioethics and Human Rights). That means access to care at any time and a standard of care that is equivalent to the general population (*see* Council of Europe Recommendation No. R(98) 7; CPT Standards). Individual health care personnel has the duty to inform prison authorities about the shortcomings – either with the consent of individual patients or providing generalized descriptions – and to denounce any violations of the principle of equivalence to their medical hierarchy and to professional organizations. The latter have, until today, not always taken up their responsibilities to stand up for medical ethics in correctional medicine. For example, the American Medical Association, in contrast to medical associations in other countries, does not recognize the principle of equivalence of care in prisons. Medical associations should follow the widely known guidelines of the British Medical Association and intervene actively in the case where individual physicians find themselves working in unethical conditions.

Conclusion

A prisoner is a human being whose right to move freely and to make certain choices has been restricted. However, a prisoner retains the right to health. Health care for prisoners must be administered independently of the reasons for imprisonment and governed by the same medical ethics

that exists outside prisons. The minimum standard of care in prison is the principle of equivalence of care. This standard should be applied universally. Unfortunately, besides the many examples from the past, health care personnel in prisons are often forced to work in conditions where they are unable to respect the basic principles of medical ethics. This shows not only that physicians working with prisoners often lack the appropriate ethical knowledge but also that they do not get enough support from professional organizations. Resources and conditions that would allow health care professionals to respect the basic principles are critical and should be provided. Whenever these conditions and resources are unavailable, their lack should be reported. Hence, transparency and both individual and professional courage are needed to expose existing ethical and human rights violations. The aim is not to blame single practitioners working in prisons for failing to proceed according to set standards. All health care personnel are responsible for educating their peers, the general population, and policy makers. They are also responsible to actively support the respect for the health care rights of every patient, including prisoners.

Cross-References

- ▶ Autonomy
- ▶ Bioethics: Medical
- ▶ Confidentiality
- ▶ Conflict of Interest
- ▶ Ethics
- ▶ Global Ethics
- ▶ Human Dignity
- ▶ Medicine and Ethics
- ▶ Professional Ethics
- ▶ Vulnerability

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Professional Ethics

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Abstract

Research in professional ethics is a fairly new field of ethics, which is concerned with the values which govern the professions and provides us with the theoretical framework for dealing with issues which arise in the context of the professions.

Keywords

Technê; Art; Craft; Episteme; Science; Profession; Trust; Care; Virtue

Introduction

Though apparently a new field of philosophical research, professional ethics, as a domain of research, can be traced back to antiquity, to the treatises of the philosophers–physicians, and even before them to the work of philosophers such as Plato, Aristotle, and Xenophon. Even though they did not actually have a notion of professional ethics, nevertheless the way they conceived professions was heavily value-laden. Unlike with what happens in our era, in antiquity professions could hardly be divested of the rich evaluative meaning they possessed.

The Origins and the Historical Development of Professions: From *Technai* to *Professions*

Plato on Technai

The notion of profession is not a modern one. Its origins can be traced back to classical antiquity and, in particular, to writers such as Plato, Aristotle, Xenophon, Hippocrates, and Galen. Plato in his dialogues mentions a great number of technai (*τέχναι*), as he calls them, which range from shoemaking, pottery, weaving, carpentry, building, shipbuilding to medicine, music, calculating, and political craft. However, things are not as simple as they may appear to be. It may be true that Plato has not written a dialogue on technai; however, he has a rather sophisticated conception of technê (*τέχνη*) and he comes back to it again and again in his dialogues. He is careful to point out that whereas in some cases the result or the product of technê is different from and independent of the activity, in other cases the result is the activity itself. Thus, in the case of medicine, the result is health, and in the case of building, the result is a house; on the other hand, in the case of calculation, there is no result or product which is independent of the activity of calculating.

Technê is initially associated with a practical component, with knowing how to do certain things or certain activities. But even in this early stage of development, technê also involves a theoretical element, a kind of knowledge or episteme (*ἐπιστήμη*). Throughout his dialogues, the two notions are used in an interchangeable manner, and it seems that Plato's foremost concern is not so much to distinguish technê from episteme as to make the concept of technê clearer.

In his conversation with Polus in the dialogue *Gorgias*, Socrates argues that rhetoric is not a technê but instead an experience (*ἐμπειρία*) (*Gorgias* 462 B-C). He draws a distinction between the industries which are assigned to each of our two parts (*Gorgias* 464 B-C). We have, on the one hand, the tendance of the body, which is divided into two branches, medicine (*ἰατρική*) and gymnastic (*γυμναστική*), while, on the other hand, we have the care of the soul, politics (*πολιτική*), which in an analogous

manner is distinguished into two parts, justice (*δικαιοσύνη*) and legislation (*νομοθετική*). As he points out, between each of these pairs there is some kind of intercommunication (*ἐπικοινωνοῦσι μὲν δὴ ἀλλήλαις*), and medicine is related to gymnastic in connection with the body in the same way that justice is related to legislation in connection with the soul.

Along with these four technai which look after what is best and advantageous for the body and the soul, respectively, there is the art of flattery (*κολακεία*) which divides itself into four parts, i.e., personal adornment (*κομμωτική*) and cookery (*ὀψοποιητικὴ*) on the one hand and sophistry (*σοφιστική*) and rhetoric (*ῥητορική*) on the other. Even though each of these parts of flattery does not possess any knowledge or episteme, it insinuates itself in the position of one of the four branches of technai. Cookery, for instance, pretends to take the role of medicine, assuming that it knows what is good and healthy for the body. But in so doing, it does not benefit the body at all, it simply presents itself as something which is not, seeking what is delightful and pleasant and ignoring what is best (*Gorgias* 464 D–E). According to Plato, flattery is something disgraceful and shameful because it is concerned with what is pleasant and not with what is beneficial. It is not a technê but a habit or an experience, "since it has no account to give of the real nature of the things it applies, and so cannot tell the cause of any of them" (*Gorgias* 465 A). Plato claims that we cannot call something an art if it is irrational (*Gorgias* 465 A). A technê, on the other hand, can give an account of the nature of its object, can explain in a scientific manner the causes of the things it does, and can justify rationally its existence (*Gorgias* 501 A).

It becomes obvious that technê, according to Plato, has two characteristics: (a) it involves knowledge and (b) it is beneficial to its object. The physician and the physical trainer look for the welfare of the body in the same way that the judge and the legislator care for the welfare of the soul (*Gorgias* 464 C).

But Plato also says something further. He claims that engaging with knowledge, acquiring an episteme or technê has a refining influence upon the technites himself. True, in the lower stages of knowledge, the knowledge of the things of the senses, a technites can come to experience certain virtues, the ones required for his expertise. So he may become brave but he has not been virtuous yet. However, as he goes up the ladder of knowledge and he acquires knowledge of the Forms, he eventually beholds (*θεᾶται*) the Idea of Good; he becomes one with it. This man participates in the Idea of Good by virtue of which all other virtues are unified.

It is now understood why the carpenter, the smith, and the farmer, who have a limited knowledge of the objects of the senses and, therefore, possess only certain virtues, will concentrate only on some partial good of the city; whereas the philosophers who have acquired the supreme form of knowledge, i.e., the dialectic, by virtue of which they can behold and thus participate in the Idea of Good, can work for the welfare of the city as a whole (*Republic* 428 b–d). This is how Plato reaches his twofold conclusion: (1) that the persons most appropriately equipped to become rulers of the city are the philosophers and (2) that the craft of ruling is the supreme of all arts.

Aristotle on Technê, Art, and Craft

The notion of technê is taken over and further elaborated by Plato's pupil, Aristotle. According to Aristotle, technê is not distinguished from knowledge, since it is a form of knowledge itself. In the *Nicomachean Ethics*, he offers an account of the different forms of knowledge or science, as he calls them, and of the way these are interrelated (*Nicomachean Ethics*, VI, I, 1138 b 34 – 1139 a 15). All forms of science are processes which take place in the rational part of the soul, unlike the instincts, drives, and desires which are included under the irrational part of it. Thus far, Aristotle emphasizes the common origin of both science and technê. However, as he hurries up to add, if knowledge proceeds by virtue of a kind of likeness or affinity between subject and object, it follows that if there are different kinds of object, then there must be different kinds of knowledge (*Nicomachean Ethics*, VI, I, 1139 a 9–12). As a consequence, if we examine and contemplate things whose principles are necessary or

invariable, then we employ the kind of knowledge which stems from the Scientific Faculty of the rational part of the soul. If, on the other hand, we contemplate things which admit of change and variation, we proceed by means of the kind of science which flows from the Calculative Faculty of the rational soul. Even though in both faculties the aim is the attainment of truth, however, the virtues or dispositions which best qualify them to achieve the truth differ in each case (*Nicomachean Ethics*, VI, II, 1139 b 12–14). Aristotle distinguishes five qualities by virtue of which man attains truth: science, wisdom, intelligence, technê or art, and prudence (*Nicomachean Ethics*, VI, II, 1139 b 15–18).

Science, wisdom, and intelligence are the three (intellectual) virtues by means of which operates the Scientific Faculty of the soul. The objects of the Scientific Faculty are the theoretical sciences which, in Aristotle's days, included first philosophy (metaphysics or theology), physics (natural science), and mathematics (*Metaphysics,* VI, I, 1026 a 18–19). In the case of theoretical sciences, the principles are in the things to be known and not in the knower. Of course, things may be either necessary and invariable (as in the case of philosophy and mathematics) or changeable and variable (as it is the case with natural science). Science or episteme implies knowing things which are necessary and invariable. It also implies knowing things which are variable by virtue of the unchanging principles which are instantiated in these changeable things (Polansky 2000). Moreover, science proceeds by means of demonstration from first principles, and it aims at truth or knowledge. In this sense, it is entirely speculative and not practical, despite the fact that Plato and Aristotle quite often connect the theoretical activity of man with eudaimonia (*εὐδαιμονία*) (*Nicomachean Ethics*, X, VII). Furthermore, precisely because it proceeds by a kind of apodeictic syllogism, science can be taught. Of course, first principles cannot be so taught, because they cannot be conceived through demonstrative syllogisms. If known at all, these first principles can only be known through a particular activity of the Theoretical Faculty initiated by intelligence, the dialectic.

Art or technical skill and prudence (phronesis, *φρόνησις*) are the particular dispositions of the Calculative Faculty of the soul by virtue of which we attain knowledge of things which could have been different from what they are (*ἐνδεχόμενα*). The builder has to think about the size of the house he is about to build in the same way that the physician has to contemplate in advance the kind of treatment he is going to prescribe to the patient. Aristotle draws a distinction between the productive and the practical sciences. The productive sciences include *crafts* like carpentry, shoemaking, building, and shipbuilding; *arts* like painting, dancing, and poetry; and *technai* like medicine (*ἰατρική*), husbandry (*γεωργία*), and warfare (*πολεμική τέχνη*). In the case of productive sciences, the artist has the form of the thing to be made in his soul, and he then realizes it gradually in some external matter. The principles of the art, therefore, reside in the artist (*Metaphysics*, VII, VII, 1032 b 1–1033 a 1). Even though the power which originates the art is in the artist, the arts have to conform to the standards of good practice which are appropriate in each field. The ultimate justification of these standards is nature, since according to Aristotle all that technê does is to imitate and compliment nature (Polansky 2000; *Physics,* II, 194 a 21–27). Productive sciences therefore are potencies, principles by virtue of which we produce things, whose origin lies in the rational soul of the producer. These rational potencies admit of contrary results, because science is a rational account which involves both the thing and its privation. The physician has knowledge of the principles of health but equally of what destroys it, of disease (*Metaphysics,* IX, II, 1046 a 36–1046 b 29). What he eventually decides to bring about, health or its privation, does not fall in the realm of productive sciences; it is a matter of choice and as such it is a concern of practical sciences.

Moreover, each productive science aims at some particular good. The art of shipbuilding aims at making a vessel, whereas the art of engineering aims at building a bridge. The particular good of each art is often wished for its own sake; however, it is also wished for some further good. This implies that there must be some ultimate

Good, the supreme Good. The Good however is not the object of a particular art but of a master-craft, as is obviously called the science of Politics (*Nicomachean Ethics,* I, I, 1094 a 27–29). Politics determines which sciences should exist in the state, which kinds of knowledge should be sought, and which forms of good should be promoted. It would seem that in so far as it includes the particular goods of the other sciences, the end of Politics is the Good of man. But since the Good of man is the same for the individual and the state, and the Good of the state is greater and more difficult to attain, it follows that the Good of the Political science is the Good of the state (*Nicomachean Ethics,* I, I, 1094 b 5–12).

Aristotle does not hesitate to restore experience to the place it deserves. He claims that art came from experience. It is through repeated experience that a universal judgment or principle is formed with regard to similar objects. Experience is the knowledge of particulars, whereas art is the knowledge of universals. This is why art remains superior to experience and the master-craftsmen (*ἀρχιτέκτονες*) are wiser than the artisans (*χειροτέχναι*) (*Metaphysics*, I, I, 981 a 26–981 b 5). For the artisans know how to do things, but they do not know the reason why they do them; the master-craftsmen, on the other hand, are wiser because they know the causes of the things which have to be done (*Metaphysics*, I, I, 981 b 5–7). This explains why art can be imparted by teaching, whereas experience cannot. The artist who wishes to teach his art can do so by knowing and explaining the reasons for which everything is done in his art. The artisan, however, does not have this option. The conclusion, therefore, is not only that art can be taught. But precisely because the sign of knowledge is that it can be taught, art turns out to be a kind of knowledge, a kind of science, something which obviously experience is not (*Metaphysics*, I, I, 981 b 7–12).

However, if the productive sciences are concerned with making a product which goes beyond the process of making, the practical sciences are concerned with choice and action. As already hinted at, the practical sciences are indirectly involved in the way the artist chooses to perform his art, since he can have knowledge of the contraries. It is practical wisdom, the capacity of the practical sciences, which will enable him to make the right choice regarding his art. Practical wisdom or prudence is the capacity the agent has to think about the human things and know what is good for him, for his house, and his community (*Nicomachean Ethics,* VI, VII, 1141 b 8–11). This is why, according to Aristotle, there is the main prudence (ethics, *ἦθος*), the prudence which concerns the house (house management, economy, *οἰκονομία*), and the prudence which concerns the city or the community (politics, *πολιτική*). The faculty by virtue of which we realize the things our prudence tells us that we ought to do, if we wish to attain our end, is cleverness. If the end is noble, then this competence is praiseworthy; if the aim is evil, then it is knavery. Now prudence is precisely related to this kind of cleverness which enables us to attain the right end. This is why, according to Aristotle, we cannot be prudent without being virtuous at the same time (*Nicomachean Ethics,* VI, XII, 1144 a 35–37).

The physician therefore who chooses to restore health rather than its opposite shows that he is a prudent man, since he consciously chooses to bring about the right end and to avoid the evil. He is the person who distinguishes what things ought to be done with regard to himself, to his house, and to his city, in the light of the right end. The aim of practical science is human happiness. This is why the practically wise man, (prudent, *σπουδαῖος*), in choosing which things ought to be done and which particular arts ought to be practiced, actually promotes the supreme Good, which is eudaimonia (*εὐδαιμονία*). Unlike the particular sciences or technai, each of which promotes a particular good, the practical science aims at the attainment of the whole Good. As a consequence, while they need to be ordered by practical science, and in particular by the superior of all practical sciences which is Politics, practical science need not be so ordered (Polansky 2000).

Practical wisdom, like virtue, is exercised rather than taught. Virtue is a state of character which lies between two extremes, the excess and the deficiency. No amount of teaching can show anyone how to achieve this medium. Only by habitually making the right acts, man gradually succeeds in developing his character to this

intermediate state and become practically wise and virtuous. This is a process which takes quite a while and in which teaching plays only a secondary role (Polansky 2000).

Not only in Aristotle's treatises but also in his successors' works there is the widely held view that certain arts are superior to others and that the highest of all is medicine. After describing how various arts developed out of experience, Aristotle draws a distinction between the arts which are related to necessities and the arts which are related to our leisure time, esteeming those who practice the latter wiser, because they do not aim at some kind of utility (*Metaphysics,* 981 b 18–22). Despite this distinction, however, Aristotle does not fail to remind us repeatedly of the various respects in which medicine excels all other arts. In his ethical treatises, in particular, the science of medicine is the standard paradigm by reference to which he compares and contrasts ethical thinking.

Xenophon on the Noble and Illiberal Arts

In a similar spirit, the famous historian Xenophon (430–354 BC), somewhat older than Aristotle, distinguishes between the illiberal (*βαναυστικαί*) and the noble arts (*Oeconomicus,* IV, 2–6). He speaks disapprovingly of the former because they force the craftsmen to sit and work all day indoors, softening in this way both their minds and bodies. Moreover, as they so work in complete isolation, they have no spare time for their friends, nor do they have the interest or the ability to defend their country. The opposite is the case with the noble arts, and the king of Persia is a good example in this respect, because he engages equally in husbandry and the art of war (*Oeconomicus,* IV, 2–6).

The Philosophers-Physicians on Technai

Along Aristotle runs a long tradition of philosophers–physicians who compose treatises on medical art praising medicine as the superior of all arts. The most popular of them was, of course, Hippocrates (460–370 BC). Among Hippocrates' medical and deontological works are included his two treatises: *On Medical Art* and *The Art* respectively. His aim in the former is to show that, even though medicine came out of philosophy, it has nevertheless developed into a new and independent art with its own principles and methods (*On Ancient Medicine,* XX 1–27; Kalokairinou 2014). In the latter, he defends medicine against its detractors' objections who claim that there is no such thing as a medical art and that all cures are due to luck, and he goes on to define the end of medicine (*The Art,* IV–VI).

The reason however Hippocrates has been renowned is because he was the first philosopher–physician to introduce a systematic kind of medical code in the form of his *Oath.* Unlike the famous Code of Hammurabi (eighteenth century BC), which is an example of a legal code issued by the supreme Assyrian leader Hammurabi, Hippocrates' *Oath* defines the standards of practice which all good physicians should follow. According to Hippocrates, the good physician should not only have knowledge of his art, but he should also practice it as a virtuous physician would (*Oath*). This implies that he should be careful about his relationship with his patients and their family members due to their vulnerable situation (*The Physician,* I, 23–27). But in order to be kind and respectful toward his patients, the physician has to be modest, reserved, of sound opinion and judgment, free from prejudice, and to be opposed to vulgarity, greed, and shamelessness. But then, if he is all that, the physician is a philosopher, i.e., equal to god (*Decorum,* V, 1–13).

By Roman times, the medical profession is further developed and formed. Aulus Cornelius (c. 25 BC - c. 50 AD) is famous for his treatise, *De Medicina.* This is a kind of encyclopedia, a lengthy work which included volumes on agriculture, law, rhetoric, military, and other arts. However, in extant parts of this, Celsus presents us with an excellent history of medical art up to his own days.

Galen of Pergamon (129–199 AD) devotes a whole treatise on exalting all arts, and particularly medicine (*Exhortation to study the Arts, especially Medicine,* 2004). He divides all technai or professions into two categories. The first, which consists of all the arts of intelligence, Galen calls honorable and liberal arts. These are medicine, rhetoric, music, mathematics and calculation,

astronomy, grammar and jurisprudence, and, of course, sculpture and painting. The second category includes all those crafts which require manual labor or the illiberal arts. Galen concludes that the young, if they are not to be vulgar, should learn how to exercise one of the arts, and especially the best of all which is medicine (Galen 2004).

The Professions in the Roman Era and the Middle Ages

Unlike the medical profession which was formed and developed in ancient Greece, the legal profession was rather exclusively a Roman achievement. Although the *Ars Oratoria* was practiced in ancient Athens, it was the Roman Republic that first introduced the expert consultants and later the advocates, when the forensic and the trial system became more complex (Siggins 1996). At the same time, in the Christian era, the Church developed its canonical laws and ecclesiastical courts. As a consequence, even though the paid legal representation was not accepted, the Church legal officials quite often undertook issues of property and family law. The result was that this mixed legal and ecclesiastical system survived through the centuries of feudalism that prevailed in Europe and reappeared in the twelfth century as University disciplines in the civil and canonical law. However, whatever the influence was between law and canon, to the standard ethical dilemma which the legal professional was facing, i.e., whether to do his duty and support the person represented or whether to do his duty to society, the Church bequeathed the injunction that he should practice his profession in accordance with law and truth (Siggins 1996).

The Professions in Modern Times

In the first millennium of the Christian era, the profession of divinity also makes its appearance. Initially this profession is used to imply the conversion to monastic life and its vows; later on it means the kind of vocation which takes care of the man's soul. More than any other, the profession of religion matches with the definition of a profession as a serious commitment or calling. To profess (the Latin verb "profiteri") means to state openly that one has a belief, to avow that one has a faith or conviction, and to declare publicly that one has a particular kind of knowledge and that he is a physician, an advocate, a teacher, or a priest and therefore one who fulfils all the conditions in order to realize his calling (Siggins 1996).

Moreover, while all these professions were preserved in the monastic order during the middle ages, with the establishment of the European University, the teachers of religion were the first that were called "professors." The title was further extended to the readers of other disciplines in the sixteenth century, when the king of England Henry VIII established five regius professorships at Oxford, contributing in this way to the process of secularization that all learned professions were going through (Siggins 1996). The distance from the modern professions was not far.

Definition: What Is a Profession?

The "profession" is often used as an umbrella term in order to include all sorts of occupations ranging from vocations, arts, to trades, jobs, etc. This is why it is difficult not only to define professions but also to distinguish them from various kinds of occupations. The distinction between illiberal and liberal arts, already present in antiquity, continues and is further enhanced in modern times: liberal are the professions that befit the "free men"; they are also "learned" in the sense that they are related to a certain body of knowledge or expertise. Moreover, the professionals "practice" their art, and they do not simply "work," something which denotes not only their autonomy but also their authority in the particular field they exercise (Chadwick 1997). As a consequence, an occupation usually means the particular activity one is engaged in for living, whereas by "profession" we imply the "white collar" activity one practices which is usually related to a certain status, e.g., physician, lawyer, clergyman, teacher.

Attempts to define a "profession" in terms of necessary and sufficient conditions have not been successful, because there will always be borderline cases which these definitions cannot account for. Definitions by means of family resemblances,

on the other hand, seem to be more to the point, since they assume that the concept of a profession is a developing one and that they are not formulated on the basis of the standard characteristics that professions such as medicine, law, or teaching have (Downie 1990). However, both of these types of definitions are wrong in that they attempt to capture the features which as a matter of sociological fact professions have, and they fail completely to conceive what is good and important about them. Unlike the sociologist, the philosopher is concerned with bringing about what is good about the professions and the professionals and with providing a partly evaluative account of them (Downie 1990).

Along these lines, it has been suggested that the ideal features the professional includes are mainly that she provides *a service* to the clients who are in need of it, by means of *a relationship* which is conceived in a twofold manner: either as *a bond* which joins two or more persons, for instance, physician–patient, teacher–pupil, marriage, etc., or as *an attitude*, a desire to help those in a vulnerable position, a feeling of beneficence. This kind of inequality which characterizes the professional relationship requires that the bond takes the form of an institutional body, all the more so especially if we take into account the degree of intimacy to which professionals come with their clients and intervene in their lives. Moreover, the professional has the social function to go beyond her duties to her clients and to express her opinion boldly on issues of public policy which relate to her expertise. Of course, she can only retain her outspokenness and integrity, if she can remain free from any influences that come either from the state or from commerce (Downie 1990).

One does not fail to notice a gradual shift *from* defining a profession by virtue of certain characteristics *to* defining a professional or a professional attitude. The professional must always be in a position to justify *with reasons* the work she does, reasons that are independent of what the consumer likes. If she chooses to sacrifice "the value of reasons" in order to save "the trade value," then she is not a professional anymore but turns into a tradeswoman. If she, on the other hand, sticks to her reasons and decides to act in accordance with them, even at the risk of losing her customer to another worker, she is a professional. Professionalism, therefore, and working in a professional way is what characterizes the professions (Davis 1991).

Because of the difficulties with which are replete the definitions of a profession, a further suggestion has been made that we should conceive of professions not in terms of traits but in terms of *a process*, that is, a process by virtue of which certain occupational groups get gradually recognized and acquire status. This conception of a profession however is heavily laden with the Anglo-American values which tie up occupation with status, unlike the European value system which associates it with education (Chadwick 1997).

According to a different kind of approach originating in sociology, a profession is not so much an orientation that offers guidance and support to vulnerable people but, instead, *an ideology.* Its ultimate goal is, through determining human needs, to make people dependent upon them and so exercise authority and control upon the whole population (Chadwick 1997).

Identifying the Problems

The first problem of professional ethics refers to the professional relationship, the way the professional is related to his client. How do we analyze the professional relationship? Is it contractual, covenantal, or a commodity transaction (Pellegrino et al. 1991)? As philosophical arguments have shown, the professional relationship is none of these. It has been viewed in different terms. Especially in the medical profession, the client has to lay herself open, both literally and metaphorically, to the prudence of the professional and ask his counsel or advice. This has come to be known as "the phenomenon of nakedness" according to which the client has to lift the cover of privacy that protects her in order to facilitate the professional's acquisition of knowledge about her state (Sokolowski 1991). It is the nakedness, openness, or vulnerability of the client

which grounds the professional's obligation to exercise his professional judgment and skills for the good of the client. This obligation does not spring out of the beneficent or virtuous character of the former. Rather it is based on the relationship which is formed between the professional and the client (Sokolowski 1991).

The professional relationship is mostly seen as a fiduciary one. The client entrusts the professional not merely her possessions but herself: the health of her body, the salvation of her soul, the education of her mind. The professional, on the other hand, is trustworthy not in the way that a friend or a relative is but in virtue of the certification he has received as an expert in a field (Sokolowski 1991). The professional relationship is mostly reciprocal: not only the client trusts the professional and entrusts her problems to his judgment but also the professional expects that the recommendation or counsel he has given will be followed and respected (Sokolowski 1991).

Viewing, however, the professional relationship as a fiduciary relationship presents certain problems. Some philosophers tend to understand the fiduciary relationship as a form of paternalism in which beneficence is the prevailing principle (Zaner 1991). The professional decides the most appropriate therapeutic method which will mostly benefit the patient. In this spirit, certain philosophers talk about the "fiduciary paternalism" (Zaner 1991).

The fiduciary paternalism, however, tends to highlight the inequality of the two parties. The patient is condemned to enter a relationship of asymmetry because she is vulnerable, while the professional possesses the knowledge and the skills to heal her (Zaner 1991). This asymmetry, which is inherent in the professional relationship, imposes certain responsibilities and obligations on the professional. In actual fact, the root of the professional's obligations lies in this very asymmetry (Zaner 1991).

While the asymmetry of the professional–client relationship constitutes a permanent temptation for the professional to employ his power in such a way as to take advantage of the client, at the same time it founds the moral imperative which is implied by the trustworthiness attached to the professional in virtue of his social certification. This is the most important issue about the fiduciary relationship. Moreover, it is true independently of whether we conceive it as paternalistic or view it in terms of informed consent, entitlement, and rights (Zaner 1991). It is argued that for the last 30–40 years, things have changed not only in medical but also in the other professional relationships. Even though at its very core the professional relationship remains fiduciary, we have gradually replaced the beneficence model with the autonomy model (Zaner 1991). However, trust and trustworthiness still remain the core concepts of the relationship. It is simply that all the issues discussed so far in the language of beneficent paternalism have by now been shifted to the language of rights, entitlements, and autonomy. This is even more obvious in the case of informed consent, the most central notion of autonomy. If a patient or a client is to be informed in an unbiased manner, in order to give his uncoerced consent to a treatment or a course of action in a relationship which is asymmetrical, trust will have to be the presupposition of the professional relationship. And this is so not only with regard to the relevant information the professional discloses to the client but also in connection with the actions he undertakes to perform regarding her vulnerable situation (Zaner 1991).

The inequality of the professional relationship implies precisely the vulnerability which the patient or the client experiences. She is in a sensitive position, and so she calls for support and help from someone she trusts that can understand her situation. In a way, she asks the professional to be in her shoes and, while remaining himself, see her situation as she herself experiences and understands it. In feeling the situation as the patient feels it, the professional is actually orientated towards her, and he feels compassion for her. In fact, he enters in a caring, sympathetic dialogue with her (Zaner 1991). It is this care and compassion which the professional feels for the patient which lifts the inequality of the relationship and transforms it into a relationship between equals. To be sure, like all dialogues, the professional–patient one has to be mutual, between two equal parts. This is why, for some philosophers,

care for the vulnerable patient is the root axiom of an ethics of trust (Zaner 1991).

The notion of trust, as a key notion in professional ethics, has been disputed by philosophers for both social and theoretical reasons. The empirical reasons may relate to a number of events and policies that have been taking place in society for the last 20–30 years such as the commercialization of medical care, the phenomenon of malpractice, the excessive income of certain professionals, the payment-in-advance-of-treatment policy which many hospitals and physicians apply, etc. Or they may have to do with the incompetent, self-interested, greedy, and insensitive character of the professional or, even, with the patient or client herself (Pellegrino 1991). The patients or clients have been recently better educated and informed and have all the means to check not only the knowledge they acquire from the professional about their situation. As the patient or the client is getting gradually more independent and he moves from a beneficent paternalistic to a patient or client autonomy trend, she can also seek to examine the professional's failure and success records and even ask his peers' opinion (Pellegrino 1991).

For all these reasons, an ethos of distrust has appeared in society. Fewer and fewer people consider the professionals trustworthy, preferring to secure the patient–professional or the client–professional relationship by employing various mechanisms: by means of contracts or by appointing an ombudsman or a healthcare manager or a patient advocate who will operate as an intermediary (Pellegrino 1991).

However, these people do not understand that by turning a professional relationship into a contractual one or by interjecting a third objective party to the relationship with the professional, they actually reduce a natural relationship into a legalistic one (Pellegrino 1991). Each party is simply concerned with protecting her own interest to the extent that this is safeguarded by a contract or a legal representative, and nobody wishes to transcend herself and act in ways that go beyond the conditions of the contract. This is a policy, however, which will automatically lead to the eradication of trust in the patient–professional relationship and to its replacement by a more complex contractual or legal system of agreement (Pellegrino 1991).

Contracts and legal agreements, however, are more difficult for a patient or a client to handle. Even though she had so far to attempt to communicate with the professional in a direct manner, now she has to get to him, be it a physician, a lawyer, or a minister, through a healthcare manager or a contractual agreement. Such a procedure not only complicates things, but it mainly brings out the ineradicability of trust in professional relationships. The patient or the client now has to trust not only the professional but also the ombudsman or the third party that intervenes between her and the professional. Contracts and third parties' interventions cannot operate and be efficient, unless they are rooted in the trustworthiness of the parties involved.

There are also conceptual reasons for which professional trust has been questioned. Robert M. Veatch provides us with three basic arguments in support of his contention that trust for the professionals is not a coherent concept (Veatch 1991). The first argument is that quite often it is impossible for the professionals to know what the best interests of the clients are. And if they cannot know them, obviously they cannot be trusted to be in a position to advise their patients accordingly (Veatch 1991). According to the second argument, trust is affected not so much because the professional cannot know the patient's or client's well-being but because he cannot provide the client with a value-free description of the facts that concern her. If the physician, the lawyer, the minister, or the teacher cannot offer the required information with regard to the client's situation in a way which is independent of his own evaluations, then he cannot be trusted as a reliable, impartial expert in the field, and the fiduciary character of the professional relationship breaks down (Veatch 1991). The last argument refers to the importance virtues have for professional trust. Even though we cannot trust the professional because he cannot know the patient's or the client's best interests, or because he cannot give a value-free account of the facts which are relevant to the patient's situation, we could perhaps still consider him trustworthy

because he was a virtuous man and because of the specific virtues which are inherent in that profession. However, it is claimed, it would be a mistake to argue in this simplistic manner. There are no virtues which are specific to each role or to each profession. This is because it would be wrong to conceptualize a certain profession in a particular manner. The conception we have of the profession of the physician, the lawyer, the minister, or the teacher is not univocal in the different cultures. The physician is certainly conceived quite differently in the Hippocratic, Christian, Talmudic, or liberal tradition. As a consequence, there cannot be specific virtues in the generic role of the physician. On the contrary, the virtues of the generic profession of the physician are different, depending on the particular conception of the physician we have in mind each time, which, in turn, is determined by the different underlying social and ethical systems (Veatch 1991).

These attempts, however, to deconstruct professional trust and to establish ethics of distrust are doomed to fail. As has been pointed out, the physician is responsible for defining the patient's good and for attempting through his advice to make her achieve it. This is the physician's expertise. He cannot however be worthy of his role, unless he is able to place it in the patient's or the client's scheme of evaluation of his total good or well-being. The latter is the patient's or the client's business. However, he would have violated his patient's autonomy, if he assumed that defining a patient's well-being was the physician's job (Pellegrino 1991). In a similar manner, the professional trust is not affected because the physician cannot give an account of the medical facts which are relevant to the patient's situation in a way which is not laden by his evaluations. In fact, even though this happens fairly often, it is not always the case. In most cases, the physician can present the medical situation and the required treatment in a way which is logically independent of any evaluations he may hold, even though admittedly at some more progressed stage the values he embraces as a physician and as a person may come into the picture and shape the way he conceives the medical facts. Such a thing however need not imply that professional trust is eradicable; the least it can mean is that trust in the professional relationships is not perfect (Pellegrino 1991). This is why philosophers like Edmund D. Pellegrino wish to prescribe a new ethics of trust in which fidelity to trust will be the logically necessary concept upon which the basic obligation in all professional relationships will be founded (Pellegrino 1991). Moreover, it would seem that in conceiving the specific virtues of a profession not only in virtue of the particular conception of the profession but also in terms of the underlying ethical and social systems, one may be criticized for holding a relativistic view of virtue. Such a view, however, need not worry us very much. As Julia Annas has pointed out, virtue is a "thick" concept; it conveys information about the situation to which it is applied and at the same time evaluates our disposition towards it (Annas 2015). The specific virtues inherent in the Talmudic conception of the medical profession may be different from the specific virtues which are inherent in the liberal conception of the same profession. However, even though they are as specific as to apply to each conception of the medical role, nothing stops them at the same time from being equally universalizable. On the contrary, the specific professional virtues are not so much a threat to the universalizability of virtues as they are a danger to what we call "the unity of virtues" (Meilaender 1991). In the very end, are the virtues of the good physician, of the good minister, or of the good teacher different from the virtues of a good citizen or a good person? Would the physician, who is committed to the healer's role and who would never kill either for compassion or for money, ever harm someone in his civic and/or private life? Do the virtues of the professions ever clash with the requirements of ordinary morality? (Meilaender 1991).

The answers to these questions are divided. On the one hand, there are those who tend to argue for the unity of virtues. Their view is logically implied by their conception of moral life as a harmonious whole in which clashes of virtues cannot even be conceived. In such a conception of morality, the professional does not simply seek a specific virtue, be it justice or spiritual care, but also seeks to promote the whole virtuous way of

life. Considered in this light, professional trust not only is not threatened by the unity of virtues but is also extended beyond the sphere of each individual profession and establishes itself at the center of moral life itself.

There are those, on the other hand, who claim that conflicts and tragic dilemmas are the essence of moral life (Meilaender 1991). They conceive moral life as fragmented, consisting of different compartments, public, professional, private, in which we are called to participate and decide. Decisions now taken in the professional sphere cannot merely depend on the virtues inherent in the particular profession or on the trustworthiness of the professional. In order to avoid the arbitrariness intruding in the professions in the contemporary liberal society, philosophers suggest that there should be at least an ethic specific to each professional group, instead of relying on every individual professional's judgment. In this way, even though the professional's autonomy will be preserved, he will always be required to give an account of his decisions and his actions not only to his clients but also and more importantly to his peers (Chadwick 1997).

Along with the internal approaches to professional ethics, consequently, there are also the external approaches. An external approach would imply that a moral theory like Kantianism, Utilitarianism, or Feminism can be applied to the issues that arise in the professional context and in connection with the professional–client relationship (Chadwick 1997). The principles of the professional codes have often been considered as a first attempt to apply Kantian ethics in the professions, even though one could also work out an interpretation according to which the codes of conduct may fit into a virtue ethics account of morality. A Kantian analysis of the codes of conduct would mean that we tend to view the contents of the codes not so much as Kantian moral principles but as a number of guidelines which prescribe the particular courses of action which the professionals ought to follow (Harris 1994). This kind of approach, however, has been disputed on the grounds that it does not constitute a genuine interpretation of the Kantian thought. Kant would object to specifying maxims and putting them down in print. This would go against the grain of morality which is primarily concerned with authenticity. According to a Kantian account, professionals would be expected to work out their principles themselves and not to take them ready from the codes of a conduct book (Harris 1994).

Moreover, the Utilitarian approach seems to fit quite well to the legal profession. This is so because in the contemporary liberal society, the lawyer is expected to know how to promote the client's best legal interests through the adversarial system, something which by definition has been set to promote the greatest interests of the whole society (Chadwick 1997). In the end, the Feminist theories of care can be construed either as an internal account, when care is conceived as a virtue which is inherent in the professions, or as an external account, when care provides the logical framework through which we can address the issues rising in the professions (Chadwick 1997).

Conclusion

In this paper, we attempted to bring out the development of the professions both historically and systematically. In so doing, we noticed that this was not easy, as these two kinds of accounts could not always be kept apart. The origins of professions can be traced back to classical antiquity, at a time when the artist was equally concerned both to achieve the truth and to benefit the others. The picture changes gradually in the renaissance and the modern era. The professional now is concerned to support, counsel, and offer spiritual care to the vulnerable client that approaches him through and by virtue of a caring and trustworthy relationship. In the liberal contemporary society, however, as life has been essentially compartmentalized and every sense of unity has been lost, external ethical accounts have been applied to professions in an effort the philosophers make to safeguard the interests of both parties in the professional process.

Our conceptions of the profession, however, and of the way we conceive the professional context are going to change further in light and under the pressure of the new technological developments in the related fields. Telemedicine or e-medicine, for instance, is going to influence the way in which we understand the professional relationship and to redefine the professional process anew.

Cross-References

- Applied Ethics
- Codes of Conduct
- Dental Ethics
- Integrity: Professional
- Journalism Ethics
- Laboratory Science Ethics
- Leadership, Ethics of
- Legal Ethics
- Media Ethics
- Medicine and Ethics
- Military Ethics
- Oath
- Organizational Ethics
- Pastoral and Spiritual Care
- Publication Ethics
- Traditional Medicine
- Trust
- Utilitarianism
- Veterinary Ethics

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Professionalism

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Abstract

Professionalism refers to the set of skills and values that, in the case of medicine, characterize the essence of humanism in professional work. Professionalism, ethics, and bioethics are closely linked. Within the framework of global bioethics, professionalism provides a comprehensive picture of the medical profession and its relationship with the major social challenges worldwide in the areas of health and education.

Keywords

Professional values; Professional behavior; Medical ethics; Health; Education

Introduction

In recent years, the world has witnessed significant changes in the medical field, not only in the areas of scientific and clinical knowledge, in the understanding of diseases, or in the development of new drugs and therapeutic treatments but also in the physician-patient relationship, the management of healthcare systems, and the general approach to health, welfare, and disease management. In practically all of these areas, the changes have been accompanied by a comprehensive approach to health. As might be expected, all these elements involve a significant ethical component that is of special interest for bioethics.

Basic Concepts

In the area of healthcare, and of medicine in particular, professional work cannot be conceived outside the framework that includes the relationship among health, welfare, and disease. Health has a biological dimension that can be measured using biomarkers (i.e., functional imaging with PET/CT or MRI, tumor markers), but it also has a psychosocial dimension that can often only be perceived (only allowing qualitative analyses of the subjective components).

In its definition of health and welfare, the World Health Organization tacitly recognizes this fact (WHO 1989). Consequently, together with the professional duties related to disease treatment, the duties directly related to patients' welfare should also be taken into account. There may be diseases that cannot be cured or even treated, but this by no means frees physicians from their professional duty of caring for the patient. While the ability to identify and use physical, clinical, and analytic facts is relevant in the patients' treatment, there are other elements that should be considered in the assessment of professional practice. These elements, such as how the healthcare professionals interact with patients or how professionals address their roles in healthcare, are also an important piece of their professional practice. Unfortunately, either the rapid spread of new technologies, especially in the most developed countries, or the overdemand of patients, in the majority of developing countries, has caused that any source of knowledge (i.e., knowledge derived from better understanding of patients' concern) outside of the exact sciences falls in practice into oblivion. This leads to the need for training to provide physicians with

the necessary tools to treat patients from a comprehensive approach based on adapting management to the particular situation of the patient, as in, for example, linguistic, social, and cultural differences. With this aim, the development of skills focused mostly on the technical part of medical and clinical aspects involved in the diagnosis and treatment of disease is not enough. It is also essential to design a model of professional behavior which should include communication and analysis skills in professional physician-patient interaction, teamwork skills to optimize the services provided by multidisciplinary healthcare teams, and, finally, information retrieval and analysis skills and competence in the application of different types of information and professional knowledge. These elements, set within a clear framework of ethical and legal action, are the source of professionalism in the context of medical practice.

History and Development

Despite professionalism as a concept used to describe the core elements of the medical practice, which can be traced back at least around 400 BC with the Hippocratic Oath, the use of this term in the medical literature is not longer than a century ago (DeAngelis 2015). According to some authors (Hu et al. 2000), behind the use of the term of medical professionalism, there are at least three common elements that medical doctors from different cultural backgrounds recognize as core part of their profession (Hu et al. 2000): (i) the existence of common ethical principles such as confidentiality, truthfulness, informed consent, respect, justice, and commitment to preventive medicine, (ii) the need for medical education based on fostering good practice, and (iii) the requirement for professional practice to comply with moral codes of good practice. These elements, which are common to the different medical traditions, are in the basis on which professionalism is built.

The concept of professionalism arises as an articulated body made up of professional traits and skills that constitute physicians' professional work, regardless of the geographical, social, or cultural settings where it is carried out.

According to David Stern (2006), the core values of medical professionalism derive from the universality of disease and begin with caring or compassion. Caring over time creates the value of responsibility. The shared responsibility for care, between physicians and patients, engenders trust and respect in both parts. The maintenance of trust demands integrity and confidentiality. In these terms, human values, traditionally viewed as central to medical practice, have a biological basis on the very nature of the human bonds established among people by the experience of disease. This human core is present in all medical traditions to date and in all the existing perspectives of professionalism. Some of these values are autonomy, service, maintenance of competence, and accountability. And these professional values, as such, are common to all professions and not only to medicine.

However, a brief historical introduction will contribute to the understanding of how human and professional values are the two main elements that lead to the birth and further development of professionalism in the medical field.

What is nowadays known as medicine is the product of countless sources of knowledge that have existed from the very origin of humankind. Sumerian, Egyptian, Greek, Roman, and later Arabic culture, as well as providing the major bases of knowledge that have been used to study the diagnosis and treatment of disease, have left a rich body of doctrine on professional practice that has survived over time. Based on a similar worldview of the relationship between human beings and their environment, the Chinese, Indian, and pre-Columbian traditions have provided the bases for a holistic understanding of medicine. The concept of person introduced by Christianity was a turning point for medical practice. Attached to this concept is that of dignity, thanks to which health acquired universal value. The foundations of what would later be professional colleges were laid in the Middle Ages, as well as those for the creation of a professional medical body. The achievement of technical rigor and the necessary tools to acquire new knowledge, thanks to the scientific

method, took place during the Renaissance and the subsequent evolution of modernity. Also in the modern era, the concept of profession becomes fully consolidated under a more formal structure that, in the case on medicine, has led to profound changes both in the physician-patient relationship and in the relationship between medicine and society.

Medical professionals are currently viewed as providers of a service based on the self-regulated and responsible application of knowledge earned through a learning process. Physicians' professional activity is always carried out seeking the benefit of others rather than their own. Nevertheless, because of the contractual nature of the job, physicians earn the right to a financial reward for their work.

So far, a brief description of the historical background for the birth and development of professionalism has been provided. However, the history of how the term was originated and subsequently acquired the significance it has nowadays in the fields of medical education and global bioethics is more recent. Toward the late 1970s, in the very early stages of bioethics, a flourishing interest was starting around the term of medical professionalism to describe a renewed paradigm of medical practice. In the field of medical education, there was an incipient interest in adapting teaching models to the needs of an increasingly globalized society (DeAngelis 2015). Within this context, the search for international performance standards necessarily entailed the need to focus reflection on the search for elements of global interest. However, it was not until the 1980s that the noncognitive attributes of physicians acquired their own space within this reflection. In 1983, the *American Board of Internal Medicine* (ABIM) took a first step toward this by establishing respect, compassion, and integrity as dimensions of humanistic medicine. In the 1990s, the ABIM began to use the term "professionalism," and, together with humanism, it explicitly included among its constituents the notions of altruism, duty and service, responsibility, and excellence (ABIM 1994). According to Stern (2006), in 2000, more than 60 medical colleges had already developed, or were in the process of developing, different types of tools for the assessment of their students' professionalism. The turning point came in 2002, when the ABIM, in collaboration with the *European Federation of Internal Medicine* and several international authors, published "Medical Professionalism in the New Millennium: A Physician Charter" in *Annals of Internal Medicine* and in *The Lancet*. That same year, other journals of international relevance echoed this initiative by reproducing the document and publishing several articles and comments at the global level. In just a few years, the concept acquired its own space within specialized literature, becoming a reference both in the field of medical training and in that of bioethics. In the latter, professionalism has taken on particular significance through its inclusion in the *Bioethics Core Curriculum*, published by the UNESCO in 2008. The countless publications, conferences, events, and initiatives that from then until now have revolved around professionalism are proof of its significant contribution both in the field of medicine and in that of global bioethics. However, incorporating medical professionalism into the curriculum for many is still not feasible. Perhaps this difficulty is a consequence of a lack of global agreement on what to expect from the meaning of the term in different cultural contexts.

Conceptual Clarification

Despite that there is certain general agreement of the core elements of medical professionalism, having a definition offers important difficulties. These can appear due to the interdisciplinary framework around the term, or be derived from the important social disparities and cultural differences that accompany the medical practice in a global context.

One of the main referents of professionalism is undoubtedly the "Physician Charter" of 2002. In this document, professionalism is described as the basis of physicians' contract with society. According to its text,

> Professionalism demands placing the interests of patients above those of the physician, setting and maintaining the standards of competence and

> integrity, and providing expert advice to society on matters of health. The principles and responsibilities of medical professionalism must be clearly understood by both the profession and society. Essential to this contract is public trust in physicians, which depends on the integrity of both individual physicians and the whole profession.

This statement sets the bases for a definition without giving a closed definition of the concept. However, this document transmits the close relationship among professionalism, professional ethics, and bioethics. In agreement with this statement, it in 2008 the *Bioethics Core Curriculum* of the UNESCO included professionalism within the area of bioethics as the behavior, aims, and qualities that both professions and professionals should have.

The need for a clear, comprehensive, and concise definition of professionalism acquires special relevance when it comes to establishing the criteria for its assessment. Given that learning involves incorporating certain types of knowledge and that acquired knowledge can be measured, it must be possible to assess professionalism as the type of knowledge to be acquired by all professionals with the purpose of determining the extent to which it has been made a part of professional practice. In this context, a definition from the field of medical research, with assessment purposes, mainly suggests that professionalism is a type of knowledge reached from a starting point of three essential bases: (i) clinical skills, (ii) communicative skills, and (iii) an appropriate understanding of ethical and legal framework of the professional behavior. These three elements are the grounds upon which the founding characteristics and core values of medical professionalism are built: excellence, humanism, accountability, and altruism (Stern 2006). This definition introduces the idea of professionalism as a virtue to which physicians continue to aspire and, therefore, provides a bridge to medical ethics. In this definition, excellence, humanism, accountability, and altruism act as aspirational principles more than a normative framework that provide an assessment tool of observable professional behaviors. Meanwhile, communication skills, which are also included in the definition of professionalism, not only involve the ability to communicate with patients and their families but also with the rest of professionals that are members of professional and interprofessional teamworks. Now, more than ever, there is a need for these skills at the global and intercultural level. Their development also facilitates the opening of new paths of knowledge born from positive cooperation between patients and their environment, as well as from the contribution made by interprofessional and multidisciplinary collaboration toward the improvement of professional care within a framework of ethical practice.

Ethical Dimension

Technical expertise, skills, ethics, and communications are all essential components of professionalism. However, ethically speaking, professionalism can be described as a status that is reached, maintained, and improved through the continuous effort and perseverance of those who practice the human principles and values that make it up. In fact, according to the ABIM's *Medical Professionalism Project*, professionalism is directly associated with the following concepts: excellence, accountability, duty, altruism, respect, compassion and empathy, honor, and integrity (ABIM 1994). Underlying all these elements is a strong ethical dimension that acts as the backbone of professionalism. In spite of the numerous differences that naturally emerge from interprofessional and intercultural dialogue, there is a degree of consensus as to acknowledge the preponderant role of all these elements and to group them, for practical purposes, into four main dimensions: excellence, accountability, altruism, and humanism.

Excellence

Within the broad framework of professionalism, excellence begins with a commitment to competence, the understanding of ethical principles and values, awareness of legal boundaries, and communication skills. However, excellence cannot be understood without a personal commitment to exceed ordinary standards, which is a specific characteristic of this principle that sets it apart

from the rest. In terms of excellence, a commitment to acquire minimum standards is not enough, since its essence lies in exercising a continued and conscious effort to go beyond ordinary expectations.

Excellence is closely linked to a permanent readiness to engage in lifelong learning, which implies more than a set of self-programmed activities. According to several authors (Hojat et al. 2003), this quality is defined as the set of skills developed by individuals to gather knowledge from different sources, catalyze it, and include it in their professional practice. It requires sustained motivation, humility in the acceptance of one's own limitations, and an attitude of wonder and openness to new forms of knowledge. Another expression of excellence is dedication to the continuous improvement of quality through reducing medical errors, working toward patients' protection and well-being, concern with the misuse of healthcare resources, and the search for efficiency.

Excellence is also closely related to interprofessional collaboration, which is a strength that every professional should cultivate, especially in multidisciplinary environments such as the healthcare area. It is not enough for professionals to be skilled at their jobs; they must also be able to create, sustain, and maintain mechanisms to encourage improvements in their team's performance. At this level, collaboration takes on a preponderant role that has been explicitly acknowledged by the World Health Organization (2010). Excellence should be an aim in healthcare, although difficult to achieve in most cases.

Finally, an essential aspect of excellence is the promotion and innovation of knowledge based on scientific evidence and medical expertise. Professionals, in this case physicians, therefore, have a duty to protect and preserve the integrity of their medical expertise and technological resources while at the same time ensuring integrity as regards their proper use. Nevertheless, this must not lead to confusing excellence with technical improvements. In the case of professionals with scarce material resources, excellence allows for the development of creative skills and innovative capacities to better cope with the limitations that may arise. For example, the skills developed by physicians in resource-constrained countries to meet the health needs of their communities are bold and impressive. Apart from being a clear example of excellence, in certain cases the development of these skills has led to significant contributions to medicine and global health (Dandonoli 2013).

Accountability

Accountability includes self-regulation, the setting of standards, the management of conflicts of interest, duty, and responsibility. All these characteristics, through accountability, closely associate professionalism with the ideal physician-patient and physician-society relationships. Certain authors define accountability as the set of procedures and processes by which individuals justify and take responsibility for their actions (Emmanuel and Emmanuel 1996). Accountability implies not only professional responsibility in patient care but also the responsibility – in the case of physicians – of ensuring that the actions undertaken at all times are consistent with the standards of the profession and with society's responsibility to safeguard the health needs of its members.

In a broader sense, responsibility is an action derived from freedom and always attached to commitment, a personal commitment to take charge of patients' health despite the hardships that might be involved in such a task and the personal risk it may involve. Commitment born from responsibility also translates into a duty to cooperate with other health professionals, taking on leadership tasks and submitting to the leadership of others when required (Stern 2006).

Self-regulation is a core element of accountability through which physicians take on their role of main primary responsibility for exercising a legal and ethical behavior in their professional practice (Emmanuel and Emmanuel 1996). In terms of education, accountability provides an adequate framework of action for designing professional training schemes through which, in addition to developing clinical skills and building sufficient medical knowledge, physicians acquire the commitment to ensure that their professional

performance is responsible toward patients and society (Gonnella and Hojat 2012).

In countries with a free market economy, the danger of erosion of physicians' ethics as a result of the financial interests of third parties is not very different to the situation of vulnerability faced by physicians in countries with scarce resources, where bribery and the unlawful use of influence are well-known problems that corrupt the very essence of the profession, including physicians, patients, and society. To address these issues, accountability provides a tool that works "from within" in protecting professionals and maintaining a healthy relationship between them and society. This dimension of accountability establishes a bridge between professionalism and social justice in the search for an equitable distribution of resources, access to healthcare, and the cultivation of an increasingly caring society.

Altruism

The third ethical dimension of professionalism is altruism, which requires physicians to give priority in the performance of their profession to the best interest of patients above their own (ABIM 1994). However, these "best interests" are not to be interpreted as uncritical subordination to patients' autonomy. Several authors agree upon the need for awareness of the potential risks involved in a misunderstanding of medical altruism. The literature states that altruism is expressed in specific situations as concern for the welfare of others. Nevertheless, whether or not this behavior is to pose a risk for the well-being of the benefactor, a certain degree of sacrifice, or renunciation to self-interest to be considered altruism strictly speaking, is an issue that remains unclear (Stern 2006). There is no question as to the strong bond between altruism and compassion. In this sense, both the operational principles of bioethics and the natural link between life, health, and disease provide an illuminating framework for action. There is no health without life, and behind the loss of health is disease. This fact tacitly provides an order of values that defines the scope of action of altruism through the regulation of physicians' professional work. As regards sick patients, physicians perceive the responsibility to take action for the benefit of an asset that is in danger. However, this performance cannot involve a higher risk than there already is, neither for the patient nor for the physician. From there on, all action taken in the direction of the "best interest" or patients' welfare is not only welcome but also necessary. This is a task where responsibility is shared among physicians, patients, and society itself.

Humanism

As already mentioned, humanism is made up of a set of elements that, throughout history, have been part of the very essence of professional medical performance. These are respect, compassion, empathy, honor, and integrity.

Humanism involves a sincere concern for the interest of humanity embodied, in the case of medicine, in the figure of the patient. It is a crucial principle in a profession that is born from the interaction between people in need of care and those who provide it.

Respect means treating others with regard, deference, and dignity. It involves sensitivity and receptivity to the characteristics and peculiarities of the personal and cultural history that is a part of the individuality of others. Furthermore, it is a basic attitude that is often termed as the essence of humanism, since it awards recognition both to the worth of human beings as individuals and to their system of beliefs and values (Abbot 1983). Therefore, both patients and other professionals are entitled to respect.

Compassion and empathy are closely complementary concepts. Empathy is the ability to understand the views of others, together with their inner experiences and feelings, without getting intensely emotionally involved (Hojat 2007). Thus, empathy is the result of the human need for understanding and is the backbone of physician-patient professional relationships. However, empathy goes beyond understanding and communication. Its more human dimension is given by compassion, which allows physicians to perceive a patient's emotional disposition toward a disease or ailment and motivates them to find the causes that might explain such condition. Compassion binds physicians and patients,

while empathy transforms this bond into an instrument for caring and healing, in the case of patients, and into an element for the protection of health and welfare, in the case of physicians. Medical literature extensively discusses both the dangers to physicians of a loss of compassion and those that might arise from excessive emotional involvement with patients. However, as with empathy, compassion is not exclusive to the physician-patient relationship, but it is also extensive to the field of interprofessional collaboration (Hojat et al. 2014).

Finally, honor and integrity refer to "being fair and truthful, keeping one's word, meeting commitments and being straightforward" (ABIM 1994). Both qualities, as is the case of the previously mentioned, apply to all human actions, including those carried out within private contexts where the only witness is one's own conscience. Professionals that assume these qualities as a general standard in their professional work acquire in practice the habit of adapting to the situation at hand or "living in the truth." Living the truth is a way of defining humility. This is recognizing their own limitations and having an objective vision of the situation without generating false expectations or being seized by self-deception. Living in the truth also means being able to acknowledge one's own mistakes, dealing with the mistakes of others, and giving due recognition to the work of others. This is conceptually opposed to plagiarism, fraud, and deception.

Conclusion

Professionalism introduces a new paradigm for professional performance consisting of a body of values and professional skills and includes, in addition to the components derived from medical humanism, others that are common to professional practice in general.

Given its common and universal nature, professionalism is not only global in its reach, covering the different geographical and cultural realities where the medical profession is practiced, but its timeless dimension makes it present at all stages of professional life. Namely, its scope of influence covers students, professionals in training, and professional physicians.

In terms of knowledge, professionalism poses the challenges of teaching, learning, and assessment, areas that are of particular interest to medical education.

The first recipients of the benefits of professional care are professionals themselves, followed by the recipients of professional service who, in the case of medicine, are patients and ultimately by the professional environment where it takes place, represented by both other professionals and society as a whole.

Within the context of an increasingly globalized society, professionalism provides a series of tools that play an extremely useful role for bioethics to meet the major challenges that concern it in terms of health and education at the global level.

Cross-References

- ▶ Access to Healthcare
- ▶ Altruism
- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Bioethics: Clinical
- ▶ Bioethics: Education
- ▶ Bioethics: Medical
- ▶ Care Ethics
- ▶ Clinical Ethics: Teaching
- ▶ Communication: Ethics
- ▶ Compassion
- ▶ Competence
- ▶ Complicity
- ▶ Confidentiality
- ▶ Conflict of Interest
- ▶ Conscientious Objection
- ▶ Consent: Informed
- ▶ Corruption
- ▶ Cultural Diversity
- ▶ Discrimination, Ethics of
- ▶ Disease
- ▶ Education: Assessment
- ▶ Education: Goals
- ▶ Education: Lifelong Learning
- ▶ Education: Methods
- ▶ Empathy

- Equality and Equity
- Ethics
- Global Ethics
- Health Education and Promotion
- Health: Concept
- Human Dignity
- Human Rights
- Integrity: Professional
- Malpractice
- Medical Humanities
- Medicine and Ethics
- Mistakes, Medical
- Paternalism
- Patient Rights
- Personalism
- Professional Ethics
- Professional-Patient Relationship
- Responsibility: Individual
- Scientific Misconduct
- Trust
- Welfare

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Professional-Patient Relationship

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Abstract

The definition of the nature of the professional-patient relationship in bioethics varies: some hold that it is a contractual or fiduciary relationship, others believe that it is a relationship based on faith in patient's autonomy. This entry firstly analyzes the definitions and connotations of the term "profession" to help its readers to understand the nature and the core features of the professional-patient relationship; then, it reveals the changes taking place in contemporary professional-patient relationship as compared to traditional relationship and the new challenges ahead through a longitudinal analysis of its evolution; and in the end, it becomes evident that today's professional-patient relationship, though much different from the traditional one, still shares the intrinsic

feature – trust, the core and the foundation of professional-patient relationship – with traditional professional-patient relationship. Under the impact of market forces, the main challenge faced by modern professional-patient relationship is the ever-intensifying conflict of interests.

Keywords

Profession; Medical professionalism; Paternalism; Trust; Conflict of interests

Introduction

Although the professional-patient relationship (PPR) is the most important relationship in all relationships in health care, uniformly defining this term proves to be very difficult. What is the nature of the professional-patient relationship? Different points of view exist in the bioethics community:

1. The professional-patient relationship is a contractual relationship. The contractual model presents the therapeutic interaction as grounded in "a private contract for supply of professional services" (Sieghart 1982, p. 26). That is, essentially, the relationship between professional and patient is a contractual one whereby the doctor undertakes to treat and advise the patient and to use reasonable skill and care in doing so. In modern societies, such a relationship, which covers the majority of interpersonal trades and contacts, requires both parties to take on the responsibilities to honor their commitments once the contract has been made. Generally speaking, the professional-patient relationship is more or less similar to the contractual relationship. However, the complexity of the professional-patient relationship extends far beyond the normal business relationship with a contract. The contractual paradigm can't reflect the obligations of fidelity and veracity owed by physicians to their patients. As Beauchamp and Childress observed, it is fidelity that distinguishes the practice of medicine from business practices that rest on contracts and marketplace relationship (2001, p. 312).
2. The professional-patient relationship is a fiduciary relationship. To characterize the doctor–patient relationship as fiduciary wherein "the law defines a fiduciary as a person entrusted with power or property to be used for the benefit of another and legally held to the highest standard of conduct" (Rodwin 1995, p. 243). It shares the main features of fiduciary relationships, of which the sole purpose is to protect the interest of the clients. Although it's justifiable for doctors to charge reasonable remuneration for the care they have provided for their patients, the establishment of the relationship aims at upholding the interests of the patients; moreover, with the ever increasing specialization of labor and the concentration of resources, doctors gained an authoritarian status of control over the knowledge and skills, healthcare resources, and the diseases.

   When the patients come to the physicians for help, they actually stand on a completely unequal footing, and therefore become vulnerable and highly dependent. Consequently, under the laws and moral systems of most of the countries, the professional medical workers – the "fiduciaries" – are subject to the laws and the highest standards of ethical codes. Each judiciary may have their own regulations, but fidelity and integrity of the fiduciaries are indispensable. A system is thus set up to ensure that the interests of the clients are prioritized.
3. Some scholars also suggest that contemporary professional-patient relationship is not a general commercial contractual relationship as medicine and business are different in nature, or a fiduciary relationship either, because the superiority of the professionals are overemphasized in the fiduciary relationship. As a matter of fact, modern patients do not generally subscribe to a paternalistic model of health care decision-making, nor are they content to treat the therapeutic exchange as a simple commercial transaction (May 2001). In essence, it is practiced under the framework

of paternalism in which the autonomy of the individual patient is neglected and suppressed. Today's professional-patient relationship should be a relationship of trust based on the respects for the patients' autonomy. On one hand, patients entrust doctors to offer advice that is not just competent but also aligned with their interests; on the other hand, they reserve the right to seek a second opinion, propose their own options, or even reject advice provided based upon what they themselves determine to be in their interests (Bending 2015).

The three perspectives mentioned above, different as they seem, all reflect the fundamental need for trust in the professional-patient relationship despite that under different models, the contents and the boundary of trust tend to vary.

This entry begins with an analysis of the definition and connotation of profession for better understanding of the nature and the core feature of the professional-patient relationship and a discussion of the attitudes, behaviors, and moral responsibilities that should be undertaken by a medical professional – the important leading party of the professional-patient relationship – to live up to the requirements and expectations of the society and protect the image of professional-patient relationship from getting stained. Secondly, this article also longitudinally analyzes the evolution of the professional-patient relationship and addresses the enormous changes taken place in contemporary professional-patient relationship as compared to the traditional one and the new challenges faced by contemporary professional-patient relationship. In the end, we will discover that today's professional-patient relationship, though very different from the way it was a long time ago, still shares some intrinsic characters with traditional relationship. The healthcare sector is characterized by high specialization, high risk, uncertainties, and vulnerabilities of the patients, which determined that only by building trust between patients and the professionals can we ensure the success of the medical practices. Trust is not only the essence of but also the foundation for the communication between profession and patients.

Medicine as a Profession and Professional-Patient Relationship

To better understand the nature and core feature of the PPR and the proper attitudes, behaviors, and moral responsibilities required of a physician, as well as the requirements and aspirations from the society, it would be helpful to analyze the definition and origin of medical profession.

The term "profession" originated in the guilds (or professional associations) in the Middle Ages. As for etymology, the term actually derived from the Latin word "Prefessio" which means to declare openly. In Western culture, the interpretation of the definition varies. For example, some scholars believe that profession is a group of self-regulated people who gained a special skill through training or education and used it in the service of others. According to Pellegrino and Relman (1999) what distinguishes profession from other occupations is its core value – altruism.

As time goes by, a consensus of the basic definition of profession was gradually formed. Cruess et al. (2004) claim that a profession is an occupation whose core element is work, based on the mastery of a complex body of knowledge and skills. It is a vocation in which knowledge of some department of science or learning, or the practice of an art founded on it, is used in the service of others. Its members profess a commitment to competence, integrity, morality, altruism, and the promotion of the public good within their domain. These commitments form the basis of a social contract between a profession and society, which in return grants the profession autonomy in practice and the privilege of self-regulation. Professions and their members are accountable to those served and to society (Cruess et al. 2004).

Accordingly, medicine, as a profession, should possess the following basic characteristics: (1) having a sophisticated body of knowledge and know-how; (2) operating in the service of others and being directly accountable to the patients; (3) the members of which are governed

by codes of ethics and profess a commitment to competence, integrity and morality, altruism, and the promotion of the public good within their domain during their lifetime; (4) the right to autonomy in practice and the privilege of self-regulation granted by social contract.

In medicine, physicians should be held accountable to the health of the public. However, under most circumstances, the central value of the medical profession is realized by establishing a sound rapport with a specific small group of patients and proffering care for them. The professional goal of the services provided by healthcare workers is to safeguard and improve the health of the patients. From this perspective, PPR is the core relationship in the healthcare sector and also the basic unit or the "building block" of it. In short, it is the cornerstone of the whole social system that is called medicine. Any social, political factors or economic forces that poses a threat to the PPR in any way will directly threaten the development of the entire health care profession and vice versa (Hui 2005).

A History of the Development of Professional-Patient Relationship

The professional-patient relationship has occurred since the birth of medicine. Medical practices always happen in the form of interactions between patients and doctors and medicine is always pushed forward by such interactions. As time passes, the professional-patient relationship has changed profoundly.

In ancient times, medical practitioners worked as individuals providing medical services for a living. There are two features in the traditional professional-patient relationship: first, it's simply a one-on-one relationship, because at the empirical stage of medicine, a doctor had to perform all the procedures, from analysis, diagnosis to treatment, all alone in person; second, patients in the traditional professional-patient relationship model, in essence, are passive receivers of healthcare.

Contemporary medical ethicists often criticize the traditional model for being "paternalistic." However, under the traditional healthcare context, the "parents" and the "children" shared common and concentric fundamental interests, and the moral standards of individual doctors and the trust from the patients constitutes the two most important pillars of the professional-patient relationship. Under the present situation, in which the professional-patient relationship is facing a series of threats and challenges, we often miss and praise the traditional professional-patient relationship and even regard that period of time as the "golden age."

In modern times, with the rapid development and extensive application of medical science, technologies and devices that became more and more sophisticated were utilized in healthcare; thus, traditional empirical medicine evolved into modern medicine that encompasses advanced technologies and equipment. Correspondingly, the traditional relationship also underwent fundamental changes.

Firstly, expanded new relationships, other than the traditional one-on-one relationship between doctors and patients, were built, transcending the realm of medicine and becoming more sophisticated. The development and clinical application of modern healthcare technologies have posed many ethical, legal, and societal problems, such as the use of life-sustaining healthcare devices, saving newborns with serious physical handicaps, and the distribution of rare donor organs and medical resources. However, the existing knowledge system, analytical methods, and problem-solving models can't solve these problems. Therefore, since the 1950s or the 1960s, people from different disciplines and professions, including doctors, lawyers, clergies, theologians, philosophers, and artists, began working with physicians and played important roles in consultation, investigation, and even decision-making in clinical care. As a result, under such a trend of socialization and institutionalization, contemporary professional-patient relationship became more complicated and alienated, turning doctors into "the strangers at the bedside" (Rothman 1991).

Secondly, patients have become more and more involved in the clinical decision-making as their sense of autonomy strengthened and medical

knowledge became more accessible to the general public. Thus, the paternalism in the professional-patient relationship faded gradually and was replaced by a new relationship based on cooperation, patients' participation, and respect for patients' autonomy.

Thirdly, with the increasingly advanced and highly effective healthcare technologies rendering medicine extremely powerful and authoritative, doctors became more and more relied on and addicted to pursuit of those technologies. Inch by inch the professional-patient relationship is replaced by a relationship between doctors and those technologies. For many doctors, they cannot see the patients but only the diseases and methods and equipment to treat them (Dugdale et al. 2008).

Besides, the healthcare service sector and the pharmaceutical industry, equipped with the innovative and advanced technologies, are no longer traditional empirical medical practice or individualistic care, but an important building block of social capital. Furthermore, with the full development of the profit-oriented market economy, the humanitarian tradition in the professional-patient relationship has been seriously compromised. In the end, the professional-patient relationship could risk degrading into a relationship between the providers and customers of healthcare.

Trust Is the Core of the Professional-Patient Relationship

Although we have seen significant changes in the professional-patient relationship in the history of social development, trust has been and will always remain the core of the relationship as long as medicine as a profession exists.

Trust is created when people engage in interactions of mutual benefits, dependence, and reliance. In those interactions, when one party makes a commitment and holds on to it for certain interests, the other party would trust and depend on him and hold expectations for his commitment. Healthcare is characterized by high specialization, high risk, uncertainties, and vulnerabilities of the patients, which determined that only by building trust between patients and doctors can we ensure the success of the medical practices.

Under most circumstances, the health of the patients is in the hands of the doctors, and this is especially true today when medical specialties have been further divided and the patients rely on their doctors even more desperately. Moreover, doctors are in an absolute superior position in terms of medical knowledge and the control of resources in the relationship. In clinical practices, patients have to not only describe their conditions truthfully but also communicate intimate information and the fear and anxiety hidden in their hearts. In certain cases, they may have to handover all the autonomy to the doctor. Under these circumstances, trust (or to become trustworthy) is of great importance. Patients must believe that doctors will, from the beginning to the end, honor the commitment that the health of their patients comes first and believe that doctors will do no harm to them or make use of their vulnerabilities to acquire power, money, and career success. Only in this way can real professional-patient relationship be established and medical practices carried out uneventfully.

Trust, the core feature of the professional-patient relationship, is of great ethical significance. It requires the professional, as the leading party of the relationship, to bear principal responsibility for building and maintaining the professional rapport with the patients. In other words, doctors should make themselves trustworthy and reliable in the first place. To this end, doctors should become professionally competent in research and in clinical care, and keep honing their skills; at the meantime, they must be committed to safeguarding and improving the health of patients and upholding the primacy of patient's well-being. Although there is no certain answer to how much sacrifice should a doctor make in order to be proved "professional," consensus is that doctors should uphold the primacy of patients' health and well-being and avoid any conflicts of interest with the patients (Pellegrino 1987).

The conflicts of interest become increasingly fiercer under the impact of market economy. In certain western countries, doctors who are in possession of healthcare facilities may overuse those

facilities for profits; or, may try to earn a gatekeeper's bonus from the insurance companies by blocking access to medical care; or, may accept various kinds of gifts and cash gifts from pharmaceutical companies. These misconducts all contribute to the potential conflicts between doctors and patients and directly undermine the professional obligations of doctors to prioritize patients' interest (Wazana 2000; Blumenthal 2004; Dana and Loewenstein 2003). When there is a shortage of healthcare resources, certain countries, including China, have adopted a healthcare system rather different from that of the western countries. However, the opportunities and ideas of turning medicine into business, driven by the free-market ideology, are more prevalent and prosperous. If we regard traditional medicine before the 1950s, which hew to the principle of upholding the primacy of patients' benefits, as benevolent, the negative impacts of the healthcare market have fundamentally jeopardized medical professionalism and have become the biggest challenge faced by contemporary professional-patient relationship in a global scale.

Conclusion

This entry started with breaking down the fundamental term of the professional-patient relationship. As a profession, medicine must be based on the mastery of a complex body of knowledge and skills; its members must profess a commitment to maintenance of competence, integrity and morality, codes of conduct, altruism, and upholding the health and well-being of the patients. Altruism of the physicians constitutes the basis of the professional-patient relationship.

Furthermore, the article discussed the evolution of the professional-patient relationship and addressed the profound changes taken place in the relationship. However, the core element of both the traditional and contemporary professional-patient relationship, the trust placed in medical profession, remains the same. Nowadays, the professional-patient relationship is faced with a trust crisis and multiple challenges in almost every country. Although medicine is a profession that is deeply connected with local cultures and national traditions, and thus shows great differences in clinical practices, the greatest challenges of contemporary professional-patient relationship, generally speaking, are the intensified conflicts of interest under the impact of market force and medical corruption. In addition, with the extensive application of high-tech, especially the rapid development of the Internet and telemedicine, a new type of professional-patient relationship may emerge in the future.

Cross-References

▶ Consent: Informed
▶ Respect for Autonomy

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Psychiatry Ethics

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Abstract

Defining psychiatry as a praxeology or science of actions, the particular tension between facts, values, and beliefs that are present in research and care are emphasized. Psychiatric history shows clashes between the custodial and the therapeutic intervention in relation to mental and behavioral disorders. Also, it highlights different emphases on the somatic or the psychodynamic dimensions. Several aspects are relevant: research, care, teaching, counseling, policymaking, and advocacy. Each presents its own ethical challenges. The most relevant issues pertaining to psychiatry and bioethics are briefly outlined.

Keywords

Psychiatry; Bioethics; Ethics; Science of actions

Introduction

Beyond its condition as a medical specialty, psychiatry may be considered a complex praxeology, or science of actions (Lolas 1996, 2006). Medical identity incorporated it to mainstream medicine, based on the natural sciences, during the XIXth century, expecting that it would acquire respectability and be considered as a scientific endeavor. Its origins may be found in the need of human societies to isolate or control deviant behavior, such as crime, poverty, or abnormality according to cultural standards. It may be said that as a practical social tool to control these conditions, it began before its scientific or philosophical foundations were sought or established. In this sense, "practical" psychiatry (sometimes identified with asylums and isolation) precedes "academic" psychiatry, based on foundational sciences such as biology, physiology, sociology, or philosophical reflection.

Different aspects deserve attention from the standpoint of ethical reflection: (a) *research*-the constitution of new and reliable knowledge; (b) *practice*-the application of knowledge to human affairs both individually and socially; (c) *teaching*-the reproduction of skills, competencies, and attitudes in new members of the profession; (d) *advocacy*-compromise with rights and duties of patients and professionals; (e) *counseling*-advice to interested parties; (f) *policymaking*-advice to planners and legislators.

As different from other fields of medicine, firmly grounded on somatic and apparently "objective" data, psychiatry has always been subjected to changes in its epistemic base. Some outspoken critics contend that the medical model is not appropriate for psychiatry. Szasz (1974) and Torrey (1974), among others, observe that there is no clear definition of what "mental illness" is not nor how the causes and cures of conditions labeled as such can be ascertained. The comparison between mental hospitals with medical institutions reveals that they do not fulfill the same roles and that the use of psychiatric diagnosis, even in the judicial system, is a form of coercion and stigmatization. Diagnostic systems are based

more on description and opinion than on universally agreed upon criteria. The introduction of so-called theoretical diagnostic systems, like the ones elaborated by the American Psychiatric Association since 1952 (Diagnostic and Statistical Manual in its versions 1–5), and the systems proposed by other groups of experts (like the ones developed in the context of the International Classification of Diseases, ICD) are validated against expert opinion, cultural traditions, and scientific knowledge. The distinction between fact, belief, and value is particularly complex in psychiatry, and the definition of abnormality does not always coincide with the definition of illness, disease, or sickness that can be supported on factual or instrumental information. Its categories, like most health-related ones, are value laden. The emergence of evidence-based medicine has been influential in adopting an objectivistic point of view and heavily influenced by the interests of industry and politics.

Throughout its history, psychiatric theorization has oscillated between a "mindless physiology" and a "brainless psychology." The emphasis on somatic processes as fundamental for behavior, mentation, and affect has clashed with philosophical and psychodynamic orientations that are as unilateral as a pure somatic stance. In recent decades, the social dimensions of mental disorders and the importance of mental health for health in general have been stressed. The relevance of culture and tradition for defining and treating mental illness is now universally accepted (Alarcón 2013). The need for integration of different methods and outlooks has been continuously stressed (Bracken et al. 2012; Cacioppo et al. 2014).

Ethics in Psychiatry: Relevant Issues

Levels of Axiological Reasoning: Regulations

In psychiatry, moral and nonmoral values are important. The first relates to intrinsic features of the human condition and are universal, albeit differently expressed in different cultures. Nonmoral values are instrumental to good practice: efficacy, truthfulness, honesty, transparency in data generation, among others. It may be said that in psychiatry, an axiological pluralism is essential for diagnosis, intervention, prognosis, and prevention.

While values are universals of meaning lending an axiological substrate to action in the areas delineated above, principles are prima facie mixtures of moral and nonmoral values. Autonomy, nonmaleficence, beneficence, and justice, along with solidarity and reciprocity incorporate moral and instrumental dimensions. Technical competence, for instance, is as essential as compassion and understanding for a correct performance in research and care.

Aside from principles related to research, practice, planning, and advocacy, other norms pertain to the professional obligations of psychiatrists. All professional societies have written codes of conduct for their members. In addition, the World Psychiatric Association – to which most psychiatric associations belong – have expressed concerns about different aspects of psychiatry worldwide, as reflected in the Madrid Declaration, dating back to 1996 and constantly revised by a standing committee on ethics. Other professional international groups, such as the World Association for Mental Health or the World Federation for Societies of Biological Psychiatry and the World Association for Social Psychiatry – among others – have also established permanent groups for the analysis of ethical problems.

Ethics, Human Rights, and Psychiatry

Similar interests can be discerned in the development of bioethical thinking in psychiatry and human rights theory. Both derive from historical events that caused public concern or horror and both relate to civilizatory and globalization processes. Scientific developments and an increase in the destructive potential of technologies, along with tensions between the private and the public spheres in social life have stimulated interest in values, professional conduct, the meaning of knowledge, and the goals of science and medicine. The distinction between facts and values is of permanent importance in medicine in general

and in psychiatry in particular. Despite attacks from anti-psychiatric movements, attacking the profession because of its links with industry and politics in some cases, psychiatrists are committed to the respect of human dignity, to equity in access to care and to the benefits of science, and to the improvement of the human condition (condemning participation of its members in torture, degrading treatments, or transgressions to the moral of the ethical professions).

Transcultural Issues

A strong development of global mental health has drawn attention to the fact that cultural competence and cultural coherence are essential components of psychiatric research and care. In point of fact, ethical issues are linked to a respect for cultures and human diversity. This includes a careful appraisal of diagnostic systems, avoiding stigmatization and discrimination without losing validity, reliability, and usefulness. Interventions are constantly examined for their cultural acceptability, rejecting the notion that a "one-size-fits-all" posture, since diversity includes ethnic background, religious beliefs, economic conditions, and political contexts. The introduction of "culture-fair" concepts has been tackled in different versions of the most commonly used diagnostic systems. The use of language is socially shaped and what can be a normal behavior in Japan may not be the same in Western countries. Progress in the direction of a more inclusive diagnostic armamentarium has been made (Alarcón 2014). In transcultural approaches, the problem of outcome is important. It needs reflecting upon what a good outcome means for whom. There may be discrepancies about what an outcome is for professionals, health administrators, participants in research, or patients. Transcultural psychiatry also considers the timespan appropriate for considering that a problematic condition has been definitely solved or to prevent undesirable effects of interventions in the long run (Thornicroft and Slade 2014).

Novel Technologies

New developments in research and care need special attention. Innovations possess their own dynamics before they become part of the standard armamentarium of resources for investigative processes or for the treatment of persons affected with mental illness or conditions. Genomics, brain research, and social analysis of the impact of innovation pose particular problems. New diagnostic and intervention possibilities are in need of careful analysis by ethics committees and in a conceptual reformulation aimed at integrating different approaches and perspectives (Cacioppo et al. 2014). The developments, applications, and ethical implications of specific technologies in the fields of psychiatry and related disciplines are extensively discussed in a comprehensive Handbook of Neuroethics (Clausen and Levy 2015).

Abuses of Psychiatry

In some parts of the world, psychiatric knowledge has been used for torturing people or for punishing political dissent. Diagnostic categories, when used inappropriately, may have devastating effects on the welfare of people (Lolas 2009). This situation, aside from developing and disseminating written codes of conduct, requires permanent examination of cultural and valoric competencies of researchers and practitioners. In some countries (e.g., former Soviet Union and China, bur probably widespread), psychiatrists have participated in tortures and "reeducation programs" under political pressure. Most psychiatric rules of conduct (e.g., APA code of ethics and the Madrid Declaration of the World Psychiatric Association) specifically address this form of censurable professional conduct.

There exists a conflict of interest whenever overt motivations and intentions are mixed with hidden, unexpressed ones. Philanthropic interests may be manifestation of economic or power motivations. While monetary conflicts are now routinely disclosed, it has to be realized that there are other forms of conflict. For instance, institutional prestige may sometimes be involved in avoiding bad publicity for mistakes or failures in research or healthcare practices. Paramount cases have involved individual and groups influenced by industry in the fields of pharmacological research and notification of complications or side effects of

interventions. Conflicts of interest are unavoidable in a complex field such as psychiatry. The important point is to be aware of their existence and to take appropriate measures to uncover them. Sometimes conflicts are directly perceived by professionals and lay people. They may be not openly evident but are disclosed by interested parties. Almost every social practice may include some form of deception, deliberate or involuntary. Individual professionals and ethics committees must consider the risks and harms involved in conflicts of interest and regulate relations with industry, political power, and other professions.

Confidentiality

Both in research and psychiatric care, confidentiality has become an important issue. Preserving anonymity of research participants may have implications in societies where some conditions lead to stigmatization or discrimination. The professional-client or doctor-patient relationship is always confidential, except in those cases where legal authorities demand disclosure.

Personal Relationships

The relationship between a professional and a person requesting services is at the same time one of compassionate desires to help and endowed with equanimity and distance. Otherwise, technical skill can be hampered by emotional factors. Most codes of professional conduct in psychiatry stress the importance of limiting this relationship to only those aspects that demand technical competence and personal support. Intimacy or sexual relations are as a rule prohibited in view of the vulnerability of persons requesting assistance and the professional power accorded by the psychiatric profession.

Informed Consent

Since the time of the so-called Nürnberg Code, with its emphasis on voluntary acceptance of interventions on the mind and the body, it is an accepted practice in research and care that subjects must provide their informed consent for any intervention. Informed consent is a process of building trust and assuring full disclosure and acceptance of risks and possible harms involved. Information, provided in an understandable form, is an essential component of consent. In addition, subjects must be free to abandon research or treatment if they express this desire. Consent forms or other ways of obtaining consent are integral part of any action taken by professionals or institutions. The procedure must consider cultural diversity, educational level, and contexts. In some societies, for instance, traditional tribal groups in Africa or the Middle East, not all members are entitled to give individual consent, and the authority of others must be sought (elders, husbands, and tutors). In the case of minors, at least in Western societies, their personal assent is also required, despite the fact that in some cases they are considered noncompetent.

Some writers distinguish between legal competence and cognitive capacity to provide consent. Both researchers and caregivers are advised to familiarize themselves with adequate procedures for assessing capacity and with legal norms applicable in the countries where they work.

Therapeutic Misunderstanding

This condition exists when, during a research study, subjects may believe that their doctors act only in their best interest. The fact is that some stages of the research process, particularly in clinical and pharmacological trials, will not offer any direct benefit to persons. Professionals should consider that in the social sphere, medical doctors are expected to always act in the benefit of their patients and provide the best available treatment or intervention. This is coupled to the social role expected from medical and other professionals and should be clarified by these when dealing with people acting as research participants. The classical distinction made by the original Declaration of Helsinki (1964) between therapeutic and nontherapeutic research was based on the personal benefits obtained and expected by research subjects, and it opens a complex set of unresolved issues. The therapeutic misconception may be

deliberate or involuntary, but it is a factor that should be taken into consideration in every research process. In particular, under conditions of cognitive impairment on the part of subjects, ethical questions raised must be resolved by careful appraisal of capacity, by resorting to substituted judgment or best-interest principle approaches either by consulting with a close relative, appointing a mentor, or judging according to the vital trajectory of the person involved.

Measures Taken Against the Will of Persons

Psychiatrists may consider forced hospitalization in cases where there is danger for the person or for people around. This has aroused much discussion in different periods, and some countries demand that formal legal authorization is sought after. This seriously hampers professional autonomy and judgment of psychiatrists and should be analyzed according to context. By definition, some mental states preclude clear judgment and prevent informed consent.

Research Integrity

As in other fields of medical science, knowledge must be acquired respecting basic principles of truthfulness and honesty. Plagiarism, falsification, or fabrication of data are damaging to the profession in its knowledge base. They should be prevented through proper training and information about accepted practices in the scientific literature: rules for authorship, definition of plagiarism, fabrication, or falsification of data.

Placebo and Nocebo

In medical and clinical research, the effects not due to an intervention or drug – and sometimes related to symbolic aspects or expectations – must be ruled out in order to correctly assess effectiveness and efficacy. It should be noted, however, that in psychiatry the placebo effect may be consciously used as part of therapeutic interventions and that trust and self-confidence on the part of the practitioner may increase the therapeutic efficacy of interventions. This is not to be confused with deceiving patients or increasing false hopes but may constitute part of the necessary skills for application of knowledge. In psychiatric research, as different from other forms of medical research, placebo effects may be more difficult to handle. During a clinical trial, for instance, the increased attention given to participants may have an impact on their responses to treatment. In addition, some investigations may be aimed not at evaluating efficacy but at uncovering modes of action of medications.

Care should also be exercised when evaluating side effects or negative effects produced by correctly prescribed drugs or interventions. Nocebo effects are to be considered both in research and in clinical practice.

Conclusion

Ethical reasoning in psychiatry covers problems and dilemmas in many different areas: clinical practice, research, teaching, advocacy, and policymaking. This ethical dimension is considered at the level of committees designed to help clinicians, to orient researchers, and to govern personal conduct of practitioners and researchers. Psychiatry is considered to be more than a medical specialty, since it has become a specialized profession. Although following the imperatives and duties of medicine in general, several problem areas acquire in psychiatry a particular aspect. Critics of the psychiatric enterprise contend that the medical model is not adequate to the goals of an ethical profession and suggest it be replaced by educational and preventive strategies. This chapter reviews some of the most salient problems confronting researchers and clinicians in the psychiatric field, insisting that deliberation and stakeholders' participation are needed at every level of the psychiatric enterprise when dealing with its axiological dimensions. Psychiatric axiology refers both to moral and nonmoral values, the latter being those related to instrumental or technical aspects, the former pertaining to universal features of human nature. The growing field of neuroethics considers the applications and moral implications of novel technologies and imposes a reappraisal of traditional ethical dilemmas. Prima

facie principles like autonomy, beneficence, and justice are considered mixtures of moral and non-moral values adapted to circumstance and persons.

Cross-References

- ▶ Applied Ethics
- ▶ Bioethics: Medical
- ▶ Clinical Ethics: Consultation
- ▶ Codes of Conduct
- ▶ Consent: Informed
- ▶ Mental Health: Services
- ▶ Mental Health: Therapies
- ▶ Mental Illness: Concept of
- ▶ Mental Illness: Treatment of
- ▶ Neurotechnology
- ▶ Scientific Misconduct

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Psychosurgery

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Abstract

Psychosurgery encompasses neurosurgical procedures that are performed to alter a person's thoughts, emotions, personality, or behavior. Although its history dates back to antiquity, psychosurgery was first introduced into modern medicine as a treatment for patients suffering from psychiatric illness in the early twentieth century. Psychosurgery reached its zenith with the widespread performance of the now notorious leucotomy and lobotomy procedures of the 1930s and 1940s. However as a result of adverse patient outcomes and questionable clinical practice, the use of psychosurgery to treat mental disorder became increasingly opposed. This opposition, in conjunction with the introduction of psychotrophic drugs for the treatment of mental illness in the mid-twentieth century, rendered psychosurgery virtually obsolete. However, over the last few decades, advances in neuroscience, imaging technology, and psychiatry have contributed to the development of a new era in psychosurgery (Robison et al 2012). These advances have also heralded a new technique that involves a nondestructive neurosurgical procedure – deep brain stimulation. This entry considers the ethical issues associated with psychosurgery, illustrating why psychosurgery was so problematic in the early twentieth century. It also discusses ethical issues that remain relevant to the contemporary practice of psychosurgery. It concludes by briefly outlining some of the approaches to regulating psychosurgery globally.

Keywords

Psychosurgery; Neurosurgery; Deep brain stimulation, mental disorder; Authenticity; Autonomy;

Coercion; Beneficence; Nonmaleficence; Identity; Behavior control

Introduction

Psychosurgery encompasses neurosurgical procedures that are performed to alter a person's thoughts, emotions, personality, or behavior. Historically such procedures involved the destruction or ablation of brain tissue in the form of leucotomy or lobotomy. While performance of these crude procedures flourished in the early-mid twentieth century – its nadir resulted after it became increasingly associated with unethical clinical practices and negative patient outcomes. The resulting public backlash as well as the introduction of psychotrophic medications to treat psychiatric illness in the 1950s effectively consigned "psychosurgery" as it then was to a notorious chapter in medicine's history.

While contemporary "psychosurgery" is similarly informed by the theory that the manifestations of neuropsychiatric illness occur as a result of underlying neurological dysfunction, its practice is substantially different. Modern psychosurgery is based on a much-improved understanding of neuroanatomy and function. It does not generally involve destructive or ablative surgery, but primarily comprises the insertion of intracerebral electrodes for the purpose of stimulating specific areas of the brain via an implantable pulse generator. These implantable electrodes allow electrical stimulation/modulation of a targeted area of the brain to attenuate the symptoms associated with the particular neurological or psychiatric illness being treated. Similar to its historical counterpart, this procedure, which is aptly termed "deep brain stimulation" (DBS), *inhibits* neural activity. However, unlike the earlier ablative lesioning procedures, DBS is nondestructive and generally reversible (Mashour et al. 2005).

This entry briefly describes the evolution of psychosurgery from ancient to modern times. It outlines the theory that underpinned the re-emergence of psychosurgery as a treatment for psychiatric illness in the late nineteenth and early twentieth centuries. It also discusses the current performance of DBS, as well as some of the "typical" approaches to regulating the performance of ablative and nonablative psychosurgery. However, the main focus of this entry is on exploring the ethical dimensions of psychosurgery.

The Early History and Subsequent Development of Psychosurgery

Attempts to modify aberrant human behavior date back to antiquity. The earliest known instance of psychosurgery was trepanation, which involved boring a hole through the person's skull to release evil spirits or to provide relief from the manifestation of illness, such as epilepsy or mental disorder (Faria 2013a). Evidence of trepanation has been discovered on prehistoric skulls dating back to 5100 BC, with archeological findings indicating that trepanation was extensively practiced in pre-Columbian Mesoamerica, particularly in Peru and Bolivia, as well as in Europe and Asia (Mashour et al. 2005). While trepanation continued to be practiced during the Middle Ages as a treatment for psychiatric illness, it dwindled during the Renaissance period (Faria 2013a).

The era of ablative psychosurgery (i.e., leucotomy and lobotomy) brought about a resurgence in trepanation. This emerged in the late nineteenth and early twentieth century as a direct result of the adoption of the concept of cerebral "localization" by the medical and scientific community. This was the notion that specific areas of the brain were responsible for particular neurological or behavioral functions. This theory was largely a response to observing the effects of traumatic brain injury or pathology on human capacities and behavior – most famously illustrated by the case of Phineas Gage in 1848.

Gage, a railway worker, was injured in an explosion that resulted in a tamping iron being driven into his cheekbone, which then passed through his temporal lobe before exiting his skull. Remarkably Gage survived, but the effect on his personality and behavior were extreme. Gage changed from being a well-balanced energetic individual, to exhibiting poor impulse control and executive functioning – a phenomenon

now identified as "frontal lobe syndrome." The insights obtained from this correlation of brain injury with aberrant behavior, along with the work of neurosurgeon Pierre Broca and neurologist Carl Wernicke who both postulated links between particular areas of damage to the brain and language deficits, contributed to the development of the localization theory of neuroanatomy. The localization theory posits that pathological or mentally disordered states are attributable to a particular area of the brain. This theory was a significant precursor to the eventual introduction of the psychosurgical procedures: frontal leucotomy and lobotomy. These procedures emerged against a backdrop of extensive, and in some cases debilitating, mental illness for which institutionalization was the only treatment option at that time (Robison et al. 2012).

The Frontal Leucotomy and Lobotomy Era

Portuguese neurologist Egas Moniz and his neurosurgeon colleague Almeida Leima first performed frontal leucotomy in 1935 to treat people suffering from mental disorder. This crude procedure originally involved injecting alcohol into the white matter of the patient's frontal lobe to cause tissue necrosis (death) – a procedure that was subsequently refined to cutting lesions in the white matter with a purpose-built instrument called the leucotome. When Moniz was awarded the Nobel Prize for physiology and medicine in 1949, the technique had become an established treatment for a range of conditions throughout Europe and the Americas. These techniques effectively severed the neural pathways in specific areas of the brain to induce personality changes in the patient.

Neurologist Walter Freeman and neurosurgeon James Watts subsequently modified the procedure, publishing a monograph in 1942 that described the results of 200 frontal lobotomies they had performed (Robison et al. 2012). The notoriety of psychosurgery reached its zenith with the subsequent popularization of the transorbital frontal lobotomy pioneered by Freeman. During this procedure, patients were rendered unconscious using electroconvulsive therapy, after which a surgical tool resembling an ice pick was inserted through the skull above the orbit of the eye and moved laterally through the prefrontal lobe (Mashour et al. 2005). It has been estimated that between 1936 and 1956, 60,000 psychosurgery procedures were performed in the USA and Europe (Robison et al. 2012).

Although the original focus was mental illness, psychosurgery was also proposed as a means of social control. In the mid-twentieth century psychosurgical intervention was suggested as a potential tool to quell civil unrest (in the context of the civil rights movement) as well as violent prisoners (Pustilnik 2009). In 1968, a military surgeon performed psychosurgery on several prisoners (one being a minor) in a United States Penitentiary (Vacaville). Electrodes were inserted into the prisoner's brains to destroy amygdalar tissue in an attempt to eradicate the prisoner's capacity for violence (Pustilnik 2009).

However, there was a rapid fall in the popularity of psychosurgery during the 1950s as the associated risks became better known, and there was greater public awareness of the context in which it was being performed. A major contributor to its demise was the discovery of psychotropic medications, such as chlorpromazine and haloperidol, in the second half of the twentieth century. An additional factor in the downfall of psychosurgery was its graphic and negative portrayal in pop culture, such as Ken Kesey's novel *One Flew Over the Cuckoo's Nest* (Robison et al. 2012).

The developing negativity with which psychosurgery came to be viewed is exemplified in the US case of *Kaimowitz v. Michigan Department of Mental Health* (Civil No. 73-19434-AW) in 1973, an unreported decision of a circuit court in the United State's Michigan County. Kaimowitz involved a prisoner, Smith, who had killed a nurse and subsequently performed necrophilia on her body. He had been detained for 17 years after being committed to an institution under the criminal sexual psychopath law. With his release imminent, Smith wished to participate in a study comparing the effectiveness of psychosurgery with drug therapy in treating uncontrollable aggression. However, Kaimowitz, a member of the Michigan Legal Services, brought an action

on behalf of the proposed study participants to halt the state-funded experiment. Although the proposed research was abandoned following the institution of legal proceedings and the negative public reaction that resulted, the court nevertheless considered that the issues involved were of sufficient public importance to warrant a declaratory judgment. Consequently, the Court sought to determine what constituted a legally adequate patient consent to an *experimental* procedure in the context of an involuntarily committed patient.

The Court premised its decision on the view that involuntarily detained mental patients reside in an environment that is intrinsically coercive. It found that the inherent subtle, but profound, psychological coercion at play in those circumstances rendered such individuals unable to reason as equals with medical and administrative personnel as to whether they should receive psychosurgery. Given this relative inequality, the Court doubted whether individuals in that situation could give voluntary informed consent. Ultimately, it invoked the Nuremberg Code's principles to justify its conclusion that individuals, while living in such an environment, could only consent to low-risk high-benefit procedures, which did not include experimental psychosurgery.

As a result of the many critical accounts that chronicled the profound side effects and personality changes experienced by many individuals who underwent psychosurgery, in conjunction with widespread concern regarding its potential use by States to control citizens' violent or antisocial behavior, a major ethical review of psychosurgery was undertaken in the USA in the mid-1970s. The US National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research released its report in 1977 (Mashour et al. 2005). Contrary to general expectations, the Commission did not recommend outright prohibition, but instead outlined guidelines for the ethical use of neurosurgical procedures for psychiatric disorder.

Over time, the performance of psychosurgery has been refined by the introduction of stereotactic surgery, which enables more precise targeting of areas for ablative lesioning. Advances in imaging techniques such as functional magnetic resonance imaging (MRI) have also enhanced the capacity to examine neuroanatomy and function in the context of illness or disease.

Contemporary Psychosurgery

Mashour et al. describe four psychosurgical procedures that are currently used to treat affective or anxiety disorders that are refractory to drug therapy, psychotherapy, or electroconvulsive therapies. These are anterior cingulotomy, subacute tractotomy, limbic leucotomy, and anterior capsulotomy (Mashour et al. 2005). However, deep brain stimulation (DBS), a relatively new psychosurgical treatment modality introduced in the 1990s, constitutes a significant alternative to ablative procedures.

DBS was introduced as a result of the myriad advances in imaging capacity, neurosurgery, neurology, and psychiatry over the latter half of the twentieth century. DBS constitutes the first nonablative and nondestructive treatment modality for movement disorders, as well as treatment-resistant psychiatric illness. Although the mechanism of DBS is still not completely clear, it is thought to modulate disordered brain networks. While by no means a mainstream procedure, it has been successfully used to treat Parkinsonian tremor, essential tremor and dystonia (Greely 2008), and has been investigated in the context of treatment-refractory psychiatric illnesses, including obsessive-compulsive disorder (OCD) and major depressive disorder (Robison et al. 2012). Indeed, Robison describes DBS as "a conceptual breakthrough in the treatment of psychiatric illness" (Robison et al. 2012). A more recent focus of DBS research is on determining the neural pathways affected in individuals suffering from treatment-refractory addiction. In this context, DBS is used in an attempt to normalize dysfunction in the "reward" circuitry of the brain. DBS, which is also termed "neuromodulation," has also been investigated as a treatment for eating disorders, as well as posttraumatic stress disorder (Robison et al. 2012). Although certainly not common, it has been estimated that more than 100,000 people

worldwide have received DBS implants (Johansson et al. 2014).

Ethical Implications of Psychosurgery

A Common Thread

The ethical controversy that was associated with psychosurgery in the early twentieth century turned on the harmfulness of leucotomy and lobotomy. It is pertinent that the kind of harm inflicted upon people at that time occurred because it "changed" many of those individuals in profound ways. The personality changes following leucotomy and lobotomy were clearly some of the most alarming harms of early psychosurgery. The extent to which DBS alters personality, or one's "sense of self," likewise continues to be an important potential harm to be considered in this context. Consequently a primary ethical question is how beneficence, that is promoting the patient's health and welfare, is to be weighed against the principle of nonmaleficence, which is the professional obligation to "do no harm." Ultimately this involves a multidisciplinary clinical team determining with the patient whether the patient's refractory clinical symptoms are severe enough to warrant the uncertain, and potentially profound, effect of psychosurgical intervention.

Accordingly, many jurisdictions internationally place considerable emphasis on the importance of consent to psychosurgery being fully informed and voluntary. These requirements are premised on the principle of respect for persons/autonomy. While consent is an important way of ensuring that human beings are accorded the moral status that they deserve, respecting an individual's "personhood" is also relevant to psychosurgery in a more subtle way. The wrongfulness of the harm caused by leucotomy and lobotomy historically was largely due to the way that it damaged very deeply the essence of those who underwent it. Respecting the personhood of those who are candidates for psychosurgery likewise suggests that clinicians and patients must consider the possibility that it might change that individual profoundly.

Consent and Psychosurgery

A fundamental principle of medical ethics and law is that, in the case of a competent patient, consent should be sufficiently informed and voluntary. This can pose challenges in this context given that some individuals who are candidates for psychosurgical interventions may be particularly vulnerable due to their severe, and refractory, neurological or psychiatric illness. Further, if they have not responded to other traditional treatment, the procedure may effectively constitute their "last hope." In addition, some individuals may have cognitive deficits associated with their illness. The issue of informed consent is also problematic given the inherently complicated nature of psychosurgery. Although assessing the benefits and risks of a procedure is part of routine clinical practice, predicting the likely effects of psychosurgical procedures such as DBS is not straightforward. As experts concede, the "unknowns" regarding the neural mechanisms underpinning many conscious processes, as well as uncertainties regarding the actions of such neurointerventions, makes predicting their likely effects, as well as calculating benefits and risks, extremely difficult (Jotterand and Giordano 2011).

Effect on Personality Traits and/or Identity: The Experience of Some DBS Recipients

Clinical reports indicate that some patients experience a change in personality and demeanor following DBS. But although certain aspects of personality may be altered as part of treatment for illness, it cannot be known in advance whether those altered states will be negatively or positively experienced (Jotterand and Giordano 2011).

Clearly the phenomenon of therapy altering an individual's demeanor or personality is not limited to DBS. For example, some patients receiving the antidepressant Prozac report that it not only relieves the symptoms of their depression, it also enhances mood, energizes and, to an extent, may alter an individual's personal identity (Jotterand and Giordano 2011). However, it is debatable how much individual "change" is required before a treatment effectively alters a person's actual

"identity," that is, their self-constructed genuine or authentic self.

When evaluating the claim that DBS may impact on an individual's identity, it is important to first define which sense of "identity" is being referred to given the competing accounts of identity. One important distinction is between personal identity in an indexical sense – literally being a *different* person, and qualitative identity – that is being a different *kind* of person (McMillan 2005).

An "indexical" sense of identity is one that tells us the conditions that make something the specific thing it is. For example, "the tie that I wore yesterday" is a specific object and "the tie that I am wearing today" is the same tie in an indexical sense if, and only if, there is continuity throughout time of that object. The tie that I am wearing today could, indexically, be the same that I wore yesterday even if I had spilled ketchup over it and cut the end off in a blender. Likewise, we would all think that we are, indexically, the same person that we were ten years ago, even though we might be qualitatively a very different person from the one that lived ten years ago.

The "indexical" and "qualitative" senses of "identity" are two different ways of talking about what it is that makes a person the person they are. However, it is difficult to see how DBS could change a person so much that they literally become a different person, without damaging them to the extent that their personhood is called into question. For example if after DBS radically changed an individual's behavior in the way that Phineas Gage's did when an iron bar passed through his prefrontal cortex, that person would become, qualitatively, a different person, but we would be in no doubt that he/she was "indexically" the same person. The point is illustrated well by Gage himself, who clearly still was "Phineas Gage." Clearly it is hard to see how there is any version of DBS that could result in an indexically different person.

One account of qualitative personal identity claims that:

> All changes and only changes in central attitudes constitute a change in individual identity, i.e., if and only if our central beliefs, values, ideals, plans, projects, ideologies etc. change, we change in a substantial way. (Witt et al. 2013, p. 503.

On this approach, altering an individual's (qualitative) identity means that a person's core beliefs (i.e., the central beliefs that inform the way that he/she interacts with the world) have changed, such that it causes a profound cognitive shift in the person, thereby disrupting the continuity of the person pre-and-post DBS (Witt et al. 2013, p. 506). However, while there would be reason to think that a person's self-conception or qualitative identity had changed markedly over time in such cases, whether changing fundamental beliefs constitutes harm is another question.

As well as having a possible effect on demeanor, or even identity, some patients have reported experiencing a sense of self-alienation following DBS. Johansson notes a study involving 29 patients, 19 of whom self-reported a feeling of not fully recognizing themselves after receiving DBS (Johansson et al. 2014). However participants in that particular study did not consider this feeling sufficient to warrant ceasing DBS therapy.

As well as being a first person phenomenon, identity ascriptions can also be made by those that know us well. Clinical reports have described how some families struggle to adapt to what they perceive as a "new" person after a family member has undergone DBS. One such report involved a woman who had received DBS for severe OCD. Her husband observed that while his wife's condition improved suddenly when the device was switched on, there was no similar button to push to assist her family to adapt to "this new person" (Jotterand and Giordano 2011, p. 481).

The extent to which DBS may impact on identity is unknown in advance. Greater understanding of how a patient's "sense of self" or identity may be affected by DBS procedures, and its effects on the patients significant others, constitutes a rich area for ongoing research. It is important so that future candidates for these procedures may be better informed regarding the possible effects of treatment. Clearly the implications of a DBS-induced alteration in an individual's personality or "sense of self" may be significant, which is

such brought sharply into focus when treatment does not go well.

The way in which DBS may impact upon an individual's behavior and personality was illustrated in a recent coronial investigation into the death of an Australian man following treatment for Parkinson disease (State Coroner of New South Wales Australia 2014). The man had exhibited rapidly fluctuating behavior and moods, as well as impulsive behavior, immediately following DBS. Consequently the DBS settings were altered. However two days after he was discharged from hospital, after experiencing on-going fluctuations in his mood and behavior, the patient unilaterally ceased his medication and turned off his stimulator. While he recommenced the stimulation on medical advice, his moods subsequently continued to swing from being settled and calm, to being agitated and "manic." Finally after an out-of-character confrontation with his wife, he left the family home and was found dead a short time later having taken his own life.

The coroner's findings emphasized the importance of fully informing and preparing patients, as well as their immediate family, for the possible side-effects of treatment. Also emphasized was the need for out-patient review, immediate access to expert care should an emergency arise, as well as postoperative psychological or psychiatric support for the patient.

Control Over Implant and the Potential for "Enhancement"

An issue that triggers some debate (in the case of devices that allow a patient to make self-adjustments) is the degree of control over treatment parameters that a patient should be given. While simply turning a device on or off is not problematic, changes to the intensity and frequency of stimulation may have various effects on the person, such as inducing a state of hypomania or elevating a patient's mood. While this is an issue that clinicians are likely to resolve by weighing the ethical considerations discussed above, patient control over an implant raises the possibility that (depending on the area of the brain targeted) DBS might be used to enhance cognitive ability such as memory or even emotional ability.

Although DBS is currently used to treat neuropathology, it remains to be determined whether it might, in the future, be used to enhance cognitive ability, or to influence an individual's mood in the absence of a clinical disorder. However it is important to bear in mind that DBS involves the insertion of electrodes into the brain. The associated risks, including the physical risks of surgery such as infection or bleeding, might suggest that it is unlikely that such procedures would be performed in the context of a person who is not suffering from a serious neurological condition. Conversely it is clear that an increasing number of people elect to undergo highly invasive cosmetic procedures in the interest of improving their appearance. It is plausible that there may be a similar attraction to undertaking invasive procedures that improve one's cognitive ability. Further, concerns arising from invasive surgery do not apply to noninvasive means of neurological electrical stimulation, as in the use of transcranial magnetic stimulation (TMS). Consequently whether or not such neurotechnologies should be used in the future to enhance physical, behavioral, and cognitive abilities in otherwise healthy individuals is very much an issue for debate.

Psychosurgery and Crime Prevention

As already indicated above, the use of psychosurgery as a means to control aggression and violent behavior in criminal offenders was a highly controversial issue in the early debates on psychosurgery. In some instances, it led to greater controls being placed on the provision of psychosurgery in particular contexts, such as when an individual is being involuntarily detained under mental health provisions or is imprisoned, because of the risk of coercion.

Of direct relevance to the criminal justice system is recent research conducted by a group of Italian researchers. Franzini and colleagues have described the long-term outcome of posterior hypothalamic DBS in the case of seven "severely impaired patients affected by refractory aggressive behavior and mental retardation" (Franzini et al. 2013). While no discussion was provided regarding the patients' capacity to give consent, all of the research participants were all described as of "below-average IQ." The research was

approved by an institutional Ethics Committee taking into account "the chronicity and severity of the condition, the related burden to families, and the refractoriness to conservative treatments." The patients' relatives provided written consent after receiving information regarding the rationale and risks of the procedure. The authors claimed that given the reversibility of the procedure and the sustained reduction in violent behavior that was witnessed in four of the patients, the procedure was ethically acceptable. It was concluded that in this context DBS "may help patients chronically isolated in mental institutions to be integrate [d] into society." Significant questions raised by the research, such as the effect of DBS on other aspects of the patient's personality, or the side effects experienced, were not discussed.

It is significant that some commentators are now considering the potential use of consensual DBS for repeat criminal offenders with neuropathology (e.g., psychopaths) who have not responded to conventional treatment (Faria 2013b). Using neuromodulation of the reward circuitry to reduce the potential of sexual and/or violent offenders to reoffend is, at least, a theoretical possibility. However one prominent commentator, who is skeptical of using DBS in the criminal context (Greely 2008, p. 1114) observes:

> Different reports have linked parts of the prefrontal cortex to criminal behavior, usually on the theory that the criminals lack appropriate impulse control because of under-activation of those regions. Other areas that have been implicated in criminal behavior include the hippocampus, the amygdala, the corpus callosum, and the hypothalamic-pituitary-adrenal axis. DBS provides a plausible method for stimulating greater activity in regions where diminished function is thought to play a role in crime. It could also provide an adjustable, intermittent, and reversible method of inhibiting activity in areas where over-activity may contribute to criminal activity and thus serve some of the functions of neurosurgery, but without its permanence. Of course, at this stage any such uses of DBS are highly speculative. On the other hand, uses of DBS seem to be spreading rapidly as neurosurgeons try different regions for different conditions. The extension of DBS research to criminality . . . seems likely. Its success is much less certain.

The view that criminal or violent tendencies may be caused by disordered neurobiology has a long and deeply troubled past (Pustilnik 2009). However in recent decades, criminologists have been instrumental in identifying the multifactorial components of criminality, emphasizing the influence of social, economic, and political factors on criminal behavior. Given the invasiveness of DBS, the lack of conclusive evidence regarding its efficacy and the potentially serious side effects of the procedure, the prospect of using DBS to ameliorate criminal tendencies, at least in the near future, seems dubious. Nonetheless, as DBS becomes more prevalent and the class of conditions for which it is offered increases, it seems likely that its use in treating aspects of criminality will at least be mooted by some (Faria 2013b).

Procedural Preconditions

Given the issues associated with psychosurgery, significant steps have been made by professional bodies to establish an internationally consistent approach to performing neurosurgery for psychiatric indications. A group representing the major international psychiatric and neurosurgical societies recently published an international multidisciplinary "consensus on best ethical practices, norms and professional behaviours," to guide professionals involved with stereotactic neurosurgery for psychiatric disorders (Nuttin et al. 2014). The contributors encompassed representatives from the World Society for Stereotactic and Functional Neurosurgery (SFN), the European Society for SFN, the Latin American Society for SFN, the Asian-Australasian Society for SFN, as well as the World Psychiatric Association. The aim of the guidelines is to establish ethical and scientific standards to promote patient safety when stereotactic ablative focal surgery or DBS is considered for the minority of patients who are affected by psychiatric illness, but who are not responsive to pharmacological or other evidence-based therapies.

The guidelines are premised on the view that "the accumulated evidence supporting the application of all neurosurgical treatments for psychiatric disorders" requires strengthening and that these neurosurgical procedures remain "at a 'proof-of-principle' investigational stage of development" (Nuttin et al. 2014). The guidelines

provide that ethical oversight must be provided by an independent Ethics Committee or Institutional Review Board for investigational neurosurgery, and be consistent with local and national regulatory requirements. Further, that care should be taken in determining when sufficient data has been accumulated to warrant a psychiatric neurosurgical procedure being designated a therapeutic, as opposed to an experimental, intervention.

Some jurisdictions have imposed a ban on psychosurgery, such as Australia's Northern Territories and the state of New South Wales (NSW). However recent amendments to NSW law specifies that "psychosurgery" does not encompass a *neurological* procedure performed to relieve the symptoms of Parkinson's disease, Gilles de la Tourette syndrome, chronic tic disorder, tremor, or dystonia. Consequently the NSW law distinguishes between DBS for neurological indications, as opposed to DBS for psychiatric indications.

After a renewed interest in psychosurgery in the early 2000s, Chinese neurosurgeons performed ablative procedures on more than 500 patients on an area of the brain thought to be associated with addiction. Around the same time, Russian neurosurgeons resected a part of the brain associated with compulsive behavior in over 300 patients (Greely 2008). Both countries have since halted the procedures. China's Ministry of Health now restricts civilian hospitals to performing ablative psychosurgery as a last resort for specific conditions for which its use is recognized (referencing refractory OCD, depression, and anxiety disorders), but imposes a ban on any "experimental" use (Xiao 2011).

Other jurisdictions only prohibit *involuntary* psychosurgery, and impose specific regulatory requirements for those patients who are subject to involuntary treatment for mental disorder under state law. Consequently in the context of civil commitment schemes, typical legislative provisions specify that psychosurgery may only be performed if the patient's voluntary and informed consent is obtained, and a designated Review Tribunal has approved the procedure (e.g., the Australian states of Victoria, Queensland; Tasmania and Western Australia; as well as New Zealand – although New Zealand's law refers to surgery or other treatment "intended to destroy any part of the brain or brain function," which may not encompass DBS). The UK requires validation of the patient's consent and their legal capacity to give consent, as well as written certification from an authorized (nontreating) medical practitioner that the treatment is appropriate in the circumstances. Additional information may also be specifically required, such as the qualifications of the doctors involved, and that all other alternative treatments have provided insufficient benefit (e.g., Western Australia and the Australian state of Victoria).

It is also not uncommon for some jurisdictions to single out psychosurgery as a treatment that, in the case of an incompetent person, a legally appointed substitute decision maker *cannot* consent to on their behalf (e.g., New Zealand). Alternatively if consent to psychosurgery is within the power of a substitute decision maker, restrictions may be imposed on the circumstances in which substituted decision making can occur, such as only permitting proxy consent in the case of a life-threatening condition which has not responded to traditional therapy.

However the extent to which a nation incorporates rights protections into mental health laws is not consistent across the globe. In an attempt to address these issues the World Health Organization developed a Mental Health Legislation Checklist in 2005 to aid in determining the extent to which a countries mental health laws complied with international human rights obligations (Drew et al. 2013). Significantly a recent examination of mental health laws in four countries on the African continent (Ghana, Uganda, Zambia, and South Africa) revealed that only South Africa had regulations governing psychosurgery, while none of the three remaining countries prevented procedures such as psychosurgery being performed without consent (Drew et al. 2013).

Conclusion

The development of psychosurgery in the early twentieth century continues to influence the way

in which modern techniques are perceived. However, the possibility of great harm being done to patients via direct neurosurgical interventions has been significantly lessened by the development of stereotactic techniques and nonablative procedures such as DBS. But while much more is now known, the outcome and side effects for individual patients is not always predictable. Given this, the importance of fully informing individuals of the potential outcomes and implications of ablative procedures or DBS (including their immediate family/support persons) is now widely recognized. It also suggests that the current cautious approach, whereby psychosurgery is generally used only as a last resort when other treatments have not been successful, is warranted. Ultimately, the requirement that its performance be reviewed on a case-by-case basis, particularly in the case of a patient who is under a civil commitment regime, is an ethically justifiable and arguably necessary requirement.

Cross-References

- ▶ Autonomy
- ▶ Behavior Modification
- ▶ Bioethics: Clinical
- ▶ Consent: Informed
- ▶ Identity
- ▶ Mental Health: Therapies
- ▶ Psychiatry Ethics
- ▶ Risk

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Public Debate

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Abstract

Ethical issues in healthcare and biomedical research are often a matter of public debate. This entry will explore several prominent views on how such debate should be conducted within pluralistic democratic societies. It begins by considering John Rawls's account of public reason. It then examines how this account applies to the controversial issues of abortion and physician-assisted suicide, where one can see why some have objected to this view, especially with regard to the way it requires citizens to bracket their comprehensive moral, religious, and philosophical doctrines. Next, this entry will consider some alternative approaches that endorse more expansive forms of public reason that allow greater room for appeals to comprehensive doctrines. While one might expect that many of the controversial issues in bioethics will continue to be a matter of public debate going forward, it will be seen that a necessary step towards making progress in these debates is to get clear on how such debate should be conducted.

Keywords

Public debate; Public reason; Public policy; Bioethics; Religion and politics; Abortion; Physician-assisted suicide; Liberalism; Natural law ethics; John Rawls; John Finnis; Michael Sandel; Charles Taylor

Introduction

In contemporary democratic societies, ethical issues in healthcare and biomedical research are often a matter of public debate – indeed, frequently heated debate – over the direction of public policy. One might think, for instance, about the debates surrounding abortion, euthanasia, physician-assisted suicide, genetic engineering, research on embryos, assisted reproduction, healthcare distribution, public health initiatives, and so on. It is no surprise that these issues are a matter of public debate since they have widespread implications for how human beings live their lives. However, there are important questions that arise about how public debate should be conducted given the plurality of moral and spiritual perspectives within contemporary democratic societies around the world. In other words: how should citizens conduct public debate when the parties to the debate hold competing conceptions of "the good life"?

Rawlsian Public Reason

One of the most influential and widely discussed views on how such debate should be conducted within contemporary democratic societies is put forward by John Rawls (Rawls 1993 [2005]). Rawls starts by affirming what he calls the "fact of reasonable pluralism," which is "the fact that a plurality of conflicting reasonable comprehensive doctrines, religious, philosophical and moral, is the result of [a democracy's] culture of free institutions" (ibid., p. 441). Given this plurality of reasonable comprehensive doctrines with competing conceptions of the good life, the question

arises: how can fair terms of social cooperation be established among citizens who are regarded as free and equal? Rawls's account of public reason is intended to provide an answer to this question. He writes: "A citizen engages in public reason [. . .] when he or she deliberates within a framework of what he or she sincerely regards as the most reasonable political conception of justice, a conception that expresses political values that others, as free and equal citizens might also reasonably be expected to endorse" (ibid., p. 450). Democratic citizens, Rawls maintains, should appeal to public reason whenever they are debating constitutional essentials and matters of basic justice with an aim to enacting public policies.

In seeking to establish his view of "justice as fairness," Rawls's own way of proceeding with respect to public reason is to suggest that citizens should think of themselves in the hypothetical scenario of the "original position," where they adopt what he calls the "veil of ignorance" (ibid., pp. 22–28). In other words, citizens should reason about the fair terms of social cooperation for mutual advantage as if they did not know the details of their particular social position, their natural abilities and proclivities, and their comprehensive doctrine with its particular conception of the good life. The function of the veil of ignorance is to assure that individuals consider their choice of the principles of justice (the fair terms of social cooperation) from a general standpoint that is not biased by the contingencies of their particular social and historical circumstances and to assure that no one has an unfair bargaining advantage. Whether one thinks that this is the best way of proceeding, Rawls believes that public reason will ultimately lead to a liberal political conception of justice, which will have three main features: first, it will identify basic rights, liberties, and opportunities; second, it will assign a priority to these basic rights, liberties, and opportunities over claims pertaining to the general good of society and ideals of the good life; and third, it will ensure all citizens adequate resources for making use of their freedoms in pursuing their particular conceptions of the good life (ibid., p. 450).

The foregoing might seem to suggest that citizens should never appeal to their comprehensive doctrines when debating constitutional essentials and matters of basic justice with an aim to enacting public policies. Although Rawls once held this restrictive view, he came to embrace a somewhat less restrictive view, which allows that citizens may "introduce into political discussion at any time [their] comprehensive doctrine, religious or nonreligious, provided that, in due course, [they] give properly public reasons to support the principles and policies [their] comprehensive doctrine is said to support" (ibid., p. 453; see also p. 247). For instance, appeals to religious reasons were common among the abolitionists and civil rights leaders in the United States, yet Rawls says that this can be in accord with public reason insofar as these religious reasons could ultimately be stated in terms of political values acceptable to all, such as "freedom and equality for all" (ibid., pp. 249–251, 464). The goal is that ultimately there will be an "overlapping consensus" around political values that different comprehensive doctrines can support for their own particular reasons. Rawls thinks that appealing to how one's comprehensive doctrine supports such political values can in fact help to strengthen the commitment to these values among citizens. Nevertheless, it is still the case that the ultimate justification for these political values should not be derived from any particular comprehensive doctrine, but rather it should be derived from the fact that these values are seen as acceptable to citizens with different comprehensive doctrines and can be supported by an overlapping consensus.

But why must citizens, in due course, appeal to public reasons whenever they are debating constitutional essentials and matters of basic justice with an aim to enacting public policies? They should do so because of the "duty of civility," which enjoins citizens to follow the "criterion of reciprocity": that is, when they propose what they think are the fair or most reasonable terms of social cooperation, they must also "think it at least reasonable for others to accept them, as free and equal citizens, and not as dominated or manipulated, or under the pressure of an inferior

political or social position" (ibid., p. 446). This in turn forms the basis of political legitimacy. The issue of political legitimacy arises because of the use of coercive political power in enforcing constitutional essentials and matters of basic justice. So the question is: how can citizens justify to one another the use of coercive political power that will be a part of any proposed political conception of justice? Rawls's answer is that citizens must be able to appeal to reasons (political values) that they can reasonably expect others to accept. He writes: "when, on a constitutional essential or matter of basic justice, all appropriate government officials act from and follow public reason, and when all reasonable citizens think of themselves ideally as if they were legislators following public reason, the legal enactment expressing the opinion of the majority is legitimate law" (ibid., p. 446). It follows that if citizens cannot provide reasons that they can reasonably expect others to accept and that would be supported by an overlapping consensus, then they must avoid using coercive political power and instead allow each person the freedom to live as he or she deems fit, so long as he or she does not infringe on the ability of others to do likewise. Rawls acknowledges that those who think that "fundamental political questions should be decided by what they regard as the best reasons according to their own idea of the whole truth [. . .] and not by reasons that might be shared by all citizens as free and equal, will of course reject the idea of public reason" (ibid., p. 447). However, he says: "Political liberalism views this insistence on the whole truth in politics as incompatible with democratic citizenship and the idea of legitimate law" (ibid.).

Two Cases

In order to explore what is at stake in Rawls's account of public reason, it is helpful to consider its application to two of the most controversial issues in bioethics, which have been and will surely continue to be the site of public debate: namely, abortion and physician-assisted suicide.

Rawls in fact uses the issue of abortion in order to illustrate his account of public reason. He suggests that the issue should be considered in terms of three key political values that one can reasonably expect others to accept: namely, (1) "the due respect for human life," (2) "the ordered reproduction of political society over time, including the family in some form," and (3) "the equality of women as equal citizens" (ibid., p. 243, n. 32) Rawls then contends: "any reasonable balance of these three values will give a woman a duly qualified right to decide whether or not to end her pregnancy during the first trimester" (ibid.). The reason that he provides for this claim is that "at this early stage of pregnancy the political value of the equality of women is overriding, and this right [to an abortion in the first trimester] is required to give it substance and force" (ibid.). Rawls goes on to claim that "any comprehensive doctrine that leads to a balance of political values excluding that duly qualified right in the first trimester is to that extent unreasonable," and indeed "it may also be cruel and oppressive" (ibid., pp. 243–244, n. 32) Moreover, he says, "we would go against the ideal of public reason if we voted from a comprehensive doctrine that denied this right" (ibid., p. 244, n. 32).

Unsurprisingly, Rawls's claims here have been highly controversial and strongly contested, especially by those who think that a "due respect for human life" means respecting the inviolability of innocent human life from conception to death (Finnis 2000, pp. 75–93; George 2001, pp. 56–61). Indeed, Rawls merely asserts precisely what is at issue in saying that "the political value of the equality of women is overriding," which is countered by the pro-life claim that the right to life of human embryos and fetuses is overriding. Nothing has been said here to show that the pro-life advocate is being "unreasonable," and one might also imagine such an advocate likewise claiming that it is the pro-choice position that is "cruel and oppressive" by violating the right to life of innocent human embryos and fetuses, if one grants that such a right to life applies equally to all human beings. So the debate seems to be far from resolved.

In response to criticisms, Rawls later came to say that in the above passage he was not intending to provide an argument for abortion rights, but

merely to illustrate and confirm a statement about how comprehensive doctrines can violate the ideal of public reason when they fail to endorse a reasonable balance of political values. He acknowledges that the passage does express his opinion but says that his stated opinion is not an argument (though the passage does have the appearance of an argument). Showing greater modesty, he writes: "a more detailed interpretation of those [three political] values may, when properly developed in public reason, yield a reasonable argument. I don't say the most reasonable or decisive argument; I don't know what that would be, or even if it exists" (Rawls 1993 [2005], p. 479, n. 80). As an example of such an argument, Rawls cites an article by Judith Jarvis Thomson (Thomson 1995), though he mentions that he would add several (unspecified) addenda to it.

Thomson's argument is based on three main ideas: "First, restrictive regulation severely constrains women's liberty. Second, severe constraints on liberty may not be imposed in the name of considerations that the constrained are not unreasonable in rejecting. And third, the many women who reject the claim that the fetus [in the non-technical sense that includes all prenatal human life] has a right to life from the moment of conception are not unreasonable in doing so" (Thomson 1995). She adds that all of these ideas seem very plausible to her and so it follows that she favors a pro-choice position. It should be noted that this argument is essentially a Rawlsian one: it is concerned with the legitimate use of coercive political power, where this is thought to be legitimate only when citizens can provide reasons for it that they can reasonably expect others to accept. Hence Thomson's argument for abortion rights depends crucially upon the claim that pro-choice women are "not unreasonable" to reject the claim that "the fetus has a right to life from the moment of conception." However, the problem is that pro-life advocates argue that it is in fact unreasonable to reject that claim, especially if one goes in for the "political value" of personhood at all, which includes a right not to be killed (at least if a person is innocent).

For instance, Patrick Lee and Robert P. George argue against dualistic views that maintain that human embryos and fetuses are not persons because they lack higher mental functioning. They contend that such views are based on a false premise: these views implicitly identify "the human person with a consciousness which inhabits (or is somehow associated with) and uses a body; the truth, however, is that we human persons are particular kinds of physical organisms. [. . .] [You] and I came to be at conception; we once were embryos, then fetuses, then infants, just as we were once toddlers, preadolescent children, adolescents, and young adults" (Lee and George 2005, pp. 15–16). There is not a difference in *kind* here, but only a difference in the *degree* to which a human being has realized the natural capacities that are definitive of the kind of being that humans are. So they conclude: "To have destroyed the human organism that you are or I am even at an early stage of our lives would have been to have killed you or me" (ibid., p. 16).

There are some who also reject the dualist view but claim that human beings only become bearers of rights at a later point when they have developed their human capacities to a sufficient degree. However, Lee and George argue that any view that regards personhood as a property that one comes to acquire some time after conception, rather than something one has simply in virtue of being human, suffers from a line drawing problem. First, it is at least several months after birth until human beings perform mental acts that nonhuman animals do not perform and thus "if full moral respect were due only to those who possess a nearly immediately exercisable capacity for characteristically human mental functions, it would follow that 6-week old infants do not deserve full moral respect" (ibid., p. 18). Second, the difference between an immediately exercisable capacity for characteristically human mental functions and the basic natural capacity for developing the ability to perform such functions must be understood in terms of a developmental continuum. Thus, any line one draws for personhood that is not at conception – say, at brain functioning or sentience or viability or birth – is going to be arbitrary: "Between the ovum and the approaching thousands of sperm, on the one hand, and the embryonic human being, on the

other hand, there *is* a clear difference in kind. But between the embryonic human being and that same human being at any later stage of its maturation, there is only a difference of degree" (ibid.). Finally, "if human beings were worthy of full moral respect (as subjects of rights) only because of [certain acquired] qualities, and not in virtue of the kind of being they are, then, since such qualities come in varying degrees, no account could be given of why basic rights are not possessed by human beings in varying degrees. The proposition that all human beings are created equal would be relegated to the status of a superstition" (ibid., p. 19).

In Thomson's more well-known article on abortion, "A Defense of Abortion," she in fact contends that even if it is granted that human embryos and fetuses are persons and thus have a right to life, nevertheless, they do not have a right to use a woman's body to sustain their lives (Thomson 1971). Thomson argues for this conclusion by appealing to a now-famous analogy. She describes a hypothetical scenario in which one becomes kidnapped and then hooked up to a famous unconscious violinist with a potentially fatal kidney ailment in order to provide life support, which is required for 9 months, after which time the violinist will be recovered. Thomson thinks that in this case, one should not be required to stay plugged into the violinist, even though becoming unplugged before the 9 months is over would mean that the violinist would die. Although the violinist is a person and has a right to life, he does not have a right to use another person's body in order to sustain his life. Now, the analogy here might seem to apply only to cases of pregnancy due to rape, but Thomson goes on to argue that it applies to all unwanted pregnancies. In any case, Thomson's violinist argument has been the subject of much debate. For instance, some argue that there is a crucial difference between unplugging the violinist and aborting a human embryo or fetus. In the former case, one is merely allowing the violinist to die of the underlying pathology and does not *intend* to kill him even though one *foresees* that death will result as a "side effect" from the unplugging. By contrast, abortion typically involves intentional killing and in a manner that is a direct act of violence against a human life, which is quite different from mere "unplugging." The right to life, it is argued, involves a right – *qua* innocent human being – not to be intentionally killed, though it allows for causing or allowing a foreseeable death so long as one does not intend the death, the death is a side effect of one's action, and there is strong enough justifying reason for one's action (as in the case of taking a terminally ill patient off of burdensome life support). Apart from the issue of intending versus foreseeing death, it is also argued that parents have a special responsibility to their biological children – even without voluntarily assuming this responsibility (something that Thomson denies) – which they do not have to a stranger. Moreover, it is claimed that the burdens of pregnancy pale in comparison to the harm inflicted on the embryo or fetus in being killed (see Lee and George 2005, pp. 20–24).

It is not possible here to pursue all of the issues that arise in this debate in full detail, but from the foregoing, one can see how public debate might proceed on the issue of abortion. In his revised view, Rawls acknowledges that the abortion debate, even with the use of public reason, is not likely to produce unanimity. Thus, citizens will simply need to vote on the matter, and the outcome of this vote is to be regarded as a legitimate law that is binding on all citizens, provided the vote was done in accord with public reason. For those who continue to disagree with such a law and argue against it, Rawls says: "they need not exercise the right to abortion. They can recognize the right as belonging to legitimate law enacted in accordance with legitimate political institutions and public reason, and therefore not resist it with force. Forceful resistance is unreasonable: it would mean attempting to impose by force their own comprehensive doctrine that a majority of other citizens who follow public reason, not unreasonably, do not accept" (Rawls 1993 [2005], p. 480). But is Rawls's view here in fact reasonable?

John Finnis, for instance, thinks it is not. He writes: "The anti-abortion citizens are claiming, with some good arguments, that abortion is rather like slave-owning: a radical, basic injustice

imposed on people deprived of the protections of citizenship. The response, '*You* free citizens need not exercise the right to [own slaves] [abort your children] in your own case, so you can and must recognize our law as legitimate as it applies to the rest of us,' is mere impudence or thoughtlessness" (Finnis 2000, p. 89). Michael Sandel also questions the reasonableness of requiring citizens to bracket their comprehensive moral, religious, and philosophical doctrines – that is, their view of the "whole truth" – on such a grave moral question in public debate. Whether it is reasonable, he contends, depends on which of those doctrines is true. For instance, if the Catholic/natural law view that personhood begins at conception is true, then "bracketing the moral-theological question [of] when [personhood] begins is far less reasonable than it would be on rival moral and religious assumptions" (Sandel 2005, p. 225). Although Rawls asserts that "political values normally outweigh whatever nonpolitical values conflict with them," Sandel says that if the Catholic/natural law view of the moral status of the embryo and fetus is correct and abortion is tantamount to murder, then "it is not clear why the political values of toleration and women's equality [construed as it is by liberals], important though they are, should prevail" (John Rawls 1993 [2005], p. 146; Sandel 2005, p. 226). Thus, Sandel maintains: "the case for abortion rights cannot be neutral with respect to that moral and religious controversy. It must engage rather than avoid the comprehensive moral and religious doctrines at stake" (Sandel 2005, p. 226). The next section will examine the viability of this more expansive account of public reason and consider whether insistence on the whole truth in politics is really incompatible with democratic citizenship and the idea of legitimate law, as Rawls maintains. But before turning to that issue, it is helpful to examine a second case: physician-assisted suicide.

Rawls and five other prominent liberal political philosophers – Ronald Dworkin, Thomas Nagel, Robert Nozick, Thomas Scanlon, and Judith Jarvis Thomson – filed a now-famous brief ("The Philosophers' Brief") with the US Supreme Court at a time when the Court was considering two cases involving state bans on physician-assisted suicide (the Court in fact upheld these bans and rejected appeals for a constitutional right to physician-assisted suicide). In their brief, the authors argue for a constitutional right to physician-assisted suicide by contending that the cases under consideration "do not invite or require the Court to make [. . .] ethical or religious judgments about how people should approach or confront their death or about when it is ethically appropriate to hasten one's own death or to ask others for help in doing so. On the contrary, they ask the Court to recognize that individuals have a constitutionally protected interest in making those grave judgments for themselves, free from the imposition of any religious or philosophical orthodoxy by court or legislature" (Dworkin et al. 1997). The authors go on to say: "Death is, for each of us, among the most significant events of life. [. . .] Most of us see death [. . .] as the final act of life's drama, and we want that last act to reflect our own convictions, those we have tried to live by, not the convictions of others forced on us in our most vulnerable moments" (ibid.). They conclude that each person has a right to make the "most intimate and personal choices central to personal dignity and autonomy," which includes "the right to exercise some control over the time and manner of one's death" (ibid.). In short, the basic idea here is that the Court should be neutral with respect to competing conceptions of the meaning and value of human life and thus allow individuals the autonomy to make these decisions for themselves according to their conscience. In Rawlsian terms, public reasoning should proceed here by bracketing comprehensive moral, religious, and philosophical doctrines and by appealing to the political value of individual autonomy in light of the fact of reasonable pluralism.

However, some have questioned whether such a view is in fact really neutral with respect to competing conceptions of the meaning and value of human life. For instance, Sandel contends that the brief actually does affirm a view of what makes life worth living: namely, "the best way to live and die is to do so deliberately, autonomously, in a way that enables us to view our lives as our own creations. The best lives are led by those who

see themselves not as participants in a drama larger than themselves but as authors of the drama itself" (Sandel 2005, p. 114). One's life is seen here as one's own possession and Sandel notes that this is "at odds with a wide range of moral outlooks that view life as a gift, of which we are custodians with certain duties. Such outlooks reject the idea that a person's life is open to unlimited use, even by the person whose life it is" (ibid., pp. 114–115). The authors of the brief might respond by saying that people who want to see their life as a gift and as part of a larger drama remain free to do so. But Sandel contends that a constitutional right to physician-assisted suicide "would not simply expand the range of options, but would encourage the tendency to view life less as a gift and more as a possession. It might heighten the prestige we accord autonomous, independent lives and depreciate the claims of those seen to be dependent" (ibid., p. 116). He worries that this could affect attitudes and treatment of the young, the old, the disabled, the infirm, and the poor. It should also be noted that the life as a gift view is often linked to a belief in the "sanctity" (i.e., inviolability) of human life, which requires, among other things, that one never intentionally kills an innocent human being, whether it be others or one's self (see Keown 2002, Chap. 4). Here the demands of the sanctity of human life are seen as overriding all considerations about quality of life and autonomy. For anyone who affirms such a view, it is unlikely that they will see it as reasonable to bracket this view when considering the direction of public policy.

Alternative Approaches

So what alternatives are there to the Rawlsian account of public reason? One is the natural law view of ethics, which is endorsed by several of the aforementioned philosophers, namely, Finnis, George, and Lee. Natural law ethics is a comprehensive doctrine that in fact claims to engage in public reason, that is, it claims to appeal to reasons (or values) that can be accepted by any citizen as free and equal. If this is so, then it can be said to fulfill what Rawls calls the "duty of civility."

Finnis, for instance, appeals to self-evident basic goods that he thinks are constitutive of human well-being and fulfillment and towards which he believes we are naturally directed as things "to be pursued": namely, life, knowledge, play, aesthetic experience, sociability (or friendship), practical reasonableness, and religion (Finnis 1980, pp. 85–90). Each of these goods demands respect but which a person will pursue at a given moment in time and in what manner is something that each person must work out in the course of his or her particular life through practical reason. Nevertheless, there are certain general requirements of practical reasonableness that everyone must follow: for instance, cultivating the virtues and avoiding acts of injustice, such as the intentional killing of an innocent human life.

Those who endorse natural law ethics seem to be among those who Rawls has in mind when he speaks of "rationalist believers," that is, those who believe that they can establish their views through reason, and who, he thinks, must deny the "fact of reasonable pluralism" (Rawls 1993 [2005], pp. 152–153). Rawls believes that they are mistaken to deny this "fact," and this is why he thinks that the fair terms of social cooperation (the principles of justice) should not be based on knowledge claims about an independent moral order – as in the case of Martin Luther King, Jr., who made appeals to the "arc of the moral universe" in articulating his view of justice – but rather they should be based on a mutually beneficial agreement that is arrived at from behind the "veil of ignorance," as articulated by his account of justice as fairness (see ibid., pp. 22–23, 97). It is mistaken to deny the "fact of reasonable pluralism," Rawls thinks, because it is "unrealistic—or worse, it arouses mutual suspicion and hostility—to suppose that all our differences are rooted solely in ignorance and perversity, or else in the rivalries for power, status, or economic gain" (ibid., p. 58).

In response, Finnis says that one must distinguish between kinds of differences in viewpoints. There are a number of reasonable differences, he says, that "arise from differences of sentiments, of

prior commitment, and of belief about likely future outcomes. In such cases there is no uniquely correct opinion, though there are many incorrect opinions" (Finnis 2000, p. 83). However, he contends: "in relation to some matters, including at least some matters of basic rights, there are correct moral beliefs, accessible to all. In relation to such matters, differing opinions can only be rooted in ignorance or some subrational influence, and it is impossible to say that there is more than one 'fully reasonable' or 'perfectly reasonable' belief" (ibid., pp. 83–84). Abortion and physician-assisted suicide are examples of cases where Finnis believes that some beliefs are more reasonable than others, indeed, where some views are simply mistaken and others are correct. Public reasoning, he contends, "should be directed to overcoming the relevant mistakes, and public deliberations should be directed to avoiding them in practice" (ibid., p. 84; see also pp. 86–93; George 2001, pp. 53–55). The problem with Rawls's critique of the rationalist believer, for Finnis, is that he seeks to declare that the rationalist believer is mistaken without examining the merits of his or her arguments, and this is due to Rawls's understanding of political legitimacy, which does not allow for ultimate justification of constitutional essentials and matters of basic justice to be based on any specific comprehensive moral, religious, or philosophical doctrine (Finnis 2000, pp. 79–80; see also George 2001, pp. 52–53). Because Rawls preemptively censors or short-circuits precisely the kind of reasoned public debates that citizens need to be having on matters of great importance, Finnis says that his view of political legitimacy is itself "illegitimate, unreasonable, and uncivil" (Finnis 2000, p. 81).

Even if one is not as confident as Finnis about the powers of reason to definitively show which moral beliefs are correct and which are incorrect, there may still be good reason to allow greater room for appeals to comprehensive moral, religious, and philosophical doctrines in public debate and to believe that this can be compatible with legitimate law and with democratic citizenship involving mutual respect between free and equal citizens (thus fulfilling the "duty of civility"). This is especially desirable if one agrees with Sandel's view that on certain grave moral questions it is not reasonable to bracket one's comprehensive moral, religious, or philosophical doctrine and that there is no truly neutral position on these questions. Hence, he says that we should "engage rather than avoid the comprehensive moral and religious doctrines at stake" (Sandel 2005, p. 226). According to Sandel, it is important to distinguish two conceptions of mutual respect between free and equal democratic citizens. One is the "liberal conception" according to which "we respect our fellow citizens' moral and religious convictions by ignoring them (for political purposes), by leaving them undisturbed, or by carrying on political debate without reference to them. To admit moral and religious ideals into political debate about justice would undermine mutual respect in this sense" (ibid., p. 246). However, there is also what he calls a "deliberative conception" of respect according to which "we respect our fellow [citizens'] moral and religious convictions by engaging, or attending to them—sometimes by challenging and contesting them, sometimes by listening and learning from them—especially if those convictions bear on important political questions" (ibid., pp. 246–247). Sandel believes that this deliberative conception of respect is the more suitable ideal for pluralistic societies and it is better able to cultivate, rather than stifle, the "moral energies of a vital democratic life" (ibid., p. 246; see also Weithman 2002).

Such deliberative engagement with one's fellow citizens' comprehensive doctrines could proceed along the lines of what Charles Taylor describes as the "ad hominem" model of practical reasoning, which, as the name suggests, appeals "to the person," especially to his or her deepest moral (and perhaps spiritual) sense of things. With respect to trying to convince others of one's perspective, Taylor writes: "I can only convince you by my description of the good if I speak for you, either by articulating what underlies your existing moral intuitions or perhaps by my description moving you to the point of making it your own" (Taylor 1989, p. 77). Whether it is intrapersonal or interpersonal, practical reasoning on Taylor's

account is "a reasoning in transitions. It aims to establish, not that some position is correct absolutely, but rather that some position is superior to some other. It is concerned [. . .] with comparative propositions" (ibid., p. 72). The most common form of this is where such a transition is brought about by "error-reducing moves," for instance, "by the identification of contradiction, the dissipation of confusion, or by rescuing from (usually motivated) neglect a consideration whose significance [one] cannot contest" (Taylor 1995, p. 53). The ultimate court of appeal here is what Taylor calls the "best account principle": "What better measure of reality do we have in human affairs than those terms which on critical reflection and after correction of the errors we can detect make the best sense of our lives? 'Making the best sense' here includes not only offering the best, most realistic orientation about the good but also allowing us best to understand and make sense of the actions and feelings of ourselves and others" (Taylor 1989, p. 57).

For instance, citizens might engage in ad hominem practical reasoning about how to best conceive of human dignity (or the sanctity of human life) and what follows from this conception (as is relevant, say, to the abortion and physician-assisted suicide debates). They may ask: what ontological account of the world and the place of human beings within it – for instance, theistic or naturalistic – can best make sense of and inform the experience of human dignity and what it morally requires? Some might then come to realize that they do not have an adequate "moral ontology" to support their "moral phenomenology" and so they must either abandon the moral phenomenology (the sense of human dignity) or seek out a more adequate ontological basis. Or some might find that articulating what their sense of human dignity consists in – for instance, being made in the image of God, or being supreme evolutionary achievements, or whatever else – empowers them (gives them "moral energies") to show greater respect and concern for human beings and also enables them to see more clearly what such human dignity morally requires (see Taylor 1989, pp. 3–11, 91–97, 515–521; Taylor 2003, pp. 316–320). But even if citizens disagree on the deeper reasons that inform their conceptions of human dignity, for political purposes they might still try to form an overlapping consensus around some such conception. Provided they go in for the concept, citizens could then try to establish – via ad hominem practical reasoning – a consensus around what seems to be the most coherent view (see Taylor 2011, Chaps. 6, 13–14).

Conclusion

This entry has considered a dominant model of how to conduct public debate: namely, John Rawls's account of public reason. It then examined how this account applies to the controversial issues of abortion and physician-assisted suicide, where one can see why some have objected to this view, especially with regard to the way it requires citizens to bracket their comprehensive moral, religious, and philosophical doctrines. Next, this entry considered some alternative approaches that endorse more expansive forms of public reason that allow greater room for appeals to comprehensive doctrines. While one might expect that many of the controversial issues in bioethics will continue to be a matter of public debate going forward, this entry has shown that a necessary step towards making progress in these debates is to get clear on how such debate should be conducted within pluralistic democratic societies.

Cross-References

- ▶ Advocacy
- ▶ Assisted Suicide
- ▶ Autonomy
- ▶ Benefit and Harm
- ▶ Bioethics: And Politics
- ▶ Bioethics: Clinical
- ▶ Bioethics: Medical
- ▶ Biolaw
- ▶ Biopolitics
- ▶ Citizenship
- ▶ Coercion
- ▶ Common Good

- Communitarian Bioethics
- Consensus
- Consent: Informed
- Death: Good Death
- Death: Meaning of
- Deliberation
- Discourse Ethics
- Double Effect
- Embryo
- Equality and Equity
- Euthanasia: Active
- Euthanasia: Passive
- Fetus
- Freedom of Press
- Freedom of Speech
- Health Policy
- Human Dignity
- Human Nature
- Human Rights
- Justice: Theories of
- Law and Bioethics
- Law and Morality
- Life: Meaning of
- Moral Status
- Person
- Personalism
- Quality of Life
- Religion and Global Bioethics
- Respect for Autonomy
- Right to Die
- Spirituality
- Suffering
- Suicide
- Tolerance

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Public Health

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Abstract

Public health ethics deals with moral aspects of development, implementation, teaching, and

research of public health. Public health practitioners have always been aware of the ethical dimension of their field, but public health ethics as a subdiscipline of bioethics has emerged only at the end of the last century, and it is still a young and unsettled field. Public health ethics studies various theories of ethics, which guide or could guide public health actions. Practical ethical decision making can be facilitated by applying one or more public health ethics frameworks. Public health encompasses a wide spectrum of activities and each of those is raising ethical issues. Challenges that particularly invite bioethics assessment are control of infectious diseases such as HIV/AIDS, SARS, influenza pandemic, and Ebola fever. Vaccination, isolation, and quarantine are examples of public health interventions that require balancing of health benefits to community with the loss of freedom for some individuals. Applying relevant moral considerations is also helpful for health-care promotions and lifestyle interventions, screening and testing, public health research, and efforts to address health equity. Public health ethics addresses also international collaboration on global health concerns.

Keywords

Public health; Community health; Vaccination; Health promotion; Prevention; Social determinants of health; Health disparities; Health equity; Influenza; Pandemic; AIDS/HIV; Ebola fever; SARS; Quarantine; Isolation; Social distancing measures; Screening and testing; Ethical theories; Justice; Autonomy; Common good; Globalization; WHO; International collaboration; Public health research

Introduction

Understanding the current state of public health ethics requires an introduction to the history of the discipline. Subsequently, the basic definition and distinctions of this discipline from related fields will be offered. Ethical theories can provide grounding and organize our thinking about ethical aspects of public health, and some theories will be briefly listed. The reader will be then introduced to frameworks for ethical decision making which provide a bridge between theories and practice. The main body of this entry will summarize ethical issues in several main programs and concerns of public health. These topics were selected either because ethical reflections have significantly shaped them or because these events or programs shaped public health ethics. Singled out for attention were infectious diseases, health equity, health promotion, screening and testing, and research with human subjects. Some cardinal points will be gathered in the concluding paragraph.

History and Development

When the contemporary bioethics emerged in the 1970s from the work of philosophers, theologians, physicians, and legal scholars, the agenda for the new discipline was driven by burning contemporary issues, prominent among them being experimentation on human subjects, use of new medical technology, and medical paternalism. Because these issues were present in clinical research and in acute care, the early bioethics was almost entirely focused on these sectors. Given those circumstances, it is not surprising that books, reports, and articles produced by founding fathers of bioethics in the 1970s and 1980s did not address public health as an academic discipline or practical activity. Scholars working in public health however became aware of the discipline of bioethics, and, for example, already in 1978, John Last in his book *Public Health and Human Ecology* provided discussion of ethical issues grounded in bioethical principles. In the 1990s, the importance of public health became once more apparent because of the spread of HIV/AIDS, concerns about novel emerging infections, and the threat of bioterrorism. It became clear that distinct ethical concerns and questions arising from the existence of infectious diseases and our attempt to control them need to be recognized and addressed (Selgelid et al. 2005, p. 3). Early in the new century, articles and books emerged that endeavored to offer ethics frameworks or

processes for ethical assessments that would designed to facilitate effective public health interventions able to reduce current mortality and morbidity while at the same time maintain individual freedoms and advance social justice. The critical period for the growth of public health ethics was the years 2002–2003 when a first new infectious disease in the twenty-first century emerged, severe acute respiratory syndrome (SARS), and this led to many ethical studies and reflections.

After SARS was brought under control, most states of the world started to prepare for the next influenza pandemic. Pandemic plans frequently referred to ethical concerns or had a section on ethics. This is for the first time when public health ethics provided significant input to major public health programs before the programs were launched. When the influenza pandemic did arrive in 2009, it was less severe than plans anticipated, and this also created certain ethical challenges.

At the present time, it is the Ebola fever epidemic and the response to it which represent the most visible face of global public health. Increasingly, a driving force of public health ethics has been concerns about health of underprivileged and vulnerable populations and disparities in terms of social determinants of health resulting in differences in health status.

Conceptual Clarifications and Definitions

Public health is a branch of health care using methods of prevention, promotion, protection, and control, concerned with physical and mental health of a population on the international, national, state, or municipal level. Public health ethics then is the subdiscipline of bioethics (or health-care ethics) that endeavors to clarify, prioritize, and justify possible public health interventions based on moral values, ethical principles and interests of stakeholders, scientific evidence, and other relevant information. Public health ethics could also be understood as a particular "lense" that allows a closer examination of public health issues and activities and assists quality improvement in this field.

Public health is distinct from personal health care, because the former is mostly using group- or community- oriented strategies and is typically delivered by dedicated organizations under governmental control. Due to this and other distinctions between personal health care and public health, each sector of health care requires some specific ethical considerations. However, boundaries between these sectors are often indistinct. For example, regular home visits to frail elderly could be considered a clinical health-care activity or a public health initiative. Public health workers and those in clinical care have to collaborate in improving people's health. As a result, public health ethics is closely related to and partially overlaps with clinical ethics.

Public health ethics is closely related to health policy ethics because public health interventions are dependent on allocation of public resources and policies created by governments. Public health ethics is distinct from health law, yet many public health actions are governed by or restricted by laws of the land. The general legal principle formulated in ancient Rome, *salus populi est suprema lex* (the welfare of the people is the supreme law), still has a persuasive power. Epidemiology and biostatistics provide important methodologies for public health, and each has its own range of ethical issues.

Ethical Dimensions

Theoretical Considerations

At the present time, there is no single ethical theory of public health ethics that would be universally accepted, and consequently there is no universal standard that could be used to measure the moral "goodness" or "badness" of actions in public health. However, there are a number of ethical theories that could guide, illuminate, or justify public health activities.

The utilitarian theories which stress consequences of actions are of importance because the practice of public health is often driven by the principle of utility, which suggests that we ought to produce the maximal balance of positive value, such as health, over negative value, such as

disease. In contrast to utilitarianism, the Kantian ethical theories have little regard for consequences of actions and instead propose that one's actions should be guided by rationally determined duty and by regards for autonomy. This approach to morality has had a strong influence on the codes of ethics of health-care professionals, including those in public health.

Many other suggestions were made for the most suitable theoretical basis of the discipline of public health, such as multidimensional social justice, civic republicanism, liberal paternalism, and relational autonomy. The personalist approach is based on our shared human nature and our common requirements for human flourishing. On this account, the primary principle is respect for persons, which includes respect for autonomy and confidentiality but also values of equity and solidarity (Petrini and Gainotti 2008, Bulletin of WHO). Virtue ethics, with its doctrine of means, can also guide us in moral assessment of public health actions.

Childress et al. (2002) proposed that all public health decisions have to take into account the following general moral considerations: producing benefits; avoiding, preventing, and removing harms; distributing benefits and burdens fairly; and respecting autonomy. To this list, the authors added four other considerations: protecting privacy and confidentiality, keeping promises and commitments, disclosing information honestly, and building and maintaining trust. In addition, these authors proposed five "justificatory conditions": effectiveness, proportionality, necessity, least infringement of general moral considerations, and public justification. Asian scholars pointed out a strong moral tradition from Buddhism to Confucianism associated with the term compassion. European scholars suggested such alternative values as solidarity, subsidiarity, and precautionary principle.

Practical Considerations and Frameworks

Different ethical theories provide sometimes complementary and sometimes contrasting interpretations or directions but are helpful for in-depth moral assessment of public health actions, preferably applying several theories to the issue at hand and comparing the guidance provided by them. Public health practitioners, however, are in need of simpler tools for practical ethical decision making on a day-to-day basis. These tools, known as "frameworks," typically incorporate principles, values, and ideas emerging from ethical theories, combine them with process-oriented approaches, and guide the user through a variety of deliberations that at the end ought to provide an indication of ethical desirability of a public health intervention under consideration. A number of frameworks have been published, and some were adopted by organizations, such as the European Union, the Nuffield Council on Bioethics in the UK, and the American Public Health Association. Ten Have et al. (2010) reviewed and compared six of such frameworks and found that none of them took into account all ethical issues that emerge in one particular program which they evaluated, a program for obesity prevention. Nevertheless, these frameworks can be very useful, if users are provided with basic education in bioethics and training in framework utilization, so that they do not expect that frameworks do automatically provide ethically correct answers with minimal effort.

Infectious Diseases

Measures to Control Infections: Vaccination, Quarantine, and Isolation

Vaccination is considered one of the most effective public health interventions and has led to eradication of smallpox, near eradication of poliomyelitis, and great reduction of morbidity and mortality from diseases such as measles, rubella, and pertussis. Ethical issues in the vaccination in developed and in developing world are different, especially as they concern immunization of children. In wealthier countries, such vaccination is widely available against about 6–12 diseases, usually paid from public funds, and is sometimes mandatory. Serious illnesses or death from those preventable diseases became rare, and this created a perception that vaccination is no longer important. In the same time, real and imagined complications from vaccination were getting more attention, public confidence waned, and vaccination rates diminished. When most people

(80–95 %) accept a particular vaccination, even those who were not vaccinated are protected because the transmission of the infectious agent was successfully interrupted. This situation is called *herd immunity* (Dawson 2007). It is an advantage for those who cannot be vaccinated for medical reason or have grave religious objections, for example, if the vaccine would be derived from human embryos. However, if someone decides not to get vaccinated in order to avoid a discomfort of the procedure or the small risks which are shared with all others, this person can be called a freeloader – he or she wants to enjoy the benefits without paying the price. Such behavior is morally problematic because it offends the principle of fairness. Low vaccination rate can be due to a low priority people assign to vaccination in their lives. There is also a forceful opposition to vaccination in some circles, which in part arises from inadequate communication of public health authorities, but there is also a legitimate concern about conflicts of interests because some promoters of public health interventions have financial ties to drug companies that profit from vaccinations recommended by those experts. Public health ethics can understand, respect, and offer to the society critical moral assessment of both perspectives while stressing the importance of solidarity and duty of fair play (Holland 2007, pp. 135–159).

The ethical concerns about vaccination of children in developing countries are different, because there are millions of children still dying from preventable diseases. At least one out of four children does not receive vaccination because of cost, shortage of staff, and lack of infrastructure, refrigeration, and transportation or due to war conflicts. In addition, even if vaccinated, children in poorer countries may not receive the same benefit because of their malnutrition, parasites, or other health factors. Finally, because vaccine research is driven by needs of richer countries, some vaccines that would be useful in developing countries do not get developed. Considerable effort to correct this inequity had been made by governmental and private agencies, but to assure full and sustained access to vaccination everywhere remains a major challenge.

Quarantine refers to enforced separation and restriction of movements of individuals suspected to be exposed to an infection for the duration of an estimated incubation period in order to prevent them to spread the infectious agent to others. It is one of the traditional public health measures documented as far back as the fourteenth century when during the Black Death epidemic in Europe, ships were required to sit in harbors for 40 days, before passengers and crew could disembark. While most countries still have quarantine laws, in modern times it has been extensively used for the first time in 2003 to control SARS (severe acute respiratory syndrome). The decision to use the measure, in retrospect judged to be correct, was justified because the new disease was caused by an unknown agent, with an unknown mode of transmission, with no effective treatment and significant mortality. Placing a person in quarantine is a serious step, because it is done not for the benefit of that person but for the benefit of others. At stake are important human rights such as freedom and opportunity to earn a living and to discharge family responsibilities. In each case the critical question to be asked is this: Is the measure going to diminish deaths, suffering, and societal and economical disruption to such a degree that the burden of the measure is justifiable? If quarantine is justified in the opinion of the population, it may not have to be always legally enforced. During the SARS outbreak in Toronto, Canada, about 30,000 people were quarantined and most accepted it voluntarily; in only 22 cases the mandatory detainment was necessary. Authorities in Hong Kong and Japan provided those sent to quarantine with electronic bracelets, to be able to monitor people's whereabouts. It was determined that the ethical obligation of public health service, acting on the reciprocity principle, will be to assure that those in quarantine are not abandoned but have adequate food and shelter, medications, and social support and receive a compensation for the lost income.

The quarantine should not be confused with isolation, which means a separation of ill persons with an infectious disease from other people. This measure is commonly used in hospital practice and is ethically less problematic, because these patients will also benefit by receiving care.

Other public health measures that aim for distancing of people to minimize transmissions are travel restrictions, school closures and cancelation of public gatherings, national border control measures, and travel advisories. Each of those measures also should be ethically justified and carried out in a responsible fashion to minimize the adverse impact on affected population.

HIV/AIDS Pandemic

This greatest epidemic in modern history represents an important ongoing public health issue, being the fourth leading cause of death in the world. At the end of 2013, 35 million people were living with AIDS, and 1.3 million people died that year from the disease. Discrimination against infected individuals in school, employment, immigration, and housing continues to be a concern, but current ethical issues are related mainly to testing, prevention, and treatment.

Testing Since AIDS emerged, most screening and testing has been voluntary, opportunistic, and often anonymous. There are exceptions, such as mandatory premarital testing in Democratic Republic of the Congo, Nigeria, and Botswana. After it was determined that early treatment of infected persons with retroviral drugs can lead to a better clinical outcome as well as effectively prevent transmission of infection, a routine screening and testing of the general population in health-care setting has been proposed. This generated some ethical controversy (Celeda et al. 2011), but a number of authorities endorsed the concept of universal testing and treatment. Whether this means that "test-and-treat" approach will be adopted everywhere as a standard of practice remains to be seen, but modeling suggests that this is the way how prevalence can be reduced from the current 35 million to 1 million in 2050. Ethical challenges are to build global solidarity and advocate for flow of funds, to overcome social and cultural barriers, and to assure that all participants are treated with dignity and respect.

Prevention The insidious onset of this pandemic mitigated against the use of traditional infection control methods such as contact tracing, obligatory testing, and isolation. Also, many people affected were already socially stigmatized, and there was a concern about further compromising their dignity and human rights. An approach to the infection later called AIDS "exceptionalism" was taken in most countries of the world. Prevention focused on education and lifestyle changes, such as the encouragement to use condoms, avoidance of risky sexual practices, and use of unclean needles. These measures were not sufficient to stop the epidemic. The number of new cases per year is now falling, but still in 2011, additional 2.5 million people acquired HIV.

Some religious organizations promoted fidelity in marriage and avoidance of sex outside of marriage, but this call had a limited appeal. Nevertheless, it was reported that the focus of prevention on promotion of condoms in Botswana had proved to be counterproductive, while the focus on behavioral change in Uganda which promoted abstinence and mutual faithfulness to a partner of known status reduced the relative burden of AIDS. A radically different approach to prevention was taken by Cuba, which in 1987 implemented a strict quarantine for all HIV-infected individuals and established mandatory testing program. Prevalence of HIV infection as reported by the government is very low in comparison with the other Caribbean countries, but this was not verified by independent studies. Syringe exchange programs for drug users were found to be an effective risk reduction measure in 1995 and are used in some jurisdictions but continue to be controversial, because they seem to condone drug addiction and undermine the illegal drug control strategy. Given that no effective vaccine protecting against HIV was yet developed, the most effective preventive measure currently is treatment of all HIV-infected people with antiviral drugs, as noted above (see section "Testing").

Treatment In 1984, a retrovirus, labeled human immunodeficiency virus (HIV), was discovered as a cause of AIDS, but it was only by 2005, when retroviral drug treatment started to decrease the death rate and AIDS was transformed from a death sentence to a manageable chronic disease. The dominant ethical issue has been the access to

therapy. Given that sub-Saharan Africa experienced the greatest burden of disease, with up to 20 % of people infected in some states, and that the economy of these countries could not afford the expensive HIV treatment for millions, international assistance was crucial. It could be said that it was slow coming, but, eventually, it became a lifesaver of the continent, with many states, nongovernmental organizations, and private foundations contributing. The Joint United Nations Programme on HIV/AIDS in 2000 established eight strategic objectives to halt and reverse HIV/AIDS. Results so far have been mixed, with some areas achieving a major reduction in infection rates, while other regions experienced escalation. The political and fiscal will to provide antiretroviral therapy is an issue, and currently only about half of the infected people around the world, including children, are receiving the treatment. In addition, there is a strong ethical element in decision making on how much resources should be devoted to treatment of people suffering from AIDS, of those infected with HIV but asymptomatic, and to pre- and postexposure prophylaxis.

SARS

In 2002, an outbreak of severe atypical pneumonia emerged in China and Hong Kong that was named severe acute respiratory syndrome (SARS). This infection eventually spread to 37 countries with over 8,000 reported cases and led to the death of about 800 people. The cause of SARS was at first unknown, the fatality rate was high, and all therapy was only supportive or experimental. Under these circumstances, public health authorities in all affected countries, with coordination by WHO, rapidly adopted traditional public health measures – surveillance and screening for febrile illnesses, isolation of patients with SARS, contact tracing and quarantine of contacts, closure of schools and affected hospitals, and restrictions on traveling. The combined effect of these measures was that the transmission of infection, which was later determined to be due to a coronavirus which jumped species from animals to man, was stopped and at least temporarily driven out of the human population.

The attention SARS received may have been out of proportion to its impact. It was noted that SARS outbreak in Toronto, Ontario, led to half of deaths that were caused without much public attention in Montreal, Quebec, at about the same time by an outbreak caused by *Clostridium difficile*.

Nevertheless, the international impact of this near pandemic demonstrated the need for intensive regional and global surveillance and cooperation. It was a call to put aside the notion that states are entirely sovereign in the governance of public health matters within their borders. The SARS outbreak also led to the understanding that quarantine, isolation, and various social distancing measures are not outmoded. While representing restrictions on personal freedom, these measures may be ethically justified by scientific data or where these are absent also by the precautionary principle. These measures should be applied in the least restrictive or intrusive manner, for example, using voluntary rather than coercive measures where possible, and carried out in a way that will minimize the negative impact on individuals and communities. Actions of public health authorities should be transparent and administered with substantive and procedural justice. This outbreak also brought to light a number of other ethical issues, such as the duty of health-care workers to provide care in an environment that is dangerous for them and creates a risk for their families (Hsin and Macer 2004), economic impact on communities and individuals by distancing measures, and collateral damage to patients with other illnesses because their care is compromised.

Influenza

Influenza virus produces illnesses around the world, with outbreaks resulting in deaths of up to 0.5 million people annually. The leading ethical issue in responding to this ongoing threat is the use of annual vaccination. There is some evidence that vaccinating health-care staff reduces the risk of them being vectors of transmission and improves outcome for their patients. Yet, voluntary uptake of such vaccination in most jurisdictions has been as low as 25 % of eligible staff. In

response, some health organizations or jurisdictions created contractual or legal obligations to accept annual vaccination. At times, this measure has been resisted by health-care workers, and the ethics of balancing human rights of workers against their professional obligations to patient safety continues to be debated.

Influenza virus is also known to produce global disease outbreaks, called pandemics, three or four times every century when the radical change in the genetic makeup of the virus makes everyone susceptible. The most devastating pandemic in the twentieth century emerged in 1918 when the highly virulent virus killed over 50 million people worldwide and public health services were overwhelmed. In 1967, the US public health service overreacted to an emergence of a new strain by starting mass vaccination which turned up to be entirely unnecessary – the so-called swine flu fiasco. In early years of this century, it becomes obvious that the world needs to prepare for the next influenza pandemic, given that the usual cycle of the disease was already overdue. Pandemic planning which in some countries started in the 1970s was intensified and was informed by the recent experience with SARS and carried out by public health agencies which often were stronger and had more resources than ever before. Pandemic preparedness in Africa and some other developing countries was a profound challenge, given often rudimentary health-care systems and the need to deal with other infections.

These national pandemic plans were diverse but commonly identified roles of various levels of governments, nongovernmental organizations, and business sector, often representing the most extensive and far-reaching public health programs ever developed in history of those countries. Some national plans had a section referring to ethical concerns. By 2006, the World Health Organization (WHO) has published guidelines on pandemic influenza preparedness and response planning and released documents outlining also relevant ethical considerations. The WHO reports put a stress on transparency, communication, education, public engagement, and social mobilization including the recommendation to obtain an agreement of the public and civil society on the use of the therapeutic and prophylactic measures and their distributions.

A number of documents in ethical issues in pandemic emerged, for example, from the Joint Centre for Bioethics at the University of Toronto (Thompson et al. 2006) and the Nuffield Council on Bioethics in the UK. The former recommended the following core values to guide a response to the pandemic: individual liberty, protection of public from harm, proportionality, privacy, equity, duty to provide care, reciprocity, trust, solidarity, and stewardship. The report also highlighted that decision-making process in planning and response should be transparent, reasonable, inclusive, responsive, and accountable. The most often debated ethical issue during planning was an anticipated shortage of vaccine and antiviral drugs and the need for their rationing. Many authorities created tentative priority lists, often listing first health-care workers, followed by people at high risk of complications and death, followed by people operating essential civic services. Other frequently debated concerns were the duty of health professionals to serve and to accept vaccination (Kotalik 2006), and the responsibility of society to provide for their effective protection.

When in 2009 the expected pandemic did materialize, events did not go according to plans. The novel virus was less lethal and less contagious than pandemic plans, based on near worst-case scenario, expected. In most places around the world, the situation was not worse than during usual annual flu outbreaks. Stockpiles of antiviral drugs were not fully used. When the vaccine for the novel virus was manufactured, there was an initial wave of interest, but soon people sensed that the flu is not a special risk, and concerns emerged about the safety and efficacy of vaccine. These concerns were not effectively addressed by authorities, and the uptake diminished in spite of official promotion. For example, vaccination coverage achieved in Australia was estimated to be 19 %, which was considered insufficient to interrupt community transmission. Reports from Canada, where over 40 % of population received pandemic vaccination, indicate that the measure prevented infections, hospitalizations, and deaths. Many elements of pandemic plans, e.g., rigorous

triage of patients and opening of temporary care facilities, were not activated.

Retrospective discussions of political, operational, and ethical aspect of the pandemic response continue. Commentators decreed the lack of regular, realistic, and credible reports of authorities on pandemic situation to public and professionals. According to an updated Cochrane Collaboration evidence review of 2014, there is no good evidence that oseltamivir, considered in planning to be the most useful antiviral drug, actually reduces admissions to hospital or complications of influenza, yet it produces significant side effects. This bring to question one chief element of pandemic response strategy and the expense of billions of dollars ($750 million just in the UK) to acquire the drug stockpiles. The problem seems to be that public health authorities made their decisions on incomplete evidence. Only 66 % of information from clinical trials supported by a major manufacturer were published, with the tendency to withhold negative findings. On the other hand, the review of side effects of the vaccine used in 2009 confirmed it to be as safe as authorities claimed. Based on monitoring of 23 million people vaccinated in the USA, only 1.6 excess cases of Guillain-Barre syndrome were reported per million of people vaccinated. Clearly, ethical elements of future plans will need considerable refinement and flexibility to be useful under conditions of uncertainty of timing, severity, and challenges of future pandemic.

Ebola Fever

Ebola fever was first time identified in Western Africa in 1997, and since that time, there have been 24 outbreaks of this viral disease, with mortality rates of about 60 %. It appears that because the outbreaks were short and limited to Africa, the rest of the world was not very concerned. Even WHO was accused of being slow to react, and journalists noted that the assistance to affected countries only materialized when a US citizen was infected. However, the outbreak of 2013 continued unabated, and by January 2015 there have been 22,000 cases and almost 9,000 deaths, more than 500 among health-care workers. By the mid-2015, the rate of new cases appears to be finally diminishing, likely due to commitment of more national and international resources. It has been suggested that the high mortality is not just due to virus virulence but also due to lack of trained personnel and adequate equipment in the countries hardest hit, because none of a handful of Ebola patients who were diagnosed in early stage in the USA and Western Europe have died. This points toward unacceptability of severe disparities in access to good-quality care which threatens not only developing countries but the world community. The people who are most at risks of death are health-care workers in Ebola units. In Sierra Leone, these workers have 100 times higher incidence of infection than the general population of the state. Yet, even in Africa, effective response is possible. Nigeria which experienced 20 cases in early 2014 was able to contain the spread by the multidimensional approach which included extensive contact tracing identified through 18,000 face-to-face visits and has been declared Ebola free 6 months later. Ethical concerns also addressed the acceptability of departing from accepted standards in testing new drugs and vaccines, given the urgency of the situation. Currently, there are two vaccines in clinical trials that appear to be promising.

This outbreak is testing the commitment to International Health Regulations and the global cooperation in infection control. Contrary to those regulations, some 30 countries, including Canada and Australia, imposed travel restrictions on affected countries in the absence of WHO recommendation. Other countries missed target dates for enhancements of their public health infrastructures. However, the recently launched new initiative Global Health Security Agenda promises to increase the societal commitment on a global scale. Direct assistance to Ebola-affected countries by states that have resources and technologies is essential to stop the outbreak, but there is also a moral obligation for assisting countries to leave behind stronger systems to identify, stop, and prevent future threats (Frieden et al. 2014).

Health Equity

National and Community Dimension

Health equity is concerned with providing equal opportunities for good health for all people and

reducing avoidable and unfair differences in health among population groups. Historically, public health practitioners have always been aware of the significance of fairness and justice, but now this moral concern has moved to the forefront (Sen 2004). Science provides an unequivocal evidence that human health is influenced to a major degree by so-called social determinants of health, such as poverty, wealth, quality of air, water, food, and housing. Some but not all of those determinants are in the public health domain. When examining a publicly funded program, it is necessary to consider, if the program will diminish or enhance the existing disparities in health or if it will be neutral in this respect. For example, improving drinking water quality will offer some benefits to everyone. A clinic providing vaccination for foreign countries will benefit to those who can afford to travel and may increase health disparity. A program focusing on the needs of homeless populations will possibly diminish health disparity. Many public health workers will maintain that their programs should at any opportunity strive to diminish health disparities, that is, to focus on underprivileged, and vulnerable, or at least not to exacerbate the existing inequalities of health status and health opportunities. Such view may not be shared by an elected board representing citizens, who may strive for efficacy and a general appeal of programs.

International Dimension

Disparities in health status within states are even more magnified between states. On the moral level, there is a question of obligation to pursue a vision of global health equity. There have been many initiatives in this direction, such as the creation of the Global Fund to Fight AIDS, Tuberculosis and Malaria in 2001 and US President's Emergency Plan for AIDS Relief in 2004 which provided billions of dollars. But national interests rather than "health for all" still dominate political and economic agendas. On the pragmatic level, globalization with its increasing integration of markets and technological advances made possible and necessary to share demographic, epidemiological, and environmental data between states and regions and take joint actions to address health threats. But some critics contend that trade agreements by enforcing deregulation are negatively affecting the reach of public health services, control of hazardous substance, oversight of worker safety, quality of drinking water, and adequacy of sanitation. Nevertheless, the first decade of the twenty-first century has been termed the "golden age of global health" because of its rapid development policies and practices (Basilico et al. 2013).

Promotion and Lifestyle Interventions

Promotion of healthy living, as understood at a particular time in a particular society, has been always part of the public health mandate. The promotion may be carried out with a variety of instruments and techniques; some of those, like providing objective, impartial information, and education based on the current knowledge, are not ethically problematic; others may raise concerns. Important criteria of ethical appropriateness will be the truthfulness of the message, the force with which it is disseminated and perceived, and actual reduction of personal freedoms. If a campaign will exaggerate the actual risk of an activity or a lifestyle and engender to create fear, it merits a disapproval even if it is driven by genuine interest in health of the public. Various means used to gradually reduce smoking of tobacco, which were instituted by now in most countries of the world, provide a good example on how education combined with regulations can be employed to change the behavior, with broad support of citizens. It is to be noted, however, that the chief argument in this prevention strategy was to protect people from the effect of secondhand smoke, rather than protect smokers from self-inflicted harm. The prevention of harm to others is a strong ethical justification for an action. On the other hand, prevention of harm to the person, who knowingly and intentionally accepted the risk of a certain activity, is ethically questionable. However, with addictive activities, like smoking and excessive alcohol use, an argument can be made that addicts no longer have a freedom to decide for

themselves and therefore society is justified to interfere. But public health decisions in this area are difficult and have to be done with sensitivity to various situations. Even when tobacco smoking policies gained a wide support, there are problematic areas, for example, whether or not to permit or to facilitate smoking of patients in health-care institutions, especially smokers with limited life expectancy for whom smoking may be one of the last life's pleasures. The role of bioethics evaluation often will be to draw attention to the segment of the population that is underprivileged, voiceless, and marginalized and assure that their interests be considered. In addition, ethical analysis may lead to identification of ways on how an extraordinary burden placed by an intervention on some individuals could be diminished, without diminishing the efficacy of the public health measure.

Obesity has been recently framed as a public health concern with ethical dimensions (ten Have et al. 2011). Restricting access to a certain kind of food to reduce obesity may be ethically problematic for a number of reasons, one being that it may unfairly penalize people living on limited income who shop for the least expensive source of calories. There will be other measures that may gain support from an ethical perspective, such as restriction on marketing of certain food products.

Screening and Testing

When physicians screen their patients to determine if they might be at a higher risk of a disease or test patients to confirm a suspected condition, then these procedures are within the realm of personal health care. However, in the last 50 years, screening and testing of not just individuals but populations and communities became an important tool for secondary disease prevention and now represents an important component of public health programs. Prenatal, neonatal, child, and adult screening all have their specific ethical issues. Genetic screening, which looks for abnormalities in one or more genes or entire chromosomes, often has an additional implication that those subjected to screening may not be at risk themselves but the risk may become manifest in their progeny. Selective abortions of unborn children with undesirable characteristics and selective implantation of embryos during assisted reproduction invite comparison with universally deplored eugenic programs of the twentieth century which aimed to improve human stock or a particular race. While eugenic programs of the past were implemented, often forcefully, by state governments, this time the similar outcome may be resulting from successful marketing of certain medical technology. All screening programs, even those that do not appear ethically problematic, while designed to provide benefits, also introduce some risks. Therefore, the ethical evaluation needs to start with critical assessment of specificity and sensitivity of tests and consider appropriate privacy of information, psychological distress, insurance discrimination, stigmatization, and disturbance of personal bonds within families or communities. Several professional bodies offered lists of conditions that need to be present to consider a screening program ethically acceptable. For example, the UK National Screening Committee in its 2003 report offered 22 such criteria. Among the conditions often cited is the acceptability by the public, and this raises questions when and how to obtain a community consent for a screening program.

Ethics of Public Health Research

As in health research in general, the chief ethical concern in public health research will be the safety and well-being of human subjects of the research intervention. The concern will be both about harms and risks to individuals and to communities. For example, research on indigenous populations in remote settings or a cultural minority group in the city may expose them to exploitation by commercial interest, to a ridicule of the public, or to unfavorable treatment by the political system. Even observational research raises the issues of data retention and of future access to the information, privacy, and confidentiality, both for individuals and communities.

When ethics, laws, and regulations were being developed for the protection of human subjects in research, most concerns were about protection of

patients in the clinical setting. Only much later considerations were given to ethics evaluation and regulation of public health research. At this time only in some jurisdictions are public health research protocols assessed by research ethics committees. One problem is that the line between public health practice and public health research is difficult to draw. It could be argued that public health interventions, if an evaluation is done of their outcome, are almost always "research" in the sense that they contribute to the increase of knowledge. For example, if a major vaccination program is started and it is desirable to determine the effectiveness of the program in terms of antibody response and frequency of the illness in vaccinated people, in comparison with unvaccinated, the project could be framed to appear either as a quality control or as a research. There are at least two approaches how to deal with these difficulties. Some will argue that careful examination of proposed public health activities allows imperfect but practically useful decisions if the project should be subject to standard ethics review or not. Others will argue that maintaining a sharp research intervention distinction may lead to overprotection of rights and interests of subjects in some cases and underprotection in others. Therefore, they will recommend that all types of public health projects whether they appear to be a quality control, economic analysis, or research be evaluated in the same way for ethical appropriateness and provided with an oversight proportional to risks and burdens that the project entails.

As in clinical research, public health research also has to deal with conflict of interest by researchers or funders which can both distort research finding and put research participants at risk. Public health research often seeks engagement with communities or stakeholders to legitimize projects and recruit participants, but this does not always assure impartial science and high regard for the interest of subjects. Community and advocacy groups may be linked with prominent experts, who strive to maintain their research agenda and have ties to industry, and advocacy groups often accept funding from commercial or political entities interested in a particular outcome of research.

Conclusion

Public health is an academic discipline and social practice with the object to protect and promote the health of a population. Practitioners in this field have always been aware that their activities have ethical or moral dimensions, but this was not often articulated. From the end of the twentieth century to the present time, the interest in this domain much increased, and the young discipline of bioethics has broadened its agenda, striving to develop a subdiscipline of public health ethics. These developments were driven by contemporary social concerns such as universal human rights, social justice, and protection of research participants but also by challenges to public health such as new and reemerging infections and by scientific advances, for example, in screening and testing for a variety of diseases. Public health ethics is trying to understand these developments and provide guidance by applying various theoretical considerations, theories, principles, and values. It is recognized that public health needs to advance and protect the public good, but how should public good be defined, who ought to do that, and how should the population be involved, with the least possible degree of infringement on freedoms of individuals, families, and communities, are questions which need to be asked and answered again and again as physical and social environments change. Best scientific evidence should inform all public health decisions, yet these decisions also need to take into account human needs, relationships, and aspirations. The case in point is vaccination, which managed to control a number of infectious diseases, yet there is a gap between the claims made by public health and the perceptions of populations about the effectivity and risk of many vaccines, and this in turn leads to suboptimal, from scientific perspective, utilization of vaccination. Public health ethics is helpful when it is critically probing justifications for actions (or nonactions) and impartially examining successes and failures of public health practice. What is the ethically appropriate approach to public health often has to be decided in each community, engaging not just experts or authorities but people who are or will be affected.

At the same time, public health ethics is striving for global collaboration and coordination and recognition of interdependence of communities and the value of solidarity.

Cross-References

- Advertising
- Bioethics: And Politics
- Bioethics: Environmental
- Brain Drain
- Common Good
- Consensus
- Consent: Community
- Cosmopolitanism
- Development Ethics
- Discrimination, Ethics of
- Epidemics
- Epidemiology
- Equality and Equity
- Global Ethics
- Justice: Global
- Justice: Theories of
- Law and Bioethics
- Medical Tourism
- Moral Relativism vs Universalism
- Obesity
- Organ Trade
- Organ Transplantation
- Public Debate
- Public Health
- Research Policy
- Research: Human Subjects
- Right to Health
- Risk
- Solidarity
- Transplantation Medicine
- Utilitarianism
- Vaccination
- Vulnerability
- WHO

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Publication Ethics

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Abstract

Publication ethics as a subject is a part of a wider field called ethics in science or responsible conduct in research. Its beginnings can be traced to the nineteenth century. This field basically encompasses research ethics, research misconduct issues, and publication ethics. Although a bit different and separate from each other, these fields are intertwined. This contribution gives a historical overview of the field and highlights the main ethical issues and documents related to publication ethics.

Keywords

Publication ethics; History; Ethics in science; Responsible conduct in research; Research misconduct; Main ethical issues

Introduction

Publication ethics is a field that emerged with the advent of science and the ethics in science. Although it seems that it is a new field, its beginnings can be traced to the nineteenth century. At first the issues surrounding publication ethics were not as prominent in public debates because the issues related to ethics of research dealing with adequate protection of research subjects and informed consent were more prominent and urgent to address. Later with the rise in publications and advent of modern scientific journals and bibliometric science and their influence on scientific advancements, the publication ethics issues became more pronounced. Nowadays, this field is a hot topic in many scientific circles. This contribution aims at presenting the development of this field and highlighting the main ethical issues in publication ethics.

History and the Development of the Field

Publication ethics as a field is a part of wider field of ethics in science or responsible conduct in research. Ethics in science applies to all types of scientific research ranging from those using bioinformatics methods and different experimental theoretical models to those involving animals and people. This field basically encompasses research ethics, research misconduct issues, and publication ethics. Although a bit different and separate from each other, these fields are intertwined (Missa 2002). One cannot publish a good scientific paper if one has not conducted research according to ethical standards that are applicable to research on human subjects and animals (see research ethics), without fabricating, falsifying, or plagiarizing data and with an appropriate authorship and publication practices in place.

The first documents dealing with ethics in science and publication ethics issues can be traced to the nineteenth century. In 1830 the British mathematician and scientist Charles Babbage published an essay entitled *The Decline of Science in England* where he addressed the issues that are nowadays very important for publication ethics – research misconduct issues (fabrication, falsification, data manipulation). The issue of plagiarism, also an important issue when it comes to publication ethics, was reported many times through the history of science (Resnik 2014). One of the first cases happened in discovering digitalis in the eighteenth century. Withering

(1741–1799), an English botanist, geologist, chemist, and physician, first noticed the use of digitalis in treating congestive heart failure in a practice of an old woman, a folk herbalist in Shropshire. This woman used a polyherbal formulation containing over 20 different ingredients. Withering deduced that digitalis was the "active" ingredient in the formulation. Over nine following years, he carefully experimented with different preparations of various parts of the plant (collected in different seasons). He documented 156 cases where he had employed digitalis, describing the effects and the best – and safest – way of using it. At least one of these cases was a patient of Darwin (1731–1802), a physician and the grandson of Charles Darwin. Darwin had asked Withering for his second opinion. In January 1785, Darwin submitted a paper entitled "An Account of the Successful Use of Foxglove in Some Dropsies and in Pulmonary Consumption" to the College of Physicians in London. A postscript at the end of the published volume of transactions containing Darwin's paper states that "Whilst the last pages of this volume were in the press, Dr Withering of Birmingham. . . published a numerous collection of cases in which foxglove has been given, and frequently with good success" (Lehrer 2007). This is a very early example of medical academic plagiarism.

With the advent and the development of scientific publications and journals especially in the second half of the twentieth century, the issues of publication ethics became even more prominent because now the quality and number of publications became important for scientific and academic advancements and post obtainment. In 1981, a scientific misconduct committee found John Darsee, a postdoctoral fellow at Harvard Medical School, guilty of fabrication and falsifying data on 17 papers and 53 abstracts coauthored with faculty at Harvard University, Emory University, and Notre Dame University. The case raised serious questions about the responsibility of his coauthors since much of his hoaxes could have been discovered and be obvious to an expert (Resnik 2014).

Through the recent years, there have been numerous cases of research misconduct that impacted scientific journals. Nowadays, when one looks at the number of retracted papers on the web, one is amazed by the stories behind those cases.

The Council of Science Editors (CSE) is a US-based nonprofit organization that supports editorial practice among scientific writers. The organization was established in 1957 by the National Science Foundation and the American Institute of Biological Sciences as the Council of Biology Editors (CBE), being renamed as the "Council of Science Editors" in 2000. The CSE publishes a style guide for scientific papers, though it is not widely used. They also deal with publication ethics issues.

In 1978 a small group of editors of medical journals met in Vancouver, British Columbia Canada, and established the format of manuscripts that were to be submitted to their journals. The group became known as Vancouver group. The requirements for the manuscripts including the format for bibliographic references developed by the National Library of Medicine (NLM) were published in 1979. The group later evolved into the International Committee of Medical Journal Editors (ICMJE). The ICMJE gradually broadened the focus of their guidelines including also the publication ethics issues. The ICMJE Recommendations (full title, Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals), formerly called the Uniform Requirements for Manuscripts Submitted to Biomedical Journals (abbreviated URMs and often shortened to Uniform Requirements), are a set of guidelines produced by the International Committee of Medical Journal Editors for standardizing the ethics and the preparation and formatting of manuscripts submitted for publication by biomedical journals.

The European Association of Science Editors (EASE) is a nonprofit membership organization for people interested in science communication and editing. It was founded in 1982 in France. EASE now has an international membership from diverse backgrounds and professional experience. They have issued EASE Guidelines for Authors and Translators of Scientific Articles which also deal with publication ethics issues.

The World Association of Medical Editors or WAME is a nonprofit voluntary association of editors of peer-reviewed medical journals from countries throughout the world. It was established in 1995. The idea of a world association of medical editors was conceived in the early 1990s because of the concerns that the International Committee of Medical Journal Editors (ICMJE), the Vancouver group, was too small, self-serving, and exclusive and that biomedical journal editors around the world needed help in developing high-quality, peer-reviewed journals. WAME's resources for editors also include publication ethics policies, general resources for editors, and ethics resources. WAME's Ethics Committee provides consultation in anonymous format for editors seeking advice on difficult ethical issues and provides an anonymized version online as an educational tool for editors. Additional consultation and discussion (in anonymous format) may be provided to the Committee on Publication Ethics which also issues written opinions.

In 1995 the National Academy of Sciences in the USA issued a document entitled *On Being a Scientist Responsible Conduct in Research* dealing with ethical issues in sciences and among them with publication ethics as well.

The Committee on Publication Ethics (COPE) was established in 1997 by a small group of medical journal editors in the UK but now has over 9,000 members worldwide from all academic fields. Membership is open to editors of academic journals and others interested in publication ethics. Several major publishers have signed up, some as COPE members. COPE provides advice to editors and publishers on all aspects of publication ethics and, in particular, how to handle cases of research and publication misconduct. It also provides a forum for its members to discuss individual cases. COPE does not investigate individual cases but encourages editors to ensure that cases are investigated by the appropriate authorities (usually a research institution or employer). In 1999 they have issued guidelines, in 2003 the Code of Conduct and in 2007 the Best Practice Guidelines. In 2001 they have combined all these documents in the Code of Conduct and Best Practice Guidelines for Journal Editors. The COPE Code of Conduct for Journal Editors is designed to provide a set of minimum standards to which all COPE members are expected to adhere.

Main Ethical Issues

Through different publications, journals, and books, people involved in their creation, publishers, editors, reviewers, and authors, should strive to eliminate in all their work the following problems that may arise.

Conflict of Interest

Conflict of interests is a very important and often a debated question because it can lead to serious mistakes in research and publication of research results. It can cause harm to the subjects, raise suspicions regarding the truthfulness and credibility of results of scientific research, but also cause irreparable damage to science and its perception in public.

The conflict of interests is a situation in which financial and other hidden influences can bias professional scientific objectivity and judgment. Both individuals and institutions can be subject to the conflict of interest, but individuals can have both personal and institutional conflict of interest in certain situations. It should be stressed that conflict of interest exists regardless of its impact on decision making. *Obvious conflict of interest* exists in situations where a reasonable person can notice partiality in professional decision making. *Potential conflict of interest* exists when there is a possibility of a conflict of interest. Conflicts of interest can be touchable or untouchable. *Untouchable conflict* of interest is mostly related to scientific advancements which is why it is commonly referred to as academic conflict of interest. *Touchable conflicts of interests* are more noticeable and can be quantified and measured. Financial gain lies at the basis of most touchable conflicts of interest.

A series of guidelines and legislation concerning conflict of interest have been put in place. The US Public Health Service, which

includes the NIH and National Science Foundation (NSF), created a document called "Responsibility of Applicants for Promoting Objectivity in Research." The Association of American Medical Colleges (AAMC), American Association of Universities (AAU), and Council on Governmental Relations (COGR) in the USA have similar sets of guidelines. Majority of prominent scientific journals demand that their authors clearly state whether the conflict of interest regarding their research exists and if so, what it is, when submitting a scientific article for publication. Same rules apply to reviewers reviewing submitted scientific papers. When accepting the scientific article for peer review, they have to report the existence of any kind of conflict of interest that might influence their peer review. It is necessary to stress that the conflict of interest does not fall under the same category of scientific misconduct as fabrication, falsification, and plagiarism.

Questions of Cooperation, Authorship

Cooperation and authorship are frequently debated issues in the publication ethics. Planning and financing of scientific projects, together with authorship and publication of scientific papers, often cause poor interpersonal relationships between scientists. Cooperating scientists frequently encounter problems at the beginning or at the end of the research, when deciding on parts of the scientific project that each of them will work on, their contribution to the specific research, or the order in which the names of authors should appear in a publication. That is why it is necessary to decide beforehand on the respective contributions of each participant and, according to it, on the order of the names of authors in a publication. Financial relationships between participants of a scientific cooperation should also be clearly defined. Authorship implies a significant intellectual contribution to the work, some roles in writing the manuscript and reviewing the final draft of the manuscript, but authorship roles can vary. Who will be an author and in what sequence should be determined by the participants early in the research process, to avoid disputes and misunderstandings which can delay or prevent publication of a paper.

The appearance of *ghost writers*, authors who write scientific papers for others without being listed as authors, is a common problem when it comes to authorship of scientific publications. Moreover, authorship is often conferred on those that did not participate in the research or made a significant contribution to the research *ghost authors. Honorary authorship* when authorship is granted as a favor to someone powerful or prestigious who would not have qualified for it otherwise should be discouraged. This description of author contributions should be printed with the article.

Citation

Citation of sources is also a problem. Due to its importance for scientific advancement, authors often reference their own papers without just cause in order to increase the number of their own citations. In some cases, other scientists' results and ideas are used but not referenced, thereby creating an illusion that they are original. There are also groups of people involved in certain research coming from certain laboratories that in order to boost citation rates of their works cite each other. There are some journals who in order to boost their citation indexes ask authors who publish in their journals to cite papers form their journal even if they have no important relevance to the issues discussed in the paper nor did the authors consider them to be relevant.

Double Publication

Another issue that from time to time plagues the scientific community is the issue of double publication. Here, we have authors who usually publish a paper and then republish it again in another journal or in a book as a book chapter without mentioning that it has been already published. This usually happens when authors publish first in their own language for the local audience and then in English thinking that the first publications, because of the foreign language that not many could read and check and the low visibility of their first publication, will not be noticed. One should avoid such situations. When one is asked or wants to republish something for the second time, one should first ask the permission of the

first publisher to republish, and then when republishing, clearly cite that this is a republication of an article that was already published in a certain journal or book.

Data Acquisition and Processing

Irregularities related to data acquisition and processing can cause a number of ethical problems when publishing a scientific paper. If those problems are not addressed in the right way, they may cause retraction of a scientific paper. Data should be acquired in an ethically acceptable way. That means that the correct methodology was applied, that no human or animal subjects were abused, and that data were appropriately processed. When using animal subjects, it is important to respect the three R principle (refinement, reduction, and replacement): to use the smallest number of animals necessary to confirm a scientific hypothesis, to apply sophisticated methods to additionally reduce that number when necessary, and to use other types of research such as bioinformatic models whenever possible. When creating research protocol for research on human subjects, it is important to include research types that cause least discomfort and damage to subjects without interfering with the ability of research to confirm scientific hypothesis. Minors and people with diminished legal capacity (psychiatric patients, people with dementia) should be avoided as subjects. If performed at all, research using these groups of subjects should be carefully planned and bring immediate benefit to individuals and groups used as subjects. When conducting research including humans, getting an informed consent from each human subject is crucial. Data collected in a research should be used only as results of that research for which the subjects have given their consent. If one wants to use them for other purposes, he or she has to ask for new consent from the subjects. During data acquisition, processing, and publication, the identity and privacy of subjects need to be protected. The journals usually, in order to see if the already mentioned data acquisition procedures have been scrutinized with competence, ask contributors if a research presented in a paper underwent through a review by a research ethics committee (Missa 2002).

Fabrication, Falsification, Plagiarism

Daniele Fanelli has conducted a systematic review and meta-analysis of surveys asking researchers about misconduct. He was able to include 21 surveys in the systematic review and 18 in the meta-analysis and found that nearly 2 % of scientists had themselves fabricated or falsified data and a third admitted to questionable research practices. (The review does not include data on plagiarism.) When asked about other researchers, those surveyed said that 14 % had fabricated or falsified data and nearly three-quarters were guilty of questionable research practices (Fanelli 2009).

Fabrication of data is forgery of parts of research or of an entire research presented to the public. This means that the data are completely fictitious and are not gathered in any research.

Falsification of data is the act of changing the data to make them better to fit the hypothesis the author wants to confirm or refute.

Plagiarism is the use of another author's data, results, language, thoughts, or ideas and their representation as one's own without crediting their true source.

Self-plagiarism refers to the practice of an author using portions of their previous writings on the same topic in another of their publications, without specifically citing it formally in quotes. This practice is widespread and sometimes unintentional, as there are only so many ways to say the same thing on many occasions, particularly when writing the methods section of an article (Borovecki and Lackovic 2014).

Inappropriate Behavior in Relation to Misconduct

This includes unfounded or knowingly false accusations of misconduct, failure to report known or suspected misconduct, withholding or destruction of information relevant to a claim of misconduct, and retaliation against persons involved in the allegation or investigation.

Peer Review

Good-quality peer review is fundamental to the scientific publication process and the dissemination of sound science. Peer reviewers are experts chosen by editors to provide written assessment of

the strengths and weaknesses of written research. Their aim is improving the reporting of research and identifying the most appropriate and highest-quality material for the journal. Reviewers selected for the journal should be required to meet minimum standards (determined by each journal). Their background in original research, publication of articles, formal training, and previous critical appraisal of manuscripts should be taken into account. Peer reviewers should be experts in the scientific topic addressed in the articles they review and should be selected for their objectivity and scientific knowledge. Individuals who do not have such expertise should not be reviewers. Individuals who have a major competing interest in the subject of the article (e.g., those working for a company whose product was tested, its competitors, those with special political or ideological agendas, etc.) should not review it. Reviews should be professional, honest, courteous, prompt, and constructive. Reviewer's quality and other performance characteristics should be periodically assessed to assure optimal journal performance. Individual performance data must be confidential. These performance measures should also be used to assess changes in process that might improve journal performance (Publication Ethics Policies for Medical Journals, the WAME Publication Ethics Committee).

Good Editorial Practice

Good editors will base their decisions about a manuscript on its importance, originality, clarity, and relevance to the journal's scope and content. They will also take into consideration studies with negative results despite adequate power or those challenging previously published work (Publication Ethics Policies for Medical Journals, the WAME Publication Ethics Committee).

Conclusion

In this overview, we tried to present the basic ethical problems of publication ethics. Science has always been considered as one of the most creative areas of human activity, the one leading to human progress. However, modern science oftentimes develops reductionist methodology, abandons an integral perspective of the world, and turns toward the pure aspects of knowledge. That perspective of the world can cause some problems. Scientists should not forget that science is firmly connected to the environment they live in and that only the science that takes into consideration its own influence on the world around is a true alternative to the reductionist value vacuum.

Cross-References

- ▶ Commercialism in Scientific Research
- ▶ Conflict of Interest
- ▶ Ghostwriting
- ▶ Integrity: Research
- ▶ Plagiarism
- ▶ Scientific Misconduct

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